

DEPARTMENT OF HUMAN SERVICES

Social Services Rules

CHILD CARE FACILITY LICENSING

12 CCR 2509-8

[Editor's Notes follow the text of the rules at the end of this CCR Document.]

7.700 CHILD CARE FACILITY LICENSING

7.701 GENERAL RULES FOR CHILD CARE FACILITIES

7.701.1 INTRODUCTION

All rules in Section 7.701 through 7.701.500, shall be known and hereinafter referred to as the General Rules for Child Care Facilities and will apply to all child care applicants and licensees subject to the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act, Sections 26-6-901 to 26-6-923, C.R.S.

7.701.11 Licensing Exemptions

- A. A license must be obtained before care begins unless such care is exempt as set forth below.
- B. Pursuant to Section 26-6-904, C.R.S., a license is not required for:
 - 1. A child care facility that is approved, certified, or licensed by another state agency or by a federal government department or agency that has standards for operation of the facility and inspects or monitors the facility;
 - 2. Occasional care of children that has no apparent pattern and occurs with or without compensation;
 - 3. Juvenile courts; or
 - 4. Nursing homes that have children as residents.

7.701.12 Civil Penalties and Injunctions

- A. Violation of any provision of the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act or intentionally false statements or reports made to the Department or to any agency lawfully delegated by the Department to make an investigation or inspection may result in fines assessed of not more than \$250 a day for the first day, \$500 a day for the second day, and \$1,000 a day for the third and subsequent days, to a maximum of \$10,000, pursuant to Section 26-6-921, C.R.S.:
1. A civil penalty will be assessed by the Department only in conformity with the provisions and procedures specified in Article 4 of Title 24, C.R.S. No civil penalty will be assessed without a hearing conducted pursuant to Section 26-6-901 through 26-6-923, C.R.S. of the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act and Article 4 of Title 24, C.R.S., before an Administrative Law Judge acting on behalf of the Department.
 2. Upon receipt of a cease and desist order from the Department and after making an investigation or inspection under the provision of the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act, any unlicensed child care facility or agency may be fined up to \$250 a day for the first day, \$500 a day for the second day, and \$1,000 a day for the third and subsequent days, to a maximum of \$10,000 for providing the type of care without a license for which a license is required.
 3. Assessment of any civil penalty under this Section will not preclude the Department from initiating injunctive proceedings pursuant to Section 26-6-918, C.R.S.
 4. A licensed child care facility or agency may be fined up to \$250 a day for the first day, \$500 a day for the second day, and \$1,000 a day for the third and subsequent days, to a maximum of \$10,000 for each violation of the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act or for any statutory grounds as listed at Section 26-6-914(2), C.R.S.
 5. Assessment of any civil penalty does not preclude the Department from also taking action to deny, suspend, revoke, make probationary, or refuse to renew that license.

6. Any person intentionally making a false statement or report to the Department or to any agency delegated by the Department to make an investigation or inspection under the provisions of the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act may be fined up to \$250 a day for the first day, \$500 a day for the second day, and \$1,000 a day for the third and subsequent days, to a maximum of \$10,000.
 7. Civil penalties assessed by the Department must be made payable to the Colorado Department of Human Services.
- B. In addition to civil penalties that may be assessed under Section 7.701.12.A, when an individual operates a facility or agency after a license has been denied, suspended, revoked, or not renewed, or before an original license has been issued, injunctive proceedings may be initiated to stop the individual from operating a facility or agency without a license.
- C. Pursuant to Section 26-6-915, C.R.S., within ten (10) days after receipt of a written notice with regard to a negative licensing action or the imposition of a fine, pursuant to Section 26-6-914(2) and (8), C.R.S., the facility or agency must provide the Department with the name(s) and mailing address(es) of the parent(s) or legal guardian(s) of each child/youth cared for at the facility or agency. Within twenty (20) days after receiving the names and addresses of parents and legal guardians, the Department shall send a written notice to notify the parent(s) or legal guardian(s) of the negative licensing action taken or the fine imposed and provide a description of the basis for the action as it relates to the impact on the health, safety, and welfare of the children/youth in the care of the facility or agency. Written notice must include the current mailing address and telephone number of the Division within the Department responsible for licensing. The facility/agency will be responsible for paying a fine to the Department that is equal to the direct and indirect costs associated with the mailing of the notice.

7.701.13 Appeals and Waivers

The Department is authorized to hear and decide two (2) kinds of appeal or waiver requests by applicants or licensees: hardship appeals in this rule set, also referred to as hardship waivers and stringency appeals according to the following procedures. For purposes of this Section 7.701.13, a county department of human/social services that certifies foster homes under Section 26-6-910, C.R.S., is a "licensee."

A. Hardship Waivers

1. Any applicant or licensee who has applied for or been issued a license to operate a child care facility or agency has a right to appeal, pursuant to Section 26-6-909(4), C.R.S., any rule or standard which, in their opinion, poses an undue hardship on the person, facility, or community.

- a. “Undue hardship” is defined as a situation where compliance with the rule creates a substantial, unnecessary burden on the applicant or licensee’s business operation or the families or community it serves, which reasonable means cannot remedy. An undue hardship does not include the normal cost of operating the business.
- b. Emergency hardship waivers are requests by applicants or licensees to excuse noncompliance with a specific child care licensing rule due to urgent, significant, and unexpected situations outside the applicant’s or licensee’s control. Specific situations that may be considered “emergencies” under this paragraph include, but are not limited to:
 - 1) Natural disasters;
 - 2) Infectious disease outbreaks;
 - 3) Mold outbreaks;
 - 4) Acts of nature or an accident resulting in structural damage to the child care facility; or,
 - 5) For foster care homes and residential child care facilities, an immediate, child(ren)/youth-specific emergency placement, a situation which may disrupt placement, or a situation posing a safety risk to child(ren)/youth in out-of-home placement.
- 2. Such a waiver must be submitted to the Department in writing within sixty (60) calendar days from the date on which the rule, standard, or emergency situation allegedly created the hardship. The applicant or licensee or their designated representative must send an appeal on the state-prescribed form to the Department. Each rule appealed requires an individual appeal and applicable fee. If the appeal is an emergency hardship waiver appeal, the applicant or licensee must mark it as such on the state-prescribed form.
- 3. When submitting a waiver, the applicant/licensee/designated representative must consider the impact on the health, safety, and wellbeing of any children/youth in care and include a proposed alternate compliance plan.
- 4. The Department must consider the impact of a waiver on the health, safety, and wellbeing of the children/youth in care, which must take priority over any undue hardship alleged, when determining whether an appeal should be granted.

5. If the Department grants a waiver for undue hardship, it will issue the applicant or licensee an official decision notification letter temporarily excusing the applicant or licensee from compliance with the appealed rule or standard and accepting the alternate compliance plan.
- B. Stringency Appeals
1. Any applicant or licensee who has applied for or been issued a license to operate a child care facility or agency has a right to appeal, pursuant to Section 26-6-909(4), C.R.S., any violation of a child care licensing rule cited in a report of inspection, on the basis that the rule has been too stringently applied by a representative of the Department.
 - a. “Stringency,” as used in this Section 7.701.13, means the Department representative applied rules too strictly, improperly, or unfairly.
 - b. Disputes over the factual accuracy of a cited violation are not reviewable under this provision and must be resolved with the licensing representative’s supervisor.
 2. Such an appeal must be submitted to the Department in writing within sixty (60) calendar days from the date of the report of inspection at issue. The applicant or licensee or their designated representative must send an appeal on the state-prescribed form to the Department. Each rule citation requires an individual appeal and applicable fee.
 3. When submitting an appeal, the applicant/licensee/designated representative must provide all evidence that it believes shows the rule was applied too stringently.
 4. The Department must consider the impact of an appeal on the health, safety, and well-being of the children/youth in care.
 5. If the Department finds a licensing rule was too stringently applied in the appealed citation, it will issue the applicant or licensee a new report of inspection with that citation removed, which shall, for all purposes, supersede the original report of inspection. If the Department grants an appeal for stringency, it will issue the applicant or licensee an official decision notification letter.
- C. The requirements of applicants and licensees that are not appealable are outlined in the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act in Section 26-6-901, C.R.S. through Section 26-6-923, C.R.S.
- D. Administrative Review and Appeal Panel Procedures

1. The applicant or licensee must comply with all child care licensing rules and standards, including the rule(s) subject to an appeal or waiver until the applicant or licensee has received a written decision granting the appeal or waiver.
2. The Department will receive, review, and schedule all appeals and waiver requests for review by the appeals review panel constituted under Section 26-6-909(4), C.R.S.
 - a. For hardship appeals/waivers, the Department may propose that the appeal panel grant one (1) or more appeals as part of a consent agenda, which the appeal panel may approve with a single vote; except if any panel member objects to the consent agenda, the appeals on such agenda must be decided individually. The appeal panel may not deny appeals by consent agenda.
 - b. For emergency hardship appeals, the Department may administratively grant the appeal if it meets the definition of an emergency and the proposed alternate compliance plan adequately protects the health, safety, and wellbeing of children/youth in care. If the Department does not administratively grant the emergency hardship appeal, it must schedule the appeal for review by the appeal panel.
3. The appeal panel will adopt a written decision recommending that the Department grant, deny, or grant with modifications an appeal request. The Department must send an official decision letter, including the written decision of the appeal panel, to the applicant or licensee, within ten (10) calendar days from the date of the appeal panel meeting.
 - a. For hardship appeal requests, the official decision letter must be posted next to the child care license until its expiration date. If there is no expiration date, the letter expires one (1) year from the date it was issued. If the official decision letter is child-specific, it must be kept in the agency/facility file and provided to the Department upon request.
 - b. If the Department approves a hardship appeal request and the applicant or licensee wishes to make changes to the alternate compliance plan or safety policy submitted with the original appeal or request, the applicant or licensee must submit a new hardship appeal request.

- c. If, after the Department approves a hardship appeal request, and the applicant or licensee violates the terms and conditions described in the approved alternate compliance plan, approved safety policy, or official decision letter, the Department's approval will immediately be rescinded and considered null and void. For purposes of this provision, any injuries, accidents, or founded complaints or investigations related to the appealed or waived licensing rule constitute a violation.
- 4. Hearing Requests
 - a. For hardship or stringency appeals, if an applicant or licensee is aggrieved by the decision of the Department, the applicant or licensee may request an administrative hearing pursuant to Section 24-4-105, C.R.S. Written requests for an administrative hearing must be received in writing within thirty (30) calendar days from the date the applicant or licensee received the Department's decision. In all such administrative hearings, the applicant or licensee will bear the burden of proof by a preponderance of the evidence.

7.701.14 Civil Rights and Other Incorporations by Reference

All facilities licensed under the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act are subject to the following federal laws and regulations: the non-discrimination provisions of Title VI of the Civil Rights Act of 1964, 42 U.S.C. Section 2000D through Section 2000D-7(2024), and its implementing regulation, 45 C.F.R. Part 80 (2024); Title VII of the Civil Rights Act of 1964, 42 U.S.C. Section 2000E through Section 2000E-17 (2024) and its implementing regulation, 29 C.F.R. Part 1606 (2024); the Age Discrimination Act of 1975, 42 U.S.C. Sections 6101-6107 (2024) and its implementing regulation, 45 C.F.R. Part 91 (2024); Section 504 of the Rehabilitation Act of 1973, 29 U.S.C. Sections 791 and 794 (2024), and its implementing regulation, 45 C.F.R. Part 84 (2024), all of which are hereby incorporated by reference. No later editions or amendments are incorporated. These statutes and regulations are available for public inspection and copying at the Colorado Department of Human Services 1575 Sherman St. Denver, Colorado 80203, during regular business hours. These statutes and regulations are also available at no cost at <http://uscode.house.gov> and <http://www.ecfr.gov>.

All facilities licensed under the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act are also subject to Titles I through V of the Americans with Disabilities Act, 42 U.S.C. Section 12101 through Section 12213 (2024), and its implementing regulation, 29 C.F.R. Part 1630 (2024), which are hereby incorporated by reference. No later editions or amendments are incorporated. These statutes and regulations are available for public inspection and copying at the Colorado Department of Human Services 1575 Sherman St. Denver, Colorado 80203, during regular business hours. These statutes and regulations are also available at no cost at <http://uscode.house.gov> and <http://www.ecfr.gov>.

Decisions related to the enrollment, placement, or dismissal of a child/youth with a disability or chronic condition must be in compliance with the Americans with Disabilities Act. The facility must provide reasonable accommodations for the child/youth with a disability who has special needs.

A lack of independent ambulation or the need for assistance in feeding, toileting, or dressing or in other areas of self-care cannot be used as sole criteria for enrollment or placement or denial of enrollment or denial of placement. Efforts must be made to accommodate the child's/youth's needs and to integrate the child/youth with their peers who do not have disabilities.

All facilities and agencies licensed under the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act are also subject to 42 C.F.R. Parts 160, 162, and 164 of the Health Insurance Portability and Accountability Act (HIPAA) (2024); which are hereby incorporated by reference. No later editions or amendments are incorporated. These statutes and regulations are available for public inspection and copying at the Colorado Department of Human Services Sherman St., Denver, Colorado 80203, during regular business hours. These statutes and regulations are also available for public inspection at no cost at the U.S. Department of Health and Human Services website <https://www.hhs.gov/HPIAA>.

All facilities and agencies licensed under the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act are also subject to all applicable rules and regulations in the International Fire Code, ICC IFC-2024, which is hereby incorporated by reference. No later editions or amendments are incorporated. These statutes and regulations are available for public inspection and copying at the Colorado Department of Human Services 1575 Sherman Street, Denver, Colorado 80203, during regular business hours. These regulations are also available for public inspection at no cost through the International Codes website at <https://codes.iccsafe.org>.

All facilities and agencies licensed under the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act are also subject to all applicable rules and regulations of the Family First Prevention Services Act (FFPSA) 42 U.S.C. Section 672 (K)(4) (2024), which is hereby incorporated by reference. No later editions or amendments are incorporated. These statutes and regulations are available for public inspection and copying at the Colorado Department of Human Services 1575 Sherman Street, Denver, Colorado 80203, during regular business hours. These statutes and regulations are also available for public inspection at no cost through the Family First Prevention Services federal website at <https://www.congress.gov/115/bills/hr253/BILLS-115hr253ih.pdf>.

All facilities and agencies licensed under the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act are also subject to all applicable rules and regulations of the Individuals with Disabilities Education Act (IDEA) at 20 U.S.C. Sections 1400 through 1482 (2024), if applicable, which is hereby incorporated by reference. No later editions or amendments are incorporated. These statutes and regulations are available for public inspection and copying at the Colorado Department of Human Services 1575 Sherman Street, Denver, Colorado 80203, during regular business hours. These statutes and regulations are also available for public inspection at no cost through the IDEA website on the U.S. Department of Education's webpage at <http://sites.ed.gov/idea/statute-chapter-33>.

Subject to 20 U.S.C. 1232G and 34 C.F.R. Part 90 of the Family Educational Rights and Privacy Act (FERPA) (2024) which is hereby incorporated by reference. No later editions or amendments are incorporated. These statutes and regulations are available for public inspection and copying at the Colorado Department of Human Services 1575 Sherman St., Denver, Colorado 80203, during regular business hours. These statutes and regulations are also available for public inspection at no cost at the U.S. Department of Education website at <https://studentprivacy.ed.gov/ferpa>.

7.701.2 DEFINITIONS

"Affiliate of a licensee," means (A) a person or entity that owns more than five (5) percent of the ownership interest in the business operated by the licensee or the applicant for a license; or (B) a person who is directly responsible for the care and welfare of children served; or (C) an executive, officer, member of the governing board, or employee of a licensee; or (D) a relative of a licensee, which relative provides care to children at the licensee's facility or agency or is otherwise involved in the management or operations of the licensee's facility or agency.

"Aftercare" means a continuum of treatment and family-based support that is developed in collaboration with the family and permanency team throughout treatment and prior to the child's/youth's discharge, is designed to support the child/youth and family during the child's/youth's transition out of the program and to safely maintain the child/youth in a lower level of care, and is provided for at least six (6) months after discharge.

“Annually” means the time frame from the initial date of hire, training, licensing, or certification, and the following twelve (12) months.

“Application” means a declaration of intent to obtain or continue a license or certificate for a residential or day treatment child care facility or child placement agency.

“Behavioral Health Administration” (BHA) is the state administration established in the Department of Human Services and charged with creating a coordinated, cohesive, and effective behavioral health system pursuant to Section 27-50-102, C.R.S. The BHA is responsible for ensuring that behavioral health programs delivered by state agencies and commercial payers are comprehensive, evidence-based, affordable, high quality, equity-focused, and easily accessible for all Coloradans pursuant to Section 27-50-103, C.R.S.

“Calendar year” means the time frame from January 1 to December 31.

“Case plan” means a plan that is developed in a day treatment center, specialized group facility, runaway and homeless youth services program or shelter services program, child placement agency, or host family home that includes goals regarding personal and group living skills, anticipated behavioral changes of the child/youth, projected length of participation in the program, and any other components as required in 12 CCR 2509-8 Sections 7.706, 7.709, 7.710, and 7.721.

“Certificate” means a legal document granting permission to operate a foster care home or a kinship foster care home.

“Certification” means the process by which a county department of human or social services, a child placement agency, or a federally recognized tribe pursuant to applicable federal law approves the operation of a foster care home or a kinship foster care home.

“Chemical restraint” as defined in Section 26-20-102(2), C.R.S., means giving an individual medication involuntarily for the purpose of restraining that individual; except that “chemical restraint” does not include the involuntary administration of medication pursuant to Section 27-65-111(5), C.R.S., or administration of medication for voluntary or life-saving medical procedures.

“Child” pursuant to Section 19-1-103(21), C.R.S. means a person under eighteen (18) years of age.

“Child abuse,” and “child neglect” mean the same as in the definition of “child abuse or neglect” set forth in Section 19-1-103(1), C.R.S., unless otherwise indicated.

“Child Care Centers,” as defined in Section 26-6-903(5), C.R.S., means a facility, by whatever name known, that is maintained for twenty-four (24)-hour care for five (5) or more children, unless otherwise specified in Section 26-6-903(5), who are not related to the owner, operator, or manager of the facility, whether the facility is operated with or without compensation for such care and with or without stated educational purposes. The term includes, but is not limited to, facilities commonly known as residential child care facilities, day treatment facilities, specialized group facilities, secure residential treatment centers, and respite child care centers.

“Child Placement Agency” is defined at Section 26-6-903(6), C.R.S. Child placement agencies are further detailed at 12 CCR 2509-8 Section 7.710, “Rules and Regulations for Child Placement Agencies”.

“Children's Habilitation Residential Program (CHRP) Waiver” is described at Section 25.5-6-903, C.R.S. Information regarding level of care evaluations and determinations are further detailed in 10 CCR 2505-10 Sections 8.400 (2024), 8.500 (2024), 8.600 (2024), and 8.700 (2024) which is hereby incorporated by reference. No later amendments or editions are incorporated. These regulations are available for public inspection and copying at the Colorado Department of Human Services 1575 Sherman Street, Denver, Colorado 80203 during regular business hours. These regulations are also available for public inspection at no cost through the state’s website page at <https://www.sos.state.co.us/ccr>.

“Citizen/legal resident” means a citizen, current legal resident, or a person lawfully present in the United States.

“Client representative” means a person designated by the facility to process grievances.

“Colorado Criminal Justice Records Act” or “CCJRA” means the maintenance, access, and dissemination, completeness, accuracy, and sealing of criminal justice records according to Section 24-72-309, C.R.S.

“Consumer Product Safety Commission”, means the National Commission that establishes standards for the safety of children’s equipment and furnishings and for playground safety as outlined under federal laws and regulations.

“Contraband” means dangerous instruments or materials that are prohibited for children/youth to have in their possession or for visitors to introduce into their facility/agency and materials that can harm the child/youth in the facility, other children/youth, or staff. Dangerous instruments include any device, material, or substance which is capable of causing or inducing fear or death or bodily injury. Contraband can include items such as lock picks, cutting tools, combustible materials, unauthorized drugs, drug paraphernalia, ropes or ladders, obscene material, sharpened instruments, firearms, knives, or any article that may pose a threat to safety and security.

“Conversion therapy” means any practice or treatment by a licensee, registrant, or certificate holder that attempts or purports to change an individual’s sexual orientation or gender identity, including efforts to change behaviors or gender expressions or to eliminate or reduce sexual or romantic attraction or feelings toward individuals of the same sex, as defined in Section 12-245-202(3.5)(a), C.R.S.

“Convicted” means a conviction by a jury or by a court and also includes a deferred judgment and sentence agreement, a deferred prosecution agreement, a deferred adjudication agreement, an adjudication, and a plea of guilty or nolo contendere as defined in Section 26-6-914(6)(a)(II), C.R.S.

“Court-appointed special advocate” or “CASA volunteer” means a volunteer appointed by a court pursuant to Part 2 of Article 1 of Title 19 to assist in advocacy for children, as defined in Section 19-1-103(46), C.R.S.

“Criminal child abuse” means the same as in the definition set forth in Section 18-6-401, C.R.S. identified in Section 26-6-905(8)(a)(I)(A), C.R.S., unless otherwise indicated.

“Critical incident” is a serious incident or concern or potential incident or concern that poses a danger to a child/youth at the facility, a staff member at the facility, or a foster parent.

“Cultural Responsiveness” means that an organization designs and implements services and practices that consider the unique culture of the individuals, families, and communities served.

“Custodian”, as defined in Section 19-1-103(50), C.R.S. means a person who has been providing shelter, food, clothing, and other care for a child in the same fashion as a parent would, whether or not by order of court.

“De-escalation” means the use of therapeutic interventions with a child/youth during the escalation phase of a crisis. The interventions are designed to support children/youth through co-regulation or allow them to regulate their own behavior so that acute physical behavior does not develop that would lead to the need to use physical restraint.

“Department” or “State Department” means the state Department of Human Services.

“Department representative” means any staff member from the Colorado Department of Human Services, Provider Services Unit to include, but need not be limited to, licensing specialists, monitoring specialists, performance management outcomes specialists or PMO specialists, licensing supervisors, monitoring supervisors, provider performance manager, provider development manager, the ICPC deputy compact administrator or ICPC compact administrator, and/or the provider services unit manager.

“Day Treatment Center,” is defined at Section 26-6-903(8), C.R.S. Day treatment centers are further detailed at 12 CCR 2509-8 Section 7.706, “Rules Regulating Day Treatment Centers”.

“Electronic signature” as defined in Section 24-71-101(1), C.R.S. and Section 24-71-101(2), C.R.S., means an electronic sound, symbol, or process attached to or logically associated with a record and executed or adopted by a person with the intent to sign the record. In any written communication in which a signature is required or used, any party to the communication may affix a signature by use of an electronic signature that complies with the requirements of Article 71.3 of this Title 24, Colorado Revised Statutes, for electronic signatures.

“Emergency” means a serious, probable, imminent threat of bodily harm to self or others where there is present ability to effect such bodily harm as defined in Section 26-20-102(3), C.R.S.

“Employee” or “applicant for employment,” for the purpose of background checks required in Section 7.701.32, Section 7.701.321, and Section 7.701.33, is defined as: an individual (other than an individual who is related to all children/youth for whom child care services are provided):

Who is employed or associated with a licensed child care provider, including individuals who have access to protected information or other individuals associated with the license who engage with the children/youth in any capacity;

Whose activities involve the care or supervision of children for a licensed child care provider or unsupervised access to children who are cared for or supervised by a licensed child care provider; or

Any individual residing in a licensed child care facility or foster home who is age eighteen (18) and older.

Employee does not include those individuals that will not have contact with children/youth while at the licensed facility or agency, such as hired tradespeople, contractors, or other maintenance service professionals where a plan for ensuring their whereabouts and activities of the individual while at the facility is documented and implemented.

“Escalation” is an increase in intensity or seriousness of a child’s/youth’s behavior.

“Facility” or “child care facility” is any business or operation established for the purpose of providing child care services that are required to be licensed pursuant to the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act, Section 26-6-901, C.R.S. through Section 26-6-923, C.R.S.

“Family and permanency team” consists of all the appropriate biological and/or legal family members, relatives, and psychological kin of the child/youth, as well as professionals who are a resource to the child/youth, such as teachers, medical and mental health providers, or clergy, etc., who review the child’s/youth’s permanency needs. In the case of a child/youth who has reached age fourteen (14), the family and

permanency team shall include the members of the permanency planning team for the child/youth that are selected by the child/youth.

“Family engagement” means joining with the family/kin of the child/youth to establish common goals of safety, well-being, and permanency, and is inclusive of other systems, as defined in 12 CCR 2509-4 Section 7.300.1.

The “family services plan” (FSP) means a case services plan completed by a county caseworker jointly with a child/youth, parent(s) and/or legal guardian(s), and providers within sixty (60) calendar days of the referral date in the automated case management system for each child/youth receiving services from a county department of human or social services pursuant to 12 CCR 2509-4 Section 7.301.21.

“Final Agency Action” means the determination made by the Department, after the opportunity for hearing to deny, suspend, revoke, or demote to probationary status a license issued pursuant to the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act or an agreement between the Department and the licensee concerning the demotion of such a license to a probationary license.

“Foster Care Home,” is defined at Section 26-6-903(10), C.R.S. Types of foster care homes are further detailed at 12 CCR 2509-8 Section 7.708, “Rules Regulating Family Foster Care Homes”.

“Foster Youth in Transition Program” as defined in Section 19-7-302(14), C.R.S. means the program established pursuant to Section 19-7-303, C.R.S., in which an eligible youth may voluntarily opt to continue or resume receiving child welfare services that may include but are not limited to foster care maintenance payments.

“Gender identity” as defined in Section 19-1-130(1)(B), C.R.S. means a person’s innate sense of the person’s own gender, which may or may not correspond with the person’s sex assigned at birth.

“Governing Body” means the individual, partnership, corporation, or association in which the ultimate authority and legal responsibility is vested for the administration and operation of a residential or day treatment child care facility or a child placement agency.

“Guardian” means a person who is entrusted by law with the care of a child under eighteen (18) years of age.

“Health Department” is the Colorado Department of Public Health and Environment (CDPHE) or the local county department of health.

“Health First Colorado” is Colorado’s Medicaid program and public health insurance for Coloradans who qualify. Medicaid is funded jointly by the federal government and Colorado state government, and is administered by the Colorado Department of Health Care Policy and Financing.

“Homeless youth shelter” means a facility that, in addition to other services it may provide, provides services and mass temporary shelter for a period of three (3) days or more to youths who are at least eleven (11) years of age or older and who otherwise are homeless youth as that term is defined in Section 26-5.7-102(2), C.R.S. Youth may reside at a homeless youth shelter for a period not to exceed twenty-one (21) days as outlined and in accordance with Section 26-5.7-105, C.R.S.

“Human trafficking for involuntary servitude” means a person who knowingly sells, recruits, harbors, transports, transfers, isolates, entices, provides, receives, or obtains by any means another person for the purpose of coercing the other person to perform labor or services commits human trafficking for involuntary servitude, as outlined in Section 18-3-503(1), C.R.S.

“Human trafficking for sexual servitude” is defined as outlined in Section 18-3-504, C.R.S.

“Independent assessment” (IA) means a process to assess the strengths and needs of the child using an age-appropriate, evidence-based, validated, functional assessment tool. The assessment determines whether treatment in a qualified residential treatment program (QRTP) provides the most effective and appropriate level of care for the child in the least restrictive environment in accordance with Department of Human Services regulations.

The “Individual child’s/youth’s plan” is based upon an assessment of the child/youth following placement at the facility or therapeutic foster home. It is developed by the facility or child placement agency for each child/youth, designed to meet or address the individualized needs of each child/youth, and must be consistent with the family service plan (FSP) for the child/youth.

“Interdisciplinary team” means staff in a psychiatric residential treatment facility (PRTF) comprised of a physician, and a licensed mental health professional, registered nurse or occupational therapist responsible for the treatment of the client, as defined in 10 CCR 2505-10 Section 8.765.1 (2024) which is hereby incorporated by reference. No later amendments or editions are incorporated. These regulations are available for public inspection and copying at the Colorado Department of Human Services 1575 Sherman Street, Denver, Colorado 80203 during regular business hours. These regulations are also available for public inspection at no cost through the state’s website page at <https://www.sos.state.co.us/ccr>.

“Interstate compact on the placement of children” (ICPC) is a statutory agreement that establishes procedures and responsibilities for those involved in placing a child/youth out of state, and is further described at Section 24-60-1801, C.R.S. through Section 24-60-1804, C.R.S.

“Kinship foster care homes” as defined at Section 26-6-903(16), C.R.S., means a kinship foster care home that has been certified pursuant to Section 26-6-910, C.R.S. to care for a relative or kin only. A kinship foster care home provides twenty-four (24)-hour foster care for a child or youth who is a relative or kin, who is less than twenty-one (21) years of age, and who is eligible for the same foster care reimbursement, assistance, and other supports as foster care homes pursuant to Section 26-6-904.5, C.R.S.

“Kinship foster care home” does not include non-certified kinship care as that term is defined in Subsection (21.5) of this Section (26-16-21.5, C.R.S.).

“License” means a legal document issued pursuant to the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act granting permission to operate a residential or day treatment child care facility or child placement agency. A license may be in the form of a provisional, probationary, permanent, or time-limited license.

“Licensed health care provider” means a physician, doctor of osteopathic medicine, physician assistant, nurse practitioner, registered nurse, or other licensed or registered health care provider.

“Licensee” means the entity or individual to which a license is issued and that has the legal capacity to enter into an agreement or contract, assume obligations, incur and pay debts, sue and be sued in its own right, and be held responsible for its actions. A licensee may be a governing body.

“Licensing” means, except as otherwise provided in Section 26-6-903(10), the process by which the Department approves a facility or agency for the purpose of conducting business as a residential or day treatment child care facility or child placement agency, as defined in Section 26-6-903(19), C.R.S.

“Licensing Specialist” is the authorized representative of the Department who inspects and audits child care facilities to ensure compliance with licensing requirements. The Licensing Specialist inspects and audits the operations of licensed agencies and facilities to ensure their compliance with related state laws and regulations. The Licensing Specialist inspects and audits the operations of licensed agencies and facilities to ensure their compliance with related state laws and regulations.

“Mechanical restraint” means a physical device used to involuntarily restrict the movement of an individual or the movement or normal function of a portion of their body pursuant to Section 26-20-102(4), C.R.S.

“Mental health professional” means a person licensed to practice medicine or psychology in this state or any person on the staff of a facility designated by the executive director of the Department of Human Services for seventy-two-hour treatment and evaluation who is authorized by the facility to do mental or behavioral health hospital placement prescreenings, as defined in Section 19-2.5-102, and who is under the supervision of a person licensed to practice medicine or psychology in this state.

“Monitoring specialist” is the authorized representative of the Department who is responsible for investigations to determine agency/facility culpability and compliance with Social Services Rules (Volume 7: Child Welfare, Child Care Facilities) and related statutes.

“Multi-disciplinary team” (MDT) means staff in a residential child care facility (RCCF) providing mental health services comprised of at least one (1) licensed mental health professional and other staff responsible for the treatment of the client and may include a staff member from the federal agency, or a group of personnel, acting within their professional role(s) and respective scope(s) of practice, who are members of different professions, working together to provide services to individuals.

“Nationally recognized criteria” means standardized practices that are acknowledged as acceptable and appropriate for use with at-risk populations, that are incorporated into the model of physical restraint utilized by the facility. The nationally recognized criteria must include, at a minimum the following:

- A. Annual staff training and/or certification, to include training upon hire, and ongoing (at least every six (6) months) refresher training or practice exercises for each staff member trained or certified in restraint, to review and refresh skills involved in positive behavior intervention, prevention, de-escalation, and physical restraint, in accordance with the model.
- B. A restraint prevention and de-escalation component, to include identifying antecedents that may cause an individual to escalate, and/or development of behavior management plans that are in alignment with individual child’s/youth’s plans if necessary.
- C. A physical restraint process that prohibits or provides alternatives to a prone position, and includes identifying primary control techniques that emphasize utilizing only the minimum amount of force necessary to gain control and keep the individual safe.
- D. A debriefing process which includes a review of physical restraint, to determine the appropriateness and effectiveness of preventive/de-escalation strategies or interventions used, the appropriateness of physical restraint, and how, or if, physical restraints are preventable.

“Negative licensing action”, as defined at Section 26-6-903(21)(A), C.R.S., means a final agency action resulting in the denial of an application, the imposition of fines, or the suspension or revocation of a license issued pursuant to this Part 9 of Article 6 of Title 26 or the demotion of such a license to a probationary license.

“Neutral reviewer” means a staff member who observes a physical restraint and is able to monitor the child/youth during the restraint and provide an objective review of the restraint during the debriefing process.

“Nonpublic home-based educational program”, as defined at Section 22-33-104.5(2)(a), C.R.S., means the sequential program of instruction for the education of a child which takes place in a home, which is provided by the child’s parent or by an adult relative of the child designated by the parent, and which is not under the supervision and control of a school district. This educational program is not intended to be and does not qualify as a private and nonprofit school.

“Performance management outcomes specialist” is the authorized representative of the Department who evaluates and analyzes the quality of programming, practices, and policies with licensed agencies and facilities, specifically through the content contained within trauma-informed care plans, file reviews, assessments, interviews, observations, and review of agency/facility data, policies, and procedures.

“Physical restraint” means the use of bodily, physical force to involuntarily limit an individual’s freedom of movement for more than one (1) minute, except that “physical restraint” does not include the holding of a child by one adult for the purposes of calming or comforting the child.

“Prone position” means a face-down position.

“Prone restraint” means a restraint in which the individual who is being restrained is secured in a prone position pursuant to Section 26-20-102(5.5), C.R.S.

“Psychiatric residential treatment facility” (PRTF), pursuant to Section 25.5-4-103(19.5), C.R.S., means a facility that is licensed as a residential child care facility, as defined in Section 26-6-903, C.R.S., that is not a hospital, and that provides inpatient psychiatric services for individuals who are less than twenty-one (21) years of age under the direction of a physician licensed pursuant to Article 240 of Title 12, and that meets any other requirement established in rule by the State Board.

“Qualified manager” means a person who is the owner or operator of the facility or a supervisor designated by the owner or operator of the facility for the purpose of implementing Section 25-1.5-303, C.R.S.; and has completed training in the administration of medications pursuant to Section 25-1.5-303, C.R.S, or is a licensed nurse pursuant to Part 1 of Article 255 of Title 12, a licensed physician pursuant to Article 240 of Title 12, or licensed pharmacist pursuant to Article 280 of Title 12. Every unlicensed person who is a “qualified manager” within the meaning of Section 25-1.5-301(4) shall successfully complete a competency evaluation pertaining to the administration of medications.

“Qualified residential treatment program” (QRTP), as defined in Section 26-6-903(26), means a licensed and accredited program that has a trauma-informed treatment model that is designed to address the child’s or youth’s needs, including clinical needs, as appropriate, of children and youth with serious emotional or behavioral disorders or disturbances in accordance with federal “Family First Prevention Services Act”, 42 U.S.C.A. 672(K)(4), and is able to implement the treatment identified for the child or youth by the assessment of the child or youth as required in Section 19-1-115(4)(E)(I), C.R.S.

“Reasonable” as used in the rules means appropriate and suitable, not excessive or extreme.

“Referral agency or placing authority/legal custodian” means the Division of Youth Services, a county department of social/human services who has custody of the child/youth, an organization or entity placing a child/youth through the Child and Youth Mental Health Treatment Act (CYMHTA), a Regional Accountable Entity (RAE), a parent or legal guardian of the child/youth, or other entity given authority by a court of jurisdiction that refers the child for the purpose of placement.

“Registered dietitian” means someone who holds a certificate or registered dietician through the Commission on Dietetic Registration.

“Relative” as defined at Section 26-6-903(28), C.R.S. means any of the following relationships by blood, marriage, or adoption: parent, grandparent, son, daughter, grandson, granddaughter, brother, sister, stepparent, stepbrother, stepsister, stepson, stepdaughter, uncle, aunt, niece, nephew, or cousin.

“Religion” when used in these regulations includes beliefs, spiritual beliefs, and other practiced ideologies.

“Residential or day treatment child care facility” or “facility” means a residential child care facility, including a qualified residential treatment program, psychiatric residential treatment program, shelter care program, and homeless youth program; specialized group facility, including a group home and group center; day treatment center; secure residential treatment center; respite child care center; or homeless youth shelter, including a host family home, as defined at Section 26-6-903(30), C.R.S.

“Residential Child Care Facility” (RCCF) as defined at Section 26-6-903(29), C.R.S. means a facility licensed by the State Department pursuant to Part 9 of Article 6 of Title 26 to provide twenty-four (24)-hour group care and treatment for five (5) or more children operated under private, public, or nonprofit sponsorship. “Residential child care facility” includes community-based residential child care facilities; qualified residential treatment programs, as defined in Section 26-5.4-102(2), C.R.S.; shelter facilities; and psychiatric residential treatment facilities as defined in Section 25.5-4-103(19.5), C.R.S. A residential child care facility may be eligible for designation by the executive director of the state Department pursuant to Article 65 of Title 27. A child who is admitted to a residential child care facility must be:

- A. Five (5) years of age or older but less than eighteen (18) years of age; or
- B. Less than twenty-one (21) years of age and placed by court order or voluntary placement; or
- C. Accompanied by a parent if less than five (5) years of age.

“Residential Child Care Provider Training Academy”, as defined in Section 26-6-923, C.R.S., also known as “Colorado Provider Training Academy”, is created in the State Department to facilitate a pipeline of high-quality staff for residential child care providers and ensure that individuals hired to work at residential child care facilities receive the necessary training to perform an individual’s job functions responsibility and effectively. The Residential Child Care Provider Training Academy is associated and a subpart of the Colorado Provider Training Academy.

The training academy is designed to facilitate, build, coach, and support providers in the promoting high-quality staff to ensure individuals hired to work in child care facilities receive the necessary training to perform their job functions competently, responsibly, and effectively, utilizing trauma-informed practices. This involves all staff who are direct care workers, supervisors, case management staff, clinical staff, managers, administrators, and directors.

“Residing in the home” means all individuals whose home base for a period of time is in the certified foster or kinship home for:

- A. Fourteen (14) consecutive calendar days; or
- B. For more than forty (40) calendar days per year.

“Respite child care center” means a facility for the purpose of providing temporary twenty-four (24)-hour group care for three (3) or more children or youth who are placed in certified foster care homes or approved noncertified kinship care homes, and children or youth with open cases through a Regional Accountable Entity. A respite child care center is not a treatment facility, but rather its primary purpose is providing recreational activities, peer engagement, and skill development to the children and youth in its care. A respite child care center serves children and youth from five (5) years of age to twenty-one (21) years of age. A respite child care center may offer care for only part of a day. “Respite child care” means an alternate form of care to enable caregivers to be temporarily relieved of caregiving responsibilities, as defined at Section 26-6-903(31), C.R.S.

“Restraint” means, in accordance with Section 26-20-102(6), C.R.S., any method or device used to involuntarily limit freedom of movement, including bodily physical force, mechanical devices, or chemicals. Restraint must not be used as a form of discipline or to gain compliance from a child/youth. If property damage might be involved, restraint may only be used when the destruction of property could possibly result in bodily harm to the individual or another person. “Restraint” includes chemical restraint, mechanical restraint, and physical restraint. “Restraint” does not include:

- A. The use of any form of restraint in a licensed or certified hospital when such use:
 - 1. Is in the context of providing medical or dental services that are provided with the consent of the individual or the individual's guardian; and
 - 2. Is in compliance with industry standards adopted by a nationally recognized accrediting body or the Conditions of Participation for federal Medicare and Medicaid programs;
- B. The use of protective devices or adaptive devices for providing physical support, prevention of injury, or voluntary or life-saving medical procedures;
- C. The holding of an individual for less than one (1) minute by a staff person for the protection of the individual or other persons; except that nothing in Section 26-20-102(6)(C) may be interpreted to permit the holding of a public school student in a prone position, except as described in Section 26-20-111(2), (3), or (4), C.R.S.; or
- D. Placement of an inpatient or resident in their sleeping room for the night.

"Seclusion" means the placement of an individual alone in a room or area from which egress is involuntarily prevented, except during normal sleeping hours, pursuant to Section 26-20-102(7), C.R.S.

"Secure Residential Treatment Center," means a facility operated under private ownership that is licensed by the Department pursuant to Part 9 of Article 6 of Title 26 to provide twenty-four (24)-hour group care and treatment in a secure setting for five (5) or more children or persons up to the age of twenty-one (21) years over whom the juvenile court retains jurisdiction pursuant to Section 19-2.5-103(6), C.R.S. who are committed by a court, pursuant to an adjudication of delinquency or pursuant to a determination of guilt of a delinquent act or having been convicted as an adult and sentenced for an act that would be a crime if committed in Colorado, or in the committing jurisdiction, to be placed in a secure facility as defined at Section 26-6-903(32), C.R.S. Secure residential treatment centers are further detailed at 12 CCR 2509-8 Section 7.713, "Minimum Rules and Regulations for Secure Residential Treatment Centers".

"Serious emotional disturbance" means a diagnosable mental, behavioral, or emotional disorder that is of sufficient duration and has resulted in a functional impairment that substantially interferes with or limits a child's/youth's role or functioning in family, school, or community activities. Serious emotional disturbances do not include developmental disorders, substance-related disorders, or conditions or problems that may be a focus or clinical attention unless they occur with another diagnosable serious emotional disturbance.

"Serious illness" means a health condition that carries a high risk of mortality and/or either negatively impacts a person's daily function or quality of life, or excessively strains their caregivers.

“Serious injury” means an injury that results in either severe laceration, a broken or distorted extremity, crush injuries, suspected skull, chest, or abdominal injury other than bruises or minor lacerations, significant burns, unconsciousness, or paralysis.

“Shelter care services” means basic short-term services for children/youth placed by a county department of human/social services, including, but not limited to, shelter, food, education, clothing, recreation, basic medical care, supervision, and guidance.

“Sibling” means one (1) or more individuals having one (1) or both parents in common.

“Social and behavioral sciences” includes sociology, psychology, social work, criminal justice, human services, human development, and counseling.

“Specialized Group Facility,” as defined at Section 26-6-903(34), C.R.S. means a facility sponsored and supervised by a county department or a licensed child placement agency for the purpose of providing twenty-four (24)-hour care for three (3) or more children, but fewer than twelve (12) children, whose special needs can best be met through the medium of a small group. A child who is admitted into a specialized group facility must be:

- A. At least seven (7) years of age or older but less than eighteen (18) years of age;
- B. Less than twenty-one (21) years of age and placed by court order or voluntary placement; or
- C. Accompanied by a parent or legal guardian if less than seven (7) years of age.

“Specialized group facility” includes specialized group homes and specialized group centers.

A “Specialized Group Home” is located in a house owned or otherwise controlled by the group home parents who are primarily responsible for the care of the children/youth and reside at the home.

A “Specialized Group Center” is located in a facility owned or controlled by a governing body that hires the group center parents or personnel who are primarily responsible for the care of the children/youth.

“Staff member” of a facility as used in these rules includes individuals working as a specialized group home parent or in a specialized group center, or residential child care facility. For Section 7.714.53 through Section 7.714.536, a “staff member” of the facility also includes an individual certified as a foster parent in a foster home or working in a day treatment facility or secure residential treatment facility.

“Therapeutic foster care”, as defined in Section 26-6-903(35) C.R.S., means a program of foster care that incorporates treatment for the special physical, psychological, or emotional needs of a child placed with specially trained foster parents, but does not include medical foster care. Therapeutic foster care is further detailed at 12 CCR 2509-8 Section 7.703, Rules Regulating Therapeutic Foster Care.

“Therapeutic wilderness program” means a residential child care program in which the program is based outside of a building or physical space and the children/youth move from one site to another throughout the course of the program. The wilderness program may originate in Colorado or move into and/or through Colorado from another state. The program operates for three (3) or more consecutive twenty-four (24)-hour days during one (1) or more seasons of the year for the care of children/youth who are at least twelve (12) years to eighteen (18) years of age and for those persons twenty-one (21) years of age who are placed by court order prior to their eighteenth (18) birthday, are a voluntary placement, or are participating in the foster youth in transition program as defined in 12 CCR 2509-3 Section 7.203.4 through Section 7.203.43. The program must have as its purpose an individualized child and family plan through group learning experience offering behavioral, mental health, substance abuse education, and recreational activities utilizing an outdoor wilderness environment.

“Trails” means the statewide comprehensive child welfare case management system.

“Transitional measure” means physical guidance, prompting techniques of short duration or an initial, temporary, approved physical positioning of an individual at the onset or in response to a re-escalation during a physical management, for the purpose of quickly and effectively gaining physical control of that individual in order to prevent harm to self or others. Momentary utilization of a short term (as quickly as possible, but not to exceed five (5) minutes) prone position is only permissible during a transitional measure.

“Treatment foster care”, as defined at Section 26-6-903(36), C.R.S., means a clinically effective alternative to a residential treatment facility that combines the treatment technologies typically associated with more restrictive settings with a nurturing and individualized family environment. Treatment foster care is further detailed at 12 CCR 250-8 Section 7.704, Rules Regulating Treatment Foster Care.

“Trauma-Informed” refers to the services to be provided to or on behalf of a child or youth under an organizational structure and treatment framework that involves understanding, recognizing, and responding to the effects of all types of trauma and in accordance with recognized principles of a trauma-informed approach and trauma-specific interventions to address trauma’s consequences and facilitate healing.

“Unaccompanied refugee minor (URM)” means a minor who has been displaced from their home country and is unable to return due to concerns for their safety, who has been determined eligible by the federal government for resettlement in the U.S., and whose parent(s) and other relatives and caregivers are unable to be located or are deceased.

“Unaccompanied refugee minor program (URM)” means a program authorized by the U.S. Office of Refugee Resettlement (ORR) to provide specialized culturally and linguistically appropriate foster care and independent living services that facilitate social, cultural, and economic integration for children and youth who have been determined eligible to resettle in the United States through foster care with an additional goal to reunify program youth with family or kinship when appropriate.

“Voluntary placement” means a child/youth who, meets the admission criteria of the RCCF, can benefit from the services provided, and is voluntarily placed in a facility/agency via a county department of human/social services’ voluntary placement agreement with the parent(s) or guardian(s) of the child/youth in accordance with Section 19-1-127, C.R.S., or through independent entities choosing to voluntarily place at a facility/agency or program.

“Vulnerable youth” means a youth who is emancipating from foster care, a runaway or homeless youth, a youth involved in the juvenile justice system, an immigrant youth and youth with limited English proficiency, a youth with physical and/or mental disabilities, a youth with mental disorders, or a youth receiving special education services. Vulnerable youth encompass children and youth who do not have the capacities to achieve a typical level of functioning in one or more life domains or to some extent have special needs in regard to support or helping aids to achieve a typical level of functioning and thriving.

“Youth” pursuant to Section 19-1-103(145), C.R.S. means an individual who is less than twenty-one years of age.

7.701.3 APPLICATION PROCESS

7.701.31 Original Application

- A. A completed, original application accompanied by the appropriate fee must be submitted to the Department a minimum of sixty (60) days prior to the proposed opening date for the facility. The application submitted must include the attachments with specific requirements completed, in accordance with the addendum associated with the license type.
- B. A licensing audit will occur only after the Department has received the completed application and appropriate fee.

- C. If a county or agency establishes and plans to sponsor a Specialized Group Facility, the governing body for the Specialized Group Facility is the licensee. A written plan for the supervision and responsibilities in accordance with 12 CCR 2509-8 Sections 7.710.46 and 7.710.47 of the Specialized Group Facility must accompany the application.

7.701.32 BIU (Background Investigation Unit) - Use of Records and Reports of Child Abuse or Neglect for Background and Employment Inquiries

- A. The Trails child abuse and neglect records request must be accompanied by the applicant's prior written or electronic authorization to obtain such information from the state automated system. The Trails child abuse and neglect records request must be accompanied by the required fee, according to Section 7.701.4, paid by check, credit card, money order, electronic payment, or other means specified by the Department.
- B. An operator of a licensed facility must submit a request to determine if an operator, applicant for employment or foster care, current employee, or any individual who has direct contact with children/youth or is associated with the license, has been found responsible for a confirmed, as defined by Section 19-1-103(33), C.R.S., report of child abuse or neglect in the Department's automated system. All requests must be submitted on or up to thirty (30) days prior to the first day of employment or service.
 - 1. For all individuals whose activities involve the care or supervision of children/youth for any length of time or who have unsupervised access to children/youth, requests must be submitted and successfully completed prior to caring for children/youth or having unsupervised contact with children/youth.
- C. Pursuant to Section 26-6-912(1)(a)(I)(B), C.R.S., and Section 26-6-912(1)(a)(I)(D)(II)(B), C.R.S., the agency must require any applicant for foster care to complete a request to determine if they have been found responsible for a confirmed, as defined by Section 19-1-103(33), C.R.S., report of child abuse or neglect in the Department's automated system. Results must be received and reviewed prior to the certification of the foster home.
 - 1. The agency must require any person eighteen (18) years of age or older who resides in the foster care home to complete a request to determine if they have been found responsible for a confirmed report of child abuse or neglect in the State Department's automated system. Residing in the home applies to all individuals whose home base for a period of time is in the foster home for fourteen (14) consecutive calendar days or for more than forty (40) calendar days per year.

2. For each adult eighteen (18) years of age or older who begins residing in the foster or specialized group home after care begins, requests must be submitted within five (5) calendar days of when the individual begins residing in the home, and these adults must not be allowed unsupervised access to children/youth in care until all background checks have been successfully completed.
- D. The Department will inform the requesting party in writing of whether the individual has been confirmed, as defined by Section 19-1-103(33), C.R.S., to be a person responsible for an incident of child abuse or neglect.
1. If the result of the inquiry is that the individual has been confirmed, as defined by Section 19-1-103(33), C.R.S., as responsible for an incident of child abuse or neglect, the Department must provide the requesting party with information regarding the date of the reported incident, the type of abuse or neglect with the severity level, and the county department that confirmed the report.
 2. If the result of the inquiry is that the individual has not been confirmed, as defined by Section 19-1-103(33), C.R.S., to be responsible for an incident of child abuse or neglect, the Department must notify the requesting party of this fact.
- E. The information provided by the Department must serve only as the basis for further investigation. The director or operator may inform an applicant or employee that the report from the Department's automated system was a factor in the director or operator's decision with regard to the applicant or employee's employment.
- F. When a facility or agency receives a result that an individual has been confirmed, as defined by Section 19-1-103(33), C.R.S., as responsible for an incident of child abuse and/or neglect, the agency or facility must notify their licensing specialist within five (5) business days. The notification must address whether the individual will continue involvement with the facility or agency and in what capacity or if they will no longer be working with that licensed facility.
- G. Any person who willfully permits or who encourages the release of data or information related to child abuse or neglect contained in the Department's automated system to persons not permitted access to such information commits a Class 1 misdemeanor and must be punished as provided in Section 18-1.3-501, C.R.S.
- H. Every five (5) years, all child abuse and neglect inquiry background checks must be renewed by resubmitting an inquiry form and current fee to the Department for processing. An updated clearance letter or verification of the submission of the inquiry form must be obtained before five (5) years from the date reflected on the current clearance letter.

- I. The results of the abuse and neglect inquiry must be maintained at the facility or agency and must be available for review upon request by a Department representative.
- J. Any person who has lived in another state in the five (5) years previous to application for employment or foster care, any person who is engaged with children/youth at a facility or agency, or any person eighteen (18) years of age or older who resides in the foster care home must provide a copy of the state child abuse and neglect registry check from each state in which they have resided
- K. A child placement agency ("CPA") must also complete a review utilizing the CPA supervisor profile in the state automated system of all existing county department case records. This includes a review of the state automated child welfare system for each applicant and persons over the age of eighteen years (18) years of age residing in a family foster care home.
- L. The Trails child abuse and neglect records checks are not required for temporarily placed children/youth who are eighteen (18) years of age or older who are not legal, permanent residents of the certified kinship or foster home, agency, or facility.

7.701.321 Required Review of Records for History of Sex Offender

An operator of a licensed facility must conduct sex offender searches in the Colorado Bureau of Investigation (CBI) sex offender registry and the national sex offender public website operated by the United States Department of Justice for each operator, applicant for employment or foster care, current employee, or any individual who has direct contact with children/youth or is associated with the license prior to the first day of employment or service at the agency/facility and annually thereafter. Sex offender searches are not required for temporarily placed youth who are eighteen (18) years of age or older who are not legal, permanent residents of the certified kinship or foster home, agency, or facility. Records of the checks must be maintained in the employee file and include the following criteria at minimum:

- A. Known current names, prior names, and addresses of each individual, and
- B. Addresses only of the foster care home, kinship care home, or specialized group home.

7.701.33 Criminal Record Check

- A. Criminal records checks are required under the following circumstances:

1. In order to obtain any Colorado Bureau of Investigation (CBI) and/or Federal Bureau of Investigation (FBI) fingerprint criminal history records, each applicant listed below must have their fingerprints taken and processed. All requests must be submitted on or up to thirty (30) days prior to the first day of employment or service. Fingerprints must be taken at a vendor approved by CBI. Approved vendors may be located using the CBI website at Colorado.gov/cbi. Payment of the fee for the criminal record check is the responsibility of the individual being checked, identified as follows:
 - a. Each applicant for an original license for a facility or agency, applicant for a certificate at a family foster home, and any adult eighteen (18) years of age or older who resides in the licensed facility or agency.
 - b. Applicants for an original certificate for a foster care home, and any adult eighteen (18) years of age or older who resides in the foster care home.
 1. For each adult eighteen (18) years of age or older, who begins residing in the foster or specialized group home after care begins, fingerprints must be submitted within five (5) calendar days of when they begin residing in the home, and these adults must not be allowed unsupervised access to children/youth in care until all background checks have been successfully completed.
 - c. Any person working in an agency or facility including any individuals who have direct contact with children/youth or have access to protected information, volunteers as stated in Section 7.701.33.C.2-3, self-employed individuals, or other individuals associated with the license.
- B. The results of the criminal records check, including at least the individual CBI and FBI response letters, must be maintained at the facility or agency and must be available for review by a department representative. Except for records of official actions, which must be available for inspection, all other criminal justice records may be open for inspection subject to the discretion of the official custodian. The Colorado Criminal Justice Records Act (CCJRA) defines an official action as an arrest; indictment; charging by information; disposition; pre trial or post trial release from custody; judicial determination of mental or physical condition; decision to grant, order, or terminate probation, parole, or participation in correctional rehabilitation programs; and any decision to formally discipline, reclassify, or relocate any person under criminal sentence according to Section 24-72-302(7), C.R.S.

1. Letters and information received by a facility or agency that include information protected in the Criminal Justice Information System (CJIS) must immediately pass the letter or information to the subject of the fingerprint check without opening and viewing the information. Such correspondence will be mailed to the facility or agency mailing address and will be marked "to applicant" in the address field.
 2. The facility or agency must not require any applicant to divulge background information that is protected in the Criminal Justice Information System (CJIS) as a requirement for employment or certification.
 3. For all individuals whose activities involve the care or supervision of children or youth or who have unsupervised access to children/youth, all background checks must be submitted prior to caring for children or youth and completed prior to allowing unsupervised access to children/youth.
- C. Each owner, employee of a facility or agency, and licensee shall complete the fingerprint process at an approved vendor. Payment of the fee for the criminal record check is the responsibility of the individual being checked or the facility or agency. The results of the criminal record check and the CBI and FBI response letters, must be maintained at the facility or agency and must be available for review upon request by a Department Representative.
1. When an individual leaves employment, the facility/agency must submit to the Department within thirty (30) days a completed notification of name removal form to request the removal of the individual's name from their facility/agency license number in the Department background flag database. School district employees who currently work at a licensed child welfare child care facility must have their criminal history report linked to the license number of the child care facility.
 2. Any adult volunteer working as a staff member to meet the required staff-child ratio or staff qualifications, who works fourteen (14) days (112 hours) or more in a calendar year must complete the fingerprint process at an approved vendor. The results of the criminal record check must be maintained at the facility or agency and must be available for inspection by a Department Representative. An individual operating as a volunteer to meet required staff-child ratio or staff qualifications that does not have a completed background check on file must be supervised at all times by a qualified staff member who has successfully completed all background checks.

3. Criminal background check requests for volunteers, whose activities involve the care and supervision of children/youth or who have unsupervised access to children/youth, must be submitted and successfully completed prior to caring for children/youth or allowing the individual unsupervised access to children/youth in a child care facility.
4. Every five (5) years, requests for FBI criminal record checks must be renewed by completing the fingerprint process at an approved vendor. An updated clearance letter or verification of the submission of the request must be obtained prior to five (5) years from the date reflected on the current clearance letter.
5. Facilities and agencies that hire individuals who have been convicted of any felony, except those listed in Subsection 6.A-F below, unlawful sexual behavior, or any misdemeanor, the underlying factual basis of which has been found by the court on record to include an act of domestic violence must inform the Department of that hiring within fifteen (15) calendar days of receiving knowledge of the conviction.
6. A child care facility shall not employ, or a child placement agency shall not employ or certify, an individual who has been convicted of:
 - a. Child abuse, as specified in Section 18-6-401, C.R.S.
 - b. A crime of violence, as defined in Section 18-1.3-406, C.R.S.
 - c. An offense involving unlawful sexual behavior, as defined in Section 16-22-102(9), C.R.S.
 - d. A felony, the underlying factual basis of which has been found by the court on the record to include an act of domestic violence, as defined in Section 18-6-800.3, C.R.S.
 - e. A felony involving physical assault, battery, or a drug-related offense within the five (5) years preceding the date of application for a license or certificate.
 - f. A pattern of misdemeanor convictions within the ten (10) years immediately preceding submission of the application. "Pattern of misdemeanor" shall include consideration of Section 26-6-914(2), C.R.S., regarding suspension, revocation and denial of a license, and shall be defined as:

1. Three (3) or more convictions of third (3rd) degree assault as described in Section 18-3-204, C.R.S., and/or any misdemeanor, the underlying factual basis of which has been found by any court on the record to include an act of domestic violence as defined in Section 18-6-800.3, C.R.S.; or
 2. Five (5) misdemeanor convictions of any type, with at least two (2) convictions of third (3rd) degree assault as described in Section 18-3-204, C.R.S., and/or any misdemeanor, the underlying factual basis of which has been found by any court on the record to include an act of domestic violence as defined in Section 18-6-800.3, C.R.S. or,
 3. Seven (7) misdemeanor convictions of any type.
- g. Any offense in any other state, the elements of which are substantially similar to the elements listed in this Subsection a-f.
- D. Payment of the fee for the FBI check is the responsibility of the individual who is obtaining the check or the facility or agency.
- E. In multi-service agencies, only individuals who are the applicant for, a person employed by the applicant, or a person who resides at the facility or home may be submitted for a fingerprint based criminal background check under the authority of Section 26-6-910(5), C.R.S.
- F. According to Section 26-6-914(2), C.R.S., the Department may deny an application or suspend, revoke, or make probationary the license, of any facility or agency regulated and licensed pursuant to Part 9 of Article 6 of Title 26 or assess a fine against the licensee pursuant to Section 26-6-921, C.R.S. If the licensee, an affiliate of the licensee, a person employed by the licensee, or a person who resides with the licensee at the facility or agency:

1. Is convicted of a felony, other than those offenses specified in Section 26-6-905(8), C.R.S., or child abuse, as specified in Section 18-6-401, C.R.S., the record of conviction being conclusive evidence thereof, notwithstanding Section 24-5-101, C.R.S., or have entered into a deferred judgment agreement or a deferred prosecution agreement to a felony, other than those offenses specified in Section 26-6-905(8), C.R.S., or child abuse, as specified in Section 18-6-401, C.R.S., or if the Department has a certified court order from another state indicating that the applicant, licensee, person employed by the licensee, or any person residing with the licensee has been convicted of a felony, other than those offenses specified in Section 26-6-905(8), C.R.S., under a law of another state or of the United States or has entered into a deferred judgment agreement or a deferred prosecution agreement in another state as to a felony, other than those offenses specified in Section 26-6-905(8), C.R.S.; or
2. Is convicted of third (3rd) degree assault, as described in Section 18-3-204, C.R.S.; any misdemeanor, the underlying factual basis of which has been found by the court on the record to include an act of domestic violence, as defined in Section 18-6-800.3, C.R.S.; the violation of a protection order, as described in Section 18-6-803.5, C.R.S.; any misdemeanor offense of child abuse, as defined in Section 18-6-401, C.R.S.; or any misdemeanor offense in any other state, the elements of which are substantially similar to the elements of any one of the offenses described in this paragraph; or
3. Is determined to be insane or mentally incompetent by a court of competent jurisdiction, and a court has entered, pursuant to Part 3 or Part 4 of Article 14 of Title 15, or Section 27-65-110(4), C.R.S. or Section 27-65-127, C.R.S., an order specifically finding that the mental incompetency or insanity is of such a degree that the licensee is incapable of operating a facility or agency, the record of such determination and entry of such order being conclusive of evidence thereof; or
4. Uses any controlled substance, as defined in Section 18-18-102(5), C.R.S., including retail marijuana, or consumes any alcoholic beverage during the operating hours of the facility or agency or is under the influence of a controlled substance or alcoholic beverage during the operating hours of the facility or agency; or
5. Is convicted of unlawful use of a controlled substance as specified in Section 18-18-404, C.R.S.; unlawful distribution, manufacturing, dispensing, sale, or possession of a controlled substance as specified in Section 18-18-403.5 or 18-18-405, C.R.S.; or unlawful offenses relating to marijuana or marijuana concentrate as specified in Section 18-18-406, C.R.S.; or

6. Consistently fails to maintain standards prescribed and published by the Department; or
7. Furnishes or makes any misleading or any false statement or report to the Department; or
8. Refuses to submit to the Department any reports or refuses to make available to the Department any records required by it in making investigation of the facility or agency for licensing purposes; or
9. Fails or refuses to submit to an investigation or inspection by the Department or to admit authorized representatives of the Department at any reasonable time for the purpose of investigation or inspection; or
10. Fails to provide, maintain, equip, and keep in safe and sanitary condition premises established or used for child care pursuant to standards prescribed by the Department of Public Health and Environment and the Department of Human Services or by ordinances of regulations applicable to the location of such facility; or
11. Willfully or deliberately violates any of the provisions of Part 9 of Article 6 of Title 26 or any of the standards prescribed and published in Department rule pursuant to Part 9 of Article 6 of Title 26; or
12. Fails to maintain financial resources adequate for the satisfactory care of children served in regard to upkeep of premises and provision for personal care, medical services, clothing, and other essentials in the proper care of children; or
13. Is charged with the commission of an act of child abuse or an unlawful sexual offense, as specified in Section 18-3-411(1), C.R.S., if:
 - a. The individual has admitted committing the act or offense and the admission is documented or uncontroverted; or
 - b. The Administrative Law Judge finds that such charge is supported by substantial evidence; or
14. Admits to an act of child abuse or if substantial evidence is found that the licensee, person employed by the licensee, or person who resides with the licensee in the licensed facility or agency has committed an act of child abuse, as defined at Section 19-1-103(1), C.R.S.; or
15. Is the subject of a negative licensing action; or

16. Misuses any public funds that are provided to a foster care home, or child placement agency that places or arranges for placement of a child in foster care, for the purposes of providing foster care services, child placement services related to the provision of foster care, or any administrative costs related to the provision of foster care services or foster-care-related child placement services.
- G. The Department may deny an application for a child care facility license or a child placement agency license if the applicant is a relative or affiliate of a licensee, as defined in Section 26-6-903(28), C.R.S. and Section 26-6-903(1), C.R.S., of a child care facility or child placement agency, which is the subject of a previous negative licensing action or is the subject of a pending investigation by the Department that may result in a negative licensing action.
- H. For all CBI fingerprint-based criminal history record information checks required in this Section 7.701.33, including those confirming a criminal history as well as those confirming no criminal history, the Department will conduct a comparison search on the State Judicial Department's court case management system and the sex offender registry of the Colorado Department of Public Safety. The court case management search must be based on name, date of birth, and address, in addition to any other available criminal history data that the Department deems appropriate, is used to determine the type of crime(s) for which a person was arrested or convicted and the disposition thereof. The sex offender registry search is used to determine whether the address of a licensee or prospective licensee is listed as belonging to a registered sex offender, except that:
 1. County departments of human/social services must conduct sex offender searches in the CBI sex offender registry and the national sex offender public website operated by the United States Department of Justice prior to certification and annually; include a copy in the provider record using the following criteria at a minimum:
 - a. Known names and addresses of each adult residing in the foster care home or kinship foster care home; and,
 - b. Address only, of the foster care home or the kinship foster care home.
 2. Child placement agencies must conduct sex offender searches in the CBI sex offender registry and the national sex offender public website operated by the United States Department of Justice prior to certification and annually, include a copy in the provider record using the following criteria at a minimum:
 - a. Known names and addresses of each adult residing in the foster care home or kinship foster care home; and,

- b. Address only of each adult residing in the foster care home or the kinship foster care home.

I. Portability of Background Checks

- 1. Where two (2) or more individually licensed facilities are wholly owned, operated, and controlled by a common ownership group or school district, a fingerprint-based criminal history records check and a check of the records and reports of child abuse or neglect maintained by the Department, completed for one (1) of the licensed facilities of the common ownership group or school district pursuant to this Section for whom a criminal records check is required under Section 26-6-912, C.R.S., may satisfy the records check requirement for any other licensed facility under the same common ownership group or school district. A new fingerprint-based criminal history records check or new check of the records and reports of child abuse or neglect maintained by the Department is not required of such an individual if the common ownership group or school district maintains a central records management system for employees of all its licensed facilities; takes action as required pursuant to Section 26-6-905, C.R.S., when informed of the results of a fingerprint-based criminal history records check or check of the of records and reports of child abuse or neglect maintained by the Department that requires action pursuant to the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act; and informs the Department whenever an additional licensed facility comes under or is no longer under its ownership or control.
- 2. When a licensee is inspected pursuant to the Foster Care, Kinship Foster Care, Residential, Day Treatment, and Child Placement Agency Licensing and Certification Act and records regarding CBI and FBI fingerprint-based criminal background checks, as well as records and reports of child abuse and neglect maintained by the Department, and the comparison search on the State Judicial Department's court case management system are held at a central records management system, the licensee must be afforded fourteen (14) calendar days to provide to the Department documentation necessary to verify that employees at the licensed facility have the required records related to fingerprint-based criminal background checks.
- 3. When an organization completes all fingerprint checks under a singular provider number and the license for that provider number is either surrendered, revoked, or closed, the organization must submit new fingerprint-based criminal history record checks under at least one (1) of its current active provider licenses for every employee working in the facility or agencies that continue to hold a child care license.

J. State-based background checks

1. The following background check requests must be submitted and successfully completed for each state an individual has resided in, in the past five (5) years, prior to an individual caring for children or allowing the individual unsupervised access to children in child care facilities:
 - a. State criminal history check.
 - b. State sex offender registry check.
 - c. State abuse and neglect registry check
- K. Any person who has lived in another state in the five (5) years previous to application for employment or involvement with the agency/facility must inform the agency/facility to obtain a copy of the state criminal history check from each state in which they had resided. This is a fingerprint or name-based criminal history check through an approved state agency of previous residence to determine if prospective employees who have lived in other states prior to living in Colorado within five (5) years of applications have been convicted of any criminal offenses that are identified in the foster care, residential, day treatment and child placement agencies licensing and certification act. This check is not required for states participating in the National Fingerprint File (NFF) program.
- L. Criminal record checks are not required for temporarily placed youth who are eighteen (18) years of age or older that are not legal, permanent, and habitual residents of the certified kinship or foster home, agency, or facility.

7.701.331 Volunteers and Interns

- A. If volunteers and/or interns are used by a facility, the facility's administration must define specifically the services to be given or provided by that individual.
- B. Facilities and agencies must ensure, prior to working in the facility, volunteers and interns:
 1. Have a completed information form including name, address, phone number, date of birth, and person to contact in case of emergency including name, address, and phone number.
 2. Have a signed acknowledgement form including the volunteer or intern's specific roles and responsibilities, and a signed mandatory abuse/neglect reporting form.
 3. A volunteer or intern must have an orientation that includes, at minimum:
 - a. A facility overview;
 - b. Information about the facility and description of the population served;

- c. The mission statement and philosophy of the facility;
 - d. Safety procedures of the facility;
 - e. The confidential nature of the work; and
 - f. Their specific job assignment, prior to the assignment.
- C. Volunteers or interns who work on behalf of the facility but do not come to the facility for the fulfillment of their duties, are excluded from background check requirements in this Section.
- D. Volunteers and interns who work directly with children/youth must be under the direct supervision of a paid staff member at all times, in addition to the requirements in Section 7.701.331.B must:
 - 1. Be subject to reference checks as performed for employment applicants.
 - 2. Be subject to a check of the Background Investigation Unit, Trails abuse and neglect system to determine if the person was found to be responsible in a confirmed, as defined by Section 19-1-103(33), C.R.S., report of child abuse or neglect reported to the Department's automated system. Parental consent must be obtained to conduct a background check on a minor under the age of eighteen (18). Background check results must be maintained at the licensed facility or agency and made available to the Department upon request.
 - 3. Individuals must not be permitted to volunteer or intern if the individual suspects they might be infected with a contagious disease.
- E. Volunteers and interns whose activities involve the care and supervision of children/youth or who have unsupervised access to children/youth must be under the professional supervision of a paid staff member and may work alone with the child/youth in individual sessions or mentoring. In addition to requirements in Section 7.701.331.B and C, they must:
 - 1. Be subject to CBI and FBI fingerprint based background checks required for all staff members. Parental consent must be obtained to conduct a background check on a minor under the age of eighteen (18);
 - 2. Be subject to a check of the Background Investigation Unit, Trails abuse and neglect system to determine if the person was found to be responsible in a confirmed report of child abuse or neglect reported to the Department's automated system. Parental consent must be obtained to conduct a background check on a minor under the age of eighteen (18);
 - 3. Be subject to training as required in Section 7.714.92.B;

4. Be currently licensed in the state of Colorado if providing professional services to children/youth;
 5. Be subject to face-to-face supervision with a paid staff member providing supervision and guidance of the volunteer or intern on at least a weekly basis; and
 6. Have a performance review, by a paid staff member on at least a semi-annual basis.
- F. Volunteers and interns are not substitutes for paid staff members, and the facility must be fully staffed according to the requirements herein.

7.701.34 Fire and Health Inspections, Zoning Codes

- A. Prior to the original license being issued, following the renovation of the facility that would affect the licensing of the facility and at least every two (2) years thereafter, all child care facilities except family foster homes must be inspected and obtain an approving inspection report from the local department of health or the Colorado Department of Public Health and Environment and from the local fire department. These reports must be maintained at the facility and be available for review upon request by a Department Representative. The fire authority will inspect each facility at annual intervals for compliance.
- B. Prior to the original license being issued, all child care facilities except for foster homes must submit to the Department written approval from the local zoning department approving operation of the facility. The approval must include the address of the child care facility and the ages and number of children/youth to be served. The facility must also submit written zoning department approval to the Department any time there is a change to the license, including moving the facility to another location, increasing the capacity, or adding different ages of children/youth.
- C. All child care facilities must operate in compliance with local planning and zoning requirements of the municipality, city, and county where the facility is located.
- D. According to Section 26-6-909(2)(e)(ii), C.R.S., a facility that provides child care exclusively to school-age children and operates on the property of a school district, district charter school, or institute charter school may satisfy any fire or radon inspection requirement required by law by providing a copy of the satisfactory fire or radon inspection report of the property of a school district, district charter school, or institute charter school where the child care is provided if the fire or radon inspection report was completed within the preceding twelve (12) months. The Department shall not require a duplicate fire or radon inspection if a satisfactory fire or radon inspection report of the property was completed within the preceding twelve (12) months.

7.701.35 Changes Requiring a New Application

Pursuant to Section 26-6-907(2), C.R.S., a license is deemed surrendered, and a new application is required in any of the following circumstances:

- A. Change of licensee, owner, or governing body;
- B. Change in classification of facility or service offered; or
- C. Change in location of the facility.

7.701.36 Types of Licenses

7.701.361 Permanent License

- A. A permanent license is granted when the Department is satisfied that the facility or agency is in compliance with the appropriate Department rules and Section 26-6-901 through Section 26-6-923, C.R.S. The permanent license remains in effect until surrendered, suspended, or revoked.
- B. Once a permanent license has been issued, the licensee must annually submit to the Department a declaration of compliance with the applicable licensing rules and notice of continuing operation on the form prescribed by the Department, along with the appropriate annual fee as set forth at Section 7.701.4.
- C. Failure to submit the annual Continuation Notice and fee will constitute a consistent failure to maintain Department standards and may result in fines or the revocation of the license.

7.701.362 Time-Limited License

- A. A time-limited license is granted for specific types of child care facilities or agencies when the Department is satisfied that the facility or agency is in compliance with the appropriate Department rules and Sections 26-6-901 through 26-6-923, C.R.S. The time-limited license will expire on a set date.
- B. Once a time-limited license has been issued, the licensee must submit a renewal application and appropriate fee prior to the expiration of the time-limited license. This will keep the license in effect until a new time-limited license can be issued.
- C. Failure to submit the renewal application prior to the expiration of the time-limited license will result in the expiration of the license and closure of the facility.

7.701.363 Provisional License

- A. Pursuant to Section 26-6-906(3), C.R.S. a provisional license or certificate may be issued only for the initial six (6) month licensing period in the event of a zoning or other delay or dispute between a facility and the municipality, city and county, or county where the facility is situated.

- B. This license permits the facility to operate while it is temporarily unable to conform to all rules upon proof by the applicant that attempts are being made to comply with the rules.
- C. If an applicant holds a valid provisional license at the time of re-application for a permanent license, the provisional license will remain in effect until the new application is acted on by the Department.

7.701.364 Probationary License

- A. The Department may make the license of any facility probationary as provided in Section 26-6-914(2), C.R.S. Making a license probationary is a negative licensing action as defined in Section 26-6-903(21), C.R.S.
- B. If the applicant holds a valid probationary license and submits the renewal application and appropriate fee for a permanent license, the current license will remain in effect until the renewal application is acted on by the Department.

7.701.365 Multiple Licenses

- A. If a licensee wishes to assume child care responsibility in more than one (1) classification of care, separate applications, fees, and licensing evaluations are required for each classification. A family child care home and a specialized group home may only be licensed as one type of classification at any one location address.
- B. If a licensee wishes to operate more than one (1) facility of the same classification but at different locations, a separate application, fee, and evaluation are required for each location.
- C. Operating multiple licenses of the same classification at a single location by the same licensee or governing body is prohibited.

7.701.4 FEES

- A. The appropriate application fee outlined in Section 7.701.4.E, must be submitted to the Department with the application for a child care facility or agency license at least ninety (90) calendar days prior to the anticipated opening date of the facility or the expiration date of the one (1) year time-limited, provisional, or probationary license.
- B. The appropriate annual continuation fee outlined in Section 7.701.4.E, must be submitted to the Department annually, at least sixty (60) calendar days prior to the anniversary date of the license, along with a completed continuation declaration.

- C. When a facility moves to a new physical address, an original application and an original fee is required. If a child placement agency moves office locations, a change fee is required along with a letter of notification.
- D. If the licensee already holds a base license of the required type and the licensee wants to add an additional service type to the existing license, the licensee must submit the original application and pay only the fee to add a new initial service to the base license.
- E. Following is a schedule of original and annual continuation/renewal fees for all types of child care facilities and agencies:

Day Treatment Center	
	Beginning July 1, 2025 and beyond*
Original Application	\$911.00
Continuation 0-12 Students	\$552.00
Continuation 13-25 Students	\$901.00
Continuation 26-50 Students	\$1,252.00
Continuation 51 or more Students	\$1,627.00
(*One year from licensed anniversary date)	

Specialized Group Facility	
	Beginning July 1, 2025 and beyond*
Original Application	\$365.00
Continuation	\$275.00
Change of Certifying Agency	\$100.00
(*One year from licensed anniversary date)	

Child Placement Agency- Foster Care	
	Beginning July 1, 2025 and beyond*
Original Application	\$1,428.00
Original Application for Therapeutic Foster Care (Plus CPA Fee Above)	\$300.00
Original Application for Treatment Foster Care (Plus CPA Fee Above)	\$300.00
Renewal 0-5 Homes	\$726.00
Renewal 6-15 Homes	\$926.00
Renewal 16-30 Homes	\$1,141.00
Renewal 31-50 Homes	\$1,352.00
Renewal 51 or More Homes	\$1,578.00

Child Placement Agency- Foster Care
<i>(*One year from licensed anniversary date)</i>
<i>(***With renewal of therapeutic foster care add \$150.00 to listed license fees)</i>
<i>(***With renewal of treatment foster care add \$150.00 to listed license fees)</i>

Child Placement Agency- Adoption	
	Beginning July 1, 2025 and beyond*
Original Application	\$1,091.00
Continuation 0-5 Finalized Adoptions	\$528.00
Continuation 6-11 Finalized Adoptions	\$615.00
Continuation 12-17 Finalized Adoptions	\$651.00
Continuation 18-23 Finalized Adoptions	\$726.00
Continuation 24 or More Finalized Adoptions	\$752.00
<i>(*One year from licensed anniversary date)</i>	

- F. A child placement agency licensed for both foster care and adoptions will pay only one fee, either the foster care fee or the adoption fee, whichever is greater. The annual report required by Section 7.710.72.B, must be attached.

Homeless Youth Shelter	
	Beginning July 1, 2025 and beyond*
Original Application	\$911.00
Continuation	\$751.00
<i>(*One year from licensed anniversary date)</i>	

Residential Child Care Facility	
	Beginning July 1, 2025 and beyond* ** ***
Original Application	\$1,803.00
Original Application for QRTF (Plus RCCF Fee Listed Above)	\$500.00
Original Application for PRTF (Plus RCCF Fee Listed Above)	\$500.00
Original Application for Runaway and Homeless Youth and Shelter Care (Plus RCCF Fee Listed Above)	\$250.00
Renewal 0- 12 Children/Youth	\$551.00
Renewal 13-25 Children/Youth	\$901.00
Renewal 26-50 Children/Youth	\$1,252.00
Renewal 51-100 Children/Youth	\$1,627.00

Residential Child Care Facility	
Renewal 101 or More Children/Youth	\$1,854.00
(*One year from licensed anniversary date)	
(**With Renewal of Qualified Residential Treatment Programs (QRTP) Add \$200.00 to Listed License Fees)	
(**With Renewal of Psychiatric Residential Treatment Facilities (PRTF) add \$200.00 to Listed License Fees)	
(**With Renewal of Runaway and Homeless Youth and Shelter Care add \$100.00 to Listed License Fees)	

Secure Residential Treatment Center	
	Beginning July 1, 2025 and beyond
Original Application	\$1,854.00
Continuation 0- 12 Children/Youth	\$1,854.00

Respite Child Care Center	
Original Application	\$365.00
Continuation	\$275.00
(*One Year From Licensed Anniversary Date)	

Changes Made to All License Types	
	Beginning July 1, 2025 and beyond*
Increases to Licensed Capacity	\$100.00
Changes to Physical Premises at Current Address	\$100.00
Duplicate Licenses	\$44.00

- G. International adoption agencies with out-of-state offices will be required to reimburse the Department for actual and necessary charges involved with travel to out-of-state offices.
- H. The appropriate fee must be submitted for each appeal request submitted within each licensing year. There will be no charge for initial hardship and stringency requests or emergency appeals. Section 26-6-907(1)(B)(V), C.R.S. allows for the collection of fees associated with the filing of appeals.

<i>Provider Appeal and Waiver Fees</i>	
County Appeals	Free
Hardship Waivers*	Free
Emergency Appeals	Free
Stringency Appeals*	Free
(*If more than five (5) hardship waivers or stringency appeals are submitted by a facility or agency per month, the sixth (6th) and subsequent appeals will each be charged a fee.)	\$25.00

7.701.5 ADMINISTRATION

7.701.51 Governing Body

- A. The governing body must be identified by its legal name on the original application and annual continuation notice or renewal application. The names and addresses of individuals who hold primary financial control and officers of the governing body must be fully disclosed to the Department.

- B. The governing body must demonstrate to the Department, upon request, that there is sufficient financial support to operate and maintain the facility in accordance with all rules in Section 7.701, the rules regulating the specific type of facility, and the goals and objectives of the facility.

7.701.52 Reports

- A. As defined under Section 7.701.2, a critical incident must be reported for licensed agencies, facilities, and day treatment centers and the licensee, facility, or agency must follow all confidentiality laws and regulations that apply to critical incident reporting.

In no case must a critical incident be reported later than twenty-four (24) hours after receiving information, excluding weekends and state-observed holidays, of the occurrence of a critical incident at the facility or within twenty-four (24) hours of a child's/youth's return to the facility. A report may include more than one (1) child, youth, and/or staff and is required per incident and category type. Incidents must be reported under the most serious category listed below.

1. Death
 - a. Report any child/youth death in the facility or foster home while a child/youth has an open placement, to include while a child/youth is on or off grounds;
 - b. Report death of a child/youth while a child/youth is on the premises of a day treatment center;
 - c. Report death of staff while on duty;
 - d. Report death of foster parent with child(ren)/youth in placement; and
 - e. Report death of volunteer or visitor while on premises.
2. Abuse and neglect (Critical incident reporting must be completed in addition to reporting pursuant to Section 19-3-304, C.R.S.)
 - a. Report any allegation, suspicion, reasonable cause to know, observation, or condition of physical, sexual, verbal, emotional, psychological, human trafficking, or financial abuse to a child/youth when they are in placement or on the premises;
 - b. Report any allegation, suspicion, reasonable cause to know, observation, or condition of physical, deprivation of needs, medical, supervisory, emotional, psychological, or financial neglect to a child/youth while they are in placement or on the premises; and

- c. Report notification of an open investigation conducted by the county department when the allegation originated outside of the agency or facility or for which the agency or facility did not report to the hotline system as set forth in Section 26-5-111, C.R.S.
- 3. Injury
 - a. Report any serious injury to a child/youth that requires emergency medical attention by a healthcare professional at a medical facility or by an emergency medical technician (EMT) or other first responder at a foster home;
 - b. Report any serious injury in which there is no known cause or due to alleged lack of supervision;
 - c. Report any injury, bruise, or abrasion on the child/youth or complaint of physical injury made by the child/youth that occurs as a result of a physical restraint;
 - d. Report any serious injury to a foster parent, staff, volunteer, or visitor as a result of an adverse physical interaction with a child/youth;
 - e. Report any serious injury of a child/youth requiring admission to a hospital; and
 - f. Report any self-injurious behavior by a child/youth that requires emergency medical attention by a healthcare professional outside of the facility or foster home.
- 4. Illness
 - a. Report any serious illness that requires emergency medical attention by a healthcare professional outside of the facility or foster home;
 - b. Report when the wrong medication or dosage is given, or when the prescribed medication is not given to the child/youth, which results in an adverse side effect (physiological or psychological) which requires treatment from a medical professional outside of the facility or foster home to address the adverse effects and ensure the safety of the child/youth to sustain life;
 - c. A communicable reportable disease, as required by the Colorado Department of Public Health and Environment, of a child/youth, or staff member;

- d. Report any suicidal attempt by a child/youth that requires emergency medical attention by a health care professional outside of the facility;
 - e. Report if a child/youth is placed on any involuntary hold;
 - f. Report if a child, foster parent, or staff on duty receives medical or emergency attention outside of the facility as a result of a drug or alcohol-related incident; and
 - g. Report any serious illness of a child/youth requiring admission to a hospital.
- 5. Emergency response
 - a. Report if a fire department responds to a fire;
 - b. Report a hazardous situation that occurs that could have possibly threatened the lives of other people around a facility or foster home;
 - c. Report incidents that result in law enforcement taking control of a situation or taking control of a facility or foster home;
 - d. Report any major or/credible threat to the security and/or safety of a facility, foster home, or child/youth in out-of-home care;
 - e. Report if a law enforcement agency files charges; issues a summons or citation to a child/youth, and/or a child/youth is arrested while the child/youth has an open placement at the facility or foster home, to include when child/youth is on or off grounds;
 - f. If a child/youth under the age of eighteen (18) is absent from the facility or foster home without consent, the facility or agency must make a report to the Department no later than twenty-four (24) hours after receiving information of when the child/youth left the facility or foster home; and
 - g. Report if a Division of Youth Services child/youth escapes the facility or foster home.
- B. A critical incident report must be submitted by the licensed provider directly through the Colorado Department of Human Services, Division of Child Welfare, Trails automated system.

- C. The residential child care facility must notify the Department in writing within five (5) business days when a determination is made by the administration to continue employing a staff member on whom the facility has received notice, either verbally or in writing, of a confirmed, as defined by Section 19-1-103(33), C.R.S., report of the staff member being found culpable for an act of abuse or neglect. The written notice must include a supervision plan as well as the justification of continuing to employ the staff member.
- D. Required Notification
 - 1. A facility must immediately notify the child's/youth's legal custodian(s) and/or the responsible placing agency of any serious illness or serious injury resulting in medical treatment away from the facility, hospitalization, or death involving a child/youth in care.
 - 2. The facility must notify the legal custodian(s) and/or placing authority as soon as possible or at least within twenty-four (24) hours excluding weekends and holidays upon discovery that a child/youth has run away.
 - 3. Critical incidents must be reported as outlined in Section 7.701.52.A.
- E. Reports Made to the Department within Ten (10) Business Days
 - 1 Any legal action against a facility, agency, owner, operator, or governing body that relates to or may impact the care or placement of children/youth;
 - 2. Change of director of facility or agency;
 - 3. Closure of the facility or agency;
 - 4 Change of placement supervisor for a child placement agency;
 - 5. Change in Trails child placement agency (CPA) supervisor or Trails public provider profile.
- F. Changes to a License Requiring Written Notification to the Department and Prior Department Approval
 - 1. Proposed change in the number, gender identity, or age of children/youth for whom the facility is licensed that differs from that authorized by the license.
 - 2. Changes in the physical facility or use of rooms for child care at a facility.
 - 3. Change of name of the facility or agency.
 - 4. Change of residents in the facility, not to include those residents placed in the facility by a county department or a child placement agency.

7.701.53 Reporting of Child Abuse/Neglect

- A. All agencies and facilities must require each foster parent or staff member of the foster home or facility to read and sign a statement clearly defining child abuse and neglect pursuant to state law and outlining the staff member's or foster parent's personal responsibility to report all incidents of child abuse or neglect according to state law.
- B. Pursuant to Section 19-3-304, C.R.S., any worker in any facility or agency that is licensed or certified pursuant to Part 9 of Article 6 of Title 26 who has reasonable cause to know or suspect that a child/youth has been subjected to abuse or neglect or who has observed the child/youth being subjected to circumstances or conditions that would reasonably result in abuse or neglect must immediately upon receiving such information report or cause a report to be made of such fact to the county department, the local law enforcement agency, or through the child abuse reporting hotline system as set forth in Section 26-5-111, C.R.S.
- C. Any caregiver or staff member in a child care facility who has reasonable cause to know or suspect that a youth ages eighteen (18) to twenty-one (21) years old, who is considered a vulnerable youth in placement, has been subjected to abuse or neglect or who have observed the child/youth being subjected to circumstances or conditions that would reasonably result in abuse or neglect, must immediately upon receiving such information, report or cause a report to be made of such fact to the state hotline, county intake department of human/social services, or local law enforcement agency.
- D. At the time of admission the facility must give the child's/youth's parent or guardian information that explains how and where to report suspected child abuse or child neglect.
- E. The facility must ensure there is no retaliation or punishment for any staff member who files a complaint or otherwise reports any concerns or issues with the facility, the administration, other staff members, or activities within the program to the Department, county department of human services, or local law enforcement.

7.701.54 Investigation of Child Abuse/Neglect

- A. Staff members of the county department of human/social services or a law enforcement agency that investigates an allegation of child abuse must be given the right to interview staff and children/youth in care, and to obtain names, addresses, and telephone numbers of parents or legal guardians of children/youth enrolled at the child care facility.
 - 1 An agency or facility must not interfere or refuse to cooperate with a child protection investigation. Cooperation includes, but is not limited to, a request for documentation, videos, contact information, and interviews with staff and child(ren)/youth.

2. An agency or facility must not interview staff or children/youth regarding the specific allegation(s) of child abuse or child neglect until the county department of human/social services and/or local law enforcement agency has had the opportunity to interview all appropriate individuals and completed their investigation.
- B. Any report made to the law enforcement authorities or a county department of human/social services of an allegation of abuse of any child/youth at the child care facility will result in the temporary suspension of duties of the alleged perpetrator to remove the risk of harm to the child(ren)/youth if there is reasonable cause to believe that the life or physical/mental health of the victim or other child(ren)/youth at the facility is in imminent danger due to continued contact between the alleged perpetrator and the child(ren)/youth at the facility. Such suspension or reassignment of duties will remain in effect pending the outcome of the investigation by the appropriate authorities.

7.701.55 Reporting of Licensing Complaints

Agencies and facilities must provide written information to parents, legal guardians, or foster parents at the time of admission and staff members at the time of employment on how to file a complaint concerning observed or suspected licensing violations. The information must include the complete name, mailing address, and telephone number of the Colorado Department of Human Services, Division of Child Welfare.

7.701.56 Posting Licensing Information

- A. At all times during the operating hours of the facility, except for foster care homes, the facility/agency must post the current child care license in a prominent and conspicuous location easily observable by those entering the child care facility or agency. For foster care homes, the certificate must be available for review/upon request of the public.
- B. At all times during the operating hours of the facility, except for foster care homes, the facility/agency must post all approved appeals and waivers in a prominent and conspicuous location easily observable by those entering the child care facility or agency. If the official decision letter is child-specific, it must be kept in the agency/facility file and provided for the department upon request.
- C. Agencies and facilities providing out-of-home care and Day Treatment centers must post in a prominent and conspicuous location information regarding the procedures for filing a complaint with the Colorado Department of Human Services, Division of Child Welfare, including the telephone number and mailing address. For foster care homes and child placement agencies, information for filing a complaint must be made available upon request.

7.701.57 Fiscal Management

- A. Facilities and Agencies with Less Than \$750,000 Total Annual Revenue

1. Each facility or agency must declare to the department the annual fiscal year used for the operation of the facility or agency.
2. A facility or agency with less than \$750,000 total annual expenditure must submit a cost report annually on the state-prescribed form within sixty (60) days of the agency's or facility's identified end of the fiscal year. Any charitable organization required by statute to conduct an audited financial statement prepared by an independent certified public accountant (CPA) must submit the audited financial statement to the Department.
3. The cost report submitted must contain an affidavit signed by the facility or agency's executive director and an officer of its board attesting to the authenticity of the information. Submission of falsified information shall be grounds for suspension of the license.

B. Facilities and Agencies with More Than \$750,000 Total Annual Revenue

1. Each facility or agency must declare to the Department the annual fiscal year used for the operation of the facility or agency.
2. A facility or agency with more than \$750,000 total annual expenditure must submit a cost report annually on the state-prescribed form within sixty (60) days of the agency's or facility's identified end of the fiscal year.
3. The cost report submitted must contain an affidavit signed by the facility or agency's executive director and an officer of its board attesting to the authenticity of the information. Submission of falsified information shall be grounds for suspension of the license.
4. Each facility or agency whose total annual expenditures are \$750,000 or more must provide an annual financial review by an independent certified public accountant in accordance with appropriate generally accepted auditing standards.
5. Annually, every facility or agency whose annual expenditures are \$750,000 or more must submit a copy of its financial review to the Department. The financial review must be submitted to the department within six (6) months of the facility's or agency's fiscal year end.
6. The financial review submitted must contain an affidavit signed by the facility's or agency's executive director and an officer of its board attesting to the authenticity of the information. Submission of falsified information shall be grounds for suspension of the facility or agency license.

7.701.6 CONFIDENTIALITY PRACTICES AND NECESSARY RECORDS AND THEIR RETENTION

7.701.61 Confidentiality Practices

- A. The records concerning the licensing of facilities and agencies are open to the public except as provided below.
- B. Anyone wishing to review a record of a child/youth in the legal custody of the state of Colorado must make a written request to the Department.
- C. The following documents are confidential and not available for review:
 - 1. Information identifying children/youth or their families;
 - 2. Scholastic records, health reports, social reports, or documentation covered under therapeutic privilege, such as psychotherapy notes, drug or alcohol treatment records, and mental health records containing protected health information. These are available only to the person to whom the records pertain or others as provided by law;
 - 3. Personal references requested by the Department; and
 - 4. Reports and records received from other agencies, including police and child protection investigation reports.
- D. The facility/agency must have a policy as to the maintenance, storage, and confidentiality of records. Records stored in an electronic format must have safeguards and controls for appropriate access to the records; have a secure back-up system to ensure records are not lost; and must be accessible for review by a representative of the Department at all times.
- E. Records are the property of the facility/agency and must be protected against loss, tampering, or unauthorized use. Records must be maintained for a minimum of three calendar (3) years plus the current year's records and accessed in accordance with all state and federal confidentiality laws, including HIPAA, as incorporated above. Access must be given to the Department upon request pursuant to Section 26-6-912(3), C.R.S. for purposes of investigations and auditing as identified within 42 C.F.R. Parts 160 through 164, as incorporated above and 42 C.F.R. Parts 1 and 2, which is hereby incorporated by reference. No later amendments or editions are incorporated. These regulations are available at www.ecfr.gov and also available for public inspection and copying at the Colorado Department of Human Services, Division of Child Welfare, 1575 Sherman Street, Denver, Colorado 80202, during regular business hours.

- F. Images of children/youth must be kept confidential in accordance with all applicable state and federal confidentiality laws. Except for surveillance cameras installed in common areas of the facility/agency, staff must not take pictures/images or recordings of children/youth. If the facility/agency utilizes surveillance cameras, it must take steps to prevent the unauthorized disclosure of any recordings and appropriately protect the identity of child/youth if the recording is released, with the exception of auditing and/or investigative purposes by the state and county departments or by court order. Data storage for surveillance camera footage specific to any critical incident reports or police investigations must be kept and stored for a minimum of three (3) prior calendar years plus the current calendar year. The only exceptions to the confidentiality for images of youth are:
1. For promotional materials, including social media, where parental and guardian(s) consent and consent of children/youth aged ten (10) and older has been given;
 2. When the facility/agency is required to have a photo of the child/youth on file for purposes of notification to law enforcement or other local emergency service agencies; and
 3. When the facility/agency uses a child's/youth's photo for internal purposes only.
- G. Facts learned about children/youth and their families must be kept confidential, with the following exceptions:
1. In medical emergencies, and then only the minimum amount of information necessary for medical professionals to provide assistance and/or expertise; or
 2. To the child/youth; the child/youth's parent(s) or guardian(s) and their respective legal counsel(s); a court having jurisdiction over the child/youth; or licensing specialist in performance of their mandated duties; or
 3. If the parent(s) or guardian(s) have given voluntary, written consent; or
 4. For purposes of mandatory reports of abuse and/or neglect to the state child protection hotline; or
 5. When otherwise required to provide those facts pursuant to law.
- H. School records must be transmitted according to Section 22-32-138(3)(A), C.R.S. through Section 22-32-138(3)(D), C.R.S.; The Rehabilitation Act of 1973 at 29 U.S.C. Sections 701 through 796L; and the Individuals with Disabilities Education Act (IDEA) at Sections 20 U.S.C. 1400 through 1482, as incorporated above, if applicable.

- I. Medical records must be kept in a secure location at the facility and only be released in accordance with the Health Insurance Portability and Accountability Act (HIPAA), as incorporated above.
- J. Any information concerning observed behavior which reasonably appears to constitute a criminal offense committed on the premises of a facility or any criminal offense committed against any person while performing or receiving services is not considered privileged or confidential.
- K. Research using identifying information of children/youth is permitted only under the following circumstances:
 - 1. The parent/guardian of the child/youth has given written permission to conduct research on a child/youth.
 - 2. Any research being conducted on a child/youth must have previously been approved by an Institutional Review Board (IRB). An Institutional Review Board is an administrative body established to protect the rights and welfare of human research subjects recruited to participate in research activities conducted under the auspices of the institution with which the IRB is affiliated.
 - 3. All researchers conducting clinical research on a child/youth must sign an oath of confidentiality.
 - 4. All information identifying children/youth by name, address, telephone number, and/or social security number collected for research purposes must not be further disclosed by the researcher without parent/guardian consent or consent of the child/youth if they are of age to hold the privilege.
- L. When names are redacted and other identifying information is disguised or redacted, material from case records may be used by the facility/agency for teaching purposes; development of the facility/agency's governing bodies' understanding; knowledge of the facilities' services; or similar educational purposes.
- M. Information regarding treatment for alcohol or drug abuse disorders may be released only in compliance with the federal regulations on Confidentiality of Substance Use Disorder Treatment Records, 42 C.F.R. Part 2 (2024), which is incorporated above.

- N. A facility or agency may make available information in the case record to the child/youth, their parent(s), or guardian(s) and their respective legal counsel including the child's/youth's guardian ad litem (GAL) and/or counsel for youth (CFY) if the information being released does not contain material which violates the right of privacy of another individual and/or that must be withheld from release according to other laws or by order of a court. If in the professional judgment of the administration of the facility, it is determined that information contained in the record would be damaging to a child/youth, that information may be withheld except under court order. In order to withhold this information, the facility must document the reasons for withholding in the child's/youth's file.

7.701.62 Maintenance and Confidentiality of Child/Youth Records

- A. Each licensed child care facility or agency shall maintain records as required by the Department pertaining to the admission, progress, health, and discharge of children/youth in care at the facility.
1. These records shall be made available to the Department upon request.
 2. These records shall be maintained and stored in a confidential format.
 3. All information regarding children/youth and their families shall be kept confidential.

7.701.63 Necessary Records

- A. The facility must maintain an organized, legible, chronological, current written or electronic record for each child/youth in care as required for the licensing of the facility in accordance with the rules regulating the facility. The record must be separated into discrete sections addressing medical, educational, clinical, milieu, and therapy.
- B. A record of admission/case file must be completed for each child/youth in care prior to or at the time of placement. The admission record must be accessible for review at the facility where the child/youth resides and must contain:
1. Child's/youth's legal name; date, and place of birth (verified by a birth certificate when possible); gender identity; race; religious preferences of parent(s) or child/youth; date and reason for placement in the current level of care.
 2. Child's/youth's address and telephone number, and parent(s) or guardian(s) address and telephone number if different from the child/youth.

3. Name; address; day and after-hours telephone number of the individual or agency placing the child/youth; the name of the individual arranging the placement.
4. The legal custody status of the child/youth and any documents pertaining to the child's/youth's custody status such as court orders, including the appointment of a guardian ad litem, counsel for youth, legal guardianship, custody agreements, or the termination of parental rights.
5. A copy of the placement agreement pursuant to Section 7.714.21.B.9.
6. Health records including a health history; chronic medical problems of the child/youth; allergies; illnesses; special diets the child/youth has had during the last six (6) months; and a complete list of all medications the child/youth is taking.
7. Current medical reports; accident, injury, or illness reports; and a record of medication administered and necessary medical care provided to the child/youth while in placement. Psychiatric and psychological reports must be obtained and placed in the child/youth's file, when available.
8. Copies of educational records, including any individualized education plan (IEP) and/or 504 plan where applicable. Reports of school work, including scholastic performance; accumulation of high school credits, if applicable; certificates of achievement or award; and extracurricular interests.
9. The individual child's/youth's plan (ICP) and family services plan (FSP) when developed, or individualized treatment plan (ITP) for committed youth. A summary of the periodic evaluations of the child's/youth's progress and resultant changes in the ICP, FSP, or ITP.
10. Written summaries of significant contacts with parent(s), guardian(s), and other involved agencies while in care.
11. Persons authorized to have contact with the child/youth while in care at the facility and persons not authorized via formal court order to have contact with the child/youth while in care at the facility.
12. Copies of all assessments used to determine admission to the facility; documentation of the need for continued care in the facility and how to determine the appropriateness of discharge from the facility, including documentation of all attempts to gain copies of relevant assessments. The assessments may include, but are not limited to, the state-approved assessment, as completed by the independent assessor and subsequent assessment revisions completed by the facility; the assessments as completed while the child/youth is receiving treatment and care within the facility; any psychological or psychiatric assessments, or assessments

completed at the request of the child's/youth's treatment team or as ordered by the court.

7.701.64 Retention of Records

- A. Facility records for children/youth must be retained for at least three (3) years at the licensed location. Facility records can include, but are not limited to: intake documentation, demographic information, education information, and/or signed policies and procedures. Facilities must maintain a child/youth mental health record for seven (7) years at the licensed location. Mental health records can include but are not limited to: treatment plans or individual child/youth plans, diagnosis, medication prescriptions, therapy sessions, clinical tests, symptoms, prognosis, and/or substance use evaluations. Retention of records for a longer period may be desirable when they reflect an accident, injury, or other unusual circumstance or as determined by agency/facility policy.
- B. The following records must be on file at the facility or administrative office:
 - 1. Governing structure, including the charter, articles of incorporation, by-laws, or other legal basis for existence;
 - 2. Insurance coverage; and
 - 3. Annual financial audit or financial statements.
- C. The following records must be on file at the licensed location:
 - 1. Current health department inspection report specific to the child care license number;
 - 2. Current fire department inspection report specific to the child care license number; and
 - 3. A list of current staff members, substitute staff members, and staffing patterns, which must include actual schedules worked, for at least six (6) months prior.

7.701.65 Personnel Records for Direct Care Staff

- A. Personnel records for any employee having direct contact with children/youth must include:
 - 1. Name, address, birth date;
 - 2. Date of hire and date and reason for separation;
 - 3. Official documents verifying education (e.g. transcripts, ged certificate, high school diploma);

4. Training and work experience;
 5. At least two (2) employment references;
 6. Current health evaluation or physical stating fitness for work prior to the start date;
 7. A written indication of awareness of agency policies; and
 8. All required background checks.
- B. Records for personnel must be obtained prior to the start date for the employee and retained for at least three (3) years after separation. Records must be immediately accessible to the Department upon request.

7.701.7 PARENTAL ACCESSIBILITY

- A. During hours of operation, a facility must allow access to parents and guardians and custodians having legal custody of a child/youth in care to those areas of the facility that are licensed for child care. Parents, legal guardians, and custodians may be given access to the facility when in the best interest of the child/youth and approved by the multidisciplinary team (MDT) when a court is not involved. If a court is involved and there is a valid court order regarding child/youth contact with their parents, legal guardians, or custodians, that court order will control facility access. Parent, legal guardian, and custodian access to the facility decisions must be done in collaboration with the MDT and on a scheduled basis.
- B. During the hours of operation, the facility's most recent licensing, fire department, and health department inspection reports must be accessible to parents and legal guardians of children/youth in care or their designee and to parents and legal guardians considering placing their children/youth in care at the facility.
- C. A facility does not violate this section when it restricts access by a parent, guardian, or their designee to a child/youth during an emergency as instructed by local authorities.

7.701.8 PERJURY STATEMENT - APPLICATION FORMS FOR EMPLOYMENT WITH A CHILD CARE PROVIDER

Every application used in the State of Colorado for employment with a child care provider, agency, or facility, or for the certification of a foster home, must include the following notice to the applicant:

"Any applicant who knowingly or willfully makes a false statement of any material fact or thing in the application is guilty of perjury in the second degree as defined in Section 18-8-503, C.R.S., and, upon conviction thereof, shall be punished accordingly."

7.701.9 GENERAL HEALTH RULES

7.701.91 Smoking and Tobacco Products

- A. Tobacco and nicotine products are prohibited by law from use in and around licensed child care facilities, as identified in Section 25-14-103.5, C.R.S.
- B. Smoking and tobacco product use is prohibited at all times while transporting children/youth.
- C. Smoking and tobacco product use is prohibited inside the foster homes when children/youth are in placement.
 - 1 Foster parents are exempt from this rule when no children/youth are in placement.
- D. A person shall not give, sell, distribute, dispense, or offer for sale a cigarette, tobacco product, or nicotine product to any person who is under twenty-one (21) years of age, as identified in Section 18-13-121(1)(a), C.R.S.

7.701.92 The Use of Restrictive Devices

- A. The use of restrictive devices, including, but not limited to, the use of handcuffs, shackles, straight jackets, posey vests, ankle and wrist restraints, craig beds, vail beds, or any other restrictive beds, hospital cribs, and chest restraints is prohibited.
- B. Children/youth may only be restrained when authorized by applicable federal and state law. In order to be approved to utilize a restrictive device of any kind, the facility/agency must submit an appeal in accordance with Section 7.701.13 or receive prior approval from the Behavioral Health Administration (BHA). BHA approval applies solely to those entities supervised or licensed by the BHA.

7.701.200 The Reasonable and Prudent Parent Standard Requirements for Facilities Providing Twenty-Four (24) Hour Out-Of-Home Care to Approve Activities for a Child or Youth in Foster Care

“Reasonable and prudent parent standard” as defined in 12 CCR 2509-1 Section 7.000.2, means careful and sensible parental decisions that maintain the health, safety, and best interests of the child or youth while encouraging the emotional and developmental growth of the child or youth that a provider shall use when determining whether to allow a child or youth in foster care to participate in extracurricular, enrichment, cultural, and social activities based up on the criteria in Section 7.701.200.

Children and youth in foster care are entitled to participate in age or developmentally appropriate extracurricular, enrichment, cultural, and social activities that are determined on an individual basis and include the child’s/youth’s input. Such activities provide opportunities for brain development, normalcy, inclusion, social interaction, social capital, permanency options, and permanent connections. Providers must use a

“reasonable and prudent parent standard” when determining whether to allow a child or youth in foster care to participate in such activities following the criteria below:

- A. When applying the reasonable and prudent parent standard and prior to approval of the activity, the provider must take reasonable steps to obtain or determine:
 - 1. Adequate information about the child or youth, including the child’s/youth’s particular religious, spiritual, cultural, social, or behavioral practices and preferences;
 - 2. Any current safety concerns for the child/youth as related to the activity;
 - 3. The age and developmental maturity of the child/youth to engage in the activity; and
 - 4. An assessment of the child’s/youth’s abilities in reference to the potential risks of the activity.
- B. Providers and counties must consider and work to approve privileges and activities appropriate and in alignment with the child’s/youth’s age, developmental level, and overall assessed readiness for increased independence and autonomy. Providers and counties must make every attempt to ensure children/youth in their care are afforded the same opportunities for independence and autonomy as their peers within the confines of personal and community safety.
- C. When appropriate and not otherwise impacted by legal requirements including, but not limited to, probation, the level of supervision for individualized activities and privileges that is required shall be based on the child’s/youth’s individually assessed level of need as informed by the child’s/youth’s age, developmental level, and overall demonstration of safety and stability.
- D. Out of home placement alone is not a reason for a child/youth to be denied participation in activities.
- E. The county department of human/social services or licensed child placement agency staff that are responsible for direct oversight of foster homes or placement of children/youth must complete the same state training in applying the reasonable and prudent parent standard at least every five (5) years.
- F. At least one (1) staff member or administrator in each specialized group facility or Residential Child Care Facility (RCCF) must have completed the reasonable and prudent parent standard training. The staff with the reasonable and prudent parent standard training shall be designated as authorized to apply the reasonable and prudent parent standard to decisions involving the participation of a child or youth in extracurricular, enrichment, cultural, or social activities.

- G. The rationale used to authorize an activity for a child or youth must be clearly documented in the facility records and provided in a timely manner to the placing entity.
1. The facility must obtain and comply with a current copy of the policy from the responsible placing entity regarding activities that are considered appropriate for the facility to approve. If there is a conflict between placing entity policies, then the facility shall consult with the placing entities.
 - a. Appropriate activities considered for children/youth to participate in may include, but are not limited to, school activities, community-provided activities, and social activities including sports, recreation, field trips, birthday parties, sleepovers, and overnight activities lasting one (1) or more days.
 - b. The responsible county department of human/social services or DYS may restrict certain activities based upon the documented exceptional needs and circumstances of a child or youth in foster care, which impacts their identified safety needs.
 2. The wishes of the parents/legal custodian must be considered, including cultural and religious implications that are not in conflict with identified safety needs.
 3. Caregivers are encouraged to approve activities requested by the child/youth, as outlined in Sections 7.701.200.A and 7.701.200.B, unless there is a likelihood of moderate to severe harm. If a caregiver can define how a requested activity would put the child/youth at risk for moderate to severe harm, the caregiver must request a meeting with the multidisciplinary team to review the request.
 4. The facility may consult with the responsible agency for guidance about individual cases.
- H. Providers must not incur liability to the Department or to the county department of human/social services because of an extracurricular, enrichment, cultural, or social activity approved by the provider if the provider demonstrates compliance with the reasonable and prudent parent standard and has made an informed judgment demonstrated through documentation. In a child welfare investigation arising out of such an activity approved by the provider, the facility must not be founded for institutional neglect if the provider demonstrates compliance with the reasonable and prudent parent standard.

7.701.300 Cultural Responsiveness For Residential Child Care Facilities, Specialized Group Facilities, Homeless Youth Shelters, Child Placement Agencies, Day Treatment Centers, Foster Care Homes, and Secure Residential Treatment Centers.

- A. All policies and procedures must reflect culturally responsive operations, and a specific policy must be developed to ensure that the provider will implement and practice culturally responsive programming that acknowledges, respects, and integrates the child's/youth's family's cultural values, beliefs, and practices.
- B. All staff members, interns, and foster parents must complete four (4) hours of initial training in cultural responsiveness and two (2) hours annually thereafter. Volunteers that work directly with youth two (2) or more times in a twelve (12) month period must complete four (4) hours of initial training in cultural responsiveness and two (2) hours annually thereafter. Members of the governing body must complete two (2) hours of annual training in cultural responsiveness. Members of the board of directors must be given training in the agency's or facility's cultural responsiveness policies and procedures at the time they are appointed to the board of directors.
- C. Assessments of children/youth and families must identify cultural factors, and services must be in alignment with cultural values assessed and presented by the child/youth and/or family of origin.

7.701.350 Cultural Competence Plan for Agencies and Facilities

Facilities and agencies must:

- A. Develop and implement policies that promote equitable access to treatment and resources for children/youth of all backgrounds, ensuring non-discrimination based on race, ethnicity, sex, sex assigned at birth, gender identity, sexual orientation, religion, ability level, or any other protected characteristic.
- B. Implement a zero-tolerance policy for any form of discrimination or harassment within a licensed program with clear reporting mechanisms and follow-up.
- C. Regularly assess clinical interventions and practices to identify and resolve any disparities related to cultural competence.
- D. Provide two (2) hours of initial training on cultural competence principles and how these principles apply to the licensed provider's practices.

7.701.400 Trauma-Informed Care For Residential Child Care Facilities, Specialized Group Facilities, Homeless Youth Shelters, Child Placement Agencies, Day Treatment Centers, Foster Care Homes, and Secure Residential Treatment Centers.

- A. All policies and procedures must be trauma-informed, and a specific policy must be developed to ensure that the provider will implement and practice trauma-informed care and services to include, at a minimum, the following:
 - 1. A safe and supportive environment;

2. Collaboration, mutuality, and transparency between provider, interested parties, and family;
 3. Family and child/youth empowerment, input, and choice in the treatment process; and
 4. Individualized family service plans, clinical interventions, crisis intervention protocols, and transition and aftercare planning.
- B. All staff members, interns, and foster parents must complete four (4) hours of initial training in trauma-informed care and two (2) hours annually thereafter. Volunteers that work directly with youth two (2) or more times in a twelve (12) month period must complete four (4) hours of initial training in trauma-informed care and two (2) hours annually thereafter. Members of the governing body must complete two (2) hours of annual training in trauma-informed care. Members of the board of directors must be given training in the agency's or facility's trauma-informed care policies and procedures at the time they are appointed to the board of directors.
- C. Assessments of children/youth and families must screen for a history of traumatic experiences and associated needs, and services must be responsive to the needs of children/youth and families, as identified in the assessments.
- D. All licensed agencies and facilities must have a trauma-informed treatment model in alignment with the standards set forth by the Department.
1. The trauma-informed treatment model must be implemented as approved by the Department.
 2. Any substantive changes to the trauma-informed treatment model must be submitted and approved by the Department prior to implementation and at the time of renewal at least one hundred twenty (120) days and no later than sixty (60) days prior to the date of the expiration of the license.
 3. The agency/facility must implement and adhere to their trauma-informed treatment model.
 4. The agency/facility must develop policies and procedures and train all staff within one hundred twenty (120) days of hire and annually for the population served and the agency's/facility's trauma-informed model.

7.701.450 Quality Improvement Practices for Agencies and Facilities

- A. Providers must develop quality improvement policies and procedures and establish a quality improvement committee that meets at least quarterly.

- B. Providers must implement quality improvement practices that assess those operations, activities, interventions, and practices to identify and resolve problems and barriers that impact positive treatment outcomes.
- C. Providers must document the quality improvement process, indicators, measures, outcomes, and revisions to improve programming. This documentation must be made available to the Department upon request.

7.701.500 Trails Data Entry For Residential Child Care Facilities, Homeless Youth Shelters, Child Placement Agencies, Day Treatment Centers, and Secure Residential Treatment Centers

- A. Each facility or agency must apply to the Department for permission to access the Trails system, to complete all functions assigned to their facility type.
 - 1. Each facility or agency must have at least two (2) employees assigned to access the Trails system to enter critical incident reports as listed at Section 7.701.52.
 - 2. Child placement agencies must have at least two (2) employees assigned to access the Trails system to enter original and renewal foster care certificates.
 - 3. Child placement agencies must have one (1) employee assigned to access the Trails system to review any child abuse and neglect history for foster care applicants, adoptive applicants, and other adults residing in the home.
- B. The child placement employee who accesses the Trails system to review any child abuse and neglect history cannot also access the Trails system for critical incident reporting or entering foster care certificates.
- C. Each facility or agency must submit to the Department the prescribed form to delete access for persons who are no longer employed within ten (10) working days of their departure from the agency or facility or immediately for employees whose access to Trails has been restricted by their employer or by the Department.
- D. All persons accessing the trails system must adhere to all guidelines identified on the application and ensure that all information added into Trails is true and accurate, in accordance with Sections 18-8-114(1)(a), C.R.S. and 26-6-914(2)(g), C.R.S.

7.702 RULES REGULATING CHILD CARE CENTERS THAT PROVIDE LESS THAN 24-HOUR CARE [Repealed eff. 03/02/2026]

7.703 RULES REGULATING THERAPEUTIC FOSTER CARE

Child placement agencies' or county department of human/social services' therapeutic foster care programs shall comply with all rules in this section, the General Rules for Child Care Facilities in section 7.701, Rules and Regulations for Child Placement Agencies in section 7.710, and the Rules Regulating Foster Care Homes in section 7.708, 7714.532.

7.703.1 DEFINITIONS

- A. "Therapeutic Foster Care" means a program of foster care that incorporates treatment for the special physical, psychological, or emotional needs of a child placed with specially trained foster parents, but does not include medical foster care. § 26-6-102(39), C.R.S. (2020).
- B. "Therapeutic Foster Care Program" means a specialized program of foster care that complies with section 7.703 and that is approved to operate by the Colorado Department of Human Services (herein referenced as the Department).
- C. "Therapeutic Foster Care Agency" means a county department of human/social services or a licensed child placement agency (CPA) that is approved by the Department to operate a therapeutic foster care program.
- D. "Therapeutic Foster Care Home" means a certified foster care home that provides therapeutic foster care to children or youth in care.

7.703.2 THERAPEUTIC FOSTER CARE AGENCY RESPONSIBILITY

A. Policies

The therapeutic foster care agency shall develop a written statement of purpose and policies for the operation and management of the therapeutic foster care program (herein referenced as program). The therapeutic foster care agency must provide the Department with the statement of purpose and policies for the Department's approval upon adoption or revision.

- 1. The statement of purpose and policies must describe or provide for:
 - a. The population of children or youth to be served, to include age, level of development, types of behavior, mental health presentation, and other needs;
 - b. Strategies for supporting emotional regulation, including de-escalation techniques, targeted to address the needs of the children/youth served;

- c. Supports to maintain placement stability including respite;
- d. Assessment, reporting, and transition services for discharge;
- e. Cultural responsiveness, as described in 7.701.300; and
- f. Trauma-informed care, as described in 7.701.400.

7.703.3 PERSONNEL REQUIREMENTS

All personnel requirements in sections 7.710.22 through 7.710.25 apply to child placement agencies. County departments of human/social services' personnel requirements apply to county departments of human/social services personnel.

A. The therapeutic foster care agency's staff shall include, at a minimum, one home supervisor.

1. Home supervisors shall meet the following minimum qualifications:

- a. Child placement agency home supervisors shall meet the requirements of a placement supervisor per section 7.710.22.A.6 (12-CCR 2509-7).

If additional home supervisors are necessary to fulfill the placement responsibilities of the agency, additional home supervisors shall meet the minimum requirements per section 7.710.22.A.7 (12-CCR 2509-7).

- b. County department of human/social services home supervisors shall meet the requirements of a social caseworker per section 7.603.1.D.2 (12-CCR 2509-7).

2. The responsibilities of the home supervisor include:

A. Coordinate, obtain, or provide services to ensure the needs of the child/youth are met, including but not limited to:

- 1. Psychiatric, psychological, or developmental evaluations;
- 2. Assessment of child's/youth's history, current functioning, and service needs;
- 3. Development and maintenance of the individual child's plan, per 7.714.4.D; and
- 4. Group, individual, or family therapy, as indicated in the individual child's plan.

- b. Consult as needed with therapeutic foster parents to provide emotional support, share information, and address problems or concerns;
 - c. Ensure that the therapeutic foster parents communicate the current progress and needs of the child/youth with the parent(s), legal custodian, or identified permanent family at the frequency determined in the individual child's plan;
 - d. Conduct face to face contact with therapeutic foster parent(s) in the therapeutic foster home at the frequency indicated in the individual child's plan, but not less than twice monthly;
 - e. Assess, at a minimum of twice monthly, the needs of the foster parents, including respite, and coordinate, obtain, or provide services to ensure needs are met;
 - f. Conduct face to face contact with the child/youth in the therapeutic foster home at the frequency indicated in the individual child's plan, but not less than monthly;
 - g. Conduct a monthly review of the individual child's plan for each child/youth and document progress and changes to goals; and
 - h. Document all assessments, services, and face to face contacts monthly.
3. Home Supervisor Training

Prior to assuming the responsibilities of a therapeutic foster care caseload, and annually thereafter, a home supervisor shall have a minimum of twenty hours (20) of training in the following areas:

- a. The Rules Regulating Therapeutic Foster Care;
- b. Strategies to maintain placement stability in therapeutic care foster homes; and
- c. Competencies related to the needs of the children/youth in the therapeutic foster care program.

7.703.3 REQUIREMENTS FOR THERAPEUTIC FOSTER CARE HOMES

A. Training for a Therapeutic Foster Parent

- 1. In addition to the certification requirements per section 7.708.65, before accepting placements and annually thereafter, therapeutic foster parents shall have 12 hours of competency-based training in the following areas:

- a. Specific content related to diagnoses, behaviors, interventions, and other identified needs of the population(s) served; and
 - b. The Rules Regulating Therapeutic Foster Care.
- B. The roles and responsibilities of a therapeutic foster parent include:
 - 1. Communicate the current progress and needs of the child/youth with the parent(s), legal custodian, or identified permanent family at the frequency determined in the individual child's plan; and
 - 2. Consult and collaborate with the home supervisor, and other therapeutic team members as needed, to address the needs of the child/youth;
- C. Qualifications

The therapeutic foster care(s) parent must have experience, as demonstrated through verified documentation, with the population(s) served.

7.703.4 PLACEMENT IN THERAPEUTIC FOSTER CARE HOMES

- A. Capacity

No foster child/youth shall be placed in a therapeutic foster care home if that placement will result in more than six total children/youth, including biological, adoptive, and other children/youth, in that home.
- B. Placement

Indicators for therapeutic foster care placement include, but are not limited to: discharge from a residential child care facility or treatment foster care home, a determination that the needs of the child/youth cannot be met in a traditional foster care home, and/or a disrupted foster care home placement.
- C. Transition

Within no more than sixty days (60) days of the placement of a child/youth into the therapeutic foster care home, the therapeutic foster care agency shall collaborate with the family engagement and permanency team to develop and implement a transition plan to address services and supports needed for the child/youth to be in a lower level of care.
- D. Reporting
 - 1. The therapeutic foster care agency shall document progress on goals, new or unmet needs, and challenges and report these to the county department of human/social services or other referring entity on a monthly basis.

2. At a minimum of every six months, the therapeutic foster care agency shall review and consider and document outcomes for each therapeutic foster care home, to include placement stability, length of stay, achievement of treatment goals, achievement of permanency, and safety of the children/youth in the home during the review period.

7.704 RULES REGULATING TREATMENT FOSTER CARE

Child placement agencies' or county department of human/social services' treatment foster care programs shall comply with all rules in this section, the general rules for child care facilities in section 7.701, and the rules regulating foster care homes in section 7.708.

7.704.1. DEFINITIONS

- A. "Applicant" means a county department of human/social services or a licensed child placement agency (CPA) that applies to the department to operate a treatment foster care program.
- B. "Care Coordination" means the integration of services and activities provided by the treatment team into a comprehensive plan. The purpose is to manage the delivery of appropriate services for and with the child/youth, treatment foster parent(s), and identified permanent family.

The qualifications, responsibilities, training, and caseload requirements for the care coordinator are identified in 7.704.3.E.
- C. "Juvenile Justice Staff" means juvenile assessment center staff, Senate Bill 94 (SB94) staff, pre-trial staff, probation officers, client managers or juvenile parole officers.
- D. "Juvenile Justice Placement" means a detention or commitment facility, operated by the division of youth services.
- E. "Treatment Foster Care" means a clinically effective alternative to residential treatment facilities that combines the treatment technologies typically associated with more restrictive settings with a nurturing and individualized family environment (§ 26-6-102 (40), C.R.S.).
- F. "Treatment Foster Care Program" means a specialized program of foster care that complies with Section 7.704 and that is approved to operate by the Colorado Department of Human Services (herein referenced as the Department).
- G. "Treatment Foster Care Agency" means a county department of human/social services or a licensed child placement agency (CPA) that is approved by the Department to operate a treatment foster care program.

- H. “Treatment Foster Care Home” means a certified foster care home that provides treatment foster care to children or youth in care.

7.704.11 Treatment Team

The treatment team members may include, and are not limited to: biological or adoptive parents, treatment foster parent(s), legal custodian(s), Guardian ad Litem (GAL) AND/OR COUNSEL FOR YOUTH Court Appointed Special Advocate (CASA), county department of human/social services caseworker or designee, child placement agency staff, current or previous treatment providers, juvenile justice staff, school district personnel, and the child/youth. If the child/youth is unable to or it is inappropriate to participate, the child/youth voice shall be represented in decision making.

7.704.2 TREATMENT FOSTER CARE AGENCY RESPONSIBILITY

A. Policies

The treatment foster care agency shall develop a written statement of purpose and policies for the operation and management of the treatment foster care program (herein referenced as program). The treatment foster care agency must timely provide the department with the statement of purpose and policies upon adoption or revision.

1. The statement of purpose must describe or provide for:
 - a. The ages of children or youth to be served;
 - b. The use of trauma informed practices and de-escalation techniques in treatment foster care homes;
 - c. Supports to maintain placement stability including respite;
 - d. Supports for treatment foster parents to remain active participants in the treatment program for the child/youth when the child/youth is clinically placed for the short-term in an RCCF or hospital (up to 30 calendar day);
 - e. Procedures for supporting the treatment foster care home to remain open for the child/youth’s return; and,
 - f. Assessment, reporting, and transition services for discharge.
2. The treatment foster care agency will adopt a personnel policy including, but not limited to job descriptions, qualifications for positions; required documentation for position; and documentation of a background check, as required in section 7.701.

7.704.3 PERSONNEL REQUIREMENTS FOR THE TREATMENT FOSTER CARE AGENCY

All personnel requirements in sections 7.710.22 through 7.710.25 apply to CPAs. County departments of human/social services' personnel requirements apply to county departments of human/social services personnel.

- A. The county director of a county department of human/social services with a treatment foster care program or her/his designee serves in the same capacity as the CPA executive director.
- B. The treatment foster care agency shall inform the department in writing, about a change in the executive director or care coordinator of a CPA or the county director/designee or care coordinator in writing within 30 calendar days.
- C. The treatment foster care agency's direct staff shall include, at a minimum one or more care coordinators.

1. Care coordinators shall meet the following minimum qualifications:

- a. County departments of human/social services care coordinators shall meet education requirements of a social caseworker as set out in Section 7.603.1.D.2 (12 CCR 2509-7).
- b. Staff CPA care coordinators shall meet educational requirements of a placement supervisor set out in section 7.710.22.A.6.

If additional care coordinators are necessary to fulfill the placement responsibilities of the agency, additional care coordinators shall meet the minimum educational requirements set out in Section 7.710.22.A.7.

2. The responsibilities of the care coordinator are:

- a. Daily leadership of the treatment team: the care coordinator:
 - 1) Organizes and manages all team meetings, and manages team decision-making regarding the care and treatment of the child/youth;
 - 2) Provides support and consultation to treatment foster parents, to the families of children and youth in care, and to other treatment team members;
 - 3) Advocates, coordinates, and links activities to assure children, youth, and families receive needed services; and,

- 4) Prepares individuals and/or providers to work with the treatment foster parents and the child/youth's family in a supportive manner of the treatment foster parents and the safety, permanency, and social and emotional well-being of the child/youth.
- b. Provision of family preservation strategies, including:
 - 1) On-call support to the treatment parent, child/youth, and family twenty-four hours/seven days a week (24/7). If the agency uses a contracted service for on-call support, the agency must provide information about the circumstances of the child/youth, treatment foster parents, and family sufficient to provide meaningful support, and must ensure the contractor shall meets the minimal educational, personnel, and training standards of a care coordinator, and,
 - 2) Support and consultations with treatment foster parents, such as problem-solving, emotional support, information sharing, and assessment of child/youth safety and well-being.
- c. Coordination of the state-approved assessment, preparation and approval of treatment foster care plan(s), revisions, and quarterly updates.
- d. Minimum contact as judged necessary and appropriate by the treatment team and described in the treatment foster care plan unless decreased under Paragraph 3) below. "Minimum contact" must consist of at least:
 - 1) Weekly face to face contact with treatment foster parent(s);
 - a) One (1) visit per month face to face with both treatment foster parents if it is a two (2) parent treatment foster care home; and,
 - b) At least one (1) visit shall occur in the treatment foster care home.
 - 2) Three (3) monthly face-to-face contacts with the child/youth in the treatment foster care home.
 - 3) At the discretion of the treatment team the weekly meetings may be decreased, depending upon the acuity level of the child/youth.

3. Caseload

The preferred maximum number of children/youth in treatment foster care assigned to a care coordinator is eight (8). Some circumstances may allow for a larger maximum caseload size, but the maximum caseload size shall not exceed twelve (12).

4. Staff training

Prior to assuming responsibilities of a treatment foster care caseload, and annually thereafter, a care coordinator shall have forty (40) hours of competency-based training in the following areas:

- a. The program's treatment philosophy and the specific treatment methodologies the program uses;
- b. Trauma-informed care, including the impact of trauma, grief, and loss;
- c. Strategies to maintain placement stability in foster care;
- d. Rights of children/youth in treatment foster care in the education system and Special Education programs;
- e. Treatment foster care rules; and,
- f. Program policies and procedures.

7.704.4 REQUIREMENTS FOR TREATMENT FOSTER CARE HOMES

A. Training for a Treatment Foster Parent

- 1. In addition to initial certification requirements (Section 7.708.2-7.708.52), before accepting placements, treatment foster parents shall have thirty-two (32) hours of competency-based training in the following areas:
 - a) Trauma-informed care, including the impact of trauma, grief, and loss;
 - b) Trauma-informed behavior management;
 - c) Child/youth-specific content related to diagnoses, behaviors, and/or other identified needs;
 - d) Intensive mental and behavioral health training;
 - e) Confidentiality and cultural responsiveness; and,
 - f) Annual requirements for foster parents (Section 7.708.65.C.1-9).

B. The Roles and Responsibilities of a Treatment Foster Parent are:

1. Cooperatively and directly working with the parent(s), legal custodian, or identified permanent family in the treatment foster care home, unless there is a documented safety issue. If there is a safety issue, the care coordinator shall work with the treatment team, including the family to address the concerns.
2. Application of treatment foster care agency programming, policies and procedures, including but not limited to protection of privacy.
3. Seeking consultation as needed from the care coordinator or other treatment team members, including requesting a family engagement meeting to maintain placement stability; should a short-term (up to thirty (30) calendar days) clinical placement be required, actively participate in the treatment program so that the treatment foster care home remains open for the return of the child/youth.
4. Completing thirty-two (32) hours of training described in a written training development plan established between the treatment foster care agency and each treatment foster care parent. The training development plan must provide for:
 - a. Training that emphasizes skill development, knowledge, acquisition, and preparation related to meeting the needs of the child/youth placed in the treatment foster care home;
 - b. Training in program policies and procedures, ethics, and cultural competency;
 - c. No less than twenty two (22) hours of required training shall take place in a classroom setting or in an interactive setting that allows the program to gauge the treatment foster care parent(s)' strengths and needed supports and facilitates the development of positive relationships between program staff and treatment foster parents, as well as between peer treatment foster parents; and,
 - d. First aid and CPR training hours do not count toward the required thirty-two (32) annual training hours.

7.704.5 PLACEMENT OF CHILDREN AND YOUTH IN TREATMENT FOSTER CARE HOMES

A. Capacity in a Treatment Foster Care Home

A treatment foster home shall not exceed two (2) children or youth placed in the home for treatment., except a sibling(s) of a child/youth who is placed for treatment in the foster home may be placed in the same foster home when it is in the best interest of all of the siblings, in order to maintain their relationship.

B. Referral

Indicators for treatment foster care placement include, but are not limited to prior placement in a RCCF, a hospital setting, a community crisis center, an acute care unit, a juvenile justice placement, a disrupted or dissolved adoption, and a disrupted foster home care placement; and,

Current behaviors that demonstrate high levels of aggression, suicidality, emotional distress, delinquent behaviors, and/or other emotional or psychological issues.

Referral steps to be completed

1. The custodial or responsible county department of human/social services or other referring agency (i.e., Division of Youth Services or mental health agency) shall make the referral to the treatment foster care agency;
2. The treatment foster care agency shall review treatment foster care homes to determine compatibility and appropriateness of placement;
3. Prior to placement and when the treatment foster care home is identified, the referring and/or treatment foster care agency shall administer the state approved assessment instrument.
4. The custodial or responsible county shall schedule a meeting for treatment team members (Section 7.704.11).
5. If the child/youth does not have an established clinical relationship with a behavioral health provider, the treatment foster care agency shall determine the appropriate behavioral health provider and follow the authorization process for treatment services in order to determine the individual needs of the child/youth and his/her family.
 - a. The child/youth may be placed during the process, and any treatment services shall begin as soon as possible.
 - b. The clinician associated with the treatment foster care agency shall work with the child/youth, treatment foster parent(s), the parent(s), legal custodian(s), or identified permanent family, and all other relevant parties to facilitate a seamless intervention and address the clinical needs holistically.

C. Transition

Upon acceptance into the treatment foster care program, the referring agency and the treatment foster care agency shall develop and implement a transition plan of services to promote reunification with the parent(s), legal custodian(s), or identified permanent family.

D. Ongoing Treatment Team Staffing

1. The county caseworker or designee shall participate in monthly treatment team meetings to review treatment progress, service delivery, and the behavioral needs of the child/youth (Section 7.704.11).

E. Reporting

1. The treatment foster care agency shall document time spent with the child/youth, progress on goals, new or unmet needs, and challenges and report these to the county department of human/social services or other referring on a monthly basis.
2. The county department of human/social services or other referring agency shall review the appropriateness of services and placement based on the treatment foster care agency's reports.
3. The treatment foster care agency shall collect child/youth's outcomes quarterly, regarding placement stability, length of stay, achievement of permanency, and current state approved assessment ratings.

7.705 RULES REGULATING RESIDENTIAL CHILD CARE FACILITIES

All Residential Child Care Facilities must comply with the “General Rules for Child Care Facilities” found in Sections 7.701 through 7.701.500, and the Quality Standards for Twenty-Four (24)-Hour Care for Children and Youth found at Sections 7.714 through 7.714.92, the Rules Regulating Special Activities found at Sections 7.719 through 7.719.8, and the Rules and Regulations Governing the Health and Sanitation of Child Care Facilities in the State of Colorado found at 6 CCR 1010-7 (2025), which is hereby incorporated by reference. No later editions or amendments are incorporated. The rules can be found at <https://www.sos.state.co.us/ccr/numericaldeptlist.do>. Copies are also available for public inspection and copying by contacting the Department of Human Services, 1575 Sherman St., Denver, Colorado 80203, during regular business hours, or any other state publications library during regular business hours.

In accordance with Section 26-6-903, C.R.S., a “residential child care facility (RCCF)” means a facility licensed by the state Department pursuant to Part 9 of Article 6 of Title 26 to provide twenty-four (24) hour group care and treatment for five (5) or more children operated under private, public, or nonprofit sponsorship. “Residential child care facility” includes community-based residential child care facilities; qualified residential treatment programs, as defined in Section 26-5.4-102(2), C.R.S.; shelter facilities; and psychiatric residential treatment facilities as defined in Section 25.5-5.4-103(19.5), C.R.S. a residential child care facility may be eligible for designation by the executive director of the state department pursuant to Article 65 of Title 27. A child who is admitted to a residential child care facility must be:

- A. Five (5) years of age or older but less than eighteen (18) years of age; or
- B. Less than twenty-one (21) years of age and placed by court order or voluntary placement; or
- C. Accompanied by a parent if less than five (5) years of age.

7.705.11 Governing Body

- A. The governing body is the individual, partnership, corporation, or association in whom the ultimate authority and legal responsibility is vested for the conduct of the residential child care facility (RCCF).
- B. When the governing body does not include a board of directors, there must be an advisory committee of at least two (2) other individuals who act in an advisory capacity to the governing body. The names of the board of directors or advisory committee members must be disclosed to the Department. The board of directors or advisory committee must meet at regularly stated intervals and must provide a written performance evaluation of the administrator on an annual basis. The performance evaluation must be available to the Department upon request.

- C. The meeting minutes of the advisory committee or the board of directors must be maintained in accordance with a record retention policy. At minimum, the minutes for the preceding three (3) years must be available to the Department upon request.
- D. The functions of the governing body must include:
 - 1. The appointment of an administrator who is responsible, according to established performance criteria, to the governing body and delegation to the administrator of executive authority and responsibility for the administration of the residential child care facility according to its defined purpose.
 - 2. The formulation and - at a minimum - regularly-planned review of policies and procedures to be followed by the facility every two (2) years, or as needed.
 - 3. The provision of necessary facilities, adequate financing, qualified personnel, services, and program functions for the welfare and safety of children/youth in accordance with these rules.
 - 4. Ensuring the agency's administrator, chief financial officer, executive director, and other officers or employees with direct responsibility for financial transactions or financial responsibility are bonded.
 - 5. Ensuring the facility/agency maintains an average sufficient cash reserve or liquid assets to meet its operating expenses for two (2) months, taking into account the agency's projected volume of placements and its size, scope, and financial commitments.
 - 6. If the facility/agency does not have sufficient funds to meet operating expenses including less than two (2) months of cash reserves or liquid assets or determines there to be a significant financial hardship with potential to impact its programs, the facility/agency must notify their Department Representative in writing of the financial hardship and any potential negative impacts due to the hardship as soon as it is identified.

7.705.12 Fiscal Management

- A. Each RCCF must develop an annual budget reflecting anticipated income by source and expenses by purpose, plus an accompanying balance sheet, which demonstrates that the RCCF has assured resources to carry out its defined purpose. The budget must be approved by the governing body or advisory committee. The first year's budget must be submitted with the original license application.
- B. An RCCF must demonstrate that it is financially sound and manages its financial affairs prudently. All funds disbursed by the facility must be expended in

accordance with the program objectives as specified by the governing body or advisory committee.

- C. An RCCF must have a double entry accounting system, and all financial transactions must be posted to this system. Financial statements, prepared from information provided by this system, must be presented in conformity with the U.S. Generally Accepted Accounting Principles (GAAP). Books and records of the RCCF must be subject, at any time the RCCF business office is open, to inspection, audit, or copying by designated Federal, State, or county personnel, or such independent auditors or accountants as may be designated by these personnel.

7.705.13 Insurance

- A. Every facility must carry public liability insurance. The applicant or licensee must submit the amount of the insurance and the name and the address of the insurance agency providing the insurance for the facility. Information about the insurance must be maintained at the facility and must be made available to the Department upon request.
- B. Facilities operating their own transportation vehicles must carry insurance in compliance with the minimum limits required by the Colorado Revised Statutes, Title 10, Article 4. Information about the insurance must be maintained at the facility and must be made available to the Department upon request.
- C. The facility must carry workers' compensation and unemployment insurance as required by law. Information about the insurance must be maintained at the facility and must be made available to the Department upon request.

7.705.2 CHILD CARE SERVICES

7.705.21 Case Management Services Provided by the Residential Child Care Facility

- A. A staff member qualified as stated at Section 7.705.44.A must be assigned the responsibility for case management for each child/youth.
- B. Each individual assigned the responsibility of providing case management services for a child/youth must:
 - 1. Participate actively in the admission procedure, treatment planning, discharge, and after care planning.
 - 2. Assure that appropriate information concerning the child/youth and their treatment needs or interventions is shared with other staff members who supervise or care for the child/youth regularly and maintain and document communication with parents, guardians, referring agency, or the agency responsible for discharge planning and follow-up care of the child/youth.

3. Assure that an individual child's/youth's plan is developed with the child/youth and required plan review and necessary modifications are made in accordance with Section 7.714.4.B.
 4. Assure that requested progress reports for court and placing agencies are completed and submitted in a timely fashion.
 5. Attend administrative reviews in person, virtually, or by conference call, if applicable.
 6. Assure that the individual child's/youth's plan developed for each child/youth is implemented, including treatment by specialists, and documented in the child's/youth's file.
- C. RCCF providers who are serving children or youth enrolled in the Children's Habilitation Residential Program (CHRP) waiver must be in compliance with rules contained in the Department of Health Care Policy and Financing's Medical Assistance Rules and Regulations at Section 8.508 (10 ccr 2505-10; 8.500) (2024), which is hereby incorporated by reference. No later amendments are incorporated. These regulations are available at <https://www.sos.state.co.us/ccr/generaterulepdf.do?ruleversionid=11637&filename=10%20ccr%202505-10%208.500>. Copies are also available for public inspection and copying by contacting the Colorado Department of Human Services, director of the Child Welfare Division, 1575 Sherman Street, Denver, Colorado, 80203, or at any state publications library during regular business hours.).

7.705.22 Recreation and Leisure Activities

- A. A residential child care facility (RCCF) must have a written plan for ensuring that a range of indoor and outdoor recreational and leisure opportunities are provided for children/youth in care. Adaptive recreation and/or adaptive recreation opportunities must be provided to those children/youth with required accommodations and to comply with the Americans with Disabilities Act (ADA), 42 U.S.C. Section 12101 through Section 12213, and its implementing regulation, 29 C.F.R. Part 1630 (2025), as incorporated by reference in Section 7.701.14, if applicable, and documented within the child's/youth's individualized plan. Such opportunities must be based on the individual interests of the children/youth, with personal and treatment needs being considered. The reasonable and prudent parent standard requirement for staff to approve activities for a child or youth in care includes the following actions:
1. At least one (1) staff or administrator in a Residential Child Care Facility (RCCF), who is trained in determining how to use and apply the Reasonable and Prudent Parent Standard, must be designated to apply the Reasonable and Prudent Parent Standard to decisions involving the

participation of a child/youth in age or developmentally appropriate activities.

2. The staff must receive training in how to determine whether approving a child's/youth's participation in an extracurricular activity, enrichment, cultural, or social activity is consistent with the Reasonable and Prudent Parent Standard based upon the criteria in Section 7.701.200.
- B. Responsibility must be assigned for supervision and coordination of the total recreation program, including training and supervision of staff and volunteers, to assure maximum safety for each participant. For programs without a total recreation program, a designated responsible adult must be assigned for supervision and coordination of any recreational activity to ensure child/youth safety.
- C. Outdoor and indoor recreational areas and equipment must be provided in sufficient variety and quantity to meet the needs of children/youth and to offer some choice of activities. Adaptive recreation equipment and/or adaptive recreation opportunities must be provided to those children/youth with required accommodations and to comply with the Americans with Disabilities Act (ADA), 42 U.S.C. Section 12101 through Section 12213, and its implementing regulation, 29 C.F.R. Part 1630 (2025), as incorporated by reference in Section 7.701.14, if applicable, and documented within the child's/youth's individualized plan. Games, toys, equipment, and arts and crafts material must be selected according to developmental appropriateness, number of children/youth, and with consideration of the needs of children/youth to engage in both active and quiet play. All equipment and materials must be of a sufficient quality to assure safety, and must be of a type which allows for imaginative play and creativity.
- D. If the facility takes residents on out-of-state trips, the facility must develop a written plan for each trip, which includes goals for participants for the trip, selection criteria of participants, necessary skills for staff members, necessary equipment to meet participants' needs on the trip, supervision plan, and a letter of information to parents and/or guardians.
1. The facility must obtain the written permission of the parent and/or guardian for each out-of-state trip or must obtain an overall written permission of the guardian for any out-of-state trip in which the child/youth will participate.
- E. Some facilities may participate in special activities which are either comprehensively integrated and salient to the total treatment program or which supplement and enhance the treatment process. When the facility participates in any special activities, such as swimming, boating, horseback riding, use of the trampoline, white water rafting, backpacking, camping, or winter sports, the facility must comply with the Rules Regulating Special Activities found at Section

7.719. The child's/youth's participation in a special activity must be included in the individual child's/youth's plan.

1. When the residents of the facility participate in a special activity, each facility must have a plan of action in case of natural disasters relevant to the terrain and activity, lost participants, injuries, and illnesses. The plan must include available means of communication during special activities. These plans must be in writing and must accompany the participants and staff, and also be on file at the facility office. The staff must have received training regarding the implementation of these plans prior to the departure of the participants.
2. First Aid supplies must be made available in vehicles, recreation areas, and in a central location in the building. First Aid supplies must be in compliance with CDPHE regulations. There must be a person holding at least a current American Red Cross First Aid/CPR/AED certificate for schools and communities or equivalent at the site of the activity when the activity is in progress.

7.705.3 PERSONAL CARE OF THE CHILD/YOUTH

7.705.31 Food and Nutrition

- A. Menus must be planned at least one (1) week at a time and must be dated, posted, and filed for at least one (1) year. Menus must be made available to the Department upon request.
 1. If menus are used on a rotating basis, they must not repeat for three (3) weeks, or if cycle menus are used, they must be at least three (3) weeks long.
 2. Residential child care facilities must make available or make provisions for each child/youth to have three (3) nutritious meals per day scheduled at regular intervals. If more than four (4) hours elapse between meals or fourteen (14) hours elapses between evening meal and breakfast, a nutritious snack must be served.
 3. Milk must be made available with all meals, and accommodations must be made for those children/youth with lactose intolerance or who are dairy free.
 4. A facility must take into account any cultural and/or religious considerations and any special dietary restrictions of the children/youth in placement when planning menus, such as, but not limited to, children/youth who are vegan, vegetarian, pescetarian, gluten free, dairy free, or who have any other specific food allergies, preferences, or dietary restrictions.

5. Any accommodations made to menus specific to allergies, preferences, or other dietary restrictions must be approved by a registered dietitian and documented for review by the Department upon request.
- B. If menus are not prepared by a registered dietitian, there must be a review and approval by a registered dietitian at least annually or when the menu is changed more than three (3) days per month.
- C. Facilities participating in the United States Department of Agriculture (USDA) food program must have the menus for non-reimbursable meals and snacks reviewed and approved by a registered dietitian.

7.705.4 HUMAN RESOURCES

7.705.41 Administrative Staff

A. Administrator

The administrator of a residential child care facility (RCCF) must be qualified as follows:

1. The administrator must have received a bachelor's degree from a regionally accredited college or university and have a minimum of three (3) years of verified experience in the human services field, one (1) of which was in a supervisory or administrative position; or,
2. The administrator must have received a master's degree from a regionally accredited college or university and have a minimum of two (2) years of verified experience in the human services field, one (1) of which was in a supervisory or administrative position.
3. The administrator must assume or oversee the following duties: human resources management; policy and procedures development and implementation; quality assurance; regulatory compliance and accountability; overall professionalism of the agency; responsibility for the daily operation of the agency; overall direction and responsibility for the children/youth, program, facility, and fiscal management; oversight of trauma-informed care plans that are approved by the Department; overall direction and responsibility for supervision of adequate staffing as defined in rule; the selection and training of a staff member who can assume responsibility for managing the facility in the administrator's absence; and the establishment of resources within the community.

B. Assistant or Acting Administrator

1. In each residential child care facility (RCCF), there must be a specifically designated staff member, age twenty-one (21) or over, capable of acting as a substitute for the administrator during their absence. The duties and

responsibilities of the substitute administrator must be aligned with the duties and responsibilities of the administrator.

2. If the administrator is regularly absent from the facility for more than fifty percent (50%) of their working hours, an assistant administrator must meet the same qualifications as the administrator found at Section 7.705.41.A. A licensed professional may act as the acting administrator.

C. Administrative Coverage

1. When there is a change in administrator, or when the administrator has left the facility permanently without a replacement, the Department must be notified in writing within twenty-four (24) hours. When a possible change in administrator is anticipated, the facility must notify the Department in writing prior to the change.
2. The administrator or the staff member to whom the administrator has delegated responsibility must be available at all times either at the facility or via a virtual or telephonic option.

7.705.42 Medical, Health, and Clinical staff

The following apply to facilities who are only residential child care facilities (RCCF). For qualified residential treatment program requirements for medical and health staff, see Section 7.705.204 through Section 7.705.206.

- A. A residential child care facility (RCCF) must have access to medical consultative services and a health care provider licensed pursuant to Article 240 of Title 12, C.R.S., in order to provide effective healthcare to meet the needs of the children/youth in the facility. A residential child care facility may employ a licensed health care provider(s) who would be responsible for the health care of the children/youth in the facility. Such contracts or terms of employment must outline the number of hours to be provided for on-site consultation and/or intervention.
- B. If the licensed medical health care provider is not an employee of the facility, there must be a description of the contracted consultative services or other medical duties to be performed, and there must be a written contract outlining the duties to be performed and their required availability to the facility.

7.705.43 Child Care Staff

A. Residential Child Care Facilities (RCCFs) and Shelter Care Facilities

1. Each child care staff working within an RCCF must be at least twenty-one (21) years of age and have a high school diploma or equivalent. An emphasis may be placed on skills-based hiring for staff as well as valuing lived experience in the child welfare system.

2. Child care staff aides must be at least eighteen (18) years old and must have a high school diploma or equivalent. Child care staff aides must work directly under the supervision of a child care staff member in activities specified by the child care staff member or with the approval of the director.
 3. Relief staff must have the same qualifications as regularly assigned child care staff.
 4. Supervisors are required to have a college degree as well as a minimum of two (2) years of experience working in a child care setting. An associate degree may be substituted for the college degree requirement. Staff members promoted into supervisory positions or people hired into supervisory positions without supervision experience must complete six (6) hours of training within ninety (90) days of hire/promotion, as provided by the provider training academy or the facility/agency.
- B. Qualified Residential Treatment Programs (QRTPs) and Psychiatric Residential Treatment Facilities (PRTFs)
1. Each child care worker must be at least twenty-one (21) years of age and have completed two (2) years of college education. An associate degree may be substituted for the college requirement. A high school diploma or equivalent and a minimum of two (2) years' experience in the human services field may be substituted for the college requirement. A child care worker may substitute each year of work experience with thirty (30) semester hours of credit earned from a regionally accredited college or university. A high school diploma or equivalent and thirty (30) semester hours and a minimum of one (1) year of experience may be substituted for the required college. An emphasis may be placed on skills-based hiring for staff as well as valuing lived experience in the child welfare system.
 2. Child care staff aides must be at least eighteen (18) years old and must have a high school diploma or equivalent. Child care staff aides must work directly under the supervision of a child care staff member in activities specified by the child care staff member or with the approval of the director.
 3. Relief staff must have the same qualifications as regularly assigned child care staff.
 4. Each child care worker and any staff member who provides direct care for children/youth must be provided with ongoing supervision. Each child care worker and staff member who provides direct care for children/youth must be provided individual supervision at least every thirty (30) days and documented and maintained in the employee file. More frequent

supervision must be provided as necessary for support or to address performance and must be documented in the individual staff file.

5. Supervisors are required to have a college degree in the human services field or related field as well as a minimum of two (2) years of experience working in a child care setting. Staff members promoted into supervisory positions or people hired into supervisory positions without supervision experience must complete six (6) hours of training within ninety (90) days of hire/promotion, as provided by the provider training academy or the facility/agency.
6. Supervision of staff should address, at a minimum, the clinical needs of the children/youth in the care or service of the staff, trauma-informed practices, performance, emotional regulation, and vicarious trauma.
7. Documentation of the discussions during supervision should be signed or acknowledged by both parties and must be maintained by the facility and made available upon request by the Department.

7.705.44 Case Management Staff and Other Professional Staff

A. Case management must be provided by:

1. A qualified professional having a master's degree in social work, psychology, or child development from a regionally accredited college or university and have a minimum of two (2) years of treatment-oriented or case management experience, or
2. A designated member of the staff who must have a bachelor's degree from a regionally accredited college or university with a major in social and behavioral sciences, and a minimum of three (3) years of experience working with children/youth. The designated case management staff must have an arrangement for consultation from a qualified professional as described above. Effective consultation must include, at a minimum, professional and documented supervision of each case management staff at least once every thirty (30) days to review pending and new admissions to the facility, initial assessments and initial individual child's/youth's plans, current discharge plans, individual child's/youth's individual plan reviews and modifications, communication strategies with partners and families, progress reports, and planning of care following discharge.

- B. The case management staff must ensure the individual child's/youth's plan, as described in Section 7.714.4.B, aligns with the reason for residential child care facility level of care and outlines anticipated length of stay, specific care, treatment, and aftercare planning guidelines for successful discharge, as approved and signed off on by the licensed clinician.**

- C. The qualified professional, as specified in Section 7.705.44.A.1 may be a full-time or part-time employee of the facility or an individual with which the facility has a contract to provide the necessary services and/or consultation.
- D. The ratio of case management staff to children/youth must be dependent on the type of facility and the needs of the children/youth therein, but there must be at least one (1) full-time case management staff member for every twenty (20) children/youth or a part-time staff member assigned for a fraction thereof.
- E. The case management staff must be responsible for providing case management services as stated at Section 7.705.21.
- F. Psychiatrists used by the child care facility must be qualified and licensed to practice and must provide, as needed, diagnosis, treatment, and consultation services.
- G. Only licensed and trained clinicians, as outlined in Section 7.705.205.A may provide testing and diagnostic services for assessment purposes and must be qualified and licensed to practice as designated by Colorado law.
- H. Other professionals providing specific therapy must be licensed professionals as designated by Colorado state law.

7.705.45 Food Services and Maintenance Staff Members

- A. All food services and maintenance staff members must comply with the general requirements for all personnel as specified in Section 7.714.92.
- B. There must be one (1) food service staff member who has basic knowledge and understanding of nutrition, food purchasing, and menu planning. If the staff member is not qualified as a registered dietitian, there must be available written consultation from a registered dietitian.
- C. Maintenance staff must be in sufficient numbers to maintain an adequate physical space as determined by the policies and procedures of the facility.

7.705.46 Child Care Staff-to-Child/Youth Ratios

- A. There must be a minimum of one (1) child care worker on duty and one (1) staff person on call who can be physically present at the facility within sixty (60) minutes, at all times when there is one (1) child/youth or more at the facility. In the event of an emergency, local emergency services must be called immediately.
- B. At night there must be at least one (1) awake staff person within each physically separate building and within hearing distance of children/youth and a second person who can be physically present at the facility within sixty (60) minutes. In

the event of an emergency, local emergency services must be called immediately.

- C. The ratio of child care staff to children/youth in care must be dependent on the needs of the children/youth, and the ratio of such staff must not be less than the following schedule:

<u>Age of Child/Youth</u>	<u>Waking Hours</u>	<u>Sleeping Hours</u>
5 - 6 years	1 staff member: 6 children/youth	1 staff member: 12 children/youth
7 -12 years	1 staff member: 8 children/youth	1 staff member: 16 children/youth
13 years and over	1 staff member: 10 children/youth	1 staff member: 20 children/youth

- D. Specialized group homes or group centers who are serving children/youth enrolled in the Children's Habilitation Residential Program (CHRP) waiver must be in compliance with rules contained within the Department of Health Care Policy and Financing (HCPF) Medical Assistance Manual at 10 CCR 2505-10 Section 8.508 (2024), which is hereby incorporated by reference. No later editions or amendments are incorporated. These regulations are available at <https://www.sos.state.co.us/ccr/generaterulepdf.do?ruleversionid=11637&filename=10%20ccr%202505-10%208.500>. Copies are also available for public inspection and copying by contacting the Colorado Department of Human Services, Director of the Division of Child Welfare, 1575 Sherman Street, Denver, Colorado, 80203 or at any state publications library during regular business hours.

Specialized Group Facility Maximum Capacity

<u>CHRP</u>	<u>Non-CHRP</u>	<u>Total Children/Youth</u>
0	11	11
1	7	8
2	6	8
3	5	
4	4	8
5	3	8
6	2	8

- E. For children/youth with intellectual and developmental disabilities, impairment to intellectual and adaptive functioning, or physical disabilities, the administrator shall ensure that there is sufficient trained staff on duty to meet the current or potential needs of all residents at all times, considering individual needs such as the risk of accidents, hazards, or other challenging events.

1. The administrator shall ensure that the facility does not depend upon residents to perform staff functions.
2. The ratio of staff to children/youth must be at least:
 - a. If a child/youth has near appropriate functioning with limited intellectual functioning, the ratio must be one (1) staff member to every five (5) children/youth during waking hours and one (1) staff member to every ten (10) children/youth during sleeping hours.
 - b. If a child/youth has limited adaptive behavior and will always need a defined level of supervision, the ratio must be one (1) staff member to every four (4) children/youth during waking hours and one (1) staff member for every eight (8) children/youth during sleeping hours.
 - c. If a child/youth has minimal independence and communication, the ratio must be one (1) staff member to every three (3) children/youth during waking hours and one (1) staff member to every six (6) children/youth during sleeping hours.
 - d. If a child/youth is completely unable to manage self, the ratio must be one (1) staff member to every two (2) children/youth during waking hours and one (1) staff member for every four (4) children/youth during sleeping hours.

7.705.47 Supervision of Children/Youth

- A. The residential child care facility (RCCF) must have a written policy for the standard supervision of children/youth in the program based on the identified population described in the admission policies. The facility must adhere to the policy, and it must include supervision of children/youth while at the facility, during transportation, and while off the premises. The policy must be approved by the Department on an annual basis. When a child's/youth's individual needs require deviation from the supervision policy, it must be documented in the individual child's/youth's plan.
- B. When a child/youth is in their bedroom or is not involved in regular programming, staff must conduct visual checks of each child/youth at irregular intervals, not to exceed fifteen (15) minutes between visual checks.
- C. During sleeping hours, visual checks may not exceed fifteen (15) minutes. Staff must ascertain that children/youth are present in their assigned bed.
- D. Visual checks may need to be conducted at more frequent intervals as determined by the clinician or direct care supervisor on shift when a child/youth presents with a history of or current high-risk behaviors, as identified in the individual child's/youth's plan, that may endanger self or others. high-risk

behaviors may include, but are not limited to, aggression, elopement, suicidal ideations, self-harming behaviors, and sexualized behaviors.

- E. The facility/agency must provide supervision and care as stated in the facility's written policies and as stated in the individual child's/youth's plan. The facility/agency must also provide any specific, time-limited safety plans, as required by the child/youth's behavior and/or needs.
- F. The use of cameras or recording devices is prohibited in children's/youth's sleeping areas and restrooms.

7.705.5 BUILDING AND FACILITIES

7.705.51 Building Plans

Prior to construction, architectural plans for new buildings or for extensive remodeling of existing buildings must be submitted for review and approval by the local fire department and local building department. If required by the Colorado Department of Public Health and Environment (CDPHE) or local public health agency, plans must be submitted for review and approval. Plans must be submitted and reviewed by the Department as to appropriateness, general adequacy, and suitability for child care as well as for compliance with all applicable federal and state laws.

7.705.52 Living Spaces and Equipment Necessary in Residential Child Care Facilities

- A. All children's/youth's sleeping arrangements must be made in consideration of their self-identified gender. Sleeping arrangements for all children/youth must be made according to their safety, clinical, and supervision needs, as documented in each child's/youth's individual child's/youth's plan. In sleeping rooms that accommodate two (2) to four (4) children/youth, sixty (60) square feet of floor space per child/youth must be provided. There must be no more than four (4) children/youth in any bedroom. Each bedroom for single occupancy must have a minimum of eighty (80) square feet of floor space. Children/youth must not be required to sleep in a common area of the residential child care facility, outside the sleeping rooms, unless the procedures for a restriction of a child's/youth's rights found in Section 7.714.31.C-E or 7.714.33.B if the youth is in foster care or participating in the foster youth in transition program, are followed.
 - 1 A closet or designated container(s) with a minimum of four (4) square feet per child/youth must be provided for storage of personal items inside the sleeping room. Provisions for storage may include open shelving units. A separate container for the storage of soiled laundry must be provided for each child/youth.
 - 2. Children/youth who are non-ambulatory or have documented limited mobility must be sleeping in rooms that are at ground level for exiting

access. The facility must comply with all accommodations required in the Americans with Disabilities Act (ADA), if applicable.

- B. Each child/youth must be provided suitable sleeping facilities consisting of individual beds or bunks complete with a mattress. Single beds must be spaced not closer than thirty-six (36) inches laterally or end-to-end. Triple-deck bunks are prohibited. Beds being used by children/youth must have an impervious, easily cleanable mattress cover or pad, clean sheets, pillows, pillowcases, and blankets. All sleeping equipment must be maintained in a clean and sanitary condition as required in 6 CCR Section 1010-7 (2025), which are incorporated by reference. No later editions or amendments are incorporated. The regulations can be found at <https://www.sos.state.co.us/ccr/generaterulepdf.do?ruleversionid=6576&filename=6%20ccr%201010-7> . Copies are also available for public inspection and copying by contacting the Colorado Department of Human Services, 1575 Sherman St., Denver, Colorado 80203, during regular business hours or at any state publications library during regular business hours.
- C. Each room of occupancy must be adequately ventilated by exterior windows or by an approved air-conditioning system. If a mechanical ventilation system is provided, it must meet requirements of local building codes and fire regulations.
- D. When child care staff live in or sleep at the facility, there must be at least one (1) room adequately furnished and a private bath facility reserved for child care staff. Arrangements where child care staff share sleeping rooms with children/youth who have special needs must be approved by the Department.
- E. Work areas for staff who are supervising youth during sleeping hours must be located on the same floor or in the general area of children/youth's sleeping rooms so that the child care worker can supervise children/youth and be accessible when needed. If physical work areas are not located on the same floor or in the general area of children's/youth's sleeping rooms, provisions must be made for line-of-sight supervision at all times between sleeping rooms.
- F. There must be a minimum of twenty-five (25) square feet of space for each child/youth for informal individual or group activities. The area must be adequately and appropriately furnished to accommodate social and recreational activities associated with such living areas.
- G. There must be a designated space distinct from children's/youth's living areas to serve as an office for administrative duties.
- H. There must be a designated room(s) to allow private discussions and counseling sessions.

7.705.53 Outdoor Space and Equipment

- A. The grounds of the residential child care facility must be maintained in a manner that is free from any hazard to health or safety.
1. Fences must be in good repair. There must be at least one (1) fence gate that remains unlocked for immediate emergency egress. The designated exit gate may have time-delay panic hardware provided that such time-delay panic hardware is approved by the fire department having jurisdiction over the facility.

The approved exit gate must release and open automatically after no more than a thirty (30) second delay without constant pressure being applied to the gate latch and must remain open for at least fifteen (15) seconds. The approved exit gate can neither automatically reset itself nor be reset from a central control panel for a period of at least fifteen (15) seconds after release. Magnetic locks in conjunction with time-delay panic hardware that meets these criteria are allowed.
 2. Areas determined to be unsafe, including steep grades, cliffs, open pits, swimming pools, high voltage boosters, or high-speed roads, must be fenced off or have natural barriers to protect children/youth.
 3. Playground equipment must be located, installed, and maintained in compliance with the Consumer Products Safety Commission standards for public playgrounds.
- B. A residential child care facility must have access to outdoor recreational space and suitable recreational equipment to include adaptive recreation spaces and equipment, as needed, for the children/youth in care.

7.705.54 Dining, Kitchen, Laundry, Bathroom Facilities

- A. The dining area, whether located in the living unit or in a separate central dining facility, must meet the following requirements and comply with all accommodations required in the Americans with Disabilities Act (ADA), 42 U.S.C. Section 12101 through Section 12213 (2024), and its implementing regulation, 29 C.F.R. Part 1630 (2024), as incorporated by reference in Section 7.701.14, if applicable:
1. The dining area and kitchen shall be kept in a clean and sanitary condition, as required by the Colorado Department of Public Health and Environment (CDPHE) in 6 CCR 1010-7 (2025), as incorporated in Section 7.705 above.
 2. It must be large enough to accommodate the children/youth and staff.
 3. Floors must be constructed and maintained with a non-slipping surface.

4. Tables and chairs must be of sturdy construction, scaled, or adjusted to the proper height and size so that children/youth can be comfortably served.
 5. Table service and settings must be of the type, size, and design appropriate to the ages and developmental abilities of children/youth served.
 6. Dining and recreation areas may be combined if regulations for dining areas are consistently met.
- B. The kitchen must be designed and equipped to meet the requirements of efficiency, fire safety, and comfortable working conditions for the staff. Each kitchen must contain a CO2 or dry powder fire extinguisher designed to extinguish fires involving organic solids, flammable liquids, and flammable gasses.
- C. Noncommercial types of laundry facilities with ample space for sorting, drying, and ironing must be made available to children/youth old enough and capable of handling their personal laundry. Laundry facilities must be supervised as defined in the agency's written policies and procedures and based on the developmental level and needs of the children/youth in care. When an agency has a central laundry facility, it must be located in an area separate from areas occupied by children/youth.
- D. There must be a ratio of at least one (1) accessible toilet, sink, bathtub and shower for every six (6) children/youth. Toilet and bath facilities for males and females and staff must be separate. Toilets and bath facilities must have doors or partitions. Residential child care facilities (RCCF) licensed after July 1, 2024 must also have available at least one (1) single occupancy, gender-neutral toilet, sink, and bath facility. Where possible, all facilities licensed prior to July 1, 2024 must attempt to provide a single occupancy, gender-neutral toilet, sink, and bath facility. Children/youth must be allowed to use the bathroom facilities of their identified gender or use the gender-neutral bathroom if they prefer. Urinals may be substituted for up to one-third of the required number of toilets in areas of the facility in which males sleep. Toilet and bath/shower areas with multiple toilets or showers in one (1) open area must have partitions between each toilet or bath/shower fixture. Toilet and bath facilities, if not located within the sleeping rooms, must be accessible from a common hallway and be on the same floor with sleeping rooms.

7.705.55 Building Safety

- A. Building Site

1. The facility must be located in an area that is accessible to health resources, public and private utilities, adequate and safe water supplies, sewage disposal, and fire and police protection. If health resources or fire and police protection are limited or not available within a reasonable distance to the facility, a plan must be submitted to the Department for approval through the waiver process, in accordance with Section 7.701.13. If health resources are limited or not available within a reasonable distance to the facility, a plan must be submitted to the Department for medical and mental health emergencies and to meet the needs of routine care.
2. The facility site must be located in accordance with local zoning department requirements.
3. The entire premises of the facility is subject to inspection for licensing purposes, including, but not limited to, the residence where care is to be provided, the grounds surrounding the facility, the basement, the attic (if accessible), any storage buildings, and a garage or carport, if applicable.
4. All stairways containing more than four (4) steps must be equipped with a handrail.
5. The facility must have an active, working telephone. Each separate living unit within a facility must have twenty-four (24)-hour telephone service or an intercom system connected with an outside telephone service. Each facility must have a back-up form of communication that is described in the facility's disaster plan and maintained in working condition at all times. Emergency numbers must be posted near the telephone, including those related to medical care, local fire, local law enforcement, ambulance, the statewide child abuse and neglect hotline, the National Suicide Hotline, the Colorado Crisis Line, Safe2Tell Hotline, and poison control where available. Numbers for the agency or person having legal custody of each child/youth must also be available.
6. If the facility is located in the same building as, or immediately adjacent to, other facilities, an adult treatment center, or nursing home, it must be so arranged that the care and activities of the children/youth residing in the facility are completely separate and independent from the other facility. The facility may not be operated on the premises of a business that might be hazardous to the health, safety, morals, or welfare of children/youth and the operation of the facility.
7. The facility, including indoor and outdoor space, must be maintained in safe condition free from hazards to health and safety.
8. There must be outdoor space available to provide a recreation area for children/youth. This area must be fenced if children/youth younger than

five (5) years old are in care or older children/youth with disabilities require either containment or protection from outside elements such as animals.

9. If in-ground swimming pools, permanent wading pools, or above ground pools, hot tubs, or spas are located on the property, each must have a barrier on all sides and have a method of access through the barrier equipped with a safety device, such as a bolt lock.

B. Maintenance

1. Buildings shall be kept in good repair and maintained in a safe condition. Good housekeeping shall be observed in all areas at all times. Needed repairs must be identified regularly and corrected immediately.
2. All areas of the facility available for children's/youth's activities, including equipment, materials, and furnishings must be maintained in safe condition and free from hazards such as sharp points or corners, splinters, protruding nails, and broken play and recreational equipment. This also includes the elimination of debris and hazardous items of all kinds and removal of broken play and recreational equipment and any other devices which are in poor repair.
3. Closets, attics, basements, cellars, furnace rooms;, and exit routes must be kept free from the accumulation of significant amounts of extraneous materials such as discarded furniture, Furnishings, newspapers, or magazines that could pose a fire or health hazard. Combustibles such as cleaning rags, mops, and cleaning compounds must be stored in well-ventilated areas. Solutions, cleaning compounds, and other hazardous substances must be properly labeled and stored in areas inaccessible to children.
4. All hazardous chemicals, tools, and other equipment, including, but not limited to, matches, paints, gasoline, sharp kitchen utensils, insecticides, cleaning materials, computer duster spray, and personal hygiene or other products containing more than five (5) percent alcohol content, must be inventoried and stored in a secured, locked area. All other household chemicals, tools, or other equipment must be secured out of reach of children/youth as dictated by the lowest level of functioning within the facility.
5. When a facility's program description and admission criteria define their population and program as preparing for independent living within the next two (2) years, and the youth in the program are at least seventeen (17) years of age, the facility may provide limited amounts of household cleaners and laundry products accessible to youth to promote independent use of these products.

6. Storage of gasoline, kerosene, fuel oil, and other flammable material must meet requirements of safety and fire codes. Indoor storage of such materials is not permitted unless fire code standards are met.
7. Storage must not be permitted around water heaters and furnaces.

C. Exits

1. Every building or structure must be provided with exits sufficient to permit the prompt escape of occupants from the building and away from the premises in case of fire or other emergency. Additional safeguards must be provided for life safety in case any single safeguard is ineffective due to some human or mechanical failure. The facility must comply with all accommodations required in the Americans with Disabilities Act (ADA). 42 U.S.C. Section 12101 through Section 12213 (2024), and its implementing regulation, 29 C.F.R. Part 1630 (2025), as incorporated by reference in Section 7.701.14, if applicable.
2. Each residential child care facility (RCCF) must have at least two (2) approved, alternate, remotely-located means of egress from each floor of the building to safe and open space at ground level.
3. Egress from each dwelling unit, sleeping room, and dormitory must be directly available without passage through another dwelling or rooming unit to the outside or to a common hallway leading to the outside. If a facility utilizes a vestibule or common hallway to move from one area to another, any doors blocking that vestibule must remain open and unlocked to allow for egress.
4. In residential child care facilities (RCCF), time-delay, panic hardware utilizing no more than thirty (30)-second delays before release may be installed on the one (1) approved exit door on each floor of the facility, provided that such time-delay, panic hardware is approved by the fire department having jurisdiction over the facility.

The one (1) approved exit doors on each floor must release and open automatically after no more than a thirty (30)-second delay without constant pressure being applied to the doors and must remain open for at least fifteen (15) seconds. The one (1) approved exit door on each floor can neither automatically reset themselves nor be reset from a central control panel for a period of at least fifteen (15) seconds after release. Magnetic locks in conjunction with time-delay panic hardware that meets these criteria are allowed. Delayed egress is only permitted in buildings set up with sprinkler systems, and the locks must release upon activation of the fire system.

5. In every building or structure, exits must be so arranged and maintained as to provide free and unobstructed egress from all parts of the building or structure at all times when it is occupied. No lock or fastening device to prevent free escape from the inside of any building can be installed unless specifically authorized pursuant to Section 7.714.53. All exit doors must be equipped with single-action hardware only.
6. Every exit must be clearly visible, or the route to reach it must be conspicuously indicated in such a manner that every occupant of every building or structure who is physically and mentally capable will know the direction of escape from any point. Each path of escape must be so arranged or marked that the way to a place of safety outside is unmistakable.
7. In every building or structure, adequate and reliable illumination must be provided for all exit facilities. Lighting must be tested monthly and maintained.
8. Every building or structure must be so constructed, arranged, equipped, maintained, and operated as to avoid undue danger to the lives and safety of its occupants from fire, smoke, fumes, or resulting panic during the period of time reasonably necessary for escape from the building or structure in case of fire or other emergency.
9. Compliance with this rule must not be construed as eliminating or reducing the necessity for other provisions for safety of persons using a structure under normal occupancy conditions, nor shall any provision of this rule be construed as requiring or permitting any condition that may be hazardous under normal occupancy conditions.
10. The local fire department must determine the adequacy of exits and other measures for life safety in accordance with the requirements of the International Building Codes and the National Fire Protection Association Codes. In cases of practical difficulty or unnecessary hardship, the local fire department may grant exceptions from the International Building Codes or the National Fire Protection Association Codes, but only when it is clearly evident that reasonable safety is thereby secured.

D. Electrical Equipment

1. Electrical wiring systems in all buildings must conform to the requirements of the state electrical board. Electrical appliances must be examined frequently for worn or otherwise defective wiring.
2. No more than one (1) extension cord or power strip may be connected between an electrical appliance and the wall outlet.

E. General Safety Practices

1. A residential child care facility (RCCF) must immediately notify the responsible emergency agency or department of a fire or other disaster which might endanger children/youth or require their removal for reasons of health and safety. The Department of Human Services must be immediately notified of a fire or other disaster in accordance with reporting requirements outlined in Section 7.701.52.
2. Security guards patrolling the grounds and structures of a residential child care facility, whether employed by the facility or by a security service under contract to the facility, must not be permitted to be in possession of any firearm, taser, knives, or chemical weapons on the grounds or within the structures of the facility.
3. Porches, elevated walkways, and elevated play areas within a residential child care facility must have barriers to prevent falls.
4. Power-driven equipment used by a residential child care facility must be kept in safe and good repair. Such equipment must be used by children/youth only under the direct supervision of a staff member and according to state law. The ignition keys for power-driven equipment and vehicles must be inventoried daily and stored in an area inaccessible to children/youth.
5. Staff and children/youth must be trained to report fires and other emergencies appropriately. Children/youth and staff must be trained in fire prevention, including proper utilization of fire extinguishers and proper storage of combustibles in proximity to ignition sources.
6. In every building or structure, fire alarm facilities must be provided to warn occupants of the existence of fire so that they may escape or to facilitate the orderly conduct of fire exit drills. Fire alarms must be maintained in accordance with International Fire Code and manufacturer standards.
7. Responsibility for the planning, conducting, evaluating, and documenting of fire drills must be assigned to an individual staff member.
8. A residential child care facility (RCCF) must maintain an active safety program, including investigation of all accidents and recommendations for prevention.
9. Smoking, including vaping, is prohibited in all areas of the licensed facility, including all buildings, vehicles, grounds, or areas otherwise owned or controlled by the facility as further outlined in Section 25-14-103.5, C.R.S.
 - a. Smoking, including vaping, is prohibited by children/youth, staff members, visitors, or other individuals at the facility, on the grounds of the facility, or in an offsite location in which the children/youth are

transported or attending. Staff shall not use tobacco products with children/youth at the facility or off grounds.

- b. No tobacco or nicotine products will be stored by the facility for off grounds use by the children/youth.
 - c. Nicotine products used to aid in the process of quitting smoking may be stored at the facility if approved by the child's/youth's multidisciplinary team, parent(s) and/or guardian(s), and the facility staff.
10. A facility must not maintain any weapons, such as firearms, air rifles, air soft or bb guns, hunting bows, hunting knives, or hunting slingshots on the grounds or within the structure of the facility. A facility must not permit any staff member or child/youth to be in possession of any firearm on the grounds or within the structures of the facility. Chemical weapons, even when carried by staff members to and from work for protection, must be in a secure, locked location when present at the facility. Weapons must not be transported in any vehicle in which children/youth are riding. Law enforcement professionals are exempt from the requirements of this section if conditions of their employment require them to carry weapons.

F. Fire Safety

- 1. Fire hazards, such as defective electrical appliances and electric cords, dangerous or defective heating equipment, or flammable material stored in such a manner as to create a risk of fire must be corrected or eliminated.
- 2. The facility must contain at least one (1) U.L.-approved fire extinguisher on each floor of the facility and in the kitchen that is highly visible, easily accessible, and in working condition, weighing not less than five (5) pounds, that has a rating of 2A, 10bC. Extinguishers are required to be maintained annually.
- 3. A smoke detector, in working condition, must be installed on each level of the facility where child care occurs and near sleeping areas, and at least one (1) carbon monoxide detector must be installed per floor, per manufacturer directions, in the facility.
- 4. All heating units, gas or electric, must be installed and maintained with safety devices to prevent fire, explosions, or other hazards. No gas or electric space heaters, open-flame gas or oil stoves, hot plates, or unvented heaters may be used in the facility for heating purposes.
- 5. Flammables, aerosol paints, insecticides, chemicals, and other dangerous materials must be locked or stored so that they are inaccessible to children/youth and must be stored in areas separated from sleeping or living areas with a fire resistance barrier of at least one (1) hour. No more

than ten (10) gallons of flammable or combustible liquids may be stored outside a flammable cabinet with the exception of materials stored in an approved safety can. A "safety can" means an approved container, of not more than five (5) gallon capacity, having a spring-closing lid and spout cover and so designed that it will safely relieve internal pressure when subjected to fire exposure.

6. Heating devices such as radiators, registers, fireplaces, and steam and hot water pipes that pose a fire or burn hazard to children/youth must be screened or otherwise protected.
7. Flammable material must not be stored near a furnace, hot water heater, or other heating device.
8. There must be no candles or other highly flammable objects permitted in sleeping areas.
9. Exit doors must be obvious, clearly identified as an exit, and marked by an approved sign that is clearly visible from any direction of exit access. No lock or fastening to prevent free escape from the inside of any room used by the children/youth may be permitted except for a Department-approved seclusion room as outlined in Sections 7.714.54 through 7.714.549.
10. Exit routes must be kept free of any barriers to free escape including, but not limited to, discarded furniture, furnishings, laundry, and stacks of newspapers or magazines that could interfere with the prompt evacuation of the facility.
11. All fencing around the perimeter of the property must have at least one (1) exit gate, and the gate(s) must remain unlocked at all times to allow unobstructed egress from the facility and grounds in case of an emergency. The gate must be clearly marked and easily accessible.

G. Disaster Drills

1. There must be written procedures for staff and children/youth to follow in case of emergency or disaster specific to occupancy type. These procedures must be developed by the facility with the assistance of qualified fire and safety personnel and must include provisions for staff roles and responsibilities during an emergency, evaluation of the facility, and the assignment of a central meeting place where each individual may be accounted for.
2. Fire exit drills, to include wildland fire drills where appropriate, must be held on each shift on a rotating basis with no less than one (1) drill per shift at least every six (6) months so that all occupants are familiar with the drill procedure and their conduct during a drill is a matter of established routine.

3. Drills must be held at unexpected times and under varying conditions to simulate the conditions of an actual fire.
4. Drills must emphasize orderly evacuation under proper discipline rather than speed. Running or horseplay must not be permitted.
5. Drills must include suitable procedures for ensuring that all persons in the facility actually participate.
6. A record of fire drills held over the past twelve (12) months must be maintained by the facility on the drill log, including the date and time of the drill, the amount of time taken to evacuate the facility, number of children/youth and staff participating in the drill, and any incidents or conditions of the drill that need follow-up for safety purposes.
7. Fire alarm systems must be regularly used in the conduct of drills. Accidental activation of the system will not count as a drill.
8. The facility must make special provisions for the evacuation of any physically handicapped children/youth in the facility.
9. The facility must take special care to help children/youth with intellectual and developmental disabilities understand the nature of such drills.
10. If appropriate to the location of the facility, tornado drills and/or drills for those facilities in wildland urban interfaces (WUIs) must be held often enough so that all occupants are familiar with the drill procedure and conduct during a drill is a matter of established routine. A record of tornado drills and/or wildland urban interface (WUI) drills held over the past twelve (12) months must be maintained by the facility.
11. Lockdown drills must be completed at least once every six (6) months so that all occupants are familiar with the procedure if there were to be a possible threat within the building. Staff are instructed to bring children/youth inside a room and lock the door. Children/youth are instructed to go to a safe space in the room and remain quiet until first responders give an "all clear".
12. A record of lockdown drills held over the past twelve (12) months must be maintained by the facility on a drill log, including the date and time of the drill, the amount of time taken to evacuate the facility, number of children/youth and staff participating in the drill, and any incidents or conditions of the drill that need follow-up for safety purposes.

7.705.56 Transportation

- A. A facility must ensure that each child/youth is provided with the transportation necessary for implementing the child's/youth's individual plan.

- B. A facility must have means of transporting children/youth in cases of emergency.
- C. Any vehicle used by the facility in transporting children/youth in care, whether such vehicle is operated by a staff member or any other person acting on behalf of the facility, must be properly licensed, including current registration and insurance on file at the agency/facility, and the vehicle must be maintained in accordance with Colorado law.
- D. Any staff member or other person acting on behalf of the facility operating a vehicle for the purpose of transporting children/youth must be properly licensed to operate the class of vehicle in accordance with Colorado law.
- E. All passengers in the vehicle must be properly fastened into a restraint system that conforms to all applicable federal motor vehicle safety standards pursuant to Colorado law.
- F. A facility must not allow the number of persons in any vehicle used to transport children/youth to exceed the number of available seats with fully functioning seatbelts in the vehicle.
 - 1. All seats in vehicles used to transport children/youth must be forward-facing and must be securely fastened to the vehicle body.
 - 2. A seat belt or booster seat for children/youth under forty (40) pounds must be provided for each seat.
 - 3. Only the number of passengers that can be comfortably seated on passenger seats shall be permitted in the vehicle.
 - 4. Each occupant must be sitting when the vehicle is moving.
 - 5. Passengers must not sit on the floor.
- G. The vehicle must be enclosed and provided with door locks and contain a First Aid kit and fire extinguisher.
- H. A facility must ascertain the nature of any need or problem of a child/youth which might cause difficulty during transportation, such as seizures, a tendency toward motion sickness, or identified behaviors that would place the driver or other children/youth in the vehicle at risk. The facility must communicate this information to the driver of any vehicle transporting the children/youth in care.
- I. The facility must have naloxone available onsite and in facility vehicles at all times. All staff must be trained on how to use naloxone. Certificates of training must be kept in each staff file.

7.705.6 POLICIES AND PROCEDURES

All policies and procedures must be in written form and available to the department for review upon request. Staff must be trained to and adhere to all written policies and procedures implemented by the facility.

All policies and procedures must be consistent with the child/youth rights outlined in Section 7.714.31. Additionally, policies and procedures must be consistent with requirements outlined in Sections 7.714.33 through 7.714.34 and Section 19-7-101(2), C.R.S. if the youth is in foster care or participating in the foster youth in transition program.

7.705.61 Child's/Youth's Rights

- A. Children's/youth's rights are listed in Sections 7.714.31 through 7.714.34.
- B. All policies and procedures must be consistent with the rights outlined in Section 7.714.31. Additionally, policies and procedures must be consistent with rights outlined in Section 7.714.33 and Section 19-7-101(2), C.R.S. if the child/youth is in foster care or participating in the foster youth in transition program.
- C. For rights listed in Section 7.714.31, the facility must have written policies for any restrictions or limitations to a child's/youth's right, and any restrictions or limitations must only limit a child's/youth's rights to ensure the safety of the child/youth. If a facility enforces any restrictions upon the child's/youth's rights, the facility must follow guidelines as outlined in Sections 7.714.31.C through 7.714.31.E. Written policies must be made available to the Department upon request.
- D. For rights listed in Section 7.714.33 and Section 19-7-101, C.R.S., all rights limitations or denials must go through the judicial system. A juvenile court shall not limit or deny a child's/youth's rights unless the court finds by clear and convincing evidence that there are extraordinary circumstances and the limitations or denial is necessary for the safety of the child/youth in foster care.
- E. The facility must have written policies regarding monies earned and spent.

All policies and procedures must be consistent with the child/youth rights outlined in Section 7.714.31. Additionally, policies and procedures must be consistent with requirements outlined in Sections 7.714.33 through 7.714.34 and Section 19-7-101(2), C.R.S. if the youth is in foster care or participating in the foster youth in transition program.

7.705.62 Individual Child's/Youth's Plan

- A. The facility must have written policies concerning the individual child's/youth's plan.

All policies must be consistent with applicable federal and state laws, including Sections 19-7-101(2)(b)(x) and 19-7-101(2)(h)(V), C.R.S. if the youth is in foster care or participating in the foster youth in transition program.

All written policies must include the following:

1. Staff responsibility for planning and implementation of the treatment procedures and techniques;
2. The range of procedures and techniques to be used and the anticipated range or types of behavior or conditions for which such procedures and techniques are to be used, including philosophy of treatment, modes of therapy, treatment modalities, positive behavior intervention, problem management, discipline, physical restraint, and seclusion, where allowed and approved by law;
3. The facility's responsibility for monitoring the safety of the children/youth during treatment;
4. Review procedures for ensuring the appropriateness of the ongoing treatment and placement for each child/youth;
5. Policies and procedures requiring termination of the treatment procedures in the event of achievement of goals identified for successful completion of treatment or when the procedures are proving to be ineffective or detrimental for a particular child/youth;
6. Policies and procedures for ensuring the child/youth, their family of origin, and/or other support persons are involved in the development of and revisions made to the individual child's/youth's plan and consent to its content, consistent with state and federal law;
7. Policies and procedures for ensuring the child/youth, their family of origin, and/or other support persons are involved in the development of and revisions made to the individual child's/youth's plan and consent to its content;
8. Policies and procedures on how the facility/agency monitors the ongoing physical and emotional safety and the privacy of a child/youth during treatment or therapy, including face-to-face or virtual telehealth interactions(s) with the child/youth;
9. Requirements, where appropriate, for medical examination of a child/youth prior to implementation of a treatment strategy on a regular basis;
10. Policies and procedures on how the facility will monitor and analyze the facility's treatment program and strategies, including how the facility will assess the overall effectiveness of the treatment program; and

11. Each facility must adopt and implement a written policy for continuity of resident care which includes, at a minimum, the following:
 - a. Ease of resident movement from one element of service to another one within the facility; and
 - b. Plans for discharge, aftercare, medication management, and referrals to community resources as a component of the individual child's/youth's plan as outlined in Section 7.714.4.B and elements of a discharge summary as found in Section 7.714.85.I. Any recommendations must involve the family of origin or other supportive individuals as defined in Section 7.714.4.B.2 if actively participating in treatment.

7.705.63 Policies for Admission, Discharge, and Confidentiality of Child/Youth Files

- A. Each facility must develop and adhere to written admission policies. Such policies must be provided to the referring agencies and parent(s) and/or guardian(s) of the child/youth referred for placement.

All policies must be consistent with applicable federal and state laws, including the child's/youth's rights and requirements outlined in Section 19-7-101, C.R.S. if the youth is in foster care or participating in the foster youth in transition program.

All policies, at a minimum, must include:

1. Policies related to intake, including how the facility assesses and provides for the child's/youth's immediate basic needs of clothing, food, and medical care, and identification of all documents that need to be completed upon admission. Documents must include, where applicable, but are not limited to:
 - a. When consistent with state and federal laws, consent to treatment according to Section 27-65-104, C.R.S.;
 - b. Consent to release confidential information;
 - c. Disclosure statements for therapists;
 - d. Consent for healthcare;
 - e. Placement agreement;
 - f. Appropriate authorizations;
 - g. Use of restraint/seclusion;

- h. Grievance procedure; and
 - i. Specific to QRTPs, consent for the BHA to release the state-approved assessment.
 - 2. The age range, sex assigned at birth, and identified gender of the children/youth accepted for care;
 - 3. Any special programming offered by the facility;
 - 4. Any pre-placement requirements for the child/youth, the parent(s) or guardian(s), and/or the placing entity;
 - 5. The criteria, situations, and specific behaviors that would result in a request to remove a child/youth from placement prior to the planned discharge;
 - 6. Specific to QRTPs and PRTFs, the facility's policy concerning the provision of mental health services to children/youth, if appropriate, and the application of Section 27-65-104, C.R.S. - Voluntary Applications for Mental Health Services - when a child/youth is receiving mental health treatment;
 - 7. A statement regarding the religious orientation or affiliation of the facility and of the religious activities of the facility;
 - 8. Opportunities for children's/youth's participation in recreational activities, religious activities, and community life;
 - 9. A statement regarding the potential use of images of the children/youth;
 - 10. A process for the completion of the requirements of the interstate compact on the placement of children (ICPC) in accordance with Section 24-60-1802, C.R.S. for any child/youth in which the placing entity is outside of Colorado; and
 - 11. A policy regarding usage of cell phones or other electronic communication devices by children/youth.
- B. As identified in Section 7.714.2.A, a facility that accepts youth who are eighteen (18) years of age and older but under twenty-one (21) years of age must have a written policy to address how the facility will ensure the youth's adult rights and duties will be accommodated, including, but not limited to, registering to vote and registering for selective service, in addition to rights identified in Sections 19-7-101(2), C.R.S. and Section 7.714.33 if the child/youth is in foster care or participating in the foster youth in transition program.

- C. Provisions must be made for the storage and protection of children's/youth's property when admitted into the facility. Provisions must be consistent with Section 19-1-701(2)(d)(II) and 19-7-101(2)(f)(III), C.R.S. If the child/youth is in foster care or participating in the foster youth in transition program the facility is responsible for creating a written policy or acknowledgement that identifies the length of time a children's/youth's property will be stored at the facility.
- D. The facility must allow a child/youth in care to bring their personal belongings to the program, as outlined in facility policy. If the child/youth is in foster care or participating in the foster youth in transition program, the facility must allow a child/youth to bring their personal belongings to the program consistent with Section 19-7-101(2)(f)(III), C.R.S.
- E. For children/youth in foster care or participating in the foster youth in transition program, the facility is responsible for creating a written policy or acknowledgment consistent with Section 19-7-101(2)(f)(III), C.R.S. that identifies the procedures for having the child's/youth's belongings transported in appropriate luggage with the child/youth during or shortly after a change in placement, with the child's/youth's personal belongings stored and maintained in the same or better condition.
- F. The facility is also responsible for creating a written policy should any belongings be left behind at the time of discharge by the child/youth, their parent(s) or guardian(s) or custodian(s), or the county department of human or social services working with the child/youth. The policy or acknowledgement must include the length of time the personal belongings are held onto or stored and how many attempts or reminders the facility will conduct before property is disposed of. Notice must be provided to the child/youth, their parent(s) or guardian(s) and custodian(s), and the county department of human or social services working with the child/youth at least forty-eight (48) working hours before any personal property is disposed of. All attempts, reminders, and notices must be documented in writing and provided to the Department upon request. This policy must be provided at the time of intake to the child/youth, the parent(s) or guardian(s) and custodian(s), and/or placing authority with signatures and dates from all involved parties acknowledging the policy.
- G. Facilities must create and adhere to a policy that guides and governs the maintenance of child/youth records to ensure they are stored and maintained in accordance with all applicable federal and state confidentiality laws, including protections for cybersecurity for those records which are stored electronically.

7.705.64 Personnel Policies, Orientation, and Training

- A. Personnel Policies
 - 1. The facility must have a written statement of personnel policies which include:

- a. Job descriptions for all positions required. The descriptions must describe duties of the job, qualifications for performance, and supervision to be provided as well as any training requirements of the position;
 - b. Salary range and provision for incremental increases;
 - c. Hours of work, holidays, paid time off, and any other types of leaves;
 - d. Conditions of employment, tenure, and promotion;
 - e. Employment benefits, including retirement plan, social security, medical benefits, hospitalization, and other insurances;
 - f. Employee performance evaluation procedures;
 - g. Grievance procedures which may be used by staff; and
 - h. Termination of employment procedures.
 - 2. A copy of the personnel policies must be given to each staff member at the start of their employment, and written verification of receipt of the personnel policies must be maintained in each staff file. Within ten (10) calendar days of a change to a personnel policy, all staff members must be provided a copy of the changed policy and written verification of receipt maintained in each staff file.
- B. Orientation and Training
- 1. The facility must have a comprehensive written policy for the hiring, retention, orientation, initial and ongoing training, and professional development of staff.
 - a. The facility must provide an introductory training and orientation program for all facility staff. This program must include orientation to emergency and safety procedures, professional boundaries and the general and specific duties and responsibilities of the job. The introductory training and orientation training must be completed within thirty (30) days of hire and before working alone with children and youth in care. The orientation training must include the facility's policy on required boundaries between staff members and the children/youth cared for or discharged from the facility.
- C. Qualified residential treatment programs must develop and adhere to policies and procedures for supervision of clinical professionals that address the requirements outlined in Section 7.705.206.B.

- D. Qualified residential treatment programs must have a written policy on how the program staff will be trained to facilitate family engagement and participation in the child's/youth's treatment and follow requirements outlined in Section 7.705.207.A.
- E. All training must include requirements outlined in sections 7.714.92 and 7.714.93.

7.705.65 Medical and Health Services Policies

A. Medication

- 1. The facility must have policies and procedures for documenting that required medication information was given to the child/youth before the child/youth took the medication. When information is given to the child/youth, the documentation must include an assessment regarding whether the child/youth understood. The policy must be consistent with the child/youth rights enumerated in Sections 19-7-101(2)(g)(III-V), C.R.S. for children/youth in foster care or participating in the foster youth in transition program.
- 2. The facility must have a written policy that addresses the process for when the count of a controlled substance is found to be inconsistent with the documentation. The policies must include the notification of administrative staff, and, if necessary, the prescribing provider and local law enforcement of the discrepancy and the actions taken.

B. Medical and Health Services

- 1. The facility must have and follow policies and procedures for informing the child/youth, parent(s) and/or guardian(s) of any follow up dental procedures as prescribed.
- 2. The facility must have a medical policy including staff training, which includes the screening of the child/youth for child abuse and signs of illness requiring immediate medical attention.

7.705.66 Safety Policies

A. Supervision of Children/Youth

- 1. The residential child care facility must have a written policy for the standard supervision of children/youth in the program based on the identified population described in the admission policies.

All policies must be consistent with applicable federal and state laws, including the child's/youth's rights and requirements outlined in Section

19-7-101, C.R.S. if the youth is in foster care or participating in the foster youth in transition program.

2. The facility must adhere to the policy, and it must include supervision of children/youth while at the facility, during transportation, and while off the premises.
3. The policy must be approved by the Department on an annual basis.
4. When a child's/youth's individual needs require deviation from the supervision policy, it must be documented in the individual child's/youth's plan.
5. As outlined in 19-7-101(2)(D)(II), C.R.S., children/youth in foster care or participating in the foster youth in transition program have a right to be free from unreasonable searches, limitation of use, or taking of personal belongings or other unreasonable invasions of privacy.

B. Recreation and Leisure

1. A residential child care facility must have a written policy for ensuring that a range of indoor and outdoor recreational and leisure opportunities are provided for children/youth in care. Such opportunities must be based on the individual interests of the children/youth with personal and treatment needs being considered.

All policies must be consistent with applicable federal and state laws, including the child's/youth's rights and requirements outlined in Section 19-7-101, C.R.S. if the youth is in foster care or participating in the foster youth in transition program.

C. Transportation

1. The facility must have a written policy concerning under what circumstances a vehicle may be driven by a licensed resident alone or with one resident passenger. Such driving privileges, if granted, must be a part of the child's/youth's individual plan.

D. Contraband

1. All facilities must have a written policy and procedures for conducting searches and control of contraband regarding children/youth, staff members, or visitors.
 - a. Searches involving children/youth must be completed upon admission to the facility, upon return from an unsupervised setting, when there is reason to believe that there is a safety risk to the facility, or when contraband may be present.

- b. Searches must be conducted in the least intrusive manner possible for the reason the search was being conducted.
- c. All contraband must be disposed of in accordance with the facility's written policy and procedures.
- d. All searches must be documented, including the reason for the search, the person(s) conducting the search, and the results.
- e. The policies and procedures must include and be in alignment with Section 7.714.51.A.

E. Feedback and Discipline

- 1. The facility must have written policies and procedures regarding preventative, supportive, and corrective feedback that must be explained in a manner appropriate for children/youth and provided to all parent(s), guardian(s), staff, and placing entities. Preventative and supportive feedback is designed to reinforce positive behavior, and coach the child/youth to develop alternative behaviors to those that interfere with positive treatment outcomes.

All policies must be consistent with applicable federal and state laws, including the child's/youth's rights and requirements outlined in Section 19-7-101, C.R.S. if the youth is in foster care or participating in the foster youth in transition program.

7.705.67 Written Policies for Physical Restraint

- A. Each facility utilizing physical restraint is to have a written policy and practices consistent with these rules.

All policies must be consistent with applicable federal and state laws, including the child's/youth's rights and requirements outlined in Section 19-7-101, C.R.S. if the youth is in foster care or participating in the foster youth in transition program.

The facility's written policies must include, at a minimum, the following information:

- 1. Documentation of the nationally recognized, research-based type of de-escalation, physical restraint model to be used in the facility. The physical restraint model must be in accordance with the requirements provided in Sections 7.714.53 through 7.714.536.
- 2. Documentation of the actual physical restraint that occurred in the physical restraint situation.

3. Documentation of which staff members by name and position will be approved by the facility to use physical restraint with children/youth in care.
4. Documentation of the training/certification staff members approved to use physical restraint and the type and number of initial hours of training each staff member is required to take as required by the model and trauma-informed prior to restraining a child/youth.
5. The type and number of hours of ongoing training each staff member will be required to take and how often they will need to become recertified, not to exceed more than every six (6) months.
6. Documentation of the individualized preventive and de-escalation techniques, trauma-informed practices or interventions considered or utilized, and positive behavioral intervention must be used by staff prior to utilizing physical restraint.
7. How the facility continuously monitors physical restraint, how the facility will be physically present, such that the staff member is able to immediately respond to the needs of the child/youth in the physical restraint.
8. The type of written documentation the facility maintains of each physical restraint that describes the details of the incident and the staff involvement.
9. The type of written documentation the facility maintains that describes the debriefing with the child/youth and staff following the physical restraint.
10. Evaluation by an objective, internal professional of the documentation of each physical restraint to determine appropriateness and effectiveness of preventive and de-escalation strategies or interventions used and effectiveness and appropriateness of the physical restraint itself. Such evaluation must also include:
 - a. Whether there was a true imminent threat and if the situation was a true emergency;
 - b. Whether the physical restraint was more dangerous than the behavior it was attempting to contain;
 - c. The documentation of avoidance of the use of movement or transport during physical restraint except to move from extreme danger (i.e., a child/youth in the middle of a busy street or road); and
 - d. If the use was justified according to Section 26-20-103, C.R.S.

11. The requirement that staff must not utilize physical restraint in any setting outside of the licensed facility. Staff must call law enforcement if there are any circumstances outside of the licensed facility that may pose a threat to the health and safety of a child/youth.
12. The requirement that staff not restrain children/youth away from the facility to include but not limited to: other businesses, public areas where people in the community are present, or private property.
13. Notification to the parent(s) and/or legal guardian(s) and child/youth in advance of the facility's physical restraint policy and methodology and following the use of the physical restraint.
14. How the facility monitors the physical well-being of the child/youth during and after the physical restraint, including but not limited to breathing, pulse, color, and signs of choking or respiratory distress.
15. Emergency medical procedures, including First Aid, that will be used if a child/youth or staff member is seriously injured during a physical restraint.
16. The internal review process of the facility to assess carefully any injuries, bruising, or death.
17. The requirement of staff to report to the county department of human/social services or local law enforcement any injury, bruising, or death that occurs as a result of the physical restraint pursuant to Colorado state law.
18. The requirement of staff to report any critical incident or child abuse and/or neglect pursuant to Colorado state law and Section 7.701.52 and Section 7.701.53.
19. A written policy around debrief of the physical restraint that is specific to the population served and included in the trauma-informed care plan. The debrief will be done in accordance with the individual needs of the child/youth and will be documented in the physical restraint report.

7.705.68 Written Policies for Seclusion

- A. A psychiatric residential treatment facility (PRTF) which is approved to use seclusion must have a written policy and practices consistent with these rules.

All policies must be consistent with applicable federal and state laws, including the child/youth rights and requirements outlined in Section 19-7-101, C.R.S. if the youth is in foster care or participating in the foster youth in transition program.

The program's written policies must include at a minimum the following:

1. A purpose and use of the room, the intake process, the evaluation of child/youth while in the room, emergency procedure while a child/youth is in seclusion and method for a child's/youth's grievance regarding the use of the room.
2. Documentation of the type of behavior management system utilized by the program.
3. Which staff members will be approved by the facility to use seclusion with children/youth in care.
4. The type of training/certification staff members approved to use seclusion will be required to have prior to secluding a child/youth.
5. The type and number of hours of ongoing training each staff member will be required to take and how often they will need to become recertified, not to exceed more than every six (6) months.
6. What preventive/de-escalation techniques and positive behavioral intervention must be used by staff prior to utilizing seclusion.
7. The release criterion (e.g. a stated period of calm behavior) that is of short duration and that is client-driven or initiated.
8. How the facility observes and evaluates the use of seclusion on a child/youth.
9. The type of written documentation the facility maintains of each seclusion that describes the details of the incident and the staff involvement.
10. The type of written documentation the facility maintains that describes the debriefing with the child/youth and staff following the seclusion.
11. Evaluation by an objective, internal professional of the documentation of each seclusion to determine appropriateness and effectiveness of preventive and de-escalation strategies or interventions used and effectiveness and appropriateness of the seclusion itself. Such evaluation must also include:
 - a. Whether there was a true imminent threat and if the situation was a true emergency;
 - b. Whether the seclusion was more dangerous than the behavior it was attempting to contain;
 - c. If the use was justified according to Section 26-20-103, C.R.S.

12. Notification to the parent(s) and/or legal guardian(s) and child/youth in advance of the facility's seclusion policy and methodology and following the use of the seclusion.
13. How the facility monitors the physical well-being of the child/youth during and after the seclusion.
14. Emergency procedures, including First Aid, that will be used if a child/youth or staff member is seriously injured during seclusion.
15. The internal review process of the facility to assess carefully any injuries, bruising, or death.
16. The requirement of staff to report to the county department of social services or local law enforcement any injury, bruising, or death that occurs as a result of the seclusion pursuant to Colorado state law.
17. The requirement of staff to report any critical incident in accordance with Section 7.701.53.
18. The method for the child's/youth's right to grieve the use of the seclusion room.
19. A written policy around debrief of the seclusion that is specific to the population served and included in the trauma-informed care plan. The debrief will be done in accordance with the individual needs of the child/youth and will be documented in the physical restraint report.

7.705.7 SPECIAL RULES AND REGULATIONS FOR SHELTER CARE SERVICES

7.705.71 Requirements of Shelter Care Services for Placements

A. Applicability of Rules

1. Any residential child care facility may provide shelter care services for children/youth in need of short-term placement resulting from such circumstances as child abuse and neglect.
2. The acceptance of children/youth for shelter care services must be stated in the written purpose of the facility and the admission policy. The facility must have a prescribed number of beds relative to each population it serves:
 - a. Residential child care services;
 - b. Homeless youth; and/or

- c. Shelter care services.
 - 3. Residential child care facilities must make every effort to assure that shelter care service placements do not exceed sixty (60) days. Exceptional circumstances must be documented in the case file. No child/youth may remain in a shelter care service program for longer than ninety (90) days. Review of placement will be required every thirty (30) days.
 - 4. These rules are specifically for shelter care services. The following rules are in addition to corresponding regulations in Sections 7.705 through 7.705.6. However, if there is a conflict between those rules and these shelter care service rules, these rules shall apply.
- B. Admission to Shelter Care Service Programs**
- 1. At the time of shelter care service placement of a child/youth, the facility must obtain at least the following information: name, birth date, if available, physical description of the child/youth, date and time of the admission, name and authority of person bringing child/youth to the facility, and reason for placement. On the next working day following shelter care service placement of the child/youth, the facility shall obtain from the county department of social or human services or the parent(s) or guardian(s) a signed, written authorization to obtain medical care for the child. The county department is authorized to give this authorization on the next working day following placement.
 - a. A child/youth who has reached the age of fifteen (15) or older may consent to shelter and services for a period not to exceed twenty-one (21) days when the following criteria are met:
 - 1. The child/youth shall voluntarily and knowingly sign a consent form that includes:
 - a. All services the child/youth may receive during their stay at the shelter and the total number of days the child/youth may consent to remain in the shelter;
 - b. Legal limits to confidentiality;
 - c. How and when the parent(s), legal guardian(s), and/or child welfare agencies will be notified of the child's/youth's stay;
 - d. An explanation that services provided by the shelter are provided free of charge to the child/youth without exchange of any goods, services, or sexual acts.

2. The consent form shall be explained in the child's/youth's preferred language or mode of communication, including augmented or facilitated communication, if necessary, and shall take into consideration the child's age, ability level, and cultural and religious background.
 3. The consent form shall be signed within twelve (12) hours of admission.
2. The facility must ensure that each newly admitted child/youth is checked at the time of admission by facility staff or a physician for signs of illness, signs of abuse or neglect, and the presence of parasitic infections.
 - a. The facility must have a medical policy, including staff training, which includes the screening of the child/youth for child abuse and signs of illness requiring immediate medical attention.
3. If a child/youth remains in a shelter care service program for longer than fourteen (14) days, a medical examination must be completed in compliance with Section 7.714.82.B.

7.705.72 Homeless Youth Shelter Care

A. Applicability of Rules

1. Any residential child care facility may provide shelter to homeless children/youth ages eighteen (18) and under for up to twenty-one (21) consecutive days. A residential child care facility may provide shelter to a homeless child/youth between the ages of eighteen (18) and twenty-one (21), if such services will not negatively impact the health, safety, and welfare of the other children/youth in care. The facility shall obtain approval from the department prior to accepting a homeless child/youth between the ages of eighteen (18) and twenty-one (21).
2. The acceptance of homeless children/youth must be stated in the written purpose of the facility and the admission policy.
3. These rules are specifically for shelter care of homeless children/youth. The following rules are in addition to corresponding regulations in Sections 7.705 through 7.705.6. However, if there is a conflict between those rules and these shelter care rules, these rules shall apply.

B. Notifications and Referrals

1. Pursuant to Section 26-5.7-105(4), C.R.S., when a youth under fifteen (15) years of age is admitted to a licensed child care facility or licensed homeless youth shelter, the director of the facility, shelter, or other person

in charge shall notify the county department within seventy-two (72) hours of the youth's admission.

2. Pursuant to Section 26-5.7-106(2)(a), C.R.S., upon admission of a youth to a licensed child care facility or licensed homeless youth shelter, the facility or shelter shall notify the youth's parent, legal guardian, or appropriate county department of the youth's placement within twenty-four (24) hours. The person making the report must also report on the physical and emotional condition of the youth, the youth's whereabouts, and the circumstances surrounding the youth's placement within twenty-four (24) hours.
3. Pursuant to Section 26-5.7-105(7), C.R.S., if a youth who is at least eleven (11) years of age but less than fifteen (15) years of age has been served up to twenty-one (21) days and returns to the licensed child care facility or licensed homeless youth shelter after leaving the facility or shelter, the director of the licensed child care facility or licensed homeless youth shelter or other person in charge must make a referral for services to the county department. The director or other person in charge should interview the child/youth and report the referral to the child's/youth's county of residence.
4. Pursuant to Section 26-5.7-105(5), C.R.S., if the director of the facility, shelter, or other person in charge determines that a referral for additional services needs to be made, the director or other person in charge shall make the referral to the appropriate county department, notify the county department of the facility's relationship to the youth pursuant to Section 19-1-307-(2)(e.5)(i), C.R.S. and notify the county department of the date when the twenty-one (21) day shelter time period will expire.
5. For a child/youth under the age of eighteen (18), if reconciliation with the child's/youth's family has not occurred within seventy-two (72) hours following admission to the shelter, and the director of the shelter or other person in charge does not anticipate that reconciliation will be achieved within twenty-one (21) consecutive days, the director or other person in charge must provide the child/youth and the child's/youth's parent with a written statement identifying:
 - a. The availability of counseling services;
 - b. The availability of longer-term residential arrangements; and,
 - c. The possibility of referral to the county department.
6. If the facility staff know the child/youth is away from home without permission, the facility must notify the child's/youth's parent(s)/legal guardian(s) or law enforcement within twenty-four (24) hours of providing shelter if the child/youth refuses to provide the shelter with contact

information for the child's/youth's parent(s) or legal guardian(s), the child's/youth's parent(s) or legal guardian(s) is deceased, or the shelter director or other person in charge believes that notifying the parent(s) or legal guardian(s) would not be in the child's/youth's best interest due to an imminent risk of abuse or neglect by the parent(s) or legal guardian(s), the shelter shall notify the appropriate county department. The notification must include the child's/youth's whereabouts, physical and emotional condition, and the circumstances surrounding the child's/youth's placement.

7. Pursuant to Section 26-5.7-106, C.R.S. the facility or licensed homeless youth shelter shall notify the youth's parent(s) or legal guardian(s) that it is the paramount concern of the facility or shelter to achieve a reconciliation between the parent or legal guardian and the youth, to reunify the family, and to inform the parent(s) or legal guardian(s) about the available alternatives.
 - a. The facility or shelter shall arrange transportation for the youth to the residence of the youth's parent(s) or legal guardian(s) when the youth and parent(s) or legal guardian(s) agree that the youth shall return to the home of the youth's parent(s) or legal guardian(s). The parent(s) or legal guardian(s) shall reimburse the party who paid for the transportation costs to the extent of the parent's or legal guardian's ability.
 - b. The facility or shelter shall arrange transportation for the youth to an alternative residential placement facility when the youth and the youth's parent(s) or legal guardian(s) agree to such placement. the parent(s) or legal guardian(s) shall reimburse the appropriate person for transportation costs to the extent of the parent's or legal guardian's ability.

7.705.73 Education and Treatment

- A. The facility shall ensure that each child/youth is provided with an educational program in accordance with federal, state, and local laws. Any educational program provided by a facility providing shelter care services shall comply with all applicable state and local laws.
- B. Daily routine schedules must be established by the facility to ensure a stable, orderly, and supportive environment for children/youth in care.

7.705.74 Case Plan and Permanency Plan

- A. If a child/youth is receiving shelter care services, the rules in this section supersede Section 7.714.4.

- B. A case plan must be developed for each child/youth who stays in shelter care services longer than two (2) weeks. The case plan must be started immediately upon admission.
 - 1. The plan will include, at least, goals regarding personal and group living skills, family contact and involvement, how family contact and involvement are to occur, and the nature and goals of care, including any specialized services or specialized treatment to be provided.
 - 2. The child/youth must be involved in the establishment of the goals.
 - 3. The comprehensive assessment, as found at Section 7.714.4.A, must serve as the basis for the plan.
- C. A case plan must include the development of a permanency plan within thirty (30) days of admission for any child/youth receiving shelter care services.
- D. A permanency plan must address options for the child/youth to discharge to a safe and stable environment, the availability of counseling and other community services that may support the permanency plan, and the anticipated discharge date and plan for the child/youth following discharge.

7.705.75 Personnel

- A. A residential child care facility that provides shelter care services must maintain the staff ratio described at Section 7.705.46.D.
- B. A residential child care facility that provides shelter care services must have awake staff during the children's/youth's sleeping hours.
- C. A residential child care facility that provides shelter care services must make adequate provisions to ensure that additional staff can be physically present at the facility within sixty (60) minutes in the event of an emergency or crisis.
- D. Training programs for staff in shelter care service programs must reflect special emphasis on the following areas:
 - 1. Desescalation techniques.
 - 2. Trauma-informed care.
 - 3. Crisis and Emergency Procedures.

7.705.76 Records

- A. A record for a child/youth accepted for care in a shelter care services program must include:
 - 1. Information obtained at admission pursuant to Section 7.705.71.B;

2. Statement from the physician about the child's/youth's physical examination pursuant to Section 7.705.71.B.3;
3. Comprehensive assessment and permanency plan;
4. Record of medication given to the child/youth while in care;
5. Record of services provided while the child/youth was in care; and
6. Date of discharge, the name to whom the child/youth was discharged and a copy of their identification, their relationship to the child/youth, and physical description of the child/youth at time of discharge.

7.705.100 REQUIREMENTS OF A PSYCHIATRIC RESIDENTIAL TREATMENT FACILITY (PRTF)

Psychiatric Residential Treatment Facility (PRTF) services are a Medicaid benefit statewide when provided in accordance with the provisions of the following sections.

7.705.101 Applicability of Rules

- A. "Psychiatric residential treatment facility (PRTF)" pursuant to Section 25.5-4-103(19.5), C.R.S., means a facility that is licensed as a residential child care facility, as defined in Section 26-6-903, C.R.S., that is not a hospital, and that provides inpatient psychiatric services for individuals who are less than twenty-one (21) years of age under the direction of a physician licensed pursuant to article 240 of title 12, and that meets any other requirement established by the State Board.
- B. These rules are specifically for psychiatric residential treatment facilities. A psychiatric residential treatment facility must maintain a license as a residential child care facility and comply with the Rules Regulating Residential Child Care Facilities in Sections 7.705 through 7.705.63, the General Rules for Child Care Facilities in Section 7.701, the Quality Standards for Twenty-Four (24) Hour Care for Children and Youth in Section 7.714, and the Rules Regulating Special Activities in Section 7.719. However, if there is a conflict between those rules and these psychiatric residential treatment facility rules, these rules shall apply.
- C. A PRTF shall comply with all rules promulgated by the Medical Services Board in the Colorado Department of Health Care Policy and Financing for clients of Psychiatric Residential Treatment Facilities (10 CCR 2505-10 Sections 8.765.1 through 8.765.12 and Section 8.765.2) (2024), which are hereby incorporated by reference. No later editions or amendments are incorporated. These regulations are available for public inspection and copying at the Colorado Department of Human Services 1575 Sherman Street, Denver, Colorado 80203 during regular business hours. These statutes and regulations are also available at no cost through the state's website at

<https://www.sos.state.co.us/ccr/generaterulepdf.do?ruleversionid=6329&filename=10%20ccr%202505-10%208.000>.

- D. A PRTF shall comply with all Conditions of Participation required by the Centers for Medicaid and Medicare Services (CMS) and regulated by the Colorado Department of Public Health and Environment (CDPHE). A PRTF shall comply with 42 C.F.R. Section 483.350 through 376 Subpart G (2025) for the Use of Restraint or Seclusion in Psychiatric Residential Treatment Facilities Providing Inpatient Psychiatric Services for Individuals Under Age Twenty-One (21), and 42 C.F.R. Section 441.151-184 (2025), General Requirements for Psychiatric Residential Treatment Facilities, which are hereby incorporated by reference. No later editions or amendments are incorporated. These statutes and regulations are available for public inspection and copying at the Colorado Department of Human Services 1575 Sherman Street, Denver, Colorado 80203 during regular business hours. These statutes and regulations are also available at no cost through the CMS website at <https://www.ecfr.gov/current/title-42/chapter-iv/subchapter-g/part-483/subpart-g>. The facility shall be designated by the Department to provide such services using the state-prescribed designation methods.

7.705.102 ACCREDITATION

The facility shall be accredited by one of the following: Joint Commission on Accreditation of Healthcare Organizations (JCAHO), the Commission on Accreditation of Rehabilitation Facilities (CARF), or by any other accrediting organization with comparable standards that is recognized by the state and approved by the U.S. Department of Health and Human Services. A copy of the facility's national accreditation certification and report shall be maintained in the facility file and made available to the Department upon request.

7.705.200 REQUIREMENTS OF A QUALIFIED RESIDENTIAL TREATMENT PROGRAM (QRTP)

7.705.201 Applicability of Rules

- A. A qualified residential treatment program (QRTP) means a facility that is licensed as a residential child care facility, as defined in Section 26-6-903(29), C.R.S. All QRTPs shall also follow all rules as outlined in 10 CCR 2505-10 Section 8.765.8 through Section 8.765.14 (2024), as incorporated by reference in Section 7.705.101.C.
- B. The program shall maintain a license as a residential child care facility (RCCF) and be in compliance with the regulations in Sections 7.701, 7.705, 7.714, and 7.719.

7.705.202 Accreditation and Attestation

- A. The program shall be accredited by one of the following organizations:

1. The Commission on Accreditation of Rehabilitation Facilities (CARF).
 2. The Joint Commission on Accreditation of Healthcare Organizations (JCAHO).
 3. Any other accrediting organization with comparable standards that is recognized by the state and approved by the U.S. Department of Health and Human Services.
 4. A copy of the facility's national accreditation certification and report must be maintained in the facility file and made available to the Department upon request.
- B. All QRTPs in the state of Colorado that plan to enroll with Health First Colorado are required to submit a signed attestation form with their Health First Colorado enrollment application that follows attestation guidelines outlined in 10 CCR 2505-10 Section 8.765.14.B (2024), as incorporated by reference above in Section 7.705.101.C.

7.705.203 Quality Improvement Plan

- A. The program must develop and implement a quality improvement (QI) plan that is designed to monitor and improve safety, quality of care, trauma-informed practices, and treatment outcomes and meets the following criteria:
1. Utilizes a data-driven approach to identify patterns and trends in the data;
 2. Analyzes the data to determine what improvements and changes must be implemented; and
 3. Is reviewed and/or modified no less than quarterly based on outcomes data.
- B. The QI plan must be submitted to the Division of Child Welfare (DCW), Provider Services Unit when requesting approval for the trauma-informed treatment model.
- C. The QI plan and all documented quarterly reviews must be submitted to the department for review on an annual basis from the date the initial plan was approved, and the QI plan and all subsequent quarterly reviews must be made available to the Department upon request.

7.705.204 Nursing Professionals

- A. The QRTP must have a nursing professional who is at least a registered or licensed nurse pursuant to Section 12-255-101 through 12-255-135, C.R.S. who:
1. Provides care within the scope of their practice as defined by state law;

2. Is on-site according to the trauma-informed treatment model, as approved by the Division of Child Welfare (DCW), Provider Services Unit; and
 3. Is available on site, via phone, or via video call twenty-four (24) hours a day, seven (7) days a week.
- B. The QRTP may contract for nursing services. These contracted services and individuals must adhere to the rules in Section 7.701.400.B. Contracted individuals are considered staff of the facility during the hours stated in the contract. Contracts must list the duties of the contracted individual with whom the program is contracting and must list the scope of services to be provided. For nursing services, there must be duties outlined in the contract that assure the services are being administered on site, via phone, or via video call twenty-four (24) hours per day, seven (7) days per week. Any contracts for nursing services must be made available to the Department upon request.
- C. The QRTP may utilize licensed physicians, licensed physician assistants, or licensed nurse practitioners in lieu of licensed nursing staff, in accordance with the trauma-informed treatment model as approved by the Division of Child Welfare (DCW), Provider Services Unit.
- D. Professional nursing staff files are the property of the facility and must remain on site and be made available to the Department upon request and remain in compliance with all federal and state confidentiality statutes and regulations.

7.705.205 Clinical/Mental Health Professionals

- A. The QRTP must have clinical staff who provide mental health services. The QRTP must have at least one (1) of the following licensed mental health professionals providing clinical services:
1. A licensed psychologist pursuant to Section 12-245-301, C.R.S. through Section 12-245-309, C.R.S.;
 2. A licensed clinical social worker (LCSW) pursuant to Section 12-245-401, C.R.S. through Section 12-245-410, C.R.S.;
 3. A licensed marriage and family therapist (LMFT) pursuant to Section 12-245-501, C.R.S. through Section 12-245-506, C.R.S.;
 4. A licensed professional counselor (LPC) pursuant to Section 12-245-601, C.R.S. through Section 12-245-607, C.R.S.;
 5. A licensed addiction counselor (LAC) pursuant to Section 12-245-801, C.R.S. through Section 12-245-806, C.R.S.
- B. The QRTP may utilize provisionally licensed mental health professionals pursuant to Section 12-245-208, C.R.S. Any provisionally licensed mental health

professionals must be supervised by a licensed mental health professional who is associated with the QRTP. If the QRTP chooses to utilize provisionally licensed mental health professionals, they must also employ/contract with a licensed mental health professional who provides clinical supervision to the provisionally licensed individual and must adhere to clinical supervision in Section 7.705.206 of this section.

- C. The QRTP may contract with licensed clinical professionals or provisionally licensed clinical professionals for the required mental health services. These contracted services and individuals must adhere to the rules of Section 7.701.400.B. Contracted individuals are considered staff of the facility during the hours stated in the contract. Contracts must list the duties of the contracted individual with whom the program is contracting and must list the scope of services to be provided. For clinical services being provided by contracted staff, the contract must outline the manner in which the clinical services are being administered. There must be assurance of the scheduled hours face-to-face on-site as well as being available via phone or video call. There must be assurance of their services being available twenty-four (24) hours a day, seven (7) days a week. Contracted professional staff must be involved in the development and review of the child's/youth's individual plan and comply with all requirements outlined in Section 7.714.4.B any contracts for clinical professionals must be made available to the department upon request.
- D. The licensed mental health professional and any provisionally licensed mental health professional must be at least a part-time employee and/or contractor of the QRTP and be responsible for the coordination and oversight of the mental health services provided in the QRTP.
- E. Clinical/mental health professional files are the property of the facility and must remain on site and be made available to the department upon request and remain in compliance with all federal and state confidentiality statutes and regulations.
- F. Clinical/mental health notes and any documentation of mental health services provided to youth residing in the QRTP are the property of the QRTP and must remain on site and be made available to the Department upon request and remain in compliance with all confidentiality statutes and regulations.

7.705.206 Administrative and Clinical Supervision of Clinical Professionals

- A. The QRTP must ensure that all licensed and provisionally licensed clinical personnel providing mental health services receive supervision as defined in this section.
- B. The QRTP will develop and adhere to policies and procedures for supervision of clinical professionals that address the following:

1. Develop and maintain the clinical professional's mandatory disclosure statement that is required as part of the intake process for children/youth admitted to a QRTP;
2. Define who is providing supervision to the clinical professionals employed by the QRTP;
3. Develop and define the requirements for regular evaluation of the clinical professional's performance with regard to clinical services provided;
4. Documentation and frequency of supervisor reviews and feedback provided;
5. For provisionally licensed professionals, the fully licensed clinical professional must provide clinical supervision at a minimum, of one (1) time every week, and, at a minimum;
 - a. Meet the standard qualifications for clinical supervision as defined by their professional practice board;
 - b. Deliver clinical supervision within the supervisor's professional practice license and ethical standards;
 - c. Dedicate time between the licensed professional and the provisionally licensed professional to instruct, model, and encourage self-reflection by the personnel receiving supervision regarding acquisition of clinical and administrative skills. The licensed clinical supervisor will determine skills through observation, evaluation, feedback, and mutual problem solving;
 - d. Address ethics and ethical dilemmas as aligned with the appropriate professional practice board;
 - e. Provide professional direction based on experience, expertise, and/or for ethical or safety concerns;
 - f. Ensure that safety and crisis management plans are followed and that clinical supervisors are available to personnel for assistance in crisis situations and processing the crisis event afterwards;
 - g. Document date, duration, and the content of supervision session for their supervisees, which may include a professional development plan. All documents pertaining to clinical supervision in a QRTP must be provided to the supervisee and the Department upon request.

7.705.207 Family Engagement

- A. The program must have a written plan on how the program staff will facilitate family engagement and participation in the child's/youth's treatment.
 - 1. All staff who participate in the child/youth plan must be trained in family engagement strategies at orientation and annually.
 - 2. The program must have a written plan that provides services that are aligned with the family's schedule. The provider must make reasonable efforts to facilitate the family's participation in treatment.
 - 3. If the family or other primary support system is clinically or legally contraindicated, then this shall be documented in the child's/youth's file.
- B. The program must have a written plan that facilitates approved contact between the child's/youth's family of origin or primary support system. The plan shall be designed to provide support to the family to ensure clinically indicated frequency of contact. This plan shall be individualized for each child/youth as determined by the family engagement and permanency team or the court.
 - 1. All efforts to arrange contact, including visits, telephone calls, or video conferences must be documented, including any reasons contact was not able to be completed.
 - 2. Contact with family or primary support systems should, in alignment with the approved plan, be used as a primary intervention when the child/youth requests contact.
- C. The program must document efforts to maintain sibling connections unless clinically contraindicated or ordered by the court. Such orders of communication must be contained within the child's/youth's file and added to the individual child's/youth's plan.

7.705.208 Placement Requirements

- A. Independent Assessment (IA)
 - 1. A child/youth referred to a QRTP by a county department or the Division of Youth Services shall have an independent assessment (IA) completed.
 - 2. The state-approved assessment shall be administered by the provider every ninety (90) days beginning with the date of placement and within fourteen (14) days of discharge to demonstrate the need for continued QRTP services.
- B. The facility must request a copy of the initial state-approved assessment upon admission using the state-prescribed form and submission to the BHA.

- C. The results of the initial state-approved assessment must be utilized, if made available, in the development of the individual child's/youth's plan.
- D. The results of the ninety (90) day reviews must be shared within five (5) days of completion with the placement authority and department through the Department-approved method.
- E. All staff completing the state-approved assessment must complete the state-approved training and pass the state-approved certification test before administering the tool. All staff completing the assessment must be recertified annually. Documentation of the training must be kept in the staff's file.

7.705.209 Clinical Record Requirements

- A. In addition to Section 7.701.63 and Section 7.701.64, a QRTP designated by the Department to provide mental health services must maintain current, organized, legible, chronological, clinical records for each child/youth. Treatment records must include:
 - 1. A mental health diagnosis;
 - 2. Copies of any assessments pertaining to the child's/youth's current need for mental health services;
 - 3. All plans of care and revisions for the current treatment episode;
 - 4. Documentation of the child's/youth's attendance at, participation in, and outcomes of QRTP mental health services;
 - 5. Documentation that the child/youth and/or the legal guardian was provided a copy of the plan of care;
 - 6. Correspondence to and from agencies and individuals involved in the child's/youth's treatment;
 - 7. An explanation whenever any member of the multidisciplinary team, child/youth, parent(s), or guardian(s), does not sign the plan of care;
 - 8. The name of the licensed mental health professional responsible for the formulation, implementation, review, and revision of the child's/youth's plan of care;
 - 9. Signed releases of information when necessary;
 - 10. Documentation of any unplanned discharges without advance notice and any discharges against the agency's or licensed mental health professional's advice;

11. Treatment entries that are signed and dated by the person providing treatment/therapy, including title or position of the person providing treatment/therapy. If the clinical services are provided by a provisionally licensed therapist, the supervisor must also sign entries;
12. All changes in diagnoses need to be documented with changes to the independent assessment;
13. All members of the clinical staff must be trained annually in the development and review of plans of care, and there must be documentation of such;
14. Clinical supervisors who are providing supervision to provisionally licensed therapists (SWP, LPP, MFP, PSP) must document supervision hours provided to supervisees.

7.705.210 Comprehensive Assessment

Qualified residential treatment programs (QRTPs) must complete a comprehensive assessment on the child/youth admitted to the program in accordance with Section 7.714.4.A.

7.705.211 Individual Child's/Youth's Plan in a QRTP

- A. In addition to Section 7.714.4.B, the following is required for an individual child's/youth's plan in a residential child care facility designated by the Department to provide mental health services.
 1. The licensed mental health professional responsible for providing clinical care must develop the mental health component of the individual child's/youth's plan.
 2. The multidisciplinary team must develop and finalize a comprehensive individual child/youth plan, as covered in Section 7.714.4.B, within fourteen (14) calendar days after the determination that mental health services are required. The plan must be signed and dated or include written documentation of participation and review by the parties listed in Section 7.714.4.b.B. The plan must be signed and dated by the mental health professional. If a provisionally licensed mental health professional is providing the services, that person and their licensed supervisor must also sign the individual child's/youth's plan. The plan must:
 - a. Address all areas listed at Section 7.714.4.B, together with mental health services and other needs including the child's/youth's presenting symptoms, physical health, emotional status, behavior support needs, community supports, available resources, and discharge/aftercare plan;

- b. Include specific goals and measurable objectives, expected dates of achievement, specific discharge and transitional/aftercare and follow-up services, criteria to be met for termination of treatment, and specific treatment goals, including, but not limited to:
 - 1. Expected reduction in the symptoms that necessitated mental health services and the specific functional achievement and skills development to be obtained in measurable terms, and
 - 2. Indications that planned services can reasonably be expected to improve the child's/youth's condition;
 - c. Specify the type, frequency, and duration of mental health services, rehabilitation services, medication management, emergency services, initial assessment, documented treatment modifications, and other services determined to be necessary to meet the child's/youth's individualized mental health service goals;
 - d. Specify that all RCCF mental health services are necessary to meet the needs of the child/youth and that the mental health services are necessary to treat the child's/youth's current diagnosis;
 - e. Identify the provision of, or the referral for, services other than RCCF mental health services and must document any court-ordered treatment, including identifying the agency responsible for providing the court-ordered treatment.
- 3. There must be a review of each plan every thirty (30) days to evaluate progress and whether goals and objectives have been achieved and/or not achieved and barriers to treatment, if applicable. The individual child's/youth's plan and the discharge plan must be revised by the designated licensed, or provisionally licensed, professional person as appropriate.
- 4. There must be documentation of the child's/youth's response to treatment approaches and changes in the individual child's/youth's plan with the reasons for such changes.
- 5. In addition to Section 7.714.4.A and Section 7.714.4.B, QRTPs must include the following components for the individual child's/youth's plan:
 - a. The findings of the assessment/evaluation, including the strengths and needs of the child/youth to be addressed in the individual child's/youth's plan. The findings of the assessment must identify which needs of the child/youth must be resolved for the child/youth to move to a less-restrictive setting/successfully discharge from the facility;

- b. Goals must be individualized, relevant to the placement of the child/youth, and address any diagnosis given. Objectives must be specific, measurable, achievable, and relevant to the goals, individualized, and time-limited. the goals and objectives must support the child's/youth's progress toward discharge;
- c. A plan for fostering and maintaining positive family relationships with the child/youth and their family;
- d. Specification of the therapeutic services, specialized services, and strategies for positive behavior intervention that will be provided directly or arranged for, frequency of services, and method for ensuring their proper integration with the child's/youth's ongoing program activities;
- e. Discharge plan, including aftercare, medication management, and referrals to community services;
- f. Identification of all persons responsible for implementation of the plan;
- g. Signature or written documentation of participation and review by a representative of the facility, a representative of the placing agency, if applicable, the child/youth, and the child's/youth's parent(s) or guardian(s);
- h. Written documentation by the facility that the plan and any subsequent revisions are explained to the child/youth in care and their parent(s) or guardian(s) in a language or manner of communication the child/youth and parent(s) or guardian(s) can understand;
- i. A review of each individual child's/youth's plan must occur to evaluate the child's/youth's progress toward achieving the goals and objectives. The review must include which goals and objectives have been achieved and which are remaining in the individual child's/youth's plan.

7.705.212 Short-Term Placements

- A. In the event the anticipated length of stay for the child/youth is less than fourteen (14) calendar days, and a service to be provided includes mental health treatment of the child/youth, the facility must complete an initial assessment and an individual child's/youth's plan for short-term placements within seventy-two (72) hours of admitting the child/youth. For short-term placements, Section 7.714.4.A and Section 7.714.4.B are not applicable.

1. The multidisciplinary team must gather historical and current information to assess and evaluate the child/youth in at least the following areas:
 - a. Reason for referral;
 - b. Safety needs of the child/youth;
 - c. Presenting needs;
 - d. Medical health and allergies, including a health history of the child/youth and child's/youth's family;
 - e. Mental health, including treatment history and history of psychotropic medications, utilizing the state-approved assessment results if required and if able to be obtained;
 - f. Family and community relationships;
 - g. Treatment/placement history; and
 - h. Alcohol/substance use history.
- B. Upon admission, the facility and placing authority must create an individualized child's/youth's plan for short-term placements that addresses safety and risk considerations, triggers, early warning signs, and strategies to assist with emotional regulation or sensory modulation strategies.
- C. The stabilization plan must include at least one (1) goal to include objectives to support the goal. The findings of the independent assessment must be incorporated.
- D. The individual child's plan must address any safety issues identified in the assessment.
- E. In the event that the child's/youth's length of stay is determined to extend to more than fourteen (14) calendar days, all requirements of Section 7.714.4 must be completed prior to the thirtieth (30) day of stay. updates to the state-approved assessments must be completed according to the required timeframes for those facilities required to complete them.

7.705.213 Mental Health Services in a QRTP

- A. Services must be provided to all children/youth admitted to the facility, and documentation of services provided to the child/youth must be kept in the file and made available to the Department upon request.
- B. Services must be individualized to the needs of the child/youth and family, as determined by the individual child's/youth's plan.

- C. The assessment needs to assess for mental health services, and the information obtained from the assessment needs to be included in the individual child's/youth's plan, unless contraindicated for the child/youth as determined by the individual child's/youth's plan.
- D. Other mental health services may include group therapy or other services as determined by the individual child's/youth's plan specified by the comprehensive assessment.
- E. Services must include medication assessment and management, if appropriate and applicable.

7.705.214 Discharge and Aftercare

A. Discharge

- 1. Qualified residential treatment programs must follow Section 7.714.85 for discharge of children/youth from the facility.
- 2. Within five (5) business days of when a child/youth is removed from placement, the facility must complete and send to the placing agency, a summary of the child's/youth's discharge from the facility, to include all components listed in Section 7.714.85.I. For QRTPs, the summary must also include a statement of an aftercare plan and identification of who is responsible for follow-up services and aftercare.
- 3. Copies of a child's/youth's file, including discharge information but excluding all medical information covered by HIPAA, as incorporated in Section 7.701.14, or educational information covered by FERPA, as incorporated in Section 7.701.14, must be provided to parent(s) or guardian(s) upon request or to others by written consent pursuant to Section 7.701.61.
- 4. If the child/youth prematurely discharges from the facility, the facility is responsible to follow through with providing aftercare services in accordance with the child/youth individualized plan. In instances where the child/youth discharges due to elopement, hospitalization, or to receive higher levels of care, the facility is responsible to ensure the next provider receives appropriate information regarding discharge and aftercare for the child/youth within five (5) business days of when a child/youth is removed from placement.

B. Aftercare

- 1. Within forty-five (45) calendar days of admitting a child/youth into care, the program must develop a discharge and aftercare plan, in collaboration with the county department of human/social services or placing entity, the child/youth, and the family engagement and permanency team.

2. The aftercare plan must describe the services and resources needed by the subsequent care provider to meet the needs of the child/youth and how those services and resources will be provided.
3. The program must provide or make referrals for family-based aftercare support for at least six (6) months post-discharge as determined in the plan utilizing the same clinician, if possible, and documenting efforts if unable to coordinate.
4. The plan must be reviewed and/or modified by the licensed clinician or provisionally licensed clinician with review and signature by the supervising licensed clinician no less than monthly during the child's/youth's placement and during aftercare services. Changes to the plan must be documented in the child's/youth's individual file.
5. The plan must be incorporated into the discharge summary as outlined at Section 7.714.85.I.
6. The program must update the state-approved-assessment no more than fourteen (14) days prior to the planned discharge date of the child/youth as outlined in Section 7.714.85.
7. The program may revise the terms of aftercare support and services with the placing entity of any child/youth not placed by a county department of human/social services or the Division of Youth Services (DYS) and must provide after-care support in accordance with the contract established with the placing entity.
8. Programs may refer to other entities to provide aftercare services and must clearly document in the aftercare plan whom they contracted with, contact information, and the specific services to be provided.

7.705.300 REQUIREMENTS OF THERAPEUTIC WILDERNESS PROGRAM SERVICES

7.705.301 Applicability of Rules

A therapeutic wilderness program means a facility that is licensed as a residential child care facility, as defined in Section 26-6-903(29), C.R.S. These rules are specifically for therapeutic wilderness program services. The following rules are in addition to corresponding regulations in Sections 7.701, 7.705, 7.711, 7.714 and 7.719. However, if there is a conflict between those rules and regulations and these special rules and regulations, these rules and regulations shall apply.

7.705.302 Personnel

- A. There must be a chief administrator of a therapeutic wilderness program that meets the qualifications for an administrator of a residential child care facility as

- stated in Section 7.705.42. The chief administrator must have at least one (1) year of verified full time experience working with children/youth in a therapeutic wilderness program and three (3) years of verified experience in staff supervision and administration. The chief administrator may also function as the field director.
- B. When the administrator of a therapeutic wilderness program is not available, an assistant or acting director who meets the same requirements as the administrator must be available.
- C. There must be a field director who is primarily responsible for the coordination, supervision, and management of field operations, field staff, and the field office. The field director must be at least twenty-five (25) years of age and must have a minimum of thirty (30) semester hours in recreational therapy or related experience, or at least one (1) year (1,820 hours) of experience working with children/youth in a therapeutic wilderness program.
- D. There must be at least one (1) senior field staff member working directly with each group of children/youth. The senior field staff must be qualified as follows:
1. Be at least twenty-one (21) years of age;
 2. Be qualified as stated in Section 7.705.44.A;
 3. Have at least six (6) months full-time experience or training in a field related to recreation and adventure activities;
 4. Have a minimum of forty (40) twenty-four (24) hour field days of program experience or equivalent working in outdoor wilderness programs; and
 5. Have current Wilderness First Responder (WFR) First Aid and CPR certification.
- E. Child care field staff must:
1. Be at least twenty-one (21) years of age;
 2. Be qualified as stated in Section 7.705.44.A;
 3. Have completed staff training and field course work as required in Sections 7.705.311 and 7.714.92;
 4. Have completed, at a minimum, a standard First Aid and CPR course.
- F. Program consultants and other multi-disciplinary resources
1. A multi-disciplinary team that has knowledge of the physical and emotional demands of the program must be available for consultation and recommendations regarding the program participants.

2. At a minimum, the multi-disciplinary team must consist of:
 - a. A physician licensed to practice in Colorado; and
 - b. A licensed mental health professional licensed to practice in Colorado, including either a licensed psychologist, licensed clinical social worker, licensed marriage and family counselor, or licensed professional counselor.

7.705.303 Staff to Child/Youth Ratios

- A. At a minimum, the following staff to child/youth ratios must be maintained:
 1. There must be at least one (1) staff member for every four (4) children/youth in care at all times;
 2. There must be at least two (2) staff present per group of eight (8) children/youth at all times;
 3. Where the gender identity or biological sex of a group is mixed, there must be staff members who represent the gender identity and/or biological sex of the members of the group at all times; and
 4. A therapeutic wilderness program must ensure the immediate availability of additional staff from the field office to be physically present in the event of an emergency or crisis. In the event of an emergency, local emergency services must be called immediately.

7.705.304 Field Office Operations and Communications

- A. A therapeutic wilderness program must have a base camp and/or field office in Colorado, hereafter referred to as a “field office”. At a minimum, the field office must:
 1. Be staffed twenty-four (24) hours a day when there are children/youth in care;
 2. Maintain current staff personnel files;
 3. Maintain a current list of the names of staff and children/youth in each field group;
 4. Maintain a master map of all activity areas used by the program and GPS coordinates for each campsite;
 5. Maintain copies of each group’s expeditionary route including schedule and itinerary, which must be made available to the department, local law enforcement, and land use agencies upon request or as required;

6. Maintain logs of all communication with each field group when away from the field office.
- B. There must be a communication system between the field office and each group away from the field office that includes:
1. Additional, fully-charged battery packs for each group;
 2. Plan for re-establishing communication in the event regular communications fail;
 3. A global positioning system (GPS) with each group when away on all expeditions;
 4. Verbal communication between each group and the field office must occur at least two (2) times in each twenty-four (24) hour period, unless alternative arrangements have been made, and documented in a log maintained at the field office.

7.705.305 Emergency and Safety

- A. A therapeutic wilderness program must establish a written policy and specific procedures for emergencies, including, but not limited to, evacuations, natural disasters, medical emergencies, casualties, and missing children/youth. At a minimum, the written policy must include:
1. Designation of authority and staff assignments;
 2. Transportation and relocation of program participants when necessary;
 3. Instruction to all participants on how to respond in the event of an emergency;
 4. Notification to the field office of the nature of the emergency and accounting of each participant's location and status;
 5. Supervision of program participants after an evacuation or relocation;
 6. Arrangements for medical care and notification of a child's/youth's parent(s) or guardian(s) and physician if necessary;
 7. Written procedures for emergency air evacuation if necessary and written agreement for air evacuation services in the event of an emergency.
- B. A therapeutic wilderness program must have written procedures regarding safety and the use of wilderness and survival equipment. The procedures must be reviewed at least annually. The written procedures must include:

1. Instruction of each program participant on environmental hazards and precautions;
2. Instructions on the proper use and storage of tools and other equipment used as a part of the program;
3. The maintenance of a First Aid kit with sufficient supplies available at all times, which must:
 - a. Include supplies specific to the location, environment, and activities of each group;
 - b. Be inventoried after each expedition and restocked as needed;
 - c. Be reviewed with each new staff member for contents and use;
 - d. Be reviewed at least annually with all staff members for content and use.

7.705.306 Admission and Assessment of Children/Youth

- A. A preadmission assessment must be done for each child/youth by a licensed mental health professional familiar with the therapeutic wilderness program prior to enrollment. The assessment must include all requirements found in Section 7.714.4.A.
- B. Subsequent assessments must be done prior to the child/youth leaving for the adventure portion of the program, away from the field office or main base of operations. The assessment must include:
 1. An interview with the child/youth conducted prior to entrance into the field program; and
 2. If previously conducted, the psychological evaluation(s) must be obtained and reviewed by an appropriate licensed mental health professional prior to the child's/youth's entrance into the field portion of the program.
- C. Each child/youth must have a physical examination within thirty (30) days prior to entrance into the therapeutic wilderness program. The program must provide a form to the examining licensed health care provider that clearly describes the type and extent of physical activity, environment, and conditions the child/youth will experience in the program. The exam must be completed by a licensed healthcare provider, who signs the form. The physical exam must, at a minimum, include:
 1. A complete blood count (cbc);
 2. A urinalysis;

3. An electrolyte screen;
 4. A physical assessment to determine fitness given the climate and activities in which the child/youth will be participating, and the child's/youth's age, weight, and physical condition;
 5. A determination whether drug and/or alcohol detoxification is indicated for the child/youth prior to entrance into the field portion of the program;
 6. Documentation of this exam signed and dated by the licensed health care provider must be reviewed prior to the child's/youth's entrance into the field portion of the program.
- D. The therapeutic wilderness program must request information regarding the last dental exam the child/youth had prior to entering the field program. If the child's/youth's last dental exam is over four (4) months prior to entrance into the program and the child's/youth's length of stay in the therapeutic wilderness program exceeds one hundred and twenty (120) days, the facility must schedule or complete a dental exam for the child/youth within fourteen (14) days after the child's one hundred twentieth (120) day of admission. If the child/youth is in the custody of a county department of human/social services or the Division of Youth Services, the therapeutic wilderness program must complete dental exams in compliance with Section 7.714.82.E. If a child/youth has dental care needs, these needs must be addressed timely, as per doctor's orders, while in care.

7.705.307 Health Care and Medication

- A. Children/youth must be observed daily for hydration, conditions of their skin and extremities, and general physical condition. The evaluation must be recorded by field staff in the daily log. Children/youth with an illness or physical complaint needing care or treatment beyond what can be provided in the field must be immediately transported to an appropriate medical care facility.
- B. Children's/youth's feet, legs, hands and arms must be observed at least two (2) times daily for signs of pain, swelling, blisters, red streaks running from cuts or abrasions, frostbite, and hydration.
- C. Complaints or reports by a child/youth of illness and injury must be recorded in the daily log along with any treatment provided.
- D. First Aid treatment must be provided in as prompt a manner as the location and circumstances allow.
- E. There must be no adverse consequences imposed on a child/youth for reporting an injury or illness or for requesting to see a healthcare professional.
- F. At least every fourteen (14) days, each child's/youth's physical condition must be assessed by a Wilderness First Responder (WFR), an Emergency Medical

Technician (EMT), or a qualified medical professional. The results of the assessment must be recorded and at a minimum include:

1. Blood pressure;
 2. Heart rate;
 3. Condition of the extremities;
 4. Condition of the skin;
 5. Hydration level;
 6. Allergies, if any;
 7. General physical condition;
 8. Provision of appropriate medical treatment, if needed.
- G. Prescription and over the counter medication, including vitamins and supplements, must be stored in a locked container, safe guarded from children/youth and the elements. Controlled medications must be double locked, and the keys to the locks must be under the control of the staff member responsible for administering medications at all times. Medications taken on field outings must be in the immediate control of a staff member qualified to administer medications at all times.
- H. The administration of psychotropic medication must be prohibited in a therapeutic wilderness program unless a qualified medical professional determines that the medication is clinically indicated. Under no circumstances will psychotropic medication be administered for disciplinary purposes, for the convenience of staff, or as a substitute for appropriate treatment services.
- I. Prescribed medications must not be stopped or changed in dosage or administration without consulting with the prescribing physician and obtaining a written order. If the prescribing physician is not available, a qualified medical professional must be consulted. Results of the consultation and any resulting medication changes must be recorded in the child's/youth's record.
- J. All controlled substances must be counted at least two (2) times per day by two (2) different staff members, and a log must be maintained that includes the signature of the individual conducting and documenting the count.
- K. All unused and expired medications must be secured until appropriately disposed of and documented.

7.705.308 Nutrition

- A. There must be a written menu, approved on an annual basis, as required in Section 7.705.31. The registered dietitian must have knowledge of the program activity levels and environmental factors. The menu must list the necessary food supplies and caloric intake for each group. The current menu must be available, and any change or substitution must be noted on the menu. menus must be dated and maintained on file for six (6) months.
- B. At least two (2) times per week, the fruits and vegetables offered in the approved menus must be fresh fruits and vegetables.
- C. Requiring a child/youth to consume food in under five (5) minutes is prohibited.
- D. Child care field staff must monitor the intake of water for each child/youth daily to ensure adequate hydration. If a child/youth exhibits signs of dehydration, this information must be reported to the field office immediately. hydration supplements with electrolytes must be available to children/youth.

7.705.309 Participant Clothing, Equipment, and Supplies

- A. Each program participant must have clothing, equipment, and supplies appropriate for the types of activities, terrain, and weather conditions likely to be encountered.
- B. Clothing, equipment, and supplies must include, at a minimum:
 - 1. Sunscreen;
 - 2. Insect repellent;
 - 3. A commercially available backpack or the materials to construct a safe backpack and/or bedroll;
 - 4. Personal hygiene items necessary for cleansing;
 - 5. Appropriate feminine hygiene supplies;
 - 6. Appropriate sleeping bag, pad, and/or sleep system for each child/youth;
 - 7. Clothing appropriate for temperature changes generally expected for the area;
 - 8. Each child/youth must be provided a clean change of clothing at least once a week or have an opportunity to wash their clothes at least once a week;
 - 9. Each child/youth must be provided clean undergarments, including socks. Additional clean undergarments and socks must be provided to a child/youth, as may be needed for health and sanitary reasons;

10. Each child/youth must be provided a means to clean their body at least two (2) times per week;
11. Appropriate well-fitting footwear for the weather, activity, and terrain the program will experience;
12. Synthetic or wool socks must be designed for outdoor activity.

7.705.310 Outings and Hiking Limits and Requirements

- A. Hiking must not exceed the physical capability of any member of the group.
- B. There must be no hiking when the temperature is above ninety (90) degrees fahrenheit or below fifteen (15) degrees fahrenheit, including windchill. If hiking in temperatures below thirty-two (32) degrees fahrenheit (including windchill), breaks must be taken at least every thirty (30) minutes in a sheltered/warmer environment.
- C. Copies of maps, routes, and anticipated schedules, including departure and arrival times, must be maintained with each group and at the field office when a group is on an outing away from the base camp.
- D. Staff must closely monitor children/youth for acclimation to the temperature, climate, altitude, environment, and situation.
- E. The maximum pack weight must not exceed thirty (30) percent of the child's/youth's physical weight.
- F. If the facility participates in special activities other than those for which regulations are found in this section, the facility must follow a written policy which includes all Sections from 7.719.1.D.

7.705.311 Training Requirements

- A. Each program staff must have completed training in each of the areas listed in Section 7.714.92.
- B. Each program staff must have an additional eighty (80) hours of training specific to therapeutic wilderness programs within the first thirty (30) days of employment and prior to working unsupervised with children/youth. The training must not be considered completed until the staff have demonstrated to the field director their proficiency in, at a minimum, the following areas:
 1. Counseling, teaching, and supervisory skills;
 2. Water, food, and shelter development;
 3. Environmental conservation skills and procedures;

4. Instruction in emergency and safety procedures including, but not limited to, medical, emergency evacuation, weather, signaling, fire, flood, and runaway or lost children/youth;
 5. Instructions on safety procedures for the use of fuel, fire, and other related tools;
 6. Sanitation procedures including water, waste, food, and toileting;
 7. Health issues related to acclimation, exposure to the environment, and other environmental issues;
 8. The contents of the First Aid kit, storage, maintenance, and use;
 9. Local environmental precautions including terrain, weather, poisonous plants, wildlife, and response to adverse situations;
 10. Required documentation, including the development and maintenance of logs and journals.
- C. Prior to a staff person being included in ratio in accordance with the requirements found in Section 7.705.46, initial staff training and demonstration of proficiency must be completed and documented.
- D. Ongoing training must be provided to staff to improve proficiency in knowledge and skills in therapeutic wilderness settings and to maintain certifications.

7.706 RULES REGULATING DAY TREATMENT CENTERS [Rev. eff. 6/1/12]

All day treatment centers must comply with the “General Rules for Child Care Facilities” found at Section 7.701, et seq., the restraint and seclusion rules found at Section 7.714.53, et seq., the applicable definitions in Section 7.714.1, and these “Rules Regulating Day Treatment Centers”.

7.706.1 DEFINITIONS [Rev. eff. 10/1/15]

A “day treatment center” is a facility which provides less than twenty-four (24) hour care for groups of five or more children from three (3) to twenty-one (21) years of age. Nothing prohibits a day treatment center from allowing a person who reaches twenty-one (21) years of age after the commencement of an academic year from attending an educational program at the day treatment center through the end of the semester in which the twenty-first birthday occurs or until the person completes the educational program, whichever comes first. The center will provide a structured program of various types of psycho-socio and/or behavioral treatments to prevent or reduce the need for placement of the child out of the home or community.

This definition shall not include special education programs operated by a public or private school system or programs which are licensed by other regulations of

the Colorado Department of Human Services for less than twenty four (24) hour care of children, such as day care centers or centers for developmentally disabled children.

“De-escalation” is the use of therapeutic interventions with a child during the escalation phase of a crisis. The interventions are designed to allow children to contain their own behavior so that acute physical behavior does not develop that would lead to the need to use a physical management.

“Reasonable” as used in these rules means appropriate and suitable, or not excessive or extreme.

7.706.11 Governing Body

- A. The governing body is the individual, partnership, corporation or association in whom the ultimate authority and legal responsibility is vested for the conduct of the day treatment center.
- B. The governing body shall be identified by its legal name. The names and addresses of individuals who hold primary financial control, members of the board of directors and officers of the governing body shall be disclosed fully to the Colorado Department of Human Services. When changes of persons holding any of these positions occur, the department shall be informed of the names and addresses of the new individuals annually.

7.706.12 Statement of Purpose

Each day treatment center shall formulate a written statement which includes the purpose and objective of the day treatment center, description of the services to be offered by the day treatment center, ages of children and type of children to be accepted by the day treatment center, and the geographic area in which children may reside for acceptance by the day treatment center.

7.706.13 Financial Operation

- A. An annual budget reflecting anticipated income by source and expenses by purpose, plus an accompanying balance sheet, must demonstrate that the agency has assured resources to carry out its defined purpose through its first year of operation and must be submitted with the original license application.
- B. The center shall maintain a competent accounting system, and the financial records of the center shall be available for inspection by staff members of the department.
- C. Each day treatment center whose total annual expenditures exceed \$100,000 shall provide for an annual audit of all accounts by a certified public accountant who is not an employee of the center nor a member of the governing body.

Centers with less than \$100,000 total annual expenditures may submit unaudited financial statements compiled by agency personnel.

7.706.14 Personnel

- A. Each staff member of the day treatment center shall evidence an interest in and knowledge of children and concern for their proper care and well-being.
- B. Each staff member of the day treatment center shall obtain a physical examination which shall be performed during the six-month period preceding employment by the center. This examination shall have been performed by a physician or a qualified nurse practitioner licensed to practice in the State of Colorado and shall verify that she/he suffers from no illness or communicable disease, including tuberculosis, that would adversely affect children in care. A statement, signed by the physician or nurse practitioner, shall be submitted to the center and retained in the personnel files. Subsequent statements shall be submitted annually and as required in a written plan signed by the physician.
- C. There shall be at least one written reference regarding each staff member of the center verifying the individual's character and suitability to work with children. This reference shall be retained in the staff member's personnel file.
- D. The administrator of the day treatment center shall have received a bachelor's degree from an accredited college and have completed two years of verifiable work experience with children in the field of psychiatry, psychology, social work, child development and child care, education, nursing, or other allied professions.

The director and governing body shall be responsible for assuring that the Rules Regulating Day Treatment Centers are being met.

- E. There shall be at least one treatment leader on the staff of the day treatment center who shall be responsible for the coordination of treatment of each child registered at the day treatment center. The treatment leader shall have at least a master's degree in the behavioral science field such as social work, psychology, psychiatric nursing; or the treatment leader shall be a psychiatrist who is qualified and licensed to practice in the State of Colorado. In addition to education, the treatment leader shall have completed three years of treatment-oriented experience.
 - 1. If properly qualified, the treatment leader and the administrator may be the same individual.
 - 2. The treatment center may contract with a recognized agency for the provision of treatment services. The staff IT Member of the agency who is to fulfill the responsibility of treatment leader must meet the requirements stated in Section 7.706.14, E.

3. There shall be at least one treatment leader employed by the day treatment center for each 25 children enrolled at the day treatment center. The treatment leader(s) shall be employed and actively involved in the services prescribed for each child through supervision or direct service for the number of hours per day the treatment program operates.
- F. There shall be counselors hired by the day treatment center, sufficient in number, to supervise the children and carry out the program of the day treatment center. Each counselor shall have completed a bachelor's degree in behavioral sciences or four years of experience with appropriate-aged children and be at least 21 years of age. The counselors shall work under the supervision of the treatment leader.
 1. The ratio of counselors and/or treatment leader(s) to children in care shall be not less than the following schedule:

<u>Age of Children</u>	<u>Staff Necessary</u>
5 years-13 years	1 staff member: 8 children
13 years-16 years & over	1 staff member: 10 children
 2. There shall be at least one counselor or treatment leader on duty at the day treatment center at any time children are present. If only one counselor is on duty, there shall be a second staff member on call and immediately available to be summoned in case of emergency.
 3. Substitute counselors shall be of the same minimum qualifications as regularly assigned counselors.
- G. Para-professionals must be at least 21 years of age and, under direct supervision, may assist the director, teachers, counselors, and treatment leaders with the children.
- H. There shall be one staff member on duty at the center at all times who holds a current Red Cross first aid card or equivalent and CPR card.

7.706.15 Child Care Services [Rev. eff. 6/1/12]

- A. Admission of the children to the day treatment center shall be limited to and in keeping with the stated purpose and goals of the center.
 1. Intake policies of the center shall be clearly defined in writing and shall be reviewed and modified as necessary.
 2. At intake the center shall obtain:
 - a. Identifying information necessary for the child's record as required at Section 7.706.18, A.

- b. Psycho-social history of the child.
 - c. A statement about the health history of the child which includes at least the immunization record; information and instruction for care of each child who has a chronic or handicapping problem such as seizures, asthma, diabetes, allergies, heart or respiratory illness or drug addiction; date of last physical examination. If child has not had a physical examination within the past twelve months, such examination shall be completed within thirty days of admission to the program. Subsequent examinations shall be obtained as required by a physician or nurse practitioner licensed to practice in Colorado.
 - d. The center shall obtain written permission for the following from the person or agency having custody:
 - 1) Permission to treat the child.
 - 2) Permission to obtain emergency medical care in the event that the child has an accident or illness requiring medical attention when parent or guardian is unavailable.
 - 3) Permission to provide transportation to and from the center or for center-sponsored activities if necessary.
 - 4) Permissions for physical management, restraint and seclusion as required in Section 7.714.53, et seq.
- B. For each child in the day treatment program, long range and short term goals shall be established and a case plan written which includes anticipated behavioral changes of the child and projected length of participation in the day treatment program. Goals and case plans shall be evaluated at least monthly. The discharge plans shall become part of the case plan and shall describe the child's behavior at the time of discharge from the day treatment program. The treatment leader shall be responsible for establishment and signing the case plans.
- 1. Persons to be involved in the process of case planning and periodic evaluation shall include a staff member of the agency having referred the child, the parent or guardian, as appropriate, and staff members of other agencies serving the child.
 - 2. A monthly progress report shall include a description of the child's behavior, summary of the treatment activities, and revision of the case plan. The report shall be forwarded to the referring agency and reviewed with parent or guardian monthly for the first six months that the child is in the center and quarterly thereafter.

3. The center shall maintain open communication with parents, guardians and/or referring agency and include each party in the treatment plan and reviews, as may be appropriate for each family and student.
- C. Treatment shall be provided to each child enrolled in the center pursuant to his/her case plan. Treatment sessions with members of the child's family shall be included, as deemed necessary and as part of the case plan. Each child and/or his/her family shall participate in at least two weekly sessions of individual therapy, group therapy or family therapy. All services shall be documented in the child's file.
 - D. If the child is attending a school operated by the day treatment program, a plan shall be established with appropriate school personnel of the school most recently or currently attended and with the child's parent(s) or guardian(s) for the education of the child based upon his/her particular needs and special ability.
 - E. Each day treatment center shall establish a schedule or a plan of activities which shall indicate the plan for educational and treatment services, as well as other services such as recreation and employment as appropriate for children in care.
 - F. Outdoor and indoor recreational equipment and material shall be provided in sufficient variety and quantity so the center may adequately sponsor and supervise necessary recreational activities. Games, toys, equipment, and arts and crafts material shall be selected according to age, number of children, and with consideration of the needs of children to engage in both active and quiet play. All equipment and materials shall be of quality to assure safety, and shall be of a type which allows for imaginative play and creativeness.
 1. The center shall establish a written policy related to the participation of children in potentially dangerous recreational activities which include the use of such equipment as trampolines, trail bikes, snowmobiles, boats, bodybuilding equipment and such activities as archery, riflery, horseback riding and backpacking (see Rules Regulating Children's Camps).
 2. A staff member shall be assigned the responsibility for supervision and coordination of the total recreational program including training and supervision of staff and volunteers.
 - G. As appropriate to the child's age and ability and as indicated in the case plan, the child shall be offered guidance and help in obtaining paid or voluntary work assignments. If employment services are part of the program of the day treatment center, one center staff member shall be assigned the responsibility of coordinating such work arrangement and shall know the employer of the child, the specific type of work and conditions of employment.
 - H. Each child shall receive assistance or supervision needed to help him/her establish good habits of personal care, grooming and hygiene.

- I. Only prescribed medicines labeled with the child's name, name and strength of the medicine, directions for use, date filled, prescription number and name of practitioner shall be given with the written consent from the child's parent or guardian. Medicines must be in the original container. A record of medication administered which includes the child's name, the date and time the medication was administered, the name and dosage of the medication, and the name or initials of the staff person who administered it shall be maintained in the child's file. There shall be one staff member on duty at all times designated to administer and record all medication. The responsible staff person must have completed State approved medication administration training and have documentation on file. All medicines and drugs shall be kept in a locked storage area inaccessible to children.
- J. Children who are in the day treatment center for more than four hours, day or evening, shall receive a meal.
 - 1. If the facility provides the meal, menus shall be kept for at least a two week period, and there shall be a documented review periodically by a qualified nutritional consultant. Children shall not be given foods which are contrary to their religious beliefs or which are known to cause an allergic reaction or health hazard.
 - 2. Drinking water shall be freely available to children.
- K. Any vehicle used for transportation of children and the vehicle driver shall meet the following regulations:
 - 1. Vehicles shall be licensed in accordance with Colorado state law, and drivers shall comply with applicable laws of the Colorado State Department of Revenue, Motor Vehicle Division, and ordinances of the municipality in which the day treatment center is located.
 - 2. The drivers shall hold a valid Standard First Aid Certificate or equivalent and CPR card.
 - 3. Only enclosed vehicles shall be used to transport children except an enclosed pickup truck shall not be permitted. All seats must be forward-facing and shall be securely fastened to the vehicle body. A seat belt or booster seat for children under 40 pounds shall be provided for each seat. Only the number of passengers that can be comfortably seated on passenger seats shall be permitted in the vehicle. Each occupant shall be sitting when the vehicle is moving. Passengers shall not sit on the floor.
 - 4. Each vehicle shall be equipped with a first aid kit, an operable fire extinguisher and seat belts in the front seats.
 - 5. The day treatment center shall carry public liability insurance on vehicles used for transportation of children in the day treatment program.

6. The staff ratio required at the day treatment center shall be maintained in any vehicle when transporting children from the day treatment center. The driver may be considered in the staff ratio.
7. The center shall receive written permission from parents or guardians for transportation of a child to and from a center or for center-sponsored excursions.

7.706.16 Behavior Management [Rev. eff. 6/1/12]

- A. Each day treatment center will have written documentation of policies and procedures that describe the treatment model upon which the behavior management system is based and who shall be responsible for disciplinary measures.
- B. The behavior management system must be explained to parents/guardians and to children taking into account the child's developmental and cognitive level.
- C. The behavior management system must include, at a minimum:
 1. Positive constructive or educational components including but not limited to diversion, withholding of privileges, talk with the child about the situation, positive reinforcement, skill training, and skill development.
 2. Clear rules and expectations with predictable, consistent consequences or interventions that will be used by staff members to reinforce positive behaviors and to address disruptive, off-task or unsafe behaviors in the program.
 3. A continuum of de-escalation and redirecting techniques to deal with disruptive, escalating or unsafe behaviors.
 4. A review process for addressing chronic behavior problems or serious acute problems in which a child is not responding to the behavior management program as being implemented.
- D. The program will have a monitoring process of the behavior management system to assure both consistency in implementation and for continuous quality improvement as needed to address changing client needs and behaviors.
- E. Physical management will be considered an extreme level of intervention at the day treatment level. The occurrence of restraints for a particular child will require a review as described in Section C, 4 above. The goal of these reviews will be to eliminate restraints for the child by reassessing the identified problem, adjusting the treatment plan, utilizing additional resources or other such interventions including the evaluation of whether day treatment is a level of care at which the child can successfully function.

- F. The behavior management system will include, but not be limited to, any of the following:
1. No child shall be punched, shaken, pinched, bitten, roughly handled or struck by an inanimate object by staff members of the day treatment center.
 2. Authority to discipline shall not be delegated to other children.
 3. Separation when used as discipline shall be brief and appropriate to the child's age and circumstances, and the child shall be within hearing and visual observation of an adult in a safe, lighted, well-ventilated room. No child shall be secluded in a locked room or closet.
 4. No child shall be punished for toileting accidents.
 5. Verbal abuse or derogatory remarks about the child, his family, his race, religion or cultural background shall not be used or permitted.
 6. No child shall be force-fed.
 7. Meals may be provided individually, but deprivation of meals shall not be used as punishment.
 8. No child shall be subjected to physical harm or humiliation.
 9. Seclusion.
- G. Disregard of any of the foregoing disciplinary rules or any disciplinary measure resulting in physical injury or abuse of any child shall be grounds for the denial, revocation, suspension, or making probationary of the license.

7.706.17 Physical Management [Rev. eff. 6/1/12]

Any physical management, restraint and seclusion shall comply with the rules found at Section 7.714.53, et seq., and the applicable definitions in Section 7.714.1.

7.706.18 Records

- A. A record of admission shall be completed for each child prior to, or at the time of, admission to the day treatment program and shall include:
1. Necessary identifying information including the child's name, address, telephone number, birth date, sex; name, address, telephone number of parent(s) or guardian(s); name and relationship of person with whom child is living, if different from parent(s) or guardian(s); address and telephone number at the location person(s) responsible for child can be reached when child is participating in the treatment program; name, address and

telephone number of individual to contact in emergency if parent(s) or guardian(s) is unavailable; name, address and telephone number of referring agency and referring worker; physical description of the child, description of child's behavior and personal habits, health and immunization record; name and address of individual who may call for the child if the child is not to leave the center on his own; name, address and telephone number of child's physician or the clinic where medical care is obtained.

2. A current medical statement, records of medication provided to the child and records of all injuries occurring while in care at the center.
 3. Written permissions as required at Section 7.706.15, A, 2, d.
 4. A case plan, a summary of the periodic evaluations of the child's progress and resultant changes in the case plan. The evaluation summary shall include the date and the individuals who participated, as well as a description of the child's progress toward the objectives outlined in the case plan.
 5. A summary of the discharge of the child from the center which includes at least the date of the discharge and reason for child's discharge from program.
- B. A daily attendance chart indicating the names of children who attended each session each day shall be maintained.
- C. Personnel record for each staff member which includes the following shall be maintained: name, address, telephone number, birth date, education and training, work experience, employment reference, statements of physician or nurse practitioner; names, addresses and telephone numbers of persons to be notified in event of an emergency; date of employment; copy of first aid card or equivalent and CPR training, and medication administration training, if appropriate.
- D. Administrative records which shall be on file at the center include: current health department inspection report, current fire department inspection report, a list of current staff members and substitutes, and a staffing pattern.

7.706.19 Physical Requirements for a Safe and Adequate Center [Rev. eff. 4/1/15]

- A. The day treatment center must declare to the Colorado Department of Human Services the address of each building which will be used by the day treatment center, the anticipated hours of use of the building by the day treatment center, and the type of use of each building by the day treatment center.

- B. The licensing representative must inspect and approve the entire premises of each facility to be used or in use by the day treatment center, including, but not limited to, the building, the grounds surrounding the building, the basement or attic, if accessible, any other buildings on the grounds, such as garage, storage areas and carports.
- C. Each facility used by the day treatment center must be equipped with adequate heat, light and ventilation for safe and comfortable occupancy.
- D. All heating units must be installed and maintained with safety devices to prevent fire, explosions, and other hazards. No open-flame gas or oil stoves, hot plates or un-vented heaters must be used or heating purposes.
- E. Closets, attics, basements, cellars, furnace rooms and exit routes must be kept free from accumulation of extraneous material such as discarded furniture, furnishings, newspapers or magazines. Combustibles, such as cleaning rags, mops, cleaning compounds, must be labeled and stored in well-ventilated areas. Storage of gasoline, kerosene, fuel oil and other flammable materials must meet requirements of the safety and fire code.
- F. In each facility there must be at least two approved, alternate, widely-separated means of egress from each floor of the building to safe and open space at the ground level.
- G. No lock or fastening to prevent free escape from the inside of the room or building used by children must be permitted. Exit hardware must be of the single-action type.
- H. Exit doors must be obvious and marked by exit signs.
- I. The route to each exit must be conspicuously indicated in such a manner that each occupant of a building or structure who is physically and mentally capable will readily know the direction of the escape from any point. Each path of escape must be so arranged or marked in such a manner that the way to a place of safety outside is unmistakable. Exitways and pathways to exitways must be unobstructed at all times.
- J. Any accessible areas beneath stairways which will be used as exitways by children must be enclosed with one-hour fire resistant material.
- K. In each building used by the treatment center, fire alarm facilities must be provided to warn occupants of the existence of a fire so that they may escape, or to facilitate the orderly conduct of fire exit drills.
- L. Each facility must have a telephone, and near the telephone must be located the numbers of an emergency medical facility, the local fire and police departments, and other emergency numbers including poison control, if available.

- M. If weapons are on the property of the facility, they must be rendered totally inoperable and must be locked up to prevent unauthorized use. Ammunition, projectiles such as arrows or other items which can be used to make the weapon operable, must be; locked separately. Weapons must not be transported in any vehicle in which children are riding unless the weapons are made inoperable and inaccessible.
- N. When a swimming pool is provided, it must meet the requirements of the Colorado Department of Public Health and Environment or local unit. Safety precautions must include protective fencing, nonskid surface of at least four feet adjoining pool sides, and winter coverage which must exclude plastic or inflatable-type domes. A lifeguard, who holds an advanced life saving certificate, must be in attendance at all times when the pool is in use.
- O. The Colorado Department of Human Services will establish the number of children who may be served at any one time in the building(s) used by the day treatment center for school, recreation or therapy, established on the basis of one child per twenty square feet of space within the rooms which are to be used. Areas within the building which are to be used for bedroom, kitchen, office, hallways, or bathroom cannot be counted as space available for use by the children at the day treatment center.
- P. Within the buildings to be used by children in the day treatment program, there must be one toilet and one lavatory for every thirty children in attendance.
- Q. Arrangement must be made for personal belongings of each child while in attendance at the center.

7.707 RULES REGULATING FAMILY CHILD CARE HOMES [Repealed eff. 03/05/2026]

7.708 RULES REGULATING FAMILY FOSTER CARE HOMES

All foster care homes must comply with the "Rules Regulating Foster Care Homes" and the "General Rules for Child Care Facilities".

7.708.1 FOSTER CARE HOMES [Rev. eff. 1/1/16]

- A. "Foster care home" (refer to Section 7.000.2 in 12 CCR 2509-1), as defined at Section 26-6-102, C.R.S., means a facility that is certified by the county department or a child placement agency for child care in a place of residence of a family or person for the purpose of providing twenty-four (24) hour family care for a child under the age of eighteen years who is not related to the head of such home, except in the case of relative care. The term includes any foster care home receiving a child for regular twenty-four (24) hour care and any home receiving a child from any state operated institution for child care or from any child placement agency.

The number and age of foster children for which a certificate may be issued is determined by the following factors:

1. No foster child/youth shall be placed in a foster home if that placement will result in more than six foster children/youth in that home, or a total of ten children/youth (foster and non- foster), or more than two children under two years of age, except in those instances in which the placement of a sibling group in a foster care home would exceed the limits. If the placement of a sibling group results in exceeding the above limits, no other foster children/youth can be placed in the home.
 - a. When a foster care home is certified for more than four children/youth in foster care, each foster parent shall demonstrate sufficient skills to meet the individual physical, mental, social, behavioral, and other trauma needs of each child/youth placed in the home, including identification of:
 - 1) Parenting experience, parenting style and the ability to meet the supervision needs of each child/youth;
 - 2) Ongoing assessment of needs shall be conducted by the certifying agency during support visits in the foster care home. The certifying agency shall provide more frequent contact during the month when that is an identified need;
 - 3) Sufficient supports, including those provided by the certifying agency to meet the daily needs of each child/youth placed in the home as identified in the family services plan. Supports could include transportation, child care, respite, and coaching for the child/youth and/or foster parent(s); and,
 - 4) Training needed to provide care to the populations served, including any increased and/or specialized training identified in the training development plan.
 - b. Space requirements in section 7.708.22 shall be maintained.
 - c. The preferences of foster parents identified in section 7.708.61 shall be maintained.
2. A foster care home may serve children enrolled in Children's Habilitation Residential Program (CHRP) if the certifying agency determines the foster home providers have the knowledge and supports to safely meet the needs of all of the children in the home. Emergency placements will not exceed maximum capacities.

<u>CERTIFIED FOSTER HOME MAXIMUM CAPACITY</u>		
CHRP	Non-CHRP	Total Children

1	3	4
2	2	4
3	1	4

3. Foster care providers who are serving foster children enrolled in the Children's Habilitation Residential Program (CHRP) waiver shall be in compliance with rules contained within the Department of Health Care Policy and Financing's Medical Assistance Manual at Section 8.508 (10 CCR 2505-10).
4. Foster care may be provided to children from birth to eighteen (18) years of age and to those persons to 21 years of age who are placed by court order prior to their eighteenth birthday.
5. When a foster care home only provides temporary emergency care for foster children/youth, the home may be certified for up to six foster children/youth. No more than two children under two years of age may reside in the foster care home, including the foster parent's children under two years of age and any children under two years of age being placed into foster care. An exception is allowed when the placement of a sibling group of twins or triplets (multiple births), etc., who are under two years of age, would exceed the limits. There can be no more than a total of six foster children/youth in the home except in sibling placements. The number of additional foster children under six years of age to be cared for shall be specified on the certificate and in the home study. Such a foster care home shall be designated as a receiving home. The designation shall appear on the certificate.
 - a. No foster child/youth shall remain in that foster care home at the receiving home rate longer than 90 consecutive days, in accordance with the requirements of Section 7.417.3.
 - b. The number of foster children/youth to be cared for at the receiving home rate and any foster children/youth to be cared for at the regular foster care home rate shall be included on the certificate.
 - c. Each receiving home parent shall have two years of experience as a foster parent, although the certifying agency may take into account other relevant education and experience.
 - d. Each receiving home parent shall complete 32 hours of on-going training every year as identified in his/her training development plan. This training shall include the following competencies:
 - 1) Issues regarding emergency and crisis placement of children/youth with unknown histories; and,

- 2) Dynamics of victimization issues, with emphasis on appropriate age and developmental levels; and,
 - 3) Cultural, spiritual, and religious awareness, consideration for, sensitivity to, and tolerance of each child/youth individually.
 - e. Because receiving homes are likely to have quick turn-over of the children/youth in care, supervision and monitoring of the receiving home shall be carried out according to the following:
 - 1) One face-to-face contact shall be made with the receiving home parent(s) at least every week when children/youth are in placement in the home, with a minimum of two visits per month occurring in the receiving home.
 - 2) Documentation of such contact shall be in the provider file, as well as in each file of all foster children/youth in the home.
 - 3) The purpose of the contact is to address any questions the receiving home parent has about the children/youth in care, to observe child care when appropriate and to provide support to the receiving home parent.
 - f. A receiving home shall have a comprehensive annual evaluation, which includes a review of any critical incidents, any allegations of institutional abuse, and the skills, needs, and competencies of the receiving home parent(s).
- 6. A foster care home may not be operated without a license or a certificate as required by law and Section 7.701.4 of these rules, and the number of foster children cared for in such facility may not exceed the number authorized by such license or certificate.
- B. Foster care homes may be licensed host family homes.
 - 1. A foster home may be a licensed host family home pursuant to the rules at Section 7.721.
 - 2. Pursuant to 26-5.7-105(6), C.R.S., a foster care home approved as a licensed host family home shall not accept a homeless youth for placement under this section if there are any foster children currently placed in the home. No foster children may be placed while a homeless youth resides at the foster home.
 - 3. A foster care home acting as a host family home for homeless youth shall receive additional training regarding homeless youth services and shall be designated as a host family home on the foster home certificate.

4. Pursuant to Section 26-5.7-105(5), C.R.S., if the county or child placement agency referring the youth to the host family home or the host home parents determine that a referral for additional services needs to be made, they shall make a referral to the county of reference of the parents of the youth.

5. Notification

Pursuant to Sections 26-5.7-105(4) and (7), C.R.S:

- a. When a youth under fifteen years of age is admitted to a host family home, the county or child placement agency referring the youth to the host family home or the host family home's parents shall notify the county department of residence of the parents of the youth within seventy-two (72) hours of the youth's admission.
- b. If a youth who is at least eleven (11) years of age but less than fifteen (15) years of age has been served up to twenty-one (21) consecutive days and returns again to the licensed host family home after leaving the home, the county or child placement agency referring the youth to the host family home or the host family home shall notify the county department of residence of the parents of the youth within seventy-two (72) hours of the youth's admission.

7.708.11 Definitions [Rev. eff. 1/1/16]

"Certifying authority" means licensed child placement agencies and county departments of human or social services that have the authority to certify foster care homes, including kinship foster care homes.

"Child specific foster care" is care where the foster child has a prior relationship to the foster parent(s).

"County designee" is the representative of a county department designated by the county executive director to make certain key decisions regarding foster children.

"De-escalation" is the use of therapeutic interventions with a foster child during the escalation phase of a crisis. The interventions are designed to allow foster children to contain their own behavior so that acute physical behavior does not develop which would lead to the need for use of a physical restraint.

The "Family Service Plan" is a case services plan completed by a county caseworker jointly with the foster child, parents, and foster parents within 60 calendar days of placement for each foster child receiving services from a county department of social/human services.

"Kin": Refer to Section 7.000.2 (12 CCR 2509-1) for the definitions of "kin" and "non-certified kinship care".

“Reasonable”, as used in these rules, means appropriate and suitable, not excessive or extreme.

“Religion”, where used in these rules, includes traditional religious beliefs and spiritual beliefs such as those of Native Americans.

“Therapeutic foster care” means a program of foster care that incorporates treatment for the special physical, psychological, or emotional needs of a child placed with specially trained foster parents.

“Treatment Foster Care” means a clinically effective alternative to residential treatment facilities that combines the treatment technologies typically associated with more restrictive settings with a nurturing and individualized family environment.

“Whole Family Placement”, also known as “shared family care”, is a situation in which adult parent(s) and foster child(ren) are placed together in the home of a family trained to mentor and support the biological parents as they develop skills and supports necessary to care for their foster child(ren) and move toward living independently.

7.708.2 REQUIREMENTS FOR CERTIFICATION OF FAMILY FOSTER CARE HOMES

7.708.21 Character, Suitability, and Qualifications of Family Foster Parents

- A. A certificate shall be denied in accordance with Section 7.500.312, D.
- B. Each foster parent in the foster care home shall demonstrate an interest in, and a knowledge of, foster children and a concern for their proper care and well-being.
- C. A certificate may be denied or revoked if the foster parent(s)' own children have been placed in foster care or a residential treatment facility under circumstances which demonstrated that the foster parent or another resident of the home was abusive, neglectful, or a danger to the health, safety, or well-being of those foster children.
- D. The foster care home parents shall be able to provide for a foster child's proper physical, mental and character development.
- E. Applicants shall demonstrate stability in family relationships within the home where foster care is to be provided.
- F. The licensing or certifying authority must receive at least three written statements which describe the applicant's character, interpersonal relations, and ability to provide care for foster children from references provided by the applicant, at least two from a source/person who are not related to the applicant and who have known the applicant one year or longer (references need not be residents of Colorado). Licensing or certifying representatives may contact others who may have knowledge or information regarding the applicant's character or suitability.

- G. The foster home parent shall possess basic knowledge of child care and good nutrition, and shall cooperate with the licensing or certifying agency in programs designed to increase such knowledge.
- H. Licenses or certificates shall not be granted to applicants who are less than twenty one years of age on the date of application for such license or who lack adequate physical stamina to care for children.
- I. Foster parents shall protect children/youth in foster care from exposure to second hand smoke. Smoking includes the use of electronic nicotine delivery systems, also known as e-cigarettes, e-cigs, vape pens, e-hookahs, e-pipe, tanks, mods, and vapes.
 - 1. Smoking is prohibited inside the foster care home at all times when a child/youth is in placement in the foster care home.
 - 2. Smoking is prohibited in a foster parent's, alternative care's (natural support), or substitute caregiver's motor vehicle when a child/youth is in placement in the foster care home.
 - 3. Smoking is prohibited in the presence of a child/youth who is in placement in the foster care home.
- J. The financial resources of foster parents shall be adequate to assure that the home where the care is provided is maintained in safe repair and in conformity with standards and that the requirements of these regulations can be fulfilled.
- K. A license or certificate shall not be granted for a foster care home unless the applicant has demonstrated the ability to manage a household so that the licensing or certifying authority may determine that the applicant is able to acquire food, materials and other equipment as may be required for child care and to maintain records pertaining to foster children, including records required by statute or regulations.
- (L) each foster parent must have a health assessment, to include a current vaccination record, within one year prior to certification or within 30 calendar days after certification and thereafter as required, in writing, by a licensed health care professional. the reports of the medical examinations shall be dated and signed by the examining physician or nurse practitioner and shall be provided to the certifying. authority. reports shall include a statement of the evaluation of the person's physical ability to care for foster children and a copy of their current vaccination. Record if the provider is providing care for infant(s) under 6 months of age, the provider must be current on the influenza and pertussis vaccinations. a medical exemption is allowed when an individual has a medical condition that prevents them from receiving a vaccine a non-medical exemption is allowed when an individual has religious beliefs whose teachings are opposed to immunizations or a personal belief that is opposed to immunizations.

- M. Children of the foster home parents and any other persons not placed by the agency and living in the foster care home shall obtain a medical statement from a licensed health care professional verifying that each such person suffers from no illness or communicable disease which would adversely affect foster children in care. This statement shall be obtained annually or as required in writing by an approved health care professional. A licensed health care professional is defined as a physician, nurse practitioner, or a physician's assistant. This statement shall have been signed within the twelve month period preceding the original license or full certificate granted to the home.
- N. Applicants will not use illegal substances under state law, will use legal prescription medications as prescribed by a licensed health care professional, and will use over the counter medications as recommended by the manufacturer or the licensed health care professional.
- O. The use of legal substances must not inhibit the foster parent's ability to provide care that is consistent with the needs of the child/youth.
- P. A foster parent must be able to communicate sufficiently to provide care for the child/youth, including the ability to communicate with the guardian ad litem (gal) and/ or counsel for youth and case worker. If needed, a foster parent may use an interpreter to assist. A child may not be used to interpret. This must be documented in the home study and updated annually.
- Q. A foster parent must be able to read and comprehend the instructions for the child's/youth's medical and mental health requirements, including but not limited to, prescription medications and the licensed health care practitioner's instructions. A foster parent may use an interpreter to assist. A child may not be used to interpret. This must be documented in the home study and updated annually.
- R. if, in the opinion of the licensed health care professional or the assessment worker, an emotional or psychological condition exists which would have a negative impact on the care of foster children, the issuance of a license must be conditioned upon the satisfactory report of a licensed mental health practitioner.

7.708.21.1 Health History Assessment:

- A. All household members must disclose to the certifying agency their current mental health and/or substance use and/or abuse issues.
- B. All household members must provide information about their physical and mental health history, including any history of drug and/or alcohol abuse or treatment.

7.708.22 Physical Requirements for a Safe and Adequate Foster Care Home
[Rev. eff. 1/1/16]

- A. Licensing or certifying representatives are authorized but not required to consult the state or county department of health regarding sanitary standards and to consult local fire departments regarding questions of fire safety. A license or certificate may be denied or revoked in the event an applicant or licensee refuses to permit an investigation by these authorities if requested by a licensing or certifying representative, or if such authority advises, that a license or certificate not be issued.
- B. The following shall be required of all foster care homes:
1. There shall be an outdoor play space free from hazards of not less than 75 square feet per child in care who is between 12 months and five years of age. This area shall be fenced or otherwise protected. If the area is not fenced, outdoor play shall be supervised by the foster care provider or designee and a specific plan for how safety is to be assured shall be documented in the case file.
 2. The presence of firearms and ammunition is strongly discouraged in any home in which foster children are 'cared for. Any weapons such as firearms, air rifles, bows, hunting knives or hunting sling shots shall be unstrung and unloaded at all times when foster children are in the home and shall be stored in locked containers out of the reach of foster children. Ammunition and arrows shall be stored in separate locked containers. Firearms which are solely ornamental are excepted from the storage requirement. Weapons shall not be transported in any vehicle in which foster children are riding unless the weapons are made inoperable and inaccessible. Law enforcement professionals are exempted from the requirements of this section if conditions of their employment require them to carry weapons.
 3. At least 35 square feet of usable indoor space exclusive of halls, baths and sleeping area shall be available for each child. However, when a sibling group is placed together in a single foster home, a variance from the minimum space standards is permitted.' All floor space shall have carpets, tile or smooth finish which may be easily cleaned, interior walls shall be constructed of solid material and be free from holes. Unfinished basements must be inaccessible to foster children when safety hazards are present.
 4. Exterior doors shall be maintained in such a manner which would permit easy exit. Interior doors shall be designed to prevent children from being trapped.
 5. A basement which will be used regularly in the care of children in any manner shall be equipped with more than one exit; such exit may be an accessible window. See Section 7.708.31.E, for additional requirements for nighttime care.

6. The foster care home shall have a kitchen with a sink that has an operating faucet, refrigerator, stove, and oven, or a way to heat and/or reheat food. Provisions shall be made for the washing, rinsing, and storing of dishes in a safe and sanitary manner.
7. The foster care home shall have at least one (1) operating toilet, sink, and either a bathtub, and/or shower. Provisions shall be made for the privacy of the toilet, bathtub, and/or shower.
8. Toys and outdoor play equipment meeting the requirements of Section 7.708.31. D.2.a shall be available.
9. A comfortable bed or crib in a clean, well-ventilated room, which is not customarily used for other purposes such as a kitchen, dining room, hall, or bathroom, shall be available for all children/youth who are in foster care in the home. There shall be a minimum of forty (40) square feet of floor space for a bed for each child/youth in foster care, and the bed should be placed at least two (2) feet apart when arranged in parallel.
10. No foster care home shall be used for a rental income business an adult foster care facility. A business of a nature which might hazardous to the health, safety, morals or welfare of foster children shall not be operated on the premises of the foster home. In order to support youth with an independent living stipend, a foster care home may provide a home for a youth that previously resided in foster care in the home on or before the youth's eighteenth (18th) birthday. The youth shall solely occupy a bedroom and shall not occupy a bedroom with a child or youth in foster care. The foster care home may accept a negotiated portion of the independent living stipend. Negotiation shall include the youth, caseworker, and foster parent(s).
11. Mobile homes used as foster care homes shall have at least two (2) exits, be skirted, and properly installed and stabilized.
12. If there is a trampoline on the foster care home property, safety issues regarding its use must be agreed upon with the foster parents and addressed in writing in the case file.
13. Safety issues related to swimming pools shall be assessed by the certification worker, any state or local requirements addressed by the county or state health department as necessary, and documented in the case file.

Use and/or access to swimming pools, including in-ground, on-ground, and above-ground models, hot tubs and spas must be safe, appropriate, hazard-free, and consistent with the age, development, and abilities of the children/youth in the foster care home. The following are required:

- a. In-ground, on-ground, and above-ground swimming pools must meet state or local safety requirements and demonstrate compliance with recommendations from the Consumer Product Safety Commission. Swimming pools must meet the following requirements to ensure they are safe and hazard free.
 - 1) Swimming pools must have a barrier on all sides.
 - 2) Swimming pools must have their methods of access through the barrier equipped with a safety device, such as a bolt lock.
 - 3) Swimming pools must be equipped with a life saving device, such as a ring buoy, float, and/or a retrieval/extension pool pole that is available and easily accessible.
 - 4) Swimming pools that cannot be emptied after each use must have a working pump and filtering system.

b. Hot tubs/spas

Hot tubs/spas must meet state and local safety requirements.

- 1) Hot tubs/spas must have safety covers and must remain locked when not in use.
- 2) Hot tubs/spas must have a working pump and filtering system if the hot tub/spa cannot be emptied after each use.
- 3) When children/youth are using the hot tub/spa, the water temperature must not exceed 104 degrees Fahrenheit. The length of time that children/youth may be in a 104 degree water temperature in the hot tub/spa must not exceed the recommendations of the manufacturer or a licensed health care professional.
- 4) Only children/youth who can stand on the bottom with their head completely out of the water are allowed in the hot tub/spa without being held or supervised by an adult in the hot tub/spa. If the child/youth is unable to stand with their head completely out of the water, the child/youth must be held by an adult at all times that the child/youth is in the water.
- 5) An adult must provide direct supervision at all times when children/youth are in the hot tub/spa.

- C. Temporary wading pools that have a water depth of less than two (2) feet are exempt from pool and hot tub/spa requirements. Children/youth shall be supervised at all times.
- D. A certificate shall be denied, suspended, revoked or made probationary for failure to repair or otherwise comply with any of the preceding requirements when a defect or noncompliance with such requirement has been noted by the licensing or certifying representative and brought to the attention of the licensee or applicant in writing.

7.708.23 Foster Home Site [Rev. eff. 1/1/16]

- A. The foster care home must be located in an area that is accessible to health resources, public and private utilities, adequate and safe water supplies, sewage disposal, and fire and police protection.
- B. The foster care home must comply with local zoning department requirements.
- C. The entire premises of the foster care home are subject to inspection for licensing or certification purposes, including, but not limited to, the residence where care is to be provided, the grounds surrounding the foster care home, the basement, the attic (if accessible), any storage buildings, and a garage or carport, if applicable.
- D. The foster care home, including indoor and outdoor space, shall be maintained in a clean and safe condition free from hazards to health and safety.

7.708.24 Foster Home Maintenance [Rev. eff. 1/1/16]

- A. The foster care home shall be kept in good repair and maintained in a safe, clean, and sanitary condition.
- B. All areas of the foster care home available to foster children's activities including equipment, materials and furnishings shall be of sturdy, safe construction, easy to clean, and free of hazards, such as sharp points or corners, splinters, protruding nails, broken play and recreational equipment, or paint that contains lead or other poisonous materials and might be dangerous to the life or health of foster children.
- C. All areas of the foster care home shall be kept free from accumulation of significant amounts of non-essential materials such as furnishings, newspapers, or magazines that could pose a fire or health hazard.
- D. Provision shall be made for collection, storage, and disposal of trash to prevent infestation by rodents.

7.708.25 Fire Safety

- A. Fire hazards, such as defective electrical appliances and electric cords, dangerous or defective heating equipment or flammable material stored in such a manner as to create a risk of fire shall be corrected or eliminated.
- B. The foster care home shall contain at least one U.L.-approved fire extinguisher, highly visible, easily accessible, and in working condition, weighing not less than five pounds, that has a rating of 2A, 10BC. This requirement may be waived if more extensive fire-control measures are required by a local fire department.
- C. A smoke detector and a carbon monoxide detector or a combination smoke/carbon monoxide detector, in working condition, must be installed on each level of the foster care home and near sleeping areas.
- D. No gas space heaters, open-flame gas or oil stoves, hot plates, or un-vented heaters shall be used in the foster care home for heating purposes. No electric space heaters shall be used in the foster care home for permanent heating purposes.
- E. Flammables aerosol paints, insecticides, chemicals, and other dangerous materials shall be locked or stored so they are inaccessible to foster children and must be stored in areas separate from sleeping or living areas. Flammables shall be stored in an approved container.
- F. Heating devices such as radiators, registers, fireplaces, wood-burning stoves, and steam and hot water pipes that pose a fire or burn hazard to foster children shall be screened or otherwise protected.
- G. Flammable material must not be stored near a furnace, hot water heater, or other heating device.
- H. There shall be no candles or other burnable objects permitted in foster children's sleeping areas. Foster children shall not be permitted to smoke inside the foster care home or in any vehicle used to transport children.
- I. Exit doors shall be clearly identified to all foster children. No lock or fastening to prevent free escape from the inside of any room used by the foster children shall be permitted.
- J. Exit routes shall be kept free of discarded furniture, furnishings, laundry, and stacks of newspapers or magazines that could interfere with the prompt evacuation of the foster care home.
- K. A written emergency evacuation plan must be posted in a prominent place in the foster care home.

7.708.26 General Comfort and Safety

- A. All hazardous chemicals, tools, and other equipment, including matches, plastic bags, paints, gasoline, medicines, insecticides, and cleaning and laundry materials, shall be stored out of reach of young foster children. Products which could cause poisoning or contamination shall not be stored in areas where food is stored or prepared.
- B. Water from any source other than a regular municipal water supply shall be tested annually for compliance with water quality requirements.
- C. The foster care home shall be equipped with adequate light, heat, ventilation, and plumbing for safe and comfortable occupancy.
- D. The foster care home must be equipped with hot and cold running water.
- E. All stairways containing more than four steps shall be equipped with a handrail.
- F. The foster care home shall have immediate access to a working telephone, and emergency numbers shall either be posted in a prominent location, including those related to medical care, fire, law enforcement, and poison control, where available. Numbers for the agency or person having legal custody of each child/youth in foster care shall also be readily available.

7.708.3 REQUIREMENT FOR THE ONGOING OPERATION OF FOSTER CARE HOMES

7.708.31 Care of Children/Youth Who Are in Foster Care

- A. Foster parents shall provide supervision and care appropriate to each child's/youth's age, level of development, and ability to accept independence and responsibility.
- B. Within twenty-four (24) hours of arrival at the foster care home, a child/youth in foster care shall be given an orientation to the home consistent with the age of the child/youth and ability to participate, which includes at least the following:
 - 1. Tour of the home and instruction on fire alarm and emergency evacuation plan or procedures, escape routes and exits.
 - 2. The rules/regulations of the home.
 - 3. Procedures affecting the foster child's behavior, including limiting or restricting a foster child's rights where allowed, the type of discipline used in the foster care home, and consequences for certain behaviors.
 - 4. The complete foster children's rights and foster children's grievance procedures as developed by the foster care home or by the certifying authority.

C. Health Care

1. Concerning behaviors which are observed by foster parents shall be reported to the certifying authority and the child's/youth's caseworker so that appropriate care may be obtained.
2. Where pets or other animals are present, additional precautions shall be taken as required to insure both safety and good hygiene. Dogs and cats shall be vaccinated as required by State law or as designated by a veterinarian. If the governor or local government declares a disaster or emergency, and because of the declared disaster or emergency the pet vaccinations cannot be completed as required by law or veterinarian, the vaccinations must be completed as soon as possible, but no later than 45 calendar days after the declared conclusion of the disaster or emergency. Children/youth in foster care shall not be permitted to mistreat animals. Any animal that poses a threat to a child's/youth's safety or health must be confined in a place away from the child(ren)/youth in foster care.

D. Home Environment and Family Activity

1. It is the purpose of foster care to provide constructive family living experiences for children/youth during the period of placement.
2. Daily activities shall be designed to encourage normal physical, mental, social and emotional development of children/youth in foster care. This requirement shall be met in the following manner:
 - a. Materials and equipment appropriate for the age of children in care shall be available for both active and quiet play.
 - b. An effort shall be made to provide for contact and friendship between children in foster care and other children of a comparable age. Opportunities shall be provided for both group and individual play.
 - c. Children shall be encouraged to relate or to communicate with each other and with adults.
 - d. Outdoor activity shall be available to each child in foster care each day, weather permitting.
3. Foster parents or a designated representative from the certifying authority with knowledge of the child/youth shall attend Administrative Reviews for the children/youth in their care and participate in the planning for such children/youth. They shall receive a copy of the child's/youth's Family Services Plan for each child/youth in their care.

E. Nighttime Care Requirements

1. Children/youth in foster care shall be provided with a bed, cot, or crib as required by Section 7.708.22.B.8.
2. Two (2) sheets and suitable warm covering shall be provided to each child/youth. Sheets shall be changed weekly or more frequently if needed, and no child shall be allowed to remain sleeping in a wet bed.
3. Except for emergency placements, children in foster care over the age of eighteen (18) months shall not sleep in the same room as unrelated adults on a regular basis. Sleeping rooms for unrelated children/youth in foster care shall not be shared by children/youth in foster care of the opposite sex when one child/youth in foster care is over four (4) years old. Siblings of the opposite sex that are over the age of four (4) years and share bedrooms shall do so only with the written approval of the certifying authority and the county designee. The written documentation must be maintained in the child's/youth's and provider's file. Adolescent parents and their children may share a room.
4. Each child/youth in foster care shall be provided with adequate sleep wear and a complete set of clean sleep wear shall be available in the event that a change is necessary.
5. Sleeping rooms for children in foster care who are under five (5) years of age shall be near the bedroom of the foster parents or other responsible person. Monitoring systems may be utilized to ensure safety. Children in foster care who are under twelve (12) years of age shall not be permitted to sleep in a detached structure unless a responsible person sleeps in the same structure. Youth in foster care who sleep in a detached structure must have written approval of the county department that placed the youth, following an assessment of the youth's abilities and needs.

F. Infant Care

1. No more than two (2) children, whether birth, adoptive, or children in foster care under the age of two (2) years, shall be cared for in a foster care home, except under unusual circumstances such as multiple births.
2. In addition to the applicable provisions of paragraphs A through E, above, infant care shall include the following:
 - a. Infants shall be held during bottle feeding and at other times during the day. Infants shall not be confined but shall be allowed freedom of movement insofar as practical and shall be provided with an environment designed to stimulate their senses.
 - b. Diapers shall be changed as required and used diapers cleaned or disposed of consistent with the practices of good hygiene. Toilet training shall not be attempted with any child in foster care less

than eighteen (18) months of age and shall be done in a non-disciplinary manner.

3. Safe Sleep and Co-sleep

- a. Infants are children who are up to one (1) year of age. They must be placed to sleep in a crib, as identified by the Consumer Product Safety Commission. A bassinet, as identified by the Consumer Product Safety Commission, may be used in lieu of a crib, only until the infant reaches the maximum weight or age as identified by the bassinet manufacturer.
 - 1) Drop side cribs and stacking cribs are prohibited.
- b. Infants must not be placed to sleep on soft surfaces including, but not limited to, a bed, chair, futon, bean bag chair, couch, sofa, waterbed, pillow, quilt, sheepskin, or blanket.
- c. Infants must not be placed to sleep in the same crib as another infant or child.
- d. Infants must not sleep on the same sleep surfaces as the foster parent(s). Sleep surfaces include, but is not limited to, a floor, mattress, bed, sofa, recliner, or chair.
- e. A foster parent must not sleep while holding an infant. This includes, but is not limited to, while sitting on a recliner or couch.
- f. Soft bedding or materials that could pose a suffocation hazard are not permitted in cribs. Soft bedding means, but is not limited to, any soft sleep surface such as bumper pads or breathable mesh liners, pillows, blankets, quilts, comforters, sleep positioning devices, sheepskins, flat sheets, cloth diapers, bibs, plush toys, and stuffed animals.
- g. Infants who fall asleep in places other than a crib, including but not limited to, a car safety seat, bouncy seat, infant seat, activity infant swing (based on the manufacturer's recommendations), jumping chair, highchair, or any other piece of equipment not approved for sleep, must be moved to a crib as soon as reasonably possible. The child may not be left to sleep in the device overnight and must be supervised at all times.
- h. Infants must be placed on their back for sleeping unless there is a different recommendation from the American Academy of Pediatrics and the latter shall be followed.

- 1) Alternate sleep positions, and any alternate sleep surface must be used only with the written approval signed by the child's physician, physician assistant, or nurse practitioner.
- i. Before swaddling is used, the certifying agency must document in the foster parent's file that the foster parent completed the state's web-based safe sleep training. The certifying agency may require additional training as appropriate. Swaddling must be discontinued as soon as the infant tries to roll unswaddled, except where a health care professional provides written direction allowing swaddling to continue. A health care professional is a licensed physician, physician assistant, or a nurse practitioner.
- j. All sleep equipment must be safe, sturdy, and free from hazards including, but not limited to, broken or loose slats, a torn mattress, chipping paint, or loose screws.
- k. Approved sleeping equipment and mattresses must be firm and must fit snugly ensuring no more than two (2) adult fingers are able to be inserted between the mattress and the side of the approved sleeping equipment.

7.708.31.1 SUBSTITUTE CARE, RESPITE CARE, AND ALTERNATIVE CARE

- A. Substitute care is provided in the foster care home where the child/youth in foster care resides. Respite care is provided in another foster home. Alternative care is provided in the home of a natural support identified by the foster parent. These caregiving services provide opportunities for foster parents to have or take breaks of varying lengths. The following apply:
 1. At least 72 business hours' notice must be provided to the caseworker, gal and/or counsel for youth and child/youth. if an emergency or an urgent situation arises, the foster parent or the certifying agency shall provide notice to the caseworker, gal and/or counsel for youth, and child/youth as soon possible.
 2. Caregivers may not consent to activities requiring a consent form or safety gear for high risk activities as defined by the custodial county. Current procedures must be followed by the foster parent to obtain these permissions prior to the activity occurring.
 3. Caregivers must ensure that all requirements related to family time, sibling time, treatment for the child/youth placed in foster care, school for the child/youth placed in foster care, and contact between child/youth and county department caseworker and GAL, are met unless other arrangements are agreed upon and consistent with court orders.

- B. Substitute care in the foster care home occurs when a foster parent is unable to provide supervision and care. The foster parent shall arrange for a qualified substitute who is familiar with these rules and with the children/youth in foster care in order to provide temporary supervision and care to the children/youth in the identified foster care home.
1. If care is to be provided for up to eight (8) hours in the foster care home, the substitute care provider must be at least sixteen (16) years of age. Exceptions based on age and maturity of the potential substitute care provider and the individual needs of the child/youth placed in foster care can be made with concurrence of the foster parent and the certifying authority, but in no case should the provider of substitute care be less than fourteen (14) years of age. The age exception must be documented in the foster parent's file.
 2. If care is provided for more than eight (8) hours in the foster care home or for overnight care in the foster care home, the substitute care provider must be at least eighteen (18) years of age, currently certified in First Aid and CPR, and the following completed checks of the substitute care provider must be placed in the foster parent's file:
 - a. Colorado Bureau of Investigation (CBI);
 - b. Federal Bureau of Investigation (FBI);
 - c. Comprehensive Child Welfare Information System (CCWIS); and,
 - d. CBI sex offender name and address check and National Sex Offender Public Website name and address check.
 3. A copy of the substitute care provider's driver's license, vehicle registration, and insurance must be given to the certifying agency before the substitute care provider can drive the child/youth in foster care.
 4. Potential caregivers with background checks consistent with Section 7.701.33.d.7 cannot be used.
- C. Respite Care

Respite care is provided in a foster care home, other than the current foster care home where the child/youth in foster care resides. Respite care is used to allow the foster parent a temporary break from providing care. Respite care is also used when children/youth in foster care need a temporary break from their current foster care home.

1. a non-emergency respite care occasion may not exceed 30 calendar days, with a maximum of 60 days per calendar year. exceptions in excess of 60 days per year may be allowed and must be approved by the

certifying agency, caseworker, child/youth and guardian ad litem and/or counsel for youth.

2. Non-emergency respite care may not exceed the identified capacity of the respite foster care home.
 - a. A sibling group may be considered a single placement; and,
 - b. Space requirements in Section 7.708.22.b.8 apply.
3. Emergency respite care that causes the foster care home to exceed the identified capacity shall not occur for more than seven (7) consecutive days per month and not exceed 28 days in a calendar year. The respite foster care home may not exceed more than two (2) children/youth in foster care above their identified capacity and age range.
4. The respite foster care home must be in compliance with all other applicable rules regulating foster care homes.

D. Alternative Care is Provided Outside of the Foster Care Home

An alternative care provider is an individual with whom the foster parent has a personal association or relationship that is typically developed in the community. This is known as a natural support and it enhances the quality and security of life, including but not limited to, family relationships, friendships, and relationships developed through participation in clubs, organizations, and other civic activities.

1. Alternative care (natural support) may be provided outside of the foster care home for up to 72 consecutive hours with a maximum of seven (7) days a month, unless approved by the certifying agency, caseworker, child/youth. and gal and/or counsel for youth.
 - a. An alternative care (natural support) provider must be familiar with the child/youth placed in foster care; and,
 - b. Be at least eighteen (18) years of age.
2. Before alternative care (natural support) is used overnight, the foster parent shall introduce the alternative care provider and the child/youth placed in foster care and arrange for the child/youth to visit the alternative care provider's home.
3. An alternative care (natural support) consent form (state prescribed) must be completed between the foster parent and the alternative care provider prior to the care being provided to the child/youth.
4. The certifying agency must review the consent form and complete background checks for all adults residing in the home in Colorado Courts

(state judicial database), CBI sex offender name and address check, and National Sex Offender Public Website name and address check. A CBI and FBI fingerprint-based criminal history record information check is required only if the adult has resided in Colorado less than five (5) years.

- a. When the county department is the certifying agency, staff will complete a child abuse/neglect background check in the CCWIS for the alternative care (natural support).
 - b. When the foster parent is certified by a child placement agency, the alternative care (natural support) must submit a request for a CCWIS check from the Background Investigation Unit.
5. A copy of the alternative care (natural support) provider's driver's license, vehicle registration, and insurance must be given to the certifying agency before the alternative care provider can drive the child/youth.
 6. Potential alternative care (natural support) with background checks consistent with Section 7.701.33.d.7 cannot be used.

7.708.32 Suspected Child Abuse

Where child abuse is suspected, the foster parent shall be alert for evidence of signs of abuse and report such evidence promptly to the county department of social/human services. A written report of any external signs of injury, such as bruising, scratching or swelling, shall be placed in the foster child's record. If there is any suspicion of abuse or illness, the foster child shall be seen by medical personnel immediately.

7.708.33 Foster Children's Rights

- A. The certifying authority shall have written policies and procedures that address and ensure the availability of each of the following core rights for foster children in residence. These rights may not be restricted or denied by the foster care home or certifying authority. Every foster child has the right to:
 1. Enjoy freedom of thought, conscience, cultural and ethnic practice, and religion.\
 2. A reasonable degree of privacy.
 3. Have his or her opinions heard and considered, to the greatest extent possible, when any decisions are being made affecting his/her life.
 4. Receive appropriate and reasonable adult guidance, support and supervision.

5. Be free from physical abuse or neglect and inhumane treatment. Every foster child has the right to be protected from all forms of sexual exploitation.
 6. Receive adequate and appropriate medical care.
 7. Receive adequate and appropriate food, clothing, and housing.
 8. Live in clean, safe surroundings.
 9. Participate in an educational program that will maximize his/her potential in accordance with existing law.
 10. Communicate with "significant others" outside the foster care home, such as a parent or guardian, caseworker, attorney or guardian ad litem and/ or counsel for youth, current therapist, physician, religious advisor, and, if appropriate, probation officer.
- B. The following foster children's rights may be limited; to reasonable periods during the day or restricted according to routine of the foster care home to ensure the protection of the foster children and foster family. Every foster child has the right to:
1. Have access to letter-writing materials, including postage, and to have a foster parent(s) assist him/her if unable to write, prepare, and mail correspondence.
 2. Have access to telephones to both make and receive calls in private.
 3. Have convenient opportunities to meet with visitors.
 4. Wear his/her own clothes, keep and use his/her own personal possessions, and keep and be allowed to spend a reasonable sum of his/her own money.
 5. Receive and send sealed correspondence.
- C. Foster care homes must develop a plan, in conjunction with the certifying authority, regarding the following rights of foster children and these rights must be explained to the foster children upon admission. The notification must be communicated in a language or mode of communication the foster child can understand. There must be plans for:
1. How and when telephone, including cell phone use, and written communications, including, but not limited to, social networking and other electronic communication, will take place.

2. Extenuating circumstances and emergency situations affecting the foster child and his/her family.

7.708.34 The Prohibited Use of Cruel and Aversive Therapy [Rev. eff. 1/1/16]

The foster care home shall refrain from engaging in all cruel and aversive behavior management, treatment or therapy including, but not limited to, the following:

- A. Any intervention designed to or likely to cause a foster child physical pain
- B. Releasing noxious, or toxic, sprays, mists, or substances in proximity to the foster child's face.
- C. Any intervention that denies a foster child sleep, food, water, shelter, access to bathroom facilities, adequate bedding, or appropriate physical comfort.
- D. Any intervention or type of treatment that subjects a foster child to verbal abuse, ridicule, humiliation or that can be expected to cause excessive emotional trauma.
- E. Interventions that use a device, material, or object that is designed to simultaneously immobilize all four of the foster child's extremities.
- F. Any treatment intervention that deprives a foster child of the use of his/her senses, including sight, hearing, touch, taste, or smell.
- G. Physical management, restraint and seclusion except as described at Sections 7.708.36 and 7.714.53.
- H. Use of rebirthing therapy or any therapy technique that may be considered similar to rebirthing therapy as a therapeutic treatment, as defined by Section 12-43-222(1)(t)(IV), C.R.S.

7.708.35 Discipline [Rev. eff. 1/1/16]

- A. The foster care home or certifying authority shall have written policies and procedures regarding discipline that must be explained to all foster children, parent(s), guardian(s), staff, and placing agencies. These policies must include positive responses to a foster child's appropriate behavior.
- B. Discipline shall be constructive or educational in nature and may include talking with the foster child about the situation, praise for appropriate behavior, diversion, separation from the problem situation, and withholding privileges.
- C. Basic rights shall not be denied as a disciplinary measure.
- D. Separation when used as discipline must be brief and appropriate to the foster child's age and circumstances. The foster child shall always be within hearing of

an adult in a safe, clean, well-lighted, well-ventilated room in the foster care home that contains at least fifty (50) square feet of floor space. No foster child shall be isolated in a bathroom, closet or pantry.

- E. Foster children in care at the foster care home shall not discipline other foster children.
- F. A foster care home shall prohibit all cruel and unusual discipline including, but not limited to, the following:
 - 1. Any type of physical hitting or any type of physical punishment inflicted in any manner upon the body of the foster child, such as spanking, striking, swatting, punching, shaking, biting, hair pulling, roughly handling a foster child, striking with an inanimate object, or any humiliating or frightening method of discipline to control the actions of any foster child or group of foster children.
 - 2. Discipline that is designed to, or likely to, cause physical pain.
 - 3. Physical exercises such as running laps, push-ups, or carrying heavy rocks, bricks, or lumber when used solely as a means of punishment.
 - 4. Assignment of physically strenuous or harsh work that could result in harm to the foster child.
 - 5. Requiring or forcing a foster child to take an uncomfortable position such as squatting or bending, or requiring a foster child to stay in a position for an extended length of time such as standing with nose to the wall, holding hands over head, or sitting in a cross-legged position on the floor, or requiring or forcing a foster child to repeat physical movements when used solely as a means of punishment.
 - 6. Verbal abuse or derogatory remarks about the foster child his/her family, his/her race, religion, or cultural background.
 - 7. Denial of any essential/basic program service solely for disciplinary purposes.
 - 8. Deprivation of meals or snacks, although scheduled meals or snacks may be provided individually.
 - 9. Denial of visiting or communication privileges with family, clergy, attorney, or caseworker solely as a means of punishment.
 - 10. Releasing noxious, toxic, or otherwise unpleasant sprays, mists, or aerosol substances in proximity to the foster child's face.
 - 11. Denial of sleep.

12. Requiring the foster child to remain silent for a period of time inconsistent with the foster child's age, developmental level, or medical condition.
13. Denial of shelter, clothing or bedding.
14. Withholding of emotional response or stimulation.
15. Discipline associated with toileting, toileting accidents or lapses in toilet training.
16. Sending a foster child to bed as punishment. This does not prohibit a family foster care home from setting individual bed times- for foster children.
17. Force feeding a foster child.
18. Physical management, restraint and seclusion.

7.708.36 Physical Management and Seclusion [Rev. eff. 1/1/16]

- A. Seclusion is prohibited.
- B. Physical management to attain and maintain control or for behavior management, treatment, or therapy is prohibited and/or seclusion is prohibited, unless the foster care home is pre-approved by the certifying authority to perform physical management and/or seclusion and the foster care home is in compliance with Section 7.708.61, K, 2, Section 7.714.53, et seq., and the applicable definitions found in Section 7.714.1. The foster care home must notify the placing caseworker when a child is subject to physical management and/or seclusion.

7.708.37 Religion [Rev. eff. 1/1/16]

- A. The foster care home shall demonstrate consideration for, and sensitivity to, the religious backgrounds of foster children in care. The foster care home shall assist a foster child's involvement in religious activities appropriate to the foster child's religious background and based upon the needs and interests of the foster child.
- B. Foster children in care at the foster care home shall be allowed and encouraged to celebrate their religious holidays.
- C. Opportunity and assistance shall be provided for each foster child to practice the chosen/preferred religious beliefs and faith of his/her family. If the family has no preference, the individual preference of the foster child shall be respected. This includes, but is not limited to, making necessary arrangements for attendance of foster children at the appropriate religious institution or at a study group for religious instruction.

- D. A foster child may be invited to participate in religious activities of the foster care home.
- E. A foster child shall not be coerced or forced to participate in the religious activities of the foster care home or to attend religious services.
- F. Any form of religious intervention used by the foster care home to control or change a foster child's behavior, or treat or heal a medical condition, must be approved, in writing, by the legal guardian(s) of the foster child prior to the use of the intervention.
- G. A foster care home cannot deny medical care to a foster child because of religious beliefs.
- H. The foster child's family and/or guardian must be consulted prior to any planned change in religious affiliation made by the foster child while he/she is in care at the foster care home.

7.708.38 Education [Rev. eff. 1/1/16]

- A. Foster children shall attend educational/vocational programs in the most appropriate and least restrictive educational setting for the foster child, including, but not limited to, attending regular classes conducted in accredited elementary, middle, and secondary schools within the community.
- B. Regular school attendance or an educational plan is required for each foster child according to school attendance laws; a suitable, quiet, well-lighted place for study shall be provided together with necessary books, papers, pencils and other equipment which are reasonably required by school-age children. Foster parents shall review grade reports and other information received from teachers or school authorities with foster children in care and shall counsel and assist foster children regarding adequate classroom performance. The parent, guardian or authority with responsibility for the foster child shall be advised of school performance. Reasonable efforts shall be made to involve a foster child in extracurricular activities. The foster parents shall attend school staffings, conferences, and Individualized Educational Plan meetings when possible.
- C. Foster children attending school shall be permitted to participate in school extracurricular activities to the extent of their interests and abilities and in accordance with each individual foster child's plan.
- D. In order to ensure that all students who may have disabilities are provided an appropriate education, the foster care home, in cooperation with the certifying authority, shall ensure that adequate "Child Find" procedures are utilized. Such procedures shall be developed cooperatively with Local Education Agencies (LEA) in accordance with Exceptional Children's Educational Act rules and regulations and LEA procedures. Child Find includes a process for screening,

referring, assessing and staffing students suspected of having a disabling condition.

7.708.39 Community Participation [Rev. eff. 1/1/16]

- A. Community activities shall be supported. The foster parent shall receive training in how to determine whether to approve a child's or youth's participation in an extracurricular, enrichment, cultural, or social activity, based upon the criteria in Section 7.701.200.
- B. The foster care home shall consider and be sensitive to identity of the child and/or youth, including, but not limited to, cultural, spiritual and/or religious needs. The foster care home shall involve a foster child in activities appropriate to his/her identity, including, but not limited to, consideration of the child's family, community, neighborhood, school activities, friends, and the child's and family's primary language.
- C. The foster care home shall utilize available services, facilities, and activity programs of the community, and foster children shall be given opportunities to participate as individuals or as a group in agency-sponsored recreational and cultural programs.
- D. With the approval of the certifying authority, the foster care home may deduct reasonable sums from a foster child's allowance as restitution for damages done by the foster child. Restitution must be negotiated with the foster child and based on the foster child's ability to pay. A written record of damages and any restitution paid by a foster child must be maintained by the foster care home and certifying authority.
- E. Chores at the foster care home are considered part of the participatory responsibility of living together. They shall provide constructive experiences in accordance with the age and ability of the foster child.
- F. All chores shall be scheduled so as not to conflict with other essential scheduled activities.
- G. The foster care home shall comply with all child labor laws and regulations in making work assignments, with consideration for agricultural work assignments in those communities.
- H. Paid or voluntary work assignments outside of the foster care home shall be approved by foster parent(s) and the county designee for the foster child, who shall know the employer, the specific type of work, and the conditions of employment.
- I. A foster child shall not be exploited. A foster child may not participate in solicitation on behalf of the foster care home or certifying authority for a fund-raising activity without the written permission of the parent(s) or guardian(s) for

each specific activity, and the foster child must be willing to participate in the activity.

7.708.4 PERSONAL CARE AND SAFETY OF THE FOSTER CHILD

7.708.41 Medical and Health Services

- A. A general medical examination for each foster child/youth must be completed or scheduled with a physician or a nurse practitioner prior to or within fourteen (14) calendar days following placement at the foster care home. If the governor or local government declares a disaster or emergency, and because of the declared disaster or emergency the medical exams cannot be completed for the child/youth in the required time frame, the medical exam must be completed as soon as possible, but no later than 45 calendar days after the declared conclusion of the disaster or emergency. If the child/youth has received a medical examination within the previous twelve (12) months, another medical examination is not required during that year as long as the foster home has written documentation of the previous medical exam. A statement from the examiner shall be retained in the foster child's/youth's file. This exam shall include the following:
 - 1. An examination for physical injury and disease.
 - 2. Vision and hearing screening.
 - 3. A current assessment of the foster child's/youth's health, including immunizations.
- B. Whenever indicated, a foster child/youth shall be referred to an appropriate specialist for either further assessment or treatment.
- C. Subsequent physical and other examinations shall be done annually or as directed, in writing, by the physician or other qualified health professional.
- D. Dental examinations, appropriate to the age of the foster child/youth, must have been completed within four months prior to placement or scheduled or completed within eight (8) weeks following placement. The foster care home or governing body shall ensure that each foster child/youth receives a dental examination every six months or as required in writing by a dentist. If the governor or local government declares a disaster or emergency, and because of the declared disaster or emergency the dental exams cannot be completed for the child/youth in the required time frame, the dental exam must be completed as soon as possible, but no later than 45 calendar days after the declared conclusion of the disaster or emergency.
- E. At all times there shall be First Aid supplies readily available at the foster care home. Each foster parent must be certified in first aid, or the equivalent, and CPR for all ages of foster children/youth in care.

1. If the governor or local government declares a disaster or emergency, and because of the declared disaster or emergency, the foster parent(s) cannot take the first aid class in a classroom with the first aid trainer, the first aid training may be completed online. The foster parent(s) must then complete the classroom training with first aid trainer as soon as possible, but no later than 45 calendar days after the declared conclusion of the disaster or emergency.
 2. If the governor or local government declares a disaster or emergency, and because of the declared disaster or emergency, the foster parent(s) cannot take the cpr class in a classroom with the cpr trainer, and the foster parent(s) has successfully completed a cpr class within the last five (5) years, the foster parent(s) may take the cpr class online. The foster parent(s) must then complete the classroom training with a cpr instructor as soon as possible, but no later than 45 calendar days after the declared conclusion of the disaster or emergency.
- F. The foster care home, in conjunction with the parent(s) or guardian(s), shall make every effort to ensure that a foster child/youth needing corrective devices such as glasses, hearing aids, etc., is provided with the necessary equipment. The placing authority for the foster child/youth shall assist with obtaining resources as necessary to fulfill this requirement.
- G. The foster care home has the right to request a statement regarding the foster child's/youth's general health from a medical examiner. In a potentially life-threatening situation, the foster care home shall refer the foster child's/youth's care to the appropriate medical and legal authority. If a foster child/youth wishes an exemption from a medical examination or medical treatment due to religious beliefs, the foster child/youth shall submit a written statement signed by his/her parent(s) or guardian(s) which states the reasons for such an exemption. The foster care home has the right to refuse admission to a foster child/youth whose parent(s) or guardian(s) refuses medical treatment or examination based upon religious convictions.
- H. Foster parents shall be aware of and shall observe foster children for signs of illness or disease and shall respond to and care for a foster child suffering from illness, accident or injury. If contagious, the foster child affected should be isolated from other children in the home and made as comfortable as possible. First aid care shall be provided as required. If additional care, medical attention or removal from the home is indicated, the appropriate person with responsibility for the foster child shall be contacted and medical assistance shall be obtained without undue delay. A written record of any illness or injury to a foster child shall be retained in each foster child's individual record.

- I. The foster care home shall regularly maintain and update a foster child's Human Services Health Passport, or a document containing all the information listed in the Health Passport, for foster children placed by a county department of human or social services. This document is to be photocopied regularly and submitted to the foster child's caseworker. The original of the document shall be given to the caseworker upon the foster child's discharge so that it can be given to the foster care home where the foster child is being admitted or to the foster child's parent(s), guardian(s), or family member(s) with whom the foster child is placed.
- J. Medications shall be administered and stored in the following manner
 - 1. When a foster child first goes into care, the foster care home shall ascertain all medication the foster child is currently taking.
 - 2. All medication must be kept in a clean storage area inaccessible to foster children and stored according to pharmacy instructions.
 - 3. All prescriptive medications shall be administered only upon the written prescription of a physician. The foster care home shall also obtain written authorization from the prescribing physician to administer any non-prescriptive medication.
 - 4. In an emergency situation, non-prescriptive medication may be administered on the verbal authorization of a physician. Written confirmation must then be obtained for the verbal authorization.
 - 5. The foster care home shall maintain for each foster child a cumulative record of all medication, both prescriptive and non-prescriptive dispensed to that foster child, including:
 - a. The name of the foster child.
 - b. The name and dosage of medication.
 - c. The time and date the medication was dispensed.
 - d. The name or initials of the person administering the medication.

7.708.42 Food and Nutrition

- A. The foster care home shall provide nutritious foods in the variety and amounts as appropriate for the age, appetite, and activity of each foster child in care.
- B. At least three nourishing, wholesome, well-balanced meals a day shall be offered at regular intervals except when foster children receive their morning and/or noon meal(s) at school. No more than fourteen (14) hours shall elapse between the evening and morning meals. Nourishing snacks shall be part of the daily food provided.

- C. Family meals including all children and adults present in the home shall be provided whenever possible.
- D. Foster children shall be encouraged to eat a variety of the food served but shall not be subjected to undue coercion, including forced feeding, or punished for refusal to eat.
- E. All food shall be from sources approved or considered satisfactory by the health authority. All foods shall be stored, prepared, and served in such a manner as to be clean, wholesome, free from spoilage, and safe for human consumption. Only pasteurized milk shall be served. Fruits, vegetables and meats may be frozen.
- F. There shall be a record made of the special diets prescribed and prepared for a foster child.
- G. Foster children must not be given foods that are contrary to their religious beliefs, or of their family, or are known to cause an allergic reaction or a health hazard.
- H. Water shall be readily accessible to foster children.
- I. Common drinking cups shall not be permitted.

7.708.43 Personal Hygiene and Daily Routine [Rev. eff. 1/1/16]

- A. The foster care home shall ensure that foster children receive training in good habits of personal care, hygiene, and grooming appropriate to their age, gender, and identity.
 - 1. There shall be supervision by foster parents to provide for proper grooming and physical cleanliness of the foster children.
 - 2. The foster care home shall ensure that foster children are provided with necessary and appropriate toiletry items, including clean, individual towels and washcloths, toothbrush, toothpaste, comb, hair brush, soap, skin lotion, and shampoo.
 - 3. Foster children shall be encouraged or assisted to maintain cleanliness or good hygiene: teeth shall be brushed each day and more frequently when possible.
- B. The foster care home shall have basic daily routines for foster children in care.
 - 1. Daily routines shall not be allowed to conflict with the implementation of a foster child's Family Services Plan.
 - 2. Daily routines shall be established for mealtimes, waking, and bedtimes.
 - 3. Opportunity for physical exercise shall be planned for each foster child.

7.708.44 Clothing and Personal Belongings [Rev. eff. 1/1/16]

- A. The foster care home shall allow a foster child in care to bring his/her personal belongings to the program, as defined by the foster care home policy, and to acquire belongings of his/her own. However, the foster care home shall, as necessary, limit or supervise the use of these items while the foster child is in care. Where extraordinary limitations are imposed, the foster child shall be informed of the reasons, in a language or manner of communication the foster child can understand.
- B. The foster care home shall ensure that each foster child in care has adequate clean, proper-fitting, attractive, and seasonable clothing as required for health, comfort, and physical well-being and as appropriate to age, gender, individual needs, and identity.
 - 1. Each foster child's clothing shall be distinguished as his/her own.
 - 2. A foster child's clothing shall be kept clean and in good repair. The foster child shall be involved, as appropriate, in the care and maintenance of his/her clothing. As appropriate, laundering, ironing, and sewing facilities shall be accessible to the foster child.
- C. The foster care home in conjunction with the placing authority shall ensure that discharge plans make provisions for clothing needs at time of discharge. The wardrobe for each foster child shall go with him/her at time of discharge.

7.708.45 Emergency Drills [Rev. eff. 1/1/16]

- A. There shall be a plan for foster parent(s) and foster children to follow in case of emergency or disaster. The plan shall include provisions for roles and responsibilities during an emergency, evacuation of the foster care home and the assignment of a central meeting place where each individual may be accounted for.
- B. Fire exit drills must be held often enough so that all occupants are familiar with the drill procedure and their conduct during a drill is a matter of established routine.
- C. Drills must be held at unexpected times and under varying conditions to simulate the conditions of an actual fire.
- D. Drills must emphasize orderly evacuation under proper discipline rather than speed. Running or horseplay shall not be permitted.
- E. Drills must include suitable procedures for ensuring that all persons in the foster care home actually participate.
- F. A record of fire drills must be recorded by the foster care home.

- G. Smoke alarm devices shall be regularly used in the conduct of drills.
- H. The foster care home shall make special provisions for the evacuation of any foster child with a disability in the foster care home.
- I. The foster care home shall take special care to help emotionally disturbed or perceptually handicapped foster children understand the nature of such drills.
- J. If appropriate to the location of the foster care home, tornado drills must be held often enough so that all occupants are familiar with the drill procedure and conducting a drill is a matter of established routine. A record of tornado drills must be recorded by the foster care home.

7.708.46 Transportation [Rev. eff. 1/1/16]

- A. A foster care home or certifying authority shall ensure that each foster child is provided with the transportation necessary for implementing the foster child's family service plan.
- B. A foster care home shall have means of transporting foster children in cases of emergency.
- C. Any vehicle used by the foster care home in transporting foster children in care, whether such vehicle is operated by a foster parent or any other person acting on behalf of the foster care home, shall be properly licensed, and the vehicle shall be maintained in accordance with Colorado law.
- D. Any foster parent or other person acting on behalf of the foster care home operating a vehicle for purpose of transporting foster children shall be properly licensed to operate the class of vehicle in accordance with Colorado law.
- E. Foster children must be properly fastened into a restraint system that conforms to all applicable Federal Motor Vehicle Safety Standards and that are pursuant to Colorado law.
- F. A foster care home shall not allow the number of persons in any vehicle used to transport foster children to exceed the number of available seats in the vehicle.
- G. The vehicle shall be enclosed and provided with door locks.
- H. A foster care home shall ascertain the nature of any need or problem of a foster child which might cause difficulty during transportation, such as seizures or a tendency toward motion sickness. The family foster care home shall communicate this information to the driver of any vehicle transporting foster children in care.

7.708.5 RECORDS AND REPORTS

7.708.51 Records

- A. The foster care home, in conjunction with the certifying authority, shall maintain complete records as required for the licensing or certification of the foster care home in accordance with the rules regulating foster care homes.
- B. Records for foster children shall be retained for at least three years. Retention of records for a longer period may be desirable when they reflect an accident, injury or other unusual circumstance.
- C. A record of admission shall be completed for each foster child in care prior to or at the time of placement. The admission record shall be maintained at the foster care home where the foster child resides and shall contain:
 - 1. Foster child's name, date and place of birth (verified by a birth certificate when possible), gender, race, religious preferences of parent(s) or foster child, date and reason for placement.
 - 2. Foster child's address and telephone number, parent(s) or guardian(s) address and telephone number if different from the foster child.
 - 3. Name, address, day and nighttime telephone number of individual or agency placing the foster child with the name of individual arranging the placement.
 - 4. Any documents pertaining to the foster child's legal status such as court orders, including the appointment of a Guardian ad litem and/or counsel for youth, legal guardianship, or custody agreements.
 - 5. A copy of the placement agreement pursuant to 7.708.61, K.
 - 6. Health records including a health history, chronic medical problems of the foster child, illnesses the foster child has had during the last six months and a complete list of all medications the foster child is taking.
- D. Each foster child's file shall also include:
 - 1. Current medical and dental reports, accident, injury, or illness reports, record of medication administered and necessary medical care provided to the foster child while in placement.
 - 2. Copies of educational records and reports of school work, including scholastic performance, certificates of achievement or award, copies of school pictures, extracurricular interests.
 - 3. The foster child's Family Services Plan, a summary of the periodic evaluations of the foster child's progress and resultant changes in the Family Services Plan.

4. Psychiatric and psychological summaries, when available.
5. Summary recording of significant contacts with parent(s), guardian(s) and other involved agencies.
6. If requested by the provider, a written notice to employees of the Department of Human Services and of county departments or other individuals with a need to know, if the foster parents do not want personally identifiable information provided to adult members of the foster child's family. Written notice may be subsequently provided to the parties aforementioned for release of personally identifiable information to the foster child's family which shall include the consent to release information, the foster parent's signature, and the date.

7.708.52 Reports [Rev. eff. 1/1/16]

The following information shall be reported consistent with the requirements in Section 7.701.52 (12 CCR 2509-8).

- A. The foster care home shall immediately notify the foster child's parent(s), guardian(s), and/or the responsible agency of any serious illness or serious injury resulting in medical treatment away from the foster care home, hospitalization or death involving a foster child in care.
- B. The foster care home shall notify the parent(s), guardian(s), or placing authority as soon as possible upon discovery that a foster child has run away.
- C. A report about a death must include:
 1. The foster child's name, birth date, address, and telephone number.
 2. The names of the foster child's parent(s) or guardian(s) and their address and telephone number if different from that of the foster child.
 3. Date of the fatality.
 4. Brief description of the incident or illness leading to the death.
 5. Names and addresses of witnesses or persons who were with the foster child at the time of death.
 6. Name and address of police department or authority to whom the report was made.
- D. The foster care home shall notify the certifying authority of any change in the status of the foster care home, police intervention or moving traffic violations that could affect care and safety of foster children.

7.708.6 CERTIFYING AUTHORITY REQUIREMENTS

7.708.61 Admission Requirements [Rev. eff. 1/1/16]

- A. Admission of a foster child to a foster care home shall be in keeping with the stated purpose of the foster care home and shall be limited to those foster children for whom the foster parent(s) is qualified and by the needs of foster children already in residence to provide the care necessary. Care must be provided in the least restrictive, most appropriate setting in order to meet the foster child's needs.
- B. Each foster care home or its certifying authority shall have a written admission policy which at a minimum must include:
 - 1. The policies and procedures related to intake.
 - 2. The age range and sex of foster children accepted/admitted for care.
 - 3. The needs, problems, situations or patterns best addressed by the foster care home.
 - 4. Any pre-placement requirements for the foster child, the parent(s) or guardian, and/or the placing agency.
 - 5. The anticipated problems or situations that would result in the foster care home or certifying authority requesting removal of a foster child from placement prior to the planned discharge.
- C. The written description of admission policies and criteria shall be provided to referring agencies.
- D. Information regarding the prospective foster child shall be discussed with the foster parent(s) as early as possible prior to placement. The foster care home shall accept a foster child into care only after a preliminary assessment/screening of presenting problems in areas such as social, physical health, mental health, psychological concerns, previous physical or sexual abuse, and concerns about previous delinquent, assaultive, or destructive behavior, if appropriate, has been conducted.
- E. It is desirable for the foster child to visit and become familiar with the foster parents, the foster care home and other persons living in the home prior to placement.
- F. For each foster child referred for placement, the foster care home shall be provided with a current comprehensive intake assessment, including a social, health, and family history, developmental assessment, mental health, and a psychological summary, if determined to be necessary by the foster care home or certifying authority. Educational records shall be provided if appropriate. As

much of this information as possible shall be provided to the foster parent prior to admission, but the total assessment shall be completed by the placing authority within one month after admission. If the foster care home or certifying authority is unable to obtain this information within these time periods or is totally unable to obtain the information, the certifying authority must document its attempts to obtain the information and reasons for not obtaining the information.

- G. At the time of placement, the foster parent(s) shall be provided with a record of admission as outlined in Section 7.708.51.C. The Medicaid card shall be given to the foster parent(s) for Medicaid eligible foster children as soon as possible after placement. If a foster child is placed at the foster care home as an emergency placement, the foster care home shall be provided with at least the following information: name, birth date, if available, and physical description of the foster child, date and time of the admission; name, address telephone number and authority of person bringing the foster child to the foster care home, and the reason for placement. Any other information that may be available should be recorded at the time of placement or as it becomes available. The date that placement terminates shall also be recorded.
- H. For all placements of foster children, previous medical records should be obtained and pertinent information from those records, including immunization records shall be given to foster parents within four weeks of the initial placement. The medical history shall contain, to the maximum degree possible, the information listed in the Colorado Department of Human Services Health Passport.
- I. Preparation of the foster child for admission shall be in a manner consistent with the foster child's age and ability to participate in the plan and to understand the reason for the placement.
- J. Prior to placement of the foster child at the foster care home, the, guardian(s), and/or placing agency must be notified of the religious practice, philosophy, and affiliation of the foster care home.
- K. The placement agreement shall be developed with the involvement of the foster child, the parent(s) or guardian(s) and the representative of the placing agency. Where the involvement of any of these is not feasible or desirable, the reasons for the exclusion shall be recorded by the certifying authority. The placement agreement shall include by reference or attachment at a minimum the following:
 - 1. Discussion of the foster child's and the parent's or guardian's expectations regarding: family contact and involvement: how family contact and involvement are to occur; the nature and goals of care, including any specialized services or specialized treatment to be provided: the religious orientation and practices of the foster child and, or family; and the anticipated planned discharge date and plan for the foster child following discharge.

2. The policy and procedure to be followed regarding the use of physical management in an emergency pursuant to Section 7.714.53, et seq., and Section 7.708.36.
3. A delineation of the respective roles and responsibilities of all agencies and persons involved with the foster child and his/her family.
4. Written authorization for care and treatment of the foster child.
5. Written authorization to obtain routine medical and dental care for the foster child and to obtain emergency medical and dental care.
6. Legal status or custody of the foster child.
7. If a foster child is being placed by a Colorado county department of human or social services, the appropriate state form or contract shall be completed. This form or contract may provide some of the required authorizations and shall require that the foster parent is trained in how to apply the reasonable and prudent parent standard.

7.708.62 Foster Children's Grievance Procedure [Rev. eff. 1/1/16]

The certifying authority must establish a written grievance procedure that provides adequate due process safeguards, spells out the appeal process, and assures that foster children and parent(s) or guardian(s) are entitled to report any grievance and shall not be subject to any adverse action as a result of filing the grievance.

- A. The foster care home and/or certifying authority for the foster care home must follow grievance procedures without alteration, interference, or unreasonable delay.
- B. If a grievance is filed with the foster care home, the grievance shall be recorded in the foster child's official case record along with the investigation findings and resulting action taken by the foster care home or certifying authority. Information regarding the grievance must be sent to the individual or agency holding legal custody of the foster child.

7.708.63 Comprehensive Program for Medical Care for the Foster Child [Rev. eff. 1/1/16]

The certifying authority shall ensure the availability of a comprehensive program of preventive, routine, and emergency medical and dental care for each foster child in care. Every reasonable effort shall be made to obtain routine and corrective dental care. The certifying authority shall have a written plan for providing such care. This plan shall include at a minimum:

- A. Ongoing appraisal of the general health of each foster child, including immunizations, in accordance with state law and regulations.

- B. Procedures for obtaining diagnostic services, emergency care, including the availability of emergency medical care on a 24-hour, seven-day-a-week basis, corrective care, recuperative care, and immunization updates.
- C. Provision of health education, which includes sex education, and birth control information and education, age appropriate to the foster child.
- D. Provision that any medical treatment administered will be explained to the foster child in a language or manner of communication understandable to him/her.
- E. The provision of dental care by a Colorado-licensed dentist, who is available to the foster care home.
- F. Procedures for dispensing medication, storage of medication, documentation of administration of all medication, disposing of medications when not needed or no longer in use, and notification to a primary physician in cases of medication errors and/or drug reactions.

7.708.64 On-Going Health of Foster Care Parents [Rev. eff. 1/1/16]

- A. A certifying authority shall not certify or continue to certify any person whose health, or emotional or psychological makeup impairs his/her ability to properly protect the health and safety of foster children.
- B. A certifying authority shall not allow a foster parent provide care for foster children if the foster parent, upon examination or as a result of tests, shows indication of a physical condition which could be hazardous to a foster child, or self, or which would prevent performance of duties.
- C. If in the opinion of a licensed health care professional or licensed mental health practitioner, a medical, emotional or psychological condition exists at any time which may jeopardize the health and/or safety of foster children or adversely affect the ability of foster home parents to care for such foster children, the issuance of a certificate shall be conditioned upon the satisfactory report of the licensed health care professional and, if foster children are in care, the certifying authority shall contact the appropriate social/ human services personnel to make satisfactory arrangements for the temporary care of foster children.
- D. The unfavorable report from any medical evaluation concerning the physical, mental health or emotional stability of any foster care parent or applicant must be evaluated and may be grounds for denial, revocation or making probationary of a foster care certificate.

7.708.65 Orientation, Training, and Certification [Rev. eff. 1/1/16]

- A. The certifying authority shall have a comprehensive written plan for the orientation, pre-certification training, certification, and ongoing training of foster parents.

1. The certifying authority shall have an introductory training and orientation program for all foster parents. This program shall include, at a minimum, twenty-seven hours of initial core training consisting of at least twelve (12) hours prior to placement of a child and the remaining hours to be completed within 3 months after placement, including the reasonable and prudent parent standard, orientation to emergency and safety procedures and the general and specific duties and responsibilities of being a foster parent.
 2. If a child is placed with a foster care family on an emergency basis, twelve (12) hours of core training must be completed and the remaining hours of training completed within 4 months from the date of placement. An emergency means that a child's safety is subject to actual or likely harm, immediate or emerging, serious or severe, which requires control.
 3. The certifying authority shall maintain written documentation of specific in-service training held, foster parents participating, the hours involved, and/or other on-going training activities in which foster parents were involved.
- B. The certifying authority shall create a training development plan for each foster parent(s) to document the strengths and competencies of the foster parent(s) and to identify those areas in which additional training is needed.
- C. The certifying authority shall document that foster parents receive quality, appropriate, competency-based training in the following areas that builds on basic competencies of the foster parent(s) established through life experiences and pre-certification training.
1. The foster care home's emergency and safety procedures, including but not limited to fire evacuation drills, tornado drills, where appropriate, and flood evacuation drills, where appropriate, on at least a semiannual basis.
 2. The principles and practices of child care, including developmentally appropriate practices.
 3. The certifying authority's administrative procedures and overall program goals.
 4. Acceptable behavior management techniques that comply with these rules and, if applicable, Section 7.708.36 and Section 7.714.53, et seq.
 5. Acceptable discipline techniques that comply with these rules.
 6. Appropriate boundaries (both physical and emotional) between foster parents and foster children while in placement at the foster care home and after discharge.

7. Positive and constructive methods of dealing with the foster child, including but not limited to, physical structuring of the environment and de-escalation of crisis situations.
 8. The foster parent is trained and knowledgeable, and has the competencies required to apply the reasonable and prudent parent standard based upon the criteria in Section 7.701.200.
 9. Annual review of these regulations by foster parents.
- D. Training requirements for the initial year of certification are as follows:
1. Twenty-Seven (27) hours of Core training, as identified in Section 7.708.65, A; and,
 2. CPR and First Aid training; and,
 3. Twenty (20) hours of ongoing specialized training.
- E. Annually, each foster parent, except therapeutic foster parents or treatment foster parents, must complete twenty hours of on-going specific training as required in his/her training development plan. Training must include at least the areas listed above. Therapeutic or treatment foster parents providing therapeutic services must complete an additional twelve hours of on-going training annually for a total of thirty-two hours of training in such areas as dynamics of victimization issues, with emphasis on appropriate age and developmental levels; and the individual needs of the foster children in care. All providers serving children funded by the Children's Habilitation Residential Program (CHRP) must complete thirty-two (32) hours of ongoing specific training as identified in their training development plan.

7.708.66 Transportation Policy [Rev. eff. 4/1/12]

The certifying authority shall have a written policy in compliance with Colorado statute(s), Department of Revenue and Department of Motor Vehicle requirements concerning the circumstances a vehicle may be driven by a licensed foster child alone or with a passenger. Such driving privileges shall be a part of the foster child's family services plan.

7.708.67 Limitation of Foster Children's Personal Belongings [Rev. eff. 1/1/16]

The decision and reasons why a foster care home would limit a foster child's access to his/her personal belongings brought to the foster home upon admission shall be recorded in the foster child's case record maintained by the placing authority.

7.708.68 Personal Allowance and Work Opportunities

Foster children shall be provided personal allowance and/or work opportunities according to the established policy of the certifying authority and shall have opportunities appropriate to the foster child's age and development to experience the use and value of money by making purchases for items according to their own choice.

- A. Money earned, received as a gift, or received as allowance by a foster child in care shall be deemed to be that foster child's personal property.
- B. Limitations may be placed on the amount of money a foster child may possess or have access to when such limitations are considered to be in the foster child's best interests.

7.708.69 Confidentiality of Records and Reports

- A. The certifying authority shall have a policy as to the maintenance, storage and confidentiality of records.
- B. Records shall be the property of the certifying authority and shall be protected against loss, tampering, or unauthorized use.
- C. Facts learned about foster children and their families shall be kept confidential, with the following exceptions:
 - 1. In medical emergencies, and then only when the assistance and/or expertise is required of that unauthorized person; or,
 - 2. The foster child, his/her parent(s) or guardian(s) and their respective legal counsel(s), a court having jurisdiction over the foster child, or an authorized public official, or certifying/licensing representative in performance of his/her mandated duties; or,
 - 3. If the parent(s) or guardian(s) has given voluntary, written consent.

7.708.7 AUTHORITY TO WAIVE NON-SAFETY CERTIFICATION STANDARDS FOR KINSHIP FAMILY FOSTER CARE PROVIDERS [Rev. eff. 1/1/16]

Pursuant to Section 26-6-106(6)(a), C.R.S., a county director or his/her designee may waive non-safety certification standards for prospective or current kinship foster care providers defined in Section 7.708.11 (12 CCR 2509-8). The safety and well-being of the child and/or youth shall not be compromised. The following non-safety certification standards may be waived case-by-case:

- A. Certification of Foster Care Homes
 - 1. Two (2) of three (3) references are not related to the applicant referenced in Section 7.708.21, F.

2. Certificates shall not be granted to applicants who are less than twenty-one (21) years of age on the date of application referenced in Section 7.708.21, H.
3. A license or certificate shall not be granted for a foster care home unless the application has demonstrated the ability to manage a household so that the certifying authority may determine that the applicant is able to acquire food, materials, and other equipment as may be required for child care referenced in Section 7.708.21, K.
4. Each foster parent shall have a health assessment within one (1) year prior to certification or within thirty (30) calendar days after certification and thereafter as required, in writing, by a licensed health care professional. The reports of the medical examinations shall be dated and signed by the examining physician or nurse practitioner and shall be provided to the certifying authority. Reports shall include a statement of the evaluation of the person's physical ability to care for children and youth in foster care.

If, in the opinion of the licensed health care professional or the assessment worker, an emotional or psychological condition exists which would have a negative impact on the care of children and youth in foster care, the issuance of a license shall be conditioned upon the satisfactory report of a licensed mental health practitioner referenced in Section 7.708.21, L.

B. Facility Standards

1. A minimum of seventy-five (75) square feet per child in foster care who is between twelve (12) months and five (5) years of age referenced in Section 7.708.22, B, 1.
2. Archery bows that require professional stringing must be unstrung referenced in Section 7.708.22, B, 2.
3. A child or youth shall sleep in a crib or bed in a clean, well-ventilated room, which is not customarily used for other purposes such as a kitchen, dining room, hall, or bathroom. There shall be a minimum of forty (40) square feet of floor space for the bed for each child or youth in foster care, and they should be placed at least two feet apart when arranged in parallel referenced in Section 7.708.22, B, 8.
4. No foster care home shall be used for rental income referenced in Section 7.708.22, B, 9.
5. All stairways containing more than four (4) steps shall be equipped with a handrail referenced in Section 7.708.26, E.

C. Ongoing Operation of Foster Care Homes

1. Except for emergency placements, children and youth in foster care over the age of eighteen (18) months shall not sleep in the same room as unrelated adults on a regular basis. Sleeping rooms for unrelated children and youth in foster care shall not be shared by others in foster care who are of the opposite sex when one of the individuals in foster care is over four (4) years old as referenced in Section 7.708.31, G, 3.
2. Not more than two (2) infants, whether birth, adoptive, or foster children under the age of two (2) years, shall be cared for in a foster care home, except under unusual circumstances such as multiple births, referenced in Section 7.708.31, H, 1.
3. **Personal Care and Safety of the Foster Child**
 - a. Home pressure-canned fruits and vegetables and canned meats cannot be served, referenced in Section 7.708.42, E.
 - b. Each foster child's clothing shall be distinguished as his/her own clothing, referenced in Section 7.708.44, B, 1.

7.708.71 Procedures for Waiving Non-Safety Certification Standards [Rev. eff. 1/1/16]

- A. Each waiver shall be considered case-by-case; and,
- B. Shall be documented on the prescribed State Department form; and,
- C. The documentation shall contain a description of the applicability of the waiver to the safety or well-being needs of the child and/or youth with a kinship/relative relationship to the prospective or current kinship foster care provider; and,
- D. The documentation shall be placed in the certification record; and,
- E. A summary of the waiver information shall be documented on the certificate.

7.708.72 Special Conditions, Restrictions, or Requirements for Certification of Prospective Current Kinship Foster Care Providers [Rev. eff. 1/1/16]

A county director or his/her designee may take the following actions to address the safety or well-being needs of a child or youth:

- A. Require special conditions for certification;
- B. Limit or restrict a certificate; and/or,
- C. Require a written agreement for compliance.

7.708.73 Circumstances When Certification Standards Shall not be Waived
[Eff. 12/1/12]

- A. The safety or well-being of a child or youth is compromised.
- B. Background checks, including:
 - 1. A fingerprint-based criminal history check of CBI and FBI records;
 - 2. A child abuse/neglect records check in every state where the adult has resided in the five (5) years preceding the date of application for each adult eighteen (18) years of age and older) living in the home;
 - 3. A comparison search in the Colorado State Courts data access, using the name and date of birth with available criminal history information for each adult eighteen (18) years and older living in the home;
- C. Twenty-seven (27) hours of initial pre-certification training and ongoing training referenced in Section 7.708.65, C and D; and/or,
- D. Required denials of applications or certificates referenced in Section 7.500.312, D and 26-6-104 (7)(a)(I)(A-F), C.R.S.

7.708.74 Appeals of Decisions [Rev. eff. 1/1/16]

Prospective and current kinship foster care home providers do not have the right to appeal the decision made by a county director or his/her designee related to waivers of non-safety certification standards.

7.709 RULES AND REGULATIONS FOR SPECIALIZED GROUP FACILITIES [Eff. 11/1/2008]

All Specialized Group Facilities must comply with the “Quality Standards for 24-Hour Child Care”, the “Rules Regulating Specialized Group Facilities”, the “General Rules For Child Care Facilities”, and the “Rules Regulating Special Activities”.

7.709.1 DEFINITIONS [Eff. 11/1/2008]

- A. The “Specialized Group Facility” (SGF) is sponsored and supervised by a county department of social/human services henceforth to be known as the county) or a licensed child placement agency (henceforth to be known as the agency) for the purpose of providing 24 hour care for three (3) or more children, but fewer than 12 children from three years old to sixteen years old and for children from 16 to 18 years old and those persons 21 years old who are placed by court order prior to their eighteenth birthday whose special needs may best be met through the medium of the small group. The two types of specialized group facilities are the specialized group home and the specialized group center.

1. A “specialized group home” is located in a house owned or otherwise controlled by the group home parents who are primarily responsible for the care of the children and reside therein.
 2. A “specialized group center” is located in a house owned or controlled by a governing body that hires the group center parents or personnel who are primarily responsible for the care of the children.
 3. The “sponsoring agency” is the county or agency that is responsible for supervising the SGF and providing support and evaluation of the facility every week by a qualified staff member.
 4. The “governing body” is the individual, partnership, corporation or association in whom the ultimate authority and legal responsibility is vested for the conduct of the specialized group center and the center's relationship with the supervising agency. The sponsoring agency and the governing body may be the same.
- B. A specialized group facility may accept children five years old and over in an emergency for a short period of time. Special rules necessitated by the emergency placement regarding intake, records, placement planning and staffing are found at Section 7.709.26.
- C. No more than two children less than five years old may be cared for in a specialized group facility at one time.
- D. In specialized group facilities, the number of natural and relative children less than sixteen years old of the caretaker plus the foster children shall not exceed eleven children.

7.709.2 REQUIREMENTS OF A SPECIALIZED GROUP FACILITY [Rev. eff. 6/1/12]

7.709.21 Operation of Specialized Group Homes or Specialized Group Centers [Rev. eff. 6/1/12]

- A. The specialized group facility shall be sponsored and supervised by a county or agency.
- B. The supervisory responsibilities of the sponsoring agency are:
1. To be knowledgeable with the Rules Regulating Specialized Group Facilities; and,
 2. Participate in the development and application process to include verifying that the original application submitted is complete with all required signatures and submitted in a timely manner; and,

3. Ongoing assessment of the specialized group facility for quality of care issues; and,
 4. Annual evaluations of the governing body, unless the governing body and the sponsoring agency are the same agency.
- C. The sponsoring agency shall be responsible to ensure that state rules are followed regarding:
1. The hiring, training and scheduling staff; and,
 2. Placement decisions including, but not limited to, appropriateness of placement and least restrictive environment; and,
 3. Documentation, reporting and corrective action of critical incidents.

7.709.22 Personnel Qualifications and Role of a County Department of Social Services or a Licensed Child Placement Agency to Establish and Supervise a Specialized Group Home or Center [Rev. eff. 11/1/15]

- A. The Specialized Group Home or Center shall be supervised by a paid staff member of the agency who holds a Master of Social Work degree or a master's degree in behavioral science with an emphasis in child development and/or family relations and a minimum of two years paid full time or equivalent part-time experience in social work; or if the staff member does not have the aforementioned education and experience, then he/she must have a bachelor's degree with a major in social work, sociology, psychology or closely related field and be directly supervised by an agency staff member holding the master degree and experience described above.
- B. In establishing a new group home or center the agency shall, with the group home parents or governing body, complete a financial feasibility study reflecting actual and realistic cost factors as well as adequate resources to insure its ongoing operation.
- C. A statement of purpose and function which includes a description of the characteristics of the youth population which is to be served by the facility shall be written by the agency and the group home parents or the group center governing body.
- D. The agency and the group home or center parents or personnel shall develop and adhere to written policies and procedures regarding the care of children which shall be reviewed annually and shall include the following:
1. Provision for emergency procedures including illness, accident, fatality and fire.
 2. Participation in special activities in compliance with Section 7.719.

- E. The group home or center parents or personnel and the agency shall develop and adhere to written policies and procedures regarding personnel including: pay (when appropriate), provision of relief time and vacation time, annual performance evaluation, training opportunities, selection of personnel and maintenance of personnel records.
- F. The agency staff member who supervises the group home or center shall obtain, provide and/or coordinate the following services for children in care:
 - 1. Group and/or individual counseling for children and their families.
 - 2. For each child, long range, intermediate and short term goals shall be established and a case plan written. The goals and case plan must include a plan for discharge and must be developed and evaluated pursuant to regulation Section 7.714.4, C,D. Goals and case plan for children three and four years old shall be evaluated monthly.
 - 3. Records for the home or center as required in Section 7.709.27.
 - 4. Psychiatric, psychological or developmental evaluations and consultations as required.
 - 5. Specialized educational resources as required.
 - 6. All necessary legal representation of child.
 - 7. Consultation with the home or center parents or personnel about methods of work with the children.
 - 8. Assessment of quality of care with the home or center parents or personnel.
- G. The agency staff member shall meet in each specialized group facility with the primary caregivers and other personnel, if available, a minimum of two hours per week exclusive of counseling services, to discuss individual children, problems, program and/or special needs.
- H. The agency shall ensure that adequate records be maintained about group home or center income and expenses, the provision of clothing, allowances, dental and medical services and medicine.
- I. Staff designated to authorize participation in activities is trained and knowledgeable, and has the competencies required to apply the reasonable and prudent parent standard based upon the criteria in Section 7.701.200.

7.709.23 Character, Suitability and Qualifications of Group Home or Group Center Parents or Personnel [Eff. 11/1/2008]

- A. The group home or group center parent's or personnel's own children shall not have been placed in foster care or a residential treatment facility under circumstances tending to show that the parent is unable or unwilling to care for his/her own children unless the placement was primarily for such reasons as the physical illness of the parent, physical or mental disability of the child or the relinquishment of an infant child for adoption.
- B. The group home or group center parents or personnel shall be able to demonstrate through satisfactory past experience and references the ability to provide for the proper physical, mental and character development of children in care.
- C. The group home or group center parents or personnel shall demonstrate stability in family or marital relationships, where applicable, within the home where the care is to be provided.
- D. The group home or group center parents or personnel shall hold a high school diploma or General Equivalency Diploma (GED).
- E. The group home or group center parents or personnel shall not be less than 21 years of age, shall be appropriate for the age of children in care and shall possess the adequate physical stamina to care for children. An individual such as the son or daughter of the group home or group center parents, who is well acquainted with the children in care and is between the ages of 18 and 21 years old may be a child care personnel and assist in the care of the children with the approval of the agency
- F. Children of the group home parents or group center parents or personnel and any other persons not placed by the agency and living in the group home or group center shall obtain a physical examination annually or as required in writing by an approved health care professional.
- G. The group home or group center parents or personnel shall include at least one primary caregiver who is a full-time employee of the facility, provides some direct care for children, supervises other child care personnel and holds major responsibility for child care.
- H. The primary caregiver shall have had verifiable working experience with children of the age to be cared for in the group facility. Such experience shall be from one or more of the following:
 - 1. One year of experience as a foster home parent.
 - 2. Two thousand hours work experience with children of the age to be cared for in the group facility.
 - 3. One year of experience as a child care worker in a residential facility for children; and,

4. The group home primary caregivers who have not previously received 12 hours of “core” training shall receive 12 hours of training within the first twelve (12) months following the submission of the applications.
- I. Other child care personnel in the group center or group home shall be supervised by a primary caregiver.
- J. The group home or group center parents or personnel who are the primary caregivers shall be evaluated by a board-eligible psychiatrist, a certified psychologist or a Licensed Clinical Social Worker, and shall obtain a statement from the evaluator which evaluates the persons.
 1. Psychosocial history to include family of origin style under which they were parented; relationships with parents and siblings; and,
 2. Child abuse and neglect assessment to include prior history of physical emotional, verbal, sexual abuse or neglect; and,
 3. Relationships with extended family, significant romantic relationships, marriages, engagements, failed relationships and estrangements; and,
 4. Work history and reason for wanting to work with children in a specialized group facility; and,
 5. Assessment of finances; and,
 6. Children, both biological and adopted, and relationships with children, including adult children; and,
 7. Support systems available to the primary caregiver; and,
 8. Areas of interest, hobbies, and activities; and,
 9. Tolerance and intolerance to include stressors, coping skills, expectations, negative experiences, survival of crisis in their life experiences and religious issues; and,
 10. Health history and medication history to include substance abuse, drugs, alcohol history and issues and current medication; and,
 11. Previous or current psychological issues including the history of the issues or therapy; and,
 12. Previous child care responsibilities and experiences; and,
 13. Legal and criminal background and/or history; and,
 14. Involvement or experiences with child protective services of a county department of social/human services as a child or as an adult; and,

- 15. Education and training; and,
 - 16. Any existing condition which may jeopardize the health or well-being of children or impair the care of the children.
- K. The licensing or certifying authority must receive at least three statements which describe the primary caregiver's character and ability to provide care for children from references, provided by the primary caregiver, who are not related to the caregiver and have known the caregiver one year or longer. References need not be residents of Colorado.
- L. References shall be obtained by the group home or center for other personnel. References shall be kept on file at the facility or the agency.

7.709.24 Physical Requirements for a Safe and Adequate Specialized Group Facility [Rev. eff. 6/1/12]

- A. The following shall be required of each specialized group care facility:
- 1. In facilities licensed after May 1, 1984, all furnaces shall be separated from living areas by one-hour fire resistive material, provided with adequate outside combustion air, installed and maintained with safety devices to prevent fire, explosions and other hazards. The space around the furnace shall not be used for storage.
 - 2. Electrical wiring systems throughout the facility shall be in good repair. There shall be no three-way plugs or extension cords in use. There shall be no exposed wiring.
 - 3. Household materials which may be dangerous to children, such as matches, plastic bags, cleaning and laundry supplies as well as household tools and equipment which may present a danger to children, shall be stored out of their reach when appropriate.
 - 4. In each facility licensed after May 1, 1984, there shall be at least two approved, alternate, widely separated means of egress from each floor of the building to safe and Open space at the ground level.
 - 5. No lock or fastening to prevent free escape from the inside of any room or building used by the children shall be permitted. Exit door hardware shall be of the single-action type.
 - 6. Exit doors shall be obvious, and where this is not so, exit signs shall be installed.
 - 7. The local fire department shall determine the adequacy of exits and other measures for life safety in accordance with the requirements of the Uniform Building Code and the National Fire Protection Codes. In cases of

practical difficulty or unnecessary hardship, the local fire department may grant exceptions from the Uniform Building Codes or the National Fire Protection Codes, but only when it is clearly evident that reasonable safety is thereby secured.

8. In new facilities licensed after May 1, 1984, any accessible areas beneath stairways shall be enclosed with one-hour fire-resistant material.
- B. Living areas for children in a specialized group home or center shall include:
1. Separate sleeping rooms for boys and girls. In sleeping rooms that accommodate two to four children, 60 square feet of floor space per child shall be provided. There shall be no more than four children in any bedroom. Each room for single occupancy shall have a minimum of 80 square feet of floor space. Closet space and drawer space for personal items sufficient for the occupants in each sleeping room shall be provided.
 2. Each child shall be provided suitable sleeping facilities consisting of individual beds or bunks complete with mattresses in good repair and constructed so as to facilitate cleaning while in use by residents, and upon each change of occupancy. Single beds shall be spaced not closer than 36 inches laterally or end to end. Triple-deck bunk facilities are prohibited. Beds being used by children shall have a mattress cover, clean sheets, pillows and pillowcases and blankets as appropriate.
 3. Sleeping rooms above or below the floor of exit travel shall not be used for sleeping purposes for children who have physical handicaps which limit mobility. Children less than eight years old shall sleep on the same floor as the group home or center parents or personnel. Children under 12 years of age shall not be permitted to sleep in a detached structure unless a responsible person sleeps in the same structure.
 4. Bedrooms, separate from those used by children, shall be provided for the group home or center parents or personnel who sleep at the facility.
 5. Facilities shall provide living areas of no less than 35 square feet per occupant of usable indoor space exclusive of halls, baths and sleeping areas which shall be available for such things as dining, recreation, reading, visiting. Interior walls and ceilings shall be constructed of solid material and shall be in good repair.
 6. There shall be a ratio of at least one toilet, lavatory and bathtub or shower for every six foster children living in the home or center. Toilet, lavatory and bath or shower facilities shall be in the same building(s) as the children's sleeping quarters and shall be accessible from the inside of the building.

7. There shall be a towel rack for each child in residence in bathroom or bedrooms.
- C. The group home or center's kitchen shall be equipped with the following:
 1. Adequate space for receiving, storage and refrigeration of food.
 2. Adequate space for eating in kitchen or adjacent room for all children to eat at the same time.
- D. The facility must have laundry facilities with adequate storage for linens.
- E. The group home or center, if located in the same building as or immediately adjacent to other residential facilities such as another group home or center, an adult treatment center or a nursing home, shall be so arranged that the care and activities of the children residing in the group home can be completely separate and independent from the other residential facility. No group home or center shall be used for purposes of a rooming or boarding house. A specialized group facility may not be operated adjacent to or on the premises of a business of a nature which might be hazardous to the health, safety, morals or welfare of children and the operation of the specialized group facility. The group home or center shall house only unrelated children of the ages mentioned on the license or certificate and of the type described in the statement of purpose. The facility shall not also be used by unrelated adults.
- F. Mobile homes shall not be used for group homes or centers.
- G. When a swimming pool is provided, it shall meet the requirements of the Colorado Department of Public Health and Environment or its local unit. Safety precautions shall include protective fencing, a nonskid surface of at least four feet adjoining poolsides, and winter coverage which shall exclude plastic or inflatable-type domes. A certified lifeguard shall be in attendance at all times when the pool is in use.

7.709.25 Care of Children [Rev. eff. 11/1/15]

- A. Group home or group center parents or personnel shall provide supervision and care appropriate to each child's age, level of development, ability to accept independence and responsibility, and according to the group home or group center's policies, procedures and the child's case plan.
 1. The group home or group center parents or personnel shall know the intended whereabouts of each child in care at all times.
 2. Children less than 14 years old shall not be left without adult supervision in the home or center. Children over 14 years old may be allowed to stay alone occasionally for short periods of time in the home or center pursuant to a case plan and the facility policy.

3. Children who are less than 16 years old who are sleeping away from the group home or center, such as during a camping trip or a slumber party, must have adult supervision.
 4. When all the children are away from the group home or center, the adult parent or personnel on duty may be away from the home or center for short periods of time.
- B. The following staff pattern shall be maintained:
1. If the primary caregiver is married, one spouse may be working full time outside the home or center. The spouse or one child care personnel shall assist the primary caregiver during times when the number and/or needs of the children require additional supervision as determined by the group facility and the supervising agency; or,
 2. If the primary caregiver is single, she/he must be at the home or center full time. There shall be one child care personnel assisting the primary caregiver during times when the number and/or needs of children require additional supervision as determined by the group home and the supervising agency; and,
 3. A relief staff member who is knowledgeable about the policies and practices of the home or center shall be available to provide child care when the primary caretaker is gone from the home and as determined by the group home or center and supervising agency. An adult shall be available in the event of an emergency; or,
 4. If the group center has rotating staff, there shall be at least one child care personnel on duty during each shift. The primary caregiver shall work together with the other child care personnel to provide modeling and supervision for at least 16 hours a week during the hours the children are at the facility.
 5. If the group home or center and supervising agency determine that additional personnel are necessary, a mutually agreed upon written plan for staffing the group home shall be followed.
- C. Health Care, Hygiene, Meals, Safety
1. There shall be a written agreement with a health care facility or medical personnel who has agreed to provide emergency medical care to children in the group home or center.
 2. Necessary medical care shall be obtained pursuant to the policy and procedures of the group home or center.

3. Menus shall be kept for at least a two-week period, and there shall be a documented review periodically by a qualified nutritional consultant.
4. One group home or center parent or personnel shall be responsible for meal planning, and a parent or personnel shall be assigned responsibility for food preparation for each meal. Children may help when appropriate.
5. Only pets permitted by local codes are acceptable at the group facility. Pets shall be housed, cared for and vaccinated according to health department regulations and other local codes. Precautions shall be taken as required to ensure both safety and good hygiene. Children shall not be permitted to mistreat animals. Animals shall not be permitted to eat in kitchen or eat from dishes in which food is prepared, served, stored or from which it is eaten.
6. An evacuation plan shall be posted in a conspicuous place.

D. School and Recreation

1. Regular school attendance is required for each child according to school attendance laws. A suitable, quiet, well-lighted place for study shall be provided together with necessary books, papers, pencils and other equipment which is reasonably required by school-age children.
2. Outdoor and indoor recreational equipment and materials shall be provided in sufficient variety and quantity to offer some choice of activities. Games, toys, equipment and arts and craft materials shall be selected according to age, number of children, and with consideration of the needs of children to engage in both active and quiet play. All equipment and materials shall be of quality to assure safety, and shall be of a type which allows for imaginative play and creativeness.

E. Participation in Community Activities

Participation in community activities shall be supported. Along with the requirements in Section 7.714.7 (12 CCR 2509-8), at least one (1) specialized group facility staff shall be trained how to determine whether to approve a child's or youth's participation in an extracurricular, enrichment, cultural, or social activity is consistent with the reasonable and prudent parent standard based upon the criteria in Section 7.701.200.

7.709.26 Required Records [Eff. 11/1/2008]

There shall be a personnel file maintained for each parent or personnel by the facility or the agency. The file shall include identifying information, references, statement from physician or qualified nurse practitioner, name and telephone number of person to contact in emergency, and verification of education and experience. The personnel file for the primary caregiver shall include a statement from a psychiatrist, a certified

psychologist or a Licensed Social Worker II. The personnel records shall be maintained pursuant to the personnel policy and procedures. If personnel records are at the agency office, there shall be maintained at the facility the address, local phone number and name, address, and phone number of persons to call in an emergency.

7.709.27 Special Rules for Emergency Placement and Care of Children [Eff. 11/1/2008]

- A. Emergency placement indicates that due to circumstances beyond the agency's control a child needs placement, yet pre-placement admission requirements have not been completed. The acceptance of a child in such an emergency status shall only be done if it is a part of the admission policy and procedures of the facility.
- B. During placement at the facility as much evaluation of the child shall be completed as possible and as is necessary for future planning for the child. As much data as possible shall be collected pursuant to the admission record.
- C. A specialized group facility which accepts only children by emergency admission shall have on duty one child care personnel for each six children or fraction thereof at the facility during the daytime and swing shift. One child care staff member will be sufficient during sleeping hours.
- D. Information gathered at the time of emergency placement and during placement shall become part of the child's file. The file shall also include the date the child was discharged from the facility, to whom the child was released, and a description of the physical condition of the child at the time of discharge.
- E. A specialized group facility operating as a shelter care facility or which accepts only children by emergency admission shall make every effort to assure that emergency placements do not exceed sixty days. Exceptional circumstances must be documented in the case file for any placement lasting over 60 days. Reimbursement at the shelter care rate shall not exceed 90 days.

7.710 RULES AND REGULATIONS FOR CHILD PLACEMENT AGENCIES

All child placement agencies shall comply with the “General Rules for Child Care Facilities” and “Rules and Regulations for Child Placement Agencies” and shall comply with the “Rules Regulating Foster Care Homes” for any homes certified by the Child Placement Agency, “Rules Regulating Host Family Homes” for any host home certified by the Child Placement Agency, and the “Specialized Group Facilities” rules for any Specialized Group Facility sponsored by the Child Placement Agency.

7.710.1 GENERAL DEFINITIONS

“Arrange for placement” means to act as an intermediary by assisting a parent or guardian or legal custodian to place or plan to place a child with other than persons related to the child for the purpose of foster care or for the purpose of adoption.

“Average sufficient cash reserve” means the computed monthly average cost over the recent ongoing twelve-(12) month period to determine the amounts spent on operating expenses for the agency including, but not limited to, staff salaries; contract reimbursements; employment, unemployment, and other taxes; insurance and retirement benefits; foster care payments; other provider reimbursement fees; health, therapy, transportation and support services for children in care; foreign country fees, office mortgage or rent payments; transportation costs; communications; or, any other expense needed for the agency to function, including a line of credit. The minimum allowable average sufficient cash reserve that an agency must maintain at all times is at least two months’ of the average monthly cost.

“Background checks” means a set of required records that are obtained and analyzed to determine whether the history of a prospective foster parent, kinship foster parent, non-certified kin, or adoptive parent meets legal and safety criteria when considering the placement or continued placement of children and in the care of the person(s). The checks include all adults residing in the home. The following individual checks are required pursuant to 19-3-406 C.R.S, 19-3-407 C.R.S, and 26-6-103, C.R.S:

1. CBI and FBI finger-print based criminal history record information;
2. Court case management system;
3. State automated case management system and child abuse and/or neglect registries in all states that adults living in the home have resided in the five years preceding the date of application; and,
4. The CBI sex offender registry and national sex offender public website operated by the United States Department of Justice using the following minimum criteria:
 - a. Known names and addresses of each adult residing in the home
 - b. Address only of the residence

“Bonding” means an insurance bond issued through a financial or insurance entity.

“Certification” means the process by which the county department of social/human services or a child placement agency approves the operation of a foster care home and/or a licensed host family home.

“Child placement” means to coordinate, arrange, and approve the process of a child entering an unrelated home or facility to be cared for on a temporary, long-term, or adoptive basis.

“Child Placement Agency (CPA)”, defined at Section 26-6-102(2), C.R.S., means any corporation, partnership, association, firm, agency, institution, or person unrelated to the child being placed, who places, facilitates placement for a fee, or who arranges for placement, any child under the age of eighteen (18) years with any family, person, or institution for the purposes of foster care, treatment and/or adoption. The natural or adoptive parents or legal guardian of any child who places that child for care with any facility licensed as a “family care home” or “child care center,” as defined by this section, shall not be deemed to be a CPA.

“Conflict of interest” means a situation that has the potential to undermine the impartiality of an individual because of the possibility of a clash between the individual’s self interest or other professional-interest.

“Cradle care home” means a facility that is certified by a child placement agency for the care of a child, or children in the case of multiple-birth siblings, who is twelve (12) months of age or younger, in a place of residence for the purpose of providing twenty-four (24) hour family care for six (6) months or less or children pursuant to Article 5 of Title 19, C.R.S., or while a county department prepares an expedited permanency plan for an infant in its custody.

“Current reference” means a reference dated within one (1) year of the time of application for employment with agency.

“Foster care home” (refer to Section 7.000.2 in 1 CCR 2509-1).

“Generally Accepted Accounting Principles” (GAAP) means the standard framework of guidelines for financial accounting.

“Licensed host family home” is defined as a home that is certified by the county department or a child placement agency as meeting the requirements for providing shelter to homeless youth.

“Licensing” means the process by which the Colorado Department of Human Services approves a facility or agency for the purpose of conducting business as a child care facility or child placement agency.

“Multi-service agency” is an organization that provides additional community services and programs other than foster care and adoption.

“Program director in a multi-services agency” is the person responsible for overseeing the foster care and/or adoption program of the organization.

“Quality improvement program” means a review of the services and outcomes of such services provided to applicants and a procedure for tracking such outcomes to determine if changes need to be made to the system to improve delivery of such services.

“Relative”, except as used in the definition of foster care home, means any of the following relationships by blood, marriage, civil union or adoption: parent, grandparent, son, daughter, grandson, granddaughter, brother, sister, stepparent, stepbrother, stepsister, stepson, stepdaughter, uncle, aunt, niece, nephew, or cousin.

“Risk assessment” means a review and assessment by an insurance or financial specialist to determine the liability an agency carries for the services it offers and the work it performs.

“SAFE (Structured Analysis Family Evaluation)” is the tool used to create a home study assessment on a family or individual, who are seeking to provide foster care or adopt a child.

“SAFE Home Study Addendum” is the format used to record and evaluate changes to the home study assessment as new information is known to the agency.

“SAFE Home Study Update” is the format used to document the annual agency review and evaluation of the home study assessment.

“Social and behavioral sciences” includes sociology, psychology, social work, criminal justice, human services, human development, and counseling.

“Specialized Group Facility Supervisor” means the professional, paid staff member of the Child Placement Agency who oversees the services provided and staff of the specialized group facility.

“Unreasonably high” means the fees, wages, or salaries paid to the directors, officers, and employees of the agency are excessively high in relation to the services actually rendered, taking into account the area in which the services are provided and norms for compensation within the community, including factors such as the location, number, and qualifications of staff, workload requirements, budget; and size of the agency or person.

“Well-being plan” means a written plan that lists the temporary changes to the child’s or youth’s daily routines or treatment plan, due to behavioral, safety, medical, or mental health needs of the child or youth. Such plan shall be signed by members of the treatment team, including the child or youth, and shall be time limited only to the duration needed to ensure the safety of the child or youth. Such plan shall be reviewed daily and overseen by the placement supervisor.

7.710.2 GOVERNING BODY

- A. Any agency from out of state assisting with, facilitating for a fee, or placing a child within Colorado for the purpose of adoption must meet the ICPC requirements of the sending state, or be licensed as a CPA by the Colorado Department of Human Services (the State Department) unless the placement services are coordinated with and provided by a county department of social services or a CPA licensed by the State of Colorado.
- B. A CPA may not be operated without a license, as required by law, which license is to be issued by the State Department in conformity with all rules and regulations contained within Section 7.710, et seq.
- C. Any Colorado entity, other than a CPA licensed in Colorado, or individual, other than a biological or legal parent of a child, may not assist or arrange for the placement of a child with a Colorado family for the ultimate purpose of adoption without first being licensed as a CPA.
- D. A child placement agency may only accept applications from and certify families for foster care within the State of Colorado.
- E. The governing body shall be the corporation, partnership, association, firm, agency, institution or person in whom the ultimate authority and legal responsibility is vested for the conduct of the CPA.
- F. The governing body shall be identified by its legal name. Each not-for profit child placement agency shall have a board of directors. If the board has community members, such community members shall neither be employed by or contracted to the CPA, nor related to any individual employed by or contracted to the CPA. Persons whose children are currently in placement (either voluntarily or involuntarily) through the CPA may serve on the board, but may not vote on any measure or issue related to the care of their child during the time such children are in placement. Minutes from all board meetings shall be maintained for a period of at least five (5) years and must be available to the State Department upon request.
- G. The governing body of the CPA shall:
 - 1. Maintain the written purpose and policies for the general operation and management of the agency. When such purpose and policies are reviewed and revised, the State Department shall be advised of such changes. The purpose and policies as a minimum shall include:
 - a. Statement of purpose of the CPA as to what type of placement of children in which the agency intends to engage, the geographic area the agency expects to serve, the ages of children to be placed, and any other specific factors regarding the children to be placed or the homes in which the children shall be placed.

- b. Personnel policy including, but not limited to, job description; qualifications for position; required documentation for position; requirement for a criminal history fingerprint background check (per section 7.701.33); a child abuse/neglect check through the State Department's designated database (per Section 7.701.32); and a name and address check of both the National and CBI Sexual Offender Registries (per section 7.701.33).
 - c. Foster care policy including, but not limited to, types of foster care homes to be certified; geographical area of proposed foster homes; good faith efforts and due diligence to be used in recruiting families who reflect the communities of all children in care; assessment, training, certification, supervision and monitoring of certified foster homes.
 - d. Adoption policy which includes types of adoption in which the agency will participate and policy and procedure for each adoption program.
 - e. Fee policy.
 - f. Volunteer policy.
- 2. Be responsible for the protection of the legal rights of children served by the CPA.
- 3. Be responsible for approval of the CPA budget and any budgets for Specialized Group Facilities sponsored by the CPA, including obtaining funds and dispersal of funds, as required in 7.710.21.
- 4. Appoint an executive director who meets requirements of Section 7.710.22 and be assured that staff members responsible for placement of children and/or certification of foster homes meet the requirements as stated in Section 7.710.22.
- 5. The Board or Chief Operating Officer, in the case of a multi-service agency, in which the program director is the person responsible for overseeing the foster care and/or adoption program shall conduct an evaluation, at least annually, to determine if the executive director is fulfilling all responsibilities as required in Section 7.710.25.
- 6. Inform the department, in writing, of:
 - a. A change in the executive director of the CPA; and,
 - b. Each agency office or change of agency office in which child placement is carried out at that location; and,

- c. The hours of operation that each CPA office is open each week and available for inspection of CPA records.
 - 7. Provide copies to the department of any legal action brought against the CPA which affects any child or children in care, personnel or conduct of the CPA.
 - 8. Maintain professional liability insurance in amounts reasonable related to its exposure to risk. The agency must provide a current risk assessment to the State Department if requested.
 - 9. Maintain and monitor a quality improvement program appropriate to the size and circumstances of the agency through which it makes systemic efforts to improve services if needed.
 - 10. Ensure that the fees, wages, or salaries paid to the directors, employees, and officers of the agency not be unreasonably high in relation to the services actually rendered.
 - 11. Ensure that the agency's Chief Executive Officer, Chief Financial Officer, Executive Director, and other officers or employees with direct responsibility for financial transactions or financial responsibility be bonded.
 - 12. Ensure that the agency maintains an average sufficient cash reserve or assets to meet its operating expenses, less foster parent payments, for two (2) months, taking into account the agency's projected volume of cases and its size, scope, and financial commitments.
- H. The governing body shall be responsible for completing the licensing renewal requirements by:
- 1. Completing and submitting the license renewal application at least ninety (90) calendar days prior to the annual expiration date of the child placement agency license; and,
 - 2. Completing, signing and submitting the required verification of compliance form; and,
 - 3. Paying the prescribed fee pursuant to Section 7.701.4; and,
 - 4. Cooperating with on-site monitoring visit(s) to assess the agency's compliance with the rules for child placement agencies.
- I. The governing body shall ensure when the Child Placement Agency accesses the state automated case management system as part of the foster home certification back ground investigation, it must do so only as listed at section 19-1-307 C.R.S. Any violation of such access may result in individual fines as listed

at section 19-1-307 (1)(C), C.R.S. and agency fines as listed at section 26-6-114 (1), C.R.S.

7.710.21 Financial Operation

- A. Each CPA shall develop an annual budget reflecting anticipated income by source and expenses by purpose, plus an accompanying balance sheet, which demonstrates that the CPA has assured resources to carry out its defined purpose. The budget shall be approved by the Board of Directors and recorded in the minutes of the Board of Directors. The first year's budget shall be submitted with the original license application.
- B. The purpose of these requirements is to provide assurance the CPA has adequate accounting and budgeting information available to allow management to maintain a financially viable enterprise and to demonstrate financial accountability to the County and State Departments of Human Services for the use of public funds.
- C. Each CPA must have a double entry accounting system and all financial transactions must be posted to this system. Financial statements, prepared from information provided by this system, shall be presented in conformity with U.S. Generally Accepted Accounting Principles (GAAP). Books and records of the CPA shall be subject, at any time the CPA office is open, to inspection, audit or copying by appropriate Federal, State or county personnel, or such independent auditors or accountants as may be designated by these personnel.

D. Annual Audits

Each CPA whose total annual foster care or adoption expenditures are \$100,000 or more shall provide for an annual audit by an independent Certified Public Accountant in accordance with appropriate generally accepted auditing standards. CPAs with less than \$100,000 total annual expenditure may submit an audit as described above or may submit compiled or reviewed financial statements, prepared in accordance with generally accepted accounting principles. All Hague accredited international adoption agencies shall submit audits as required for Hague accreditation.

- 1. Every CPA shall submit supplementary information as prescribed by the state on the required supplementary information form and the administrative expenses for foster care as defined by the State Department.
- 2. The supplementary information submitted shall contain an affidavit signed by the CPA's Executive Director and an officer of its board attesting to the authenticity of the information. Submission of falsified information shall be grounds for suspension of the CPA license.

3. The audit and supplementary information shall be submitted to the State Department within six (6) months of the CPA's fiscal year end.
4. CPAs that are a subsidiary of a parent organization must submit separate audited financial statements for the subsidiary that detail each of the CPA's facilities or programs that provide services for the State or county department.
5. If a CPA does not submit its annual audit or refuses to disclose financial information regarding the operation of the program in a timely manner, the State Department may send notice to withhold payment until the audit and/or requested information is submitted.
6. Upon receipt of adequate written notice that a county department or the State Department plans to recover or withhold unallowable or misused funds from a CPA, a CPA may file a written request for review of the decision with the State Department.
 - a. The written request for review must be submitted within thirty (30) calendar days of the receipt of the notice to recover or withhold the misused funds.
 - b. The State Department shall convene a committee to review the written request from the CPA. The committee shall consist of three members representing:
 - 1) The State Department's Child Care Division; and,
 - 2) The State Department's Child Welfare Division; and,
 - 3) An independent representative of an auditor from another State Department or a certified independent accountant referred from the Colorado Society of Certified Public Accountants.
 - c. The State Department reserves the right to receive legal consultation regarding the written request for review.
 - d. The committee shall review all relevant information and make a decision within sixty (60) calendar days of receipt of the request.
 - e. The committee shall send its findings to the Executive Director of the State Department or his/her designee, who shall make the final agency decision for the State Department.

E. Allowable Expenditures

1. A Child Placement Agency expenditure shall be considered allowable if it meets all of the following criteria:

- a. Reasonable

The expenditure is reasonable in nature or amount and does not exceed the cost that would be incurred by a prudent person under the circumstances prevailing at the time the decision was made to incur the cost.

- b. Ordinary and Necessary

The expenditure is of a type generally recognized as ordinary and necessary for the operation of a CPA or the performance of a contract for CPA services.

- c. Prudent

The individuals concerned acted with prudence in the particular circumstances, considering their responsibilities to the CPA, its employees, clients, foster families, foster children, the public at large, the State and the CPA's responsibilities pursuant to the contract for services, and must not be contrary to Federal or State law.

- d. Documented

A CPA has the responsibility to ensure that proper documentation procedures are followed for the funds that they control, and to ensure that only authorized expenditures are made. Lack of appropriate documentation will be considered cause for non-payment or non-reimbursement of expenditures. Expenditures must be adequately documented in writing. Documentation requirements shall include all of the following information:

- 1) Descriptive verification that the expenditure was for a business purpose directly related to the foster care or adoption program.
 - 2) Date of event for which the funds were used.
 - 3) Names of persons attending when an expenditure has the "appearance of personal benefit".
 - 4) Location of event or place where funds were spent.

- 5) When documentation includes purchases for multiple items and only some of the items relate to an appropriate expenditure for the CPA, a detailed itemization and reconciliation of the expenditures shall be evident on the face of the document.

e. Made in Accordance with GAAP

An expenditure must be recorded in accordance with Generally Accepted Accounting Principles (GAAP).

2. Generally Allowable Expenditures

The following list of expenditures shall be a general list of expenditures that would be permissible for CPAs to incur in order to further the goals and objectives of their agencies. This list includes, but is not limited to, the categories described below, and shall be used as a reference for guiding the expenditure of CPA funds.

When a specific expenditure is not listed, yet management deems it to be in the best interests of the CPA to make the expenditure of the funds, then the criteria above for an allowable expenditure shall be followed. The CPA shall document the purpose of the expenditure, as listed at 7.710.21, E, 1, d, so that a “reasonable person” test can be made in the event the expenditure is audited. Each CPA shall allocate expenditures in accordance with its internal policies.

- a. Salaries and wages, and related benefits, employment taxes, unemployment taxes, worker's compensation taxes, retirement benefits, and insurance benefits.
- b. Retention and recruitment of staff, contractors, foster care parents, and volunteers, including advertising, background checks and other evaluations, recruiting, relocation, recognition, and food bank administrative expenses.
- c. Foster care, including provider reimbursement, respite for foster parents, child enrichment, clothing, health services, therapy, transportation, and other supportive services.
- d. Professional fees, including audit and accounting fees, consulting, legal fees, and other professional fees. These fees must be consistent with what the market would pay if paid to a related party.
- e. Contracted services, including contract labor, human resource services, payroll services, and temporary employment agencies.

- f. Occupancy expenditures, including office rent comparable to given market rates, meeting room rental, electricity and gas, water and sewer, janitorial service, property repair and maintenance, storage unit rent, and waste removal. If the CPA owns the building, then reasonable depreciation shall be allowed based on the estimated useful life of the building.
- g. Depreciation and amortization, based on estimated useful life of the asset, and prorated if the CPA only uses a portion of that asset.
- h. Child care during client meetings, support groups, or training functions.
- i. Training for staff, foster parents, and volunteers, including honorariums and conference registration fees.
- j. Travel and transportation provided that there is some contemporaneous record to support the expense, including airfare, fuel, lodging, meals, mileage, parking, per diem, travel incidentals, vehicle rental, and vehicle repair and maintenance.
- k. Telephone, including cellular and paging, data communication services, and telephone services.
- l. Postage and shipping.
- m. Office services, including coffee, tea, and water.
- n. Printing and supply expenditures, including computer and office supplies, printing, and program supplies.
- o. Dues, memberships, licenses, and subscriptions, including accreditation fees, dues and memberships, licenses and fees, and subscriptions and publications.
- p. Insurance, including automobile, directors and officers, volunteer, general and professional liability commercial packages, and umbrella.
- q. Marketing and promotion, including advertising, booths and fairs, graphics and editorial, and printed material/mailling house.

F. Unallowable Expenditures

A CPA expenditure shall be unallowable if it does not meet the criteria and documentation requirements as specified under the definition of an allowable expenditure as referenced in Section 7.710.21, E, or is a direct violation of federal law.

G. Remedies

Remedy for unallowable expenditures may include any or all of the following solutions:

1. Repayment to the State or Counties of identified unallowable expenditures.
2. Reclassification of the accounting entry to record the expenditure correctly, if the transaction can be appropriately reallocated to another cost center of the CPA, or affiliated agency, parent company, etc.
3. "Adverse licensing action" which could result in the denial, suspension, or revocation of a license issued, pursuant to the Child Care Licensing Act or the demotion of such a license to a probationary license.
4. Any other appropriate remedy based upon the facts and circumstances of the unallowable expenditure.

H. Intentional Mis-Use of Funds

Intentional mis-use of funds implies that the individual(s) making the expenditure decision had deliberate, willful, and intentional disregard for the fiduciary responsibility for how public funds are to be used for purposes of placing children in foster care or adoptive homes, or arranging for the placement of children in foster care or adoptive homes, considering their responsibilities to the CPA, its employees, clients, foster families, foster children, the public at large, the State Department's and the CPA'S responsibilities pursuant to the contract for services.

These rules do not preclude the State or county department(s) from pursuing other remedies available at law; for example:

1. Referral for prosecution; or,
2. Referral to the Internal Revenue Service for issues that violate Internal Revenue codes; or,
3. Repayment to the State and/or counties of identified unallowable expenditures; or,
4. "Adverse licensing action" which could result in the denial, suspension, or revocation of a license issued, pursuant to the Child Care Licensing Act, or the demotion of such a license to a probationary license.

7.710.22 Personnel Requirements

- A. Each CPA shall have staff members in sufficient number to meet the needs of individuals served. Such staff members shall meet the following requirements:

1. The Executive Director and placement supervisor must:
 - a. Reside in Colorado.
 - b. Submit proof of qualifications and certified transcripts from a regionally accredited college or university to the State Department within thirty (30) calendar days of accepting the position.
2. The Executive Director or in a multi-services agency, the program director, shall possess a knowledge of the type of child welfare services in which the CPA engages and shall be able to demonstrate administrative skill and leadership qualities. The Executive Director must have:
 - a. Graduated with a Bachelor's degree from a regionally accredited college or university with a minimum of thirty (30) semester credits or equivalent quarter credits in the social or behavioral sciences, and,
 - b. At least five (5) years' full time or equivalent part time work experience in an administrative capacity, obtained after the Bachelor's degree was conferred, at least two (2) of which included supervision of professional staff and budget or fiscal management; or,
 - c. Graduated with a Master's degree or greater from a regionally accredited college or university with a minimum of thirty (30) semester credits or equivalent quarter credits in the social or behavioral sciences, and have two (2) years full time or equivalent part time work experience in an administrative capacity, all of which must have included supervision of professional staff and budget or fiscal management; a minimum of nine (9) semester hours or equivalent quarter hours in business or finance classes from a regionally accredited college or university may substitute for budget or fiscal management experience.
3. Administrative capacity includes, but is not limited to, policy and procedure development and implementation, strategic planning, budget responsibility, fiscal management, quality assurance, networking, human resources management, program development and oversight, and business management.
4. The State Department must receive at least three (3) current written statements or telephone references from individuals unrelated to the applicant, at least one (1) of whom has been the employer or supervisor of the applicant, which describes the executive director's character, reliability, knowledge of child welfare services and ability to perform the tasks of the Executive Director as outlined in the duties of the Executive Director at Section 7.710.25, A. If the Executive Director or in a multi-service agency,

the program director, is also to have responsibility for placement supervision, she/he shall also meet the requirements for placement supervisor as set forth in Section 7.710.22, A, 6.

5. References checked through a telephone call must include the:
 - a. Name and position of individual called; and,
 - b. Name and position of individual spoken with; and,
 - c. Agency; and,
 - d. Telephone number; and,
 - e. Time and date the call(s) was made; and,
 - f. Relationship of individual providing reference for prospective employee (all references must be from individuals unrelated to the applicant); and,
 - g. Whether this was a personal or professional reference; and,
 - h. Summary of conversation that describes the character, reliability, knowledge, experience, and ability to perform the tasks of the position as outlined in the job description; and,
 - i. Name, position, date and signature of individual checking the reference.
6. Placement supervisors shall have, at a minimum, a Bachelor's degree in the social or behavioral sciences, and three (3) years full time child placement experience obtained after the bachelor's degree was conferred.
 - a. If the placement supervisor has a Master's degree in the social or behavioral sciences, the placement supervisor must have a minimum of one (1) year of child placement experience.
 - b. The State Department must receive at least three (3) current written statements from individuals unrelated to the applicant, one (1) of whom has been the employer or supervisor of the applicant, which describes the character, reliability, knowledge of child welfare services and the ability to perform the tasks of the placement supervisor as outlined in the duties for that position at Section 7.710.25, B.
7. If additional placement workers are necessary to fulfill the placement responsibilities of the agency, such workers shall be supervised by a qualified placement supervisor and shall hold a Bachelor degree in the

social or behavioral sciences from a regionally accredited college or university. If an individual has a Bachelor's degree in a non-related field, he/she shall have at least two (2) years experience supervised by an individual with a Master of Social Work or other Master's degree in the social or behavioral sciences.

- B. There shall be a sufficient number of placement supervisors and placement workers to meet the needs of the individuals being served in a timely manner. Each placement supervisor shall not supervise more than nine (9) FTE.
- C. There shall be sufficient support staff to comply with record keeping, bookkeeping and reporting requirements as necessary.
- D. Foster care paraprofessionals, interns or trainees who do not meet placement worker qualifications may assist qualified placement workers, but may not complete family or child assessments or conduct home supervision. Qualifications shall be established by the CPA.
- E. there shall be a sufficient number of Specialized Group Facility supervisors to meet the needs of the individuals being served in the group facilities in a timely manner. Each Specialized Group supervisor shall not supervise more than five (5) specialized group facilities.

7.710.23 Personnel Policy

- A. A written statement of personnel policy shall be provided to each employee or qualified applicant. This statement shall, as a minimum, contain the following information: a job description which outlines the duties, responsibilities, qualifications; policy on outside agency employment; and educational requirements for the position, as well as an organizational chart for the agency.
- B. The Board of Directors must approve a conflict of interest policy regarding outside employment.
- C. If an individual is employed or contracted, as a placement supervisor at more than one (1) child placement agency, the total number of individuals supervised by the placement supervisor at all agencies may not exceed nine (9) FTE.
- D. The agency must have a written policy regarding the use of volunteers. The policy must include:
 - 1. The duties the volunteers may perform;
 - 2. The requirement of background checks, including CBI and FBI criminal fingerprint histories, child abuse and neglect records, and CBI and National Sexual Offender Registry checks as listed in 7.701.32 and .33;
 - 3. Mandatory reporting of child abuse and neglect;

4. The requirement to maintain confidentiality.

7.710.24 Personnel File

- A. A personnel file shall be maintained for each employee and contract worker and shall be available to authorized representatives of the State Department.
- B. Each file for the employees and contract WORKERS shall include:
 1. Original certified transcript from a regionally accredited college or university if a degree is required by the position.
 2. Employment application showing qualifications and experience.
 3. A minimum of three (3) current written signed statements obtained from previous employers and personal references at the time of hire to show that the person has the qualifications required in Section 7.710.22.
 4. Evaluations of job performance.
 5. Results of the review of records and reports of child abuse or neglect as listed at Section 7.701.32 and criminal record check as listed at Section 7.701.33.
- C. Each file for the volunteers shall include a signed copy of the agency policy regarding the use of volunteers, a signed copy of the agency confidentiality agreement, signed mandatory abuse reporting requirement form, and copies of the CBI and FBI fingerprint history and record checks, child abuse and neglect background records checks, and CBI and National Sexual Offender Registry Checks as applicable in 7.701.32 and .33.

7.710.25 Duties of the Executive Director, Placement Supervisor, Placement Worker, Paraprofessionals/Trainees

- A. The responsibilities of the Executive Director are:
 1. Human resources management; and,
 2. Policy and procedures development and implementation; and,
 3. Accountability for being in compliance with regulations; and,
 4. Fiduciary requirements; and,
 5. Quality assurance; and,
 6. Regulatory compliance and accountability; and,
 7. Overall professionalism of the agency; and,

8. Responsibility for the daily operation of the agency; and,
 9. In a multi-service agency, the program director may have responsibility for administering the adoption and/or foster care unit and may not be responsible for the budget and accounting duties.
- B. The responsibilities of the placement supervisor are:
1. To oversee the study of potential foster or adoptive families using the Structured Analysis Family Evaluation (SAFE) instrument(s) to determine the character and suitability of the applicant(s), appropriateness of the home, and child care practices; and,
 2. To ensure certification of foster care homes and placement of children in foster care homes; and into specialized Group Facilities sponsored by the CPA, and/or,
 3. To oversee the study of the child for adoption; and,
 4. To oversee the placement of children in homes for adoption; and,
 5. To complete the SAFE Supervisor training and then review, sign, and maintain final authority of the SAFE assessment, post placement reports, and the issue certificate form;
 6. To ensure oversight of appropriate medical services for the children placed in the care of any facility certified or sponsored by the CPA; and,
 7. To provide supervision of placement workers at least monthly to review, at a minimum, all SAFE assessments in process, certifications in renewal status, and current placement activity; such supervision shall be documented in writing by the placement supervisor.
 8. To approve and monitor well-being plans developed for children in foster homes or group facilities.
- C. The placement worker, under the direct supervision of the placement supervisor, is responsible for the monitoring and protection of children, and may:
1. Provide case management for individual children; and,
 2. Coordinate services for child(ren) and their family; and,
 3. Provide monitoring and support to foster homes; and,
 4. Conduct SAFE assessments to determine the ability of foster homes to meet children's needs.

- D. The placement worker in an adoption agency may function as an adoption caseworker or birth parent counselor as listed in 7.710.52.
- E. Foster care paraprofessionals, interns, or trainees may assess the physical environment for foster homes for compliance with regulations; act as a liaison with courts, schools, foster parents and all peripheral parties under the direct supervision of a placement supervisor or placement worker.

7.710.3 CERTIFICATION OF FOSTER CARE HOMES

7.710.31 Legal Base

- A. Licensed Child Placement Agencies are authorized under Section 26-6-102, Colorado Revised Statutes, to certify foster care homes.
- B. A foster care home certified by a CPA may not accept placements from any source other than the certifying CPA as to each such child, unless the certifying CPA gives written consent and approval for the placement.
- C. A child placement agency shall complete a background check for foster care homes and kinship foster care homes pursuant to Sections 26-6-106.3(5), (6), and 19-3-406, C.R.S.; and the results shall be documented in the resource section of the state automated case management system. No children and/or youth may be placed in the foster care home or kinship foster care home until the checks have been completed. Failure to comply shall result in a corrective action process, and may result in sanctions described in Section 7.701.12. In addition, county departments of human or social services, as part of their contracting responsibilities, may take action to recoup foster care payments from the agency if a background check was not completed pursuant to statute.

7.710.32 Minimum Regulations

- A. Operation
 - 1. The regulations for operation of a foster care home shall be met before a certificate can be issued.
 - 2. The CPA shall audit the foster care home files on an annual basis to verify that all required information is present in the file. The CPA shall attest in writing that the required information is present.
 - 3. The CPA shall notify the Colorado Department of Human Services in writing within three (3) business days of the closure of a foster home, either verbally or in writing or via email, due to a confirmed report of child abuse or neglect.
 - 4. The CPA shall notify the Colorado Department of Human Services in writing within three (3) business days when a determination is made by

the CPA to continue to certify a foster care home that the CPA has received notice, either verbally or in writing, of a confirmed report for abuse or neglect. The written notice shall include the justification for continuing to certify the foster care home.

B. Certification

The law states that foster care certificates issued by CPAs are considered licenses; the regulations which are established by the State Department for foster care homes are therefore applicable to any such facility being certified by a licensed CPA. Copies of these rules shall be made available to each applicant for certification and to each foster care home.

C. Conflict of Interest

1. Staff members or members of the governing board or relatives of staff members or relatives of any officer, executive or member of the governing board of a CPA shall not be certified by the CPA to operate a foster care home, except for a person who is employed for the sole purpose of providing foster care and who serves in no other capacity for the agency.
2. No owner, officer, executive, member of the governing board, or employee of a CPA or any relative of said owner, officer, executive, member or employee shall hold a beneficial interest in any property operated or intended to be operated as a foster care home when the property is certified by the CPA as a foster care home.
3. A licensed CPA may not provide birth parent counseling, home study assessments, or post placement assessments on any officer, owner, board member, staff member, contract staff member, or relatives of such individuals.
4. An individual certified for foster care services by a CPA may not function as a staff member or volunteer at any specialized group facility sponsored by the same certifying child placement agency.

D. A CPA:

1. Must demonstrate to the State Department that it provides child placement services ethically and in accordance with Colorado state regulations and statutes, interstate compact requirements, intercountry requirements and Hague accreditation, as applicable, to ensure that foster and adoptive placements take place in the best interests of children.
2. Shall not knowingly and willfully:

- a. Disseminate or cause directly or indirectly to be disseminated, statements regarding services which are untrue, deceptive, or misleading; or,
- b. Make any statement or prepare or use any document that is known to be false; or,
- c. Conceal or misrepresent any material fact in connection with the provision of services to birth parents, foster or adoptive parents, foster or adoptive parent applicants, or children.

7.710.33 Application and Inspection for Certification of Foster Care Homes

- A. Any application accepted by the CPA from an individual(s) or couple who wishes to be certified to operate a foster care home shall be on the Department approved form and shall include:
 - 1. The names and addresses of child placement agencies and county departments of social services that had previously certified the applicant. Information as to whether the applicant has been licensed or certified for child care in the past or is licensed or certified for child care at the time of the application, what agency issued the certificate or license, and the type of child care the license or certificate authorizes.
 - 2. Information about an applicant or individual living in the proposed foster care home who has been convicted of a felony or charged or convicted of child abuse or an unlawful sexual offense.
 - 3. Information about whether the applicant is currently licensed by the State Department to provide day care.
 - 4. A statement on the application for certification as a foster care home that states:

“Any applicant who knowingly or willfully makes a false statement of any material fact or thing in this application commits perjury in the second degree as defined in Section 18-8-503, C.R.S., and, upon conviction thereof, shall be punished accordingly.”
 - 5. The social security number or individual taxpayer identification number issued by the government for each applicant.
- B. No application shall knowingly be accepted from an individual who is currently certified by another county or CPA to operate a foster care home until that individual has terminated the certification by the other county or CPA, or the current certifying CPA has given written notice to the foster homes of the agency’s closure.

- C. No board member, director or staff member of a CPA shall knowingly contact or recruit foster homes currently certified by another county department or CPA.
- D. A CPA must take an application from an applicant(s) before the CPA has authority to complete the family assessment, and background checks.
- E. An applicant may apply to become a dual care provider to operate a family child care home as well as a foster care home. The foster home will be certified by the CPA and the family child care home will be licensed by the Colorado Department of Human Services. Both sets of standards shall be met. The CPA will monitor the foster care standards and the State Department will monitor the family child care home standards. The CPA shall counsel the family if it believes such a situation is not in the best interest of any foster child who may be placed in the home. The CPA must approve the home to be licensed as a family child care home when the home is certified for foster care.
- F. A home that is licensed as a family child care home may only be certified for foster care for one child or for a group of siblings. A foster care home dually licensed as a family child care home shall not be certified as a host family home providing shelter to homeless youth.
- G. A CPA that has a foster/adoptive home that is certified for foster care and also licensed as a family child care home must notify the Division of Child Care when any of the following situations occur in the foster/adoptive home:
 - 1. A complaint is received; or,
 - 2. A child abuse investigation occurs; or,
 - 3. A Stage II investigation occurs; or,
 - 4. A foster/adoptive child is placed in the home; or,
 - 5. A foster/adoptive child(ren) is removed from the home because of abuse allegations; or,
 - 6. The foster home certificate is changed to probationary; or,
 - 7. The foster home certificate is revoked or closed.
- H. A CPA that has a foster/adoptive home that is certified for foster care and also licensed as a family child care home must submit the following reports to the Division of Child Care:
 - 1. All complaint investigation reports; and,
 - 2. All child abuse investigation reports; and,

3. All Stage II investigation reports.
- I. Reference checks for the applicant and all adults residing in the home:

A CPA shall conduct a reference check of each applicant and all adults residing in the home by contacting all of the previous certifying authorities listed on the application or for whom an application was submitted before issuing the certification for that foster care home. The CPA shall sign an affidavit that certification is appropriate.
- J. Initial Training for Foster Care Homes
 1. Each applicant listed on the application must complete a minimum of twenty-seven (27) hours of initial training consisting of at least twelve (12) hours of core training prior to the certificate being issued and completion of the remaining training within three (3) months after the placement of a child. The training shall be provided through the statewide core curriculum training, by the CPA, or by a county department. The core training shall include the following ten primary topic areas:
 - a. General overview of foster care; and,
 - b. Administrative and legal issues; and,
 - c. Why children get placed in out-of-home care; and,
 - d. Parenting and family dynamics; and,
 - e. Key concepts of child growth and development; and,
 - f. Importance of the team approach; and,
 - g. Individual differences such as ethnicity and culture; and,
 - h. Discipline; and,
 - i. Effects of fostering on the foster family; and,
 - j. Working with the biological family; and,
 - k. Reasonable And Prudent Parent Standard.
 2. For homes dually certified as a family foster care home under this Section and Section 7.708, and a host family home under Section 7.721, an additional three (3) hours of training specifically related to the subject of providing shelter to the homeless youth populations is required.

- K. Cradle Care applicant listed on the application must complete a minimum of twenty (20) hours of initial training prior to the certificate being issued. The training shall include the following primary topic areas:
1. Attachment/bonding issues; and,
 2. Loss and grief issues, as applicable for all parties to the adoption; and,
 3. Adoption as a lifelong issue as it pertains to all parties to the adoption; and,
 4. Key concepts of child growth and development; and,
 5. Limit setting and safety; and,
 6. Caring for a child of a different cultural or racial background, if applicable; and,
 7. Understanding adoption laws and procedures, including termination of parental rights and the expedited relinquishment process pursuant to Section 19-5-103.5, C.R.S., if applicable; and,
 8. Possible current and/or future use of community resources, including help with parenting techniques; and,
 9. Infant care to include, but not be limited to, basic care and feeding of the infant and Shaken Baby Syndrome; and,
 10. Fetal alcohol/substance abuse syndrome, if applicable; and,
 11. General overview of the adoption process; and,
 12. Why children get placed for adoption; and,
 13. The importance of the team approach; and,
 14. Effects of fostering on the cradle care family; and,
 15. Potential communication with biological family and/or adoptive family.

The cradle care provider must also hold a current infant/toddler CPR and first aid card and complete four (4) hours of on-going training a year on topics related to adoption.

- L. After the application is received and prior to the certificate being issued, a family assessment using the Structured Analysis Family Evaluation (SAFE) instrument(s) to determine the character and suitability of the applicant(s), appropriateness of the home, determination of the behaviors, diagnoses, and

disabilities of the foster children that the foster parents could care for, and child care practices must be completed.

1. An assessment of character and suitability must include at least a review of the State Department's automated system as to applicants and persons who reside in the home of the applicants with written consent of the individuals. A review of all existing child placement agency and county department case records including the automated system must be completed. An investigation of any concerns raised from the application and/or the aforementioned sources of information and a personal assessment of the applicant must be conducted. A review of the CBI and National Sexual Offender Registry must be completed as listed at section 7.701.33. A review of the abuse and neglect records must be completed as listed at section 7.701.32.
2. The agency will require any applicant or any person eighteen (18) years of age or older who resides in the foster care home to submit a complete set of fingerprints taken by a qualified law enforcement agency. The fingerprints and appropriate processing fee must be submitted to the Colorado Bureau of Investigation (CBI) to obtain any record of arrest or conviction which is held by the CBI and FBI.
 - a. The agency must send an applicant card (FD 258) to the individual required to provide fingerprints. The fingerprints must be taken by a police or sheriff's department. The individual or agency must send the card to the CBI with a certified check or money order for the amount of the current processing fee for both the CBI and FBI records. The CBI report must be sent to the child placement agency certifying the foster home.
 - b. A certificate cannot be issued to a foster care home until the fingerprints and current processing fee are submitted to the CBI and a clearance or hit/match is received from both the CBI and the FBI. If a hit/match with a criminal history arrest is received, the formal court disposition must be received and reviewed before a decision is made to issue a certificate.
3. The family assessment must be completed using the Structured Analysis Family Evaluation (SAFE) home study format. The SAFE home study assessment must be completed using all the tools and processes required by the SAFE format. Persons completing the home studies must be qualified, as a minimum, as a placement worker, with a bachelor's degree in the social or behavioral sciences, and must complete the Department-required training prior to performing the home studies. Persons approving the home studies must be qualified as a placement supervisor and must comply with the Department-required training prior to reviewing and approving the home studies.

4. Other Requirements
 - a. The application and medical records must be reviewed; and any issues that are identified be discussed with the applicants. No physical examination shall be required of any person who in good faith relies upon spiritual means or prayer in the free exercise of religion to prevent or cure a disease unless there is a reason to believe such person's physical condition is such that he/she would be unable to care for a child, or such person has a communicable illness.
 - b. The agency shall not perform a family home assessment on its own staff member, board member, or family member of a staff member or board member or relatives of such.
 - c. A current photograph of the family shall be requested and maintained in the file.
 - d. In kinship care, the ability to provide a permanent home through adoption, guardianship or permanent custody, including the ability to meet the individualized needs of the specified child(ren), assessment of the relationship with birth parents and extended family members as they impact capacity of the applicants to care for the child(ren), and The ability to set boundaries with birth parents to maintain safety for the child(ren) in care.
 - e. Statements from references and physician must be obtained.
5. As part of the assessment, the agency must:
 - a. Conduct a minimum of one joint interview with a couple, one individual interview with each adult member of the household and an age/developmentally appropriate interview with all children residing in the home. For single applicants, a minimum of two (2) interviews will be required.
 - b. Conduct at least one (1) interview in the applicant's home.
 - c. Ensure the second interview, and any subsequent interviews, of the adults shall not be performed until at least three (3) calendar days after the previous interview.
6. Following the completion of the assessment, a narrative report must be completed that summarizes and evaluates the information obtained and lists the characteristics of child(ren) the home is approved for. Age, sex, race, legal risk, and special needs (such as medical, physical, behavioral, emotional) and any limitations or restrictions on placement of a child(ren).

7. If there are changes to the marital or civil union status, or significant change of health status for persons in the foster family, or additions of adults or children to the foster family, a visit to the home must be conducted and such persons interviewed. A re-evaluation of the family must be completed and the safe assessment revised in the form of an addendum. If there are changes in the age, sex, and special characteristics of child(ren) which will be considered for placement with the foster family, the SAFE home study assessment must be revised in the form of an addendum. Such addendums shall be signed by the applicants or a notice provided to the applicants to inform them of their option to review such addendum.
 8. An on-site home inspection is required to determine its compliance with the Rules Regulating Foster Care Homes and, if the applicant is applying to be dually certified as a foster home and licensed as a family child care home, with the rules regulating Family Child Care Homes. Written documentation of the home inspection including square footage measurements of the sleeping areas shall be in the foster care home file at the child placement agency.
- M. An annual on-site, unannounced, home inspection must be made to the foster care home to determine compliance with the Rules Regulating Foster Care Homes and, if the foster care home is dual certified, with the rules regulating Family Child Care Homes. A written report of the supervisory visit must be given to the foster parent and a copy maintained at the child placement agency. A written notice of noncompliance with the regulations will be left with the foster parents or sent to the foster parents within fifteen (15) calendar days of the supervisory visit if there is noncompliance. Compliance must be achieved within the time frames indicated on the written compliance notice.

7.710.34 Issuance/Denial of Certificate

- A. After the completion of the family assessment/home study, one of the following certification actions must be taken:
1. A one (1) year time-limited certificate will be issued when it is determined that the applicant is competent, has completed the necessary training, and has met the Rules Regulating Foster Care Homes. The certificate issue date is the date that the assessment/study is completed and the foster home is in compliance with the Rules Regulating Foster Care Homes.
 2. A provisional certificate may be issued, upon the written approval of the State Department for a kinship foster care home or child specific placement when requested by a county department of social/human services.

The reasons for the issuance of a provisional certificate must be displayed on the certificate. The provisional certificate will be issued for up to ninety (90) calendar days from the date it is determined that time will be needed to comply with the appropriate kinship foster care home or child specific regulations. Only one original provisional certificate may be issued to a foster care home at one location address.

3. The original application will be denied. The renewal application will not be acted upon. The applicant will not be certified as a foster home.
- B. Upon issuance of the certificate, the child placement agency shall submit data entry information to the department on forms prescribed by the State Department.
 - C. The application will be withdrawn when the applicant no longer chooses to pursue certification.
 - D. An applicant shall be denied if the person(s) applying for the certificate has been determined to be insane or mentally incompetent by a court of competent jurisdiction and, should a court enter an order pursuant to Part 3 or Part 4 of Article 14 of Title 15, C.R.S., or Section 27-65-109(4) or 27-65-127, C.R.S., specifically finding that the mental incompetency or insanity is of such degree that the applicant is incapable of operating a family child care home, foster care home, child care center, or child placement agency, the record of such determination and entry of such order being conclusive evidence thereof. "Convicted" means a conviction by a jury or a court and shall also include a deferred judgment and sentence agreement, a deferred prosecution agreement, a deferred adjudication agreement, an adjudication, and a plea of guilty or nolo contendere for E, 1-6, below. This does not apply to a diversion, deferral or plea for a juvenile who participated in diversion (defined in 19-1-103(44), C.R.S.), and does not apply to an adult who successfully completed the child abuse and/or neglect diversion program (defined in 19-3-310, C.R.S.).
 - E. The application must be denied if the person(s) applying for the certificate has been convicted of any issues as listed AT 7.701.33,D,7.
 - F. The application may be denied for one (1) or more of the following reasons if the applicant(s), an affiliate of the applicant, or any person living with or employed by the applicant(s) has:
 1. Been determined to meet any of the criteria as listed at 7.701.33,F.
 2. Been assessed by the certifying agency to not meet the character and suitability requirements.
 - G. The denial of the original application must be carried out in accordance with the Colorado Revised Statutes, Section 26-6-108.

- H. The agency shall not deny to any person the opportunity to become a foster/adoptive parent on the basis of race, color, religion, sex, age, sexual orientation, gender identity, or national origin of the person or of the child involved.

7.710.35 Renewal or Continuation Notice

A renewal notice must be sent to the foster parents at least ninety (90) calendar days prior to the expiration of the certificate.

- A. If the foster parents wish to continue to provide care, the renewal notice must be completed and returned to the child placement agency prior to the expiration of the certificate.
- B. If the renewal notice is received by the child placement agency prior to the expiration of the certificate, the renewal notice is timely, and the certificate continues valid until action is taken by the child placement agency.
- C. If the renewal notice is received after the expiration of the certificate, the renewal notice is untimely, and the certificate is no longer valid. The untimely renewal notice must be acted upon as an original application.

7.710.36 Recertification Action

- A. Upon receipt of a timely renewal application for a certificate, and prior to the expiration of the current certificate, the child placement agency must complete the following actions:
 - 1. Evaluate the foster care homes' current and past compliance with the Rules Regulating Foster Care Homes.
 - 2. Conduct an unannounced inspection of the foster care home in accordance with Section 7.710.33, M.
 - 3. Review the following information, for the applicants) and all individuals residing in the home, to determine if continued certification is appropriate:
 - a. Any child abuse allegations or investigations in the previous year;
 - b. Any arrest or conviction records in the previous year;
 - c. Any information from the local county department concerning county involvement with the foster family, including information from caseworkers;
 - d. Current health status of each household member;

4. Review and update the SAFE home study assessment in the form of an update. The update shall include at least one (1) home visit. Such updates shall be signed by the applicants or a notice provided to the applicants to inform them of their option to review such update.
 5. Conduct a search on the CBI sex offender registry and national sex offender public website operated by the United States Department of Justice, and include a copy in the provider record using the following criteria at a minimum:
 - a. Known names and addresses of each adult residing in the home; and,
 - b. Address only of the home.
 6. If the foster parent or any adult living in the foster home left the state for three (3) consecutive months or longer, a new FBI fingerprint-based criminal history record information check shall be conducted.
- B. At the time of the renewal of the certificate, one of the following must be completed:
1. A provisional certificate for a kinship or child specific placement will be changed to a one year time-limited certificate as soon as the foster family has completed the items listed as reasons for the provisional certificate.
 2. A new one (1) year time-limited certificate is issued. The certificate issue date will be the date that the foster care home is in compliance with the Rules Regulating Foster Care Homes, as found at Section 7.708.
 3. The renewal application for the certificate is denied. The process for denial of a renewal application is the same as the process for denial of an original application as listed at 7.710.34.
- C. Upon issuance of the one (1) year time-limited certificate, the child placement agency must submit data entry information to the department on forms prescribed by the department.
- D. A foster care home certificate is no longer valid whenever one of the following situations exists:
1. A certified foster family moves to a new address.
 2. A foster family decides to withdraw from the foster care home program and confirms same in writing.
 3. A certificate has been revoked or denied.

4. A certificate has expired.

7.710.4 PLACEMENT OF CHILDREN IN FOSTER CARE

7.710.41 Acceptance of Children for Placement

- A. No agency shall accept a child for placement, except as provided in paragraph B below, from any source other than the child's parent(s) or guardian(s), a court of competent jurisdiction or a county or tribal department of social services and upon a specific written authorization by one of these to place the child, as only these have the right under the law to contract for a child's placement.
- B. If a law enforcement officer places a child in case of an emergency, when the parent or guardian cannot be located, in a facility which has been designated as a shelter facility in accordance with the law, and the law enforcement officer is unable to locate the child's parent, guardian or the persons with whom the child was living, the agency shall petition the appropriate court within forty-eight (48) hours for legal custody.
- C. If a child placement agency no longer chooses to place children in the foster care home, the child placement agency shall follow one of the following procedures:
 1. A provisional certificate may be allowed to expire if the foster family chooses not to submit a renewal application; or,
 2. The child placement agency must send a written statement to the foster home explaining that the agency will no longer place children in the home for foster care, that the home must not accept any children for care from any other source; or,
 3. The child placement agency must meet with or send a letter to the foster parents requesting them to sign a statement that they are withdrawing from the foster home program.

7.710.42 The Placement Process

- A. In addition to an agency's responsibility to inspect and to supervise the ongoing operation of certified foster care homes, agencies shall comply with the following minimum standards applicable to the placement process.
- B. When accepting a referral, the agency shall conduct an assessment to determine whether placement of an individual child is appropriate and desirable. The following shall be included in this determination:
 1. The reason(s) for placement; and,
 2. Information relating to the child's natural parents or family situation including religious, educational, economic and cultural background and

other factors which should include, but are not limited to, consideration of the child's family, community, neighborhood, faith or religious beliefs, school activities, friends, and child's and family's primary language; and,

3. Information relating to the child's legal status, medical or health history, his/her physical condition, personality, school placement and adjustment, previous placements outside the home, attitude toward removal from the home, and family relationships, and preferences of the child when age and developmentally appropriate; and,
 4. Coordination with other social service agencies or departments which may have information relating to the child; and,
 5. Additional information, if any, designed to assist the agency in determining what type of placement, if any, will best meet the needs of such child.
- C. The agency must make available the original written family assessment, home study, and background checks of the foster parent(s) to the placing authority upon request. Updates to the family assessment, home study, and background checks must be sent to the placing authority.
- D. A child who is to be placed in a foster care home or child care center pursuant to Sections 7.500.21 and 7.500.22 respectively shall only be placed in a home or center that is licensed by the State Department or certified by a licensed placement agency or a county department of social services.
- E. The agency shall discuss information deemed necessary regarding a prospective foster child with foster parents as early as possible prior to placement. It is desirable for the child to visit and become familiar with the foster parents and other persons living therein prior to the time of placement.
- F. At the time of foster placement, the agency shall complete a record of admission for the foster care home parents as outlined in, Section 7.708.51, C, and be assured that the foster care home parents have a copy of a signed authorization for foster parents to obtain emergency medical care for the foster child, if necessary.
- G. No placement shall be continued where it is not in the best interest of the child.
- H. If a child is placed in a family child care home, the placement agency shall be assured that the child's admission record and all admission procedures as stated Sections 7.707.5 and 7.707.51, are completed.

7.710.43 Responsibility of Placement Agency While the Child is in Care

- A. The placement agency shall work as closely as possible with the child's natural parent(s), guardian(s), individual or agency with legal responsibility for each child

- with the view to maintaining a child in his own home, placing the child in foster care temporarily or appropriate permanent substitute care.
- B. The placement agency shall be assured that care is provided the child in placement in accordance with the applicable regulations, which shall include but not be limited to applicable medical, dental and optical care, and participation in appropriate educational and recreation experiences.
 - C. The placement agency shall require the foster home to maintain written documentation of all medical, dental, and optical care appointments of foster children, including the need for follow-up, next visit scheduled, and the reason the child was taken to the doctor. The written documentation shall be submitted by the foster home to the agency on at least a monthly basis. The agency is responsible to send the written documentation of the medical appointments to the caseworker for each foster child in care on a at least a monthly basis.
 - D. The agency shall provide on-going training for foster care parents to help improve their ability to care for children in placement. The training may include orientation and group meetings, publications, institutes, workshops, and consultation with experts.
 - E. Professional staff of the agency shall visit the home at least monthly to observe the interaction of the foster children with the foster parent(s) and to generally assess the safety of the home. Such visits shall be documented and maintained in each foster child's file.
 - F. The agency is responsible to notify all placing agencies when a child is removed from care because of concern for the child's health, welfare, and safety and other children remain in care.
 - G. The agency and/or the foster parent(s) shall attend Administrative Reviews in person or by conference call.
 - H. The reimbursement rate for child maintenance agreed upon between the CPA and the county department shall be paid to the CPA foster parents for the care of the child.
 - I. The reasonable and prudent parent standard requirements for any foster parent to approve activities for a child or youth in foster care requires the following action:
 - 1. The child placement agency shall train foster parents how to determine whether to approve a child's or youth's participation in an extracurricular, enrichment, cultural, or social activity consistent with the reasonable and prudent parent standard based upon the criteria in section 7.701.200.]
 - J. Child placement agencies contracting for foster care services when a county department of human/social services does not have a foster care certification

program and it involves the placement of a child or youth for foster care through the interstate compact placement for children (ICPC) shall take the following steps:

1. Contract with the sending state to provide all services for a private placement; or,
2. Enter into a written agreement with the county department of human/social services with ICPC responsibility that includes but is not limited to, the services and duration of the services to be provided by the child placement agency.

7.710.44 Termination of Placement

- A. The agency shall counsel and assist parent(s) or guardian(s), foster parents and foster children in preparing for the termination of placement.
- B. When a placement is terminated, the child shall be released only to his parent(s) or guardian(s) or to a court of competent jurisdiction. If a child has been received for care upon court order, the child shall be released only upon direction of the court.
- C. Upon termination of placement, the agency shall provide the parents or the agency receiving the child with pertinent health information and other records, such as school reports, which may be useful to persons with responsibility for such child.

7.710.45 Required Records

- A. A placement agency shall be responsible for maintaining an individual case record for each child accepted for care. Records for siblings in care shall be individual and not co-mingled. All records are confidential and shall be protected from unauthorized examination.
- B. The State Department staff shall have access to such records upon request,
- C. Foster care home parents are to receive necessary information regarding the foster child in care pursuant to Section 7.710.42; and such foster parents shall be given detailed instructions regarding the confidential nature of information which they receive.
- D. The agency record for each child shall contain at a minimum:
 1. Report of the original intake study; and,
 2. An agreement signed by the parent or guardian authorizing the agency to place the child in foster care and consenting to necessary medical and

- surgical care. A court order transferring legal custody to the agency will fulfill this requirement; and,
3. If the child is placed in a foster care home or specialized group facility, a copy of the record of admission form as required in Section 7.708.51, C; and,
 4. Documentation of the legal custody and responsibility for the child; and,
 5. Reports completed by the care provider of the child's progress under care; and,
 6. School reports including records of scholastic achievement and social adjustment; and,
 7. Individual medical records for each child including reports of the admission examination and a complete and continuous record of illness, immunization, communicable diseases and follow-up treatment and examination; and,
 8. Reports of psychological tests, psychological or psychiatric examination and follow-up treatment if obtained; and,
 9. Record of visits to the child and record of the contacts with child's own family and services to be provided or for which arrangements have been made; and,
 10. Copy of the treatment plan for the child in specialized group care, or the family services plan for the child in foster home care.
- E. The record for each child placed by the agency in foster care shall be maintained at least three (3) years after foster care has been terminated.
- F. Each placement agency shall maintain a separate record for each foster care home certified by the agency. This record shall include: the application; all relevant information obtained at the time of certification and recertification summary reports of subsequent visits to the home; and, a list of the children placed in the home, including names, birth dates, dates of placement and reasons for removal. This record shall be available to the staff of the State Department for inspection.
- G. Records relating to foster care homes whose certificates have expired shall be retained for at least FIVE (5) years after the expiration of the last-issued certificate.

7.710.46 Personnel Qualifications and Duties as Sponsoring Agency of Specialized Group Facilities

- A. The specialized group home or center shall be supervised by a paid staff member of the agency (specialized group facility supervisor) who holds a Master of Social Work degree or a Master's degree in behavioral science with an emphasis in child development and/or family relations and a minimum of two years paid full time or equivalent part-time experience in social work. If the staff member does not have the aforementioned education and experience, then he/she must have a Bachelor's degree with a major in social work, sociology, psychology or closely related field and be directly supervised by an agency staff member holding the Master degree and experience described above.
 - 1. The placement supervisor may serve as the specialized group facility supervisor.
 - 2. The placement supervisor is responsible to approve all placements into the specialized group facility.
- B. The specialized group facility supervisor shall obtain, provide and/or coordinate the following services:
 - 1. Group and/or individual counseling for children in care and their families.
 - 2. For each child, intermediate, short term, and long term goals shall be established and a case plan written. The goals and case plan must include a plan for discharge and must be developed and evaluated pursuant to regulation section 7.714.70, D. Goals and case plan for children three and four years old shall be evaluated monthly.
 - 3. Staff records for the home or center as required in section 7.709. 26.
 - 4. Psychiatric, psychological or developmental evaluations and consultations as required.
 - 5. Specialized educational resources as required.
 - 6. Consultation with the home or center parents or personnel about methods of working with the children.
 - 7. Assessment of quality of care with the home or center parents or personnel.
 - 8. Children's records as required in 7.714.932.
- C. The specialized group facility supervisor shall meet with the group home or center parents or primary caregiver a minimum of two hours per week exclusive

of counseling services, to discuss individual children, discharge planning including any barriers, problems, program and/or special needs. Such supervision of the group facility shall be documented in writing by the specialized group supervisor and shall be available to the department upon request.

7.710.47 Role of a Licensed Child Placement Agency to Establish and Supervise a Specialized Group Home or Center

- A. The supervisory responsibilities of the sponsoring agency are:
1. To be knowledgeable with the rules regulating specialized group facilities;
 2. To participate in the development and application process to include verifying that the original application submitted is complete with all required signatures and submitted in a timely manner;
 3. To provide ongoing assessment of the specialized group facility for quality of care issues; and
 4. Provide annual evaluations of the governing body, unless the governing body and the sponsoring agency are the same agency.
 5. To provide training to all staff members on the Reasonable and Prudent Parent Standard to approve activities for a child or youth in care. Such training shall include how to determine whether to approve a child's or youth's participation in an extracurricular, enrichment, cultural, or social activity consistent with the Reasonable and Prudent Parent Standard based upon the criteria in section 7.701.200.
- B. The sponsoring agency shall be responsible to ensure that state rules are followed regarding:
1. Hiring, training and scheduling of staff;
 2. Placement decisions including, but not limited to, appropriateness of placement and least restrictive environment; and
 3. Documentation, reporting and corrective action of critical incidents.
- C. The sponsoring agency, governing body, and group home or center parents or primary caregiver shall develop and adhere to a statement of purpose and function, which includes a description of the characteristics of the child or youth population which is to be served by the facility.
- D. The sponsoring agency and the group home or center parents or primary caregiver shall develop and adhere to written policies and procedures regarding the care of children which shall be reviewed annually and shall include the following:

1. Provision for emergency procedures including illness, accident, fatality and fire; and
 2. Participation in special activities in compliance with section 7.719.
- E. The group home or center parents or personnel and the sponsoring agency shall develop and adhere to written policies and procedures regarding personnel including: pay, provision of relief time and vacation time, annual performance evaluation, training opportunities, selection of personnel and maintenance of personnel records.
- F. Financial operation and oversight of the sponsored specialized group facilities shall include:
1. Each CPA shall ensure that each specialized group facility that is sponsored by the CPA develops an annual budget reflecting anticipated income by source and expenses by purpose, plus an accompanying balance sheet, which demonstrates that the Specialized Group Facility has resources to carry out its defined purpose. The budget shall be approved by the Executive Director of CPA and documentation of such maintained at the CPA primary office. The first year's budget shall be submitted with the original license application and to the CPA annually thereafter at the time of license continuation.
 2. Each specialized group facility sponsored by a CPA whose total annual foster care or adoption expenditures are \$100,000 or more shall provide for an annual audit by an independent certified public accountant in accordance with appropriate generally accepted auditing standards. Specialized group facilities sponsored by a CPA with less than \$100,000 total annual expenditure may submit an audit as described above or may submit compiled or reviewed financial statements, prepared in accordance with generally accepted accounting principles.
 - A. The supplementary information submitted shall contain an affidavit signed by the child placement agency Executive Director, the primary care provider of the specialized group facility, and the governing body authority of the specialized group facility attesting to the authenticity of the information. Submission of falsified information shall be grounds for suspension of the child placement agency and specialized group facility licenses.
 - B. The audit and supplementary information shall be submitted to the state department within six (6) months of the specialized group facility's fiscal year end.
 - C. If a specialized group facility sponsored by a CPA does not submit its annual audit or refuses to disclose financial information regarding the operation of the program in a timely manner, the state

department may send notice to withhold payment until the audit and/or requested information is submitted.

- D. Upon receipt of adequate written notice that a county department or the state department plans to recover or withhold unallowable or misused funds from the CPA sponsoring a specialized group facility, the CPA may file a written request for a review of the decision with the state department as listed at 7.710.21, d, 6, a-e.
 - E. Allowable expenditures are listed at 7.710.21, e.
 - F. Unallowable expenditures are listed at 7.701.21, f.
 - G. Remedies are listed at 7.710.21, g.
 - H. Intentional misuse of funds are listed at 7.701.,21, h.
3. The CPA shall ensure proper vehicle, property, and business insurance for each specialized group facilities they sponsor, and maintain current copies of such at the licensed child placement agency location.

7.710.48 RULES REGULATING THE CARE OF CHILDREN IN FOSTER HOMES WHEN CARE IS ALSO PROVIDED FOR ADULTS WITH DEVELOPMENTAL DISABILITIES

- A. No agency shall accept a child for placement from any source other than the child's parent(s) or guardian(s), a court of competent jurisdiction or a county or tribal department of social/human services and upon a specific written authorization by one of these to place the child. Such written authorization must contain notification that the child is to be placed in a foster home where adults with developmental disabilities are also receiving care.
- B. The foster home shall meet all regulations as listed in 7.710.33.
- C. The capacity of the foster home when adults with developmental disabilities are also in care shall not exceed a total of five persons requiring care through the foster care system and/or the adult intellectual and developmental disabilities (IDD) system.
 - 1. When a foster child turns eighteen and is eligible for the adult residential system through the Division of Intellectual and Developmental Disabilities, the child shall be considered an adult receiving care for the purpose of capacity. If the county/state Department of Social/Human Services has legal responsibility for the care and placement of the foster child turning eighteen, the individual will be considered a child for the purpose of capacity.

2. For foster children/youth enrolled in the Children's Habilitation Residential Program (CHRP), a maximum of three children receiving CHRP waiver services, and a total of two other persons requiring care, either an adult with developmental disabilities or a non-CHRP child/youth is allowed.
 3. All children under the age of eighteen residing in the home count in the total foster home capacity of eight (8) people needing care. Also refer to 7.708.1A.
- D. When a foster child in the home turns eighteen, if such child is eligible for the adult host system, such person must complete background checks as listed at 7.701.32 and 7.701.33, and a home study addendum as listed at 7.710.33, I, 7 must be completed.

7.710.49 Rules Regulating Host Family Homes

Child placement agencies are authorized to certify host family homes to provide shelter to homeless youth. All child placement agencies that certify host family homes must follow the rules regulating host family homes as listed at 7.721.

7.710.5 PLACEMENT OF CHILDREN FOR ADOPTION

7.710.51 Legal Basis

- A. A CPA that places children or arranges for the placement of children for the purpose of adoption shall comply with the provisions of the Child Care Licensing Act, Sections 26-6-101 through 26-6-114, C.R.S., and the Colorado Children's Code, Sections 19-5-101 through 19-5-403, C.R.S.
- B. A CPA must be approved in writing by the State Department for the type of adoption in which it engages, either domestic and/or intercountry, prior to engaging in that type of adoption. The agency must have current written policies and qualified staff at all times if engaging in intercountry and/or domestic adoption.
- C. A CPA cannot work with a facilitator to place or arrange for the placement of a child if that facilitator is not licensed as an adoption agency in Colorado or the resident state of the birth mother or adoptive parent(s).
- D. In the case of intercountry adoptions, a child placement agency is permitted to work with an in-country coordinator who resides in the foreign country and is authorized by the foreign country to provide in-country services.
- E. Foreign visiting children's programs in which children who are eligible or may become eligible for adoption and who reside with Colorado families must comply with all applicable child placement agency rules and regulations. All families participating in such programs must meet all required standards and be certified as foster parents during such time the children reside with them.

7.710.52 Adoption Definitions

“Adoption exchange” is an agency whose membership includes county departments and child placement agencies and is an organized means of sharing information among agencies about children for whom an adoptive resource is not immediately available and about potential adoptive families for whom an agency does not have a child waiting. The purpose of an exchange is to facilitate permanence as quickly as possible so that a child does not wait while an agency develops a resource.

“Administrative notice procedure” applies only to expedited relinquishments filed under Section 19-5-103.5, C.R.S., and allows the CPA counseling the birth mother to provide notice to the presumed Birth father of an anticipated expedited relinquishment prior to the filing of the relinquishment petition. The notice to the presumed birth father cannot be made more than sixty (60) days prior to the birth of the child.

“Agency adoption” means an adoptive placement in which the CPA is responsible to counsel the birth parent(s), place the child(ren), supervise the placement, and provide reports to the court as required by law. The CPA is granted custody of the child(ren) with the right to place for adoption either as legal risk placement or through relinquishment or termination of parental rights by court order and places the child(ren) for adoption with adoptive parent(s) who have a CPA approved adoptive family assessment.

An “adoption caseworker” must be qualified as a placement worker and may complete family assessments, place a child with a family, provide post placement supervision and training for adoptive parents, all under the direct supervision of a placement supervisor.

“Birth parent counseling”, here-in-after referred to as “counseling”, means the required decision making counseling that shall be provided to a birth parent(s) or legal parents prior to a decision regarding whether or not to relinquish a child(ren) as required at 7.710.57.

A “birth parent counselor” must be qualified as a placement worker and may have responsibility for relinquishment/decision making counseling with birth parent(s) dealing with an unplanned or crisis pregnancy or the relinquishment of a child, all under the direct supervision of a placement supervisor.

“Closed adoption” means an adoptive placement when the adoptive parent(s) and the birth parent(s) do not share identifying information or communicate with each other before or after the finalization of the adoption.

“Concurrent adoption” means more than one (1) adoption being processed simultaneously either through the same agency or multiple agencies.

“Convention” means the Convention on protection of children and cooperation in respect of intercountry adoption done at the Hague on May 29, 1993.

“Convention adoption” means the adoption of a child resident in a Convention country by a United States citizen or an adoption of a child resident in the United States by an individual or individuals residing in a Convention country when in connection with the adoption the child has moved or will move between the United States and the Convention country.

“Convention country” means a country that is a party to the Convention and with which the Convention is in force for the United States.

“Country of origin” means the country in which a child is a resident and from which a child is emigrating in connection with his or her adoption.

“Designated adoption” means an adoptive placement in which adoptive parent(s) have been designated by the birth parent(s) prior to either parties involvement with a CPA licensed for adoptions. The CPA is responsible for completing the birth parent(s) counseling and to ensure an adoptive family assessment is completed for the designated parent(s). The CPA is granted temporary custody of the child for the purpose of placement.

“Dissolution” means the termination of the adoptive parent(s)’ parental rights after an adoption.

“Disrupted adoption” means the interruption of a placement for adoption during the post-placement period.

“Domestic adoptions” means collectively identified or designated adoptions, interstate adoptions, and intrastate adoptions.

“Expedited Relinquishment” means the legal process which a parent desiring to relinquish his or her child under one (1) year of age may follow to obtain an expedited order terminating his or her parent-child legal relationship without the necessity of a court hearing.

“Facilitator” means a person, partnership, corporation, association, firm, agency or institution, other than an adoption exchange, county department or child placement agency, who offers, gives, charges or receives any money or other consideration or thing of value in connection with locating or identifying for purposes of adoption any child, birth parent, expectant natural parent or prospective adoptive parent.

“Foreign national” means a child who was born outside the United States of America (U.S.A.), at the time of placement lacks U.S.A. citizenship, and is a resident outside the U.S.A.

“Foreign visiting children’s program” means a program in which children visit Colorado from other countries and may or may not be available for adoption.

“Foster care adoption” means a placement in which the child(ren)’s parental rights have not been terminated at the time of the foster care placement. The placement is made as

a long-term foster care placement with the intention of adoption if or when the parental rights are terminated.

“ICPC” means the Interstate Compact for the Placement of Children which is an agreement that has been enacted into law by all fifty (50) states in the United States and the District of Columbia, which controls the lawful movement of children from one state to another for the purposes of adoption. Both the originating state, where the child is born, and the receiving state, where the adoptive parents live and where the adoption of the child will take place, must approve the child's movement in writing before the child can legally leave the originating state. This Compact regulates the interstate movement of both foster children and adoptive children. For further information or to obtain a copy of “Guide to the Interstate Compact on the Placement of Children”, contact the: American Public Human Services Association, 1133 Nineteenth Street, NW, Suite 400, Washington, DC 20036. Phone: 202-682-0100; Fax: 202-289-6555.

“Intercountry adoption” means the placement of children emigrating from their country of origin. The placement for the purpose of adoption of foreign national children with approved adoptive parents in Colorado is considered an incoming or immigrating case. The adoption may be finalized in Colorado or in the foreign country depending on the requirements of the foreign country. If the foreign country in this instance is party to the Hague Convention on Intercountry Adoption, the adoption must be in compliance with the Intercountry Adoption Act of 2000 and with federal regulations 22 CFR Parts 96, 97, and 98. U.S. children placed in a foreign country for the purpose of adoption is considered an outgoing or emigrating case. If the foreign country is party to the Hague Convention, the adoption must be in compliance with the Intercountry Adoption Act of 2000 and Federal Regulations 22 CFR Parts 96, 97, and 98.

“Interstate adoption” means a placement of a child into or from Colorado with a person(s) for the purpose of adoption. Placement shall be in conformity with Colorado Revised Statutes, including the Child Care Licensing Act, the Colorado Children's Code and the Interstate Compact on Placement of Children. Services to the child and adoptive family shall be provided under the laws of each state.

“Intrastate adoption” means an adoptive placement of a Colorado child with a person(s) who is a resident of Colorado.

“Legal risk adoption” means an adoptive placement where parental rights have not been relinquished and/or terminated and the child(ren) is not yet legally free for adoption at the time of placement. This includes infants placed directly after birth. The child(ren) is placed in an approved adoptive home. If the adoptive family resides in Colorado, the home shall be certified as a foster home. The placement is with the intention of adoption when or if the child becomes available for adoption. The placement remains a legal risk placement until parental rights have been relinquished or terminated.

“Non-agency adoption” means a placement in which the child(ren) is placed by the birth parent(s) with person(s) unrelated to the child(ren). The placement is made with the intention of adoption. There has been no CPA participation, birth parent counseling, or

adoptive family assessment completed before the placement of the child(ren). In order for the adoption to be finalized, the birth parent counseling and approved family assessment shall be completed by a CPA or county department of social services.

“Open adoption” means an adoptive placement where the adoptive parent(s) and the birth parent(s) choose to communicate with each other and share identifying information. The communication and sharing of information may occur before and/or after the finalization of the adoption. The degree of openness is determined by the parties involved.

“Relative adoption” means a placement for the purpose of adoption in which a child(ren) is placed with a person(s) related to the child(ren) as a grandparent(s), aunt, uncle, brother(s), or sister(s).

“Selected agency” means the non-public agency selected by, the State Department, pursuant to Section 19-5-205.5, C.R.S., to perform the administrative review and approval or denial functions required by the Interstate Compact on the Placement of Children (ICPC) and statutes governing foreign adoptions. Agencies selected by the State Department to provide these functions shall hereafter be referred to as “selected agencies”.

“Semi-open adoption” means an adoptive placement when the adoptive parent(s) and the birth parent(s) choose to share non-identifying information with each other either before and/or after the finalization of the adoption. The amount of information shared is determined by the parties involved. All information shared is sent to the adoption CPA.

“USCIS” means the United States citizenship and Immigration Services.

7.710.53 Disclosure and Fee Information

- A. Prior to signing the adoption services contract or other specific agreement for adoptive services or payment of any fees, each agency must provide in writing the following information to any applicant(s) approaching the agency with an interest in adopting a child(ren). The information must be easily understandable to any applicant. Applicants must sign a statement affirming that they have received the following information:
1. The most current child care license for the agency; and,
 2. The philosophical perspective and/or religious affiliation of the agency; and,
 3. Information as to where any prospective adoptive parent may obtain:
 - a. A list of all licensed adoption agencies in the State of Colorado; and,

- b. Information as to how to review the official licensing file for the agency; and,
 - c. Information as to how to file a complaint regarding the agency with the Colorado Department of Human Services; and,
 - d. Information as to how to obtain the rules regulating child placement agencies in the State of Colorado.
- 4. For what programs the agency has been approved (foster care, domestic adoption, intercountry adoption) by the State Department including a detailed description of the services offered and the agency's role in those services; and,
- 5. A statement that parents are paying for services provided by the agency, not for children; and,
- 6. A copy of the most current annual report submitted to the State Department as found at Section 7.710.73, A; and,
- 7. A fee schedule listing all the costs of the adoption itemized by services; including incidental fees, additional expenses, and post-finalization fees if included in the cost for the adoption, as listed in Section 7.710.53, C; and,
- 8. Average time frame for the different services provided, including the family assessment and for the complete finalization of the adoption; and,
- 9. Description of required training for adoptive parents, as required in Section 7.710.55; and,
- 10. The process of sharing available information regarding the child and the birth family as provided in accordance with Colorado statute pertaining to identifying and non-identifying information; and,
- 11. Provide information about the birth parent contact preference form and the medical history statement that the birth parent can file with the State Registrar, including the ability of the birth parent to change the preference form at a later date; and,
- 12. Any policy that would result in the denial of services or would preclude placement of a child with an adoptive family; and,
- 13. The grievance/appeal process of the agency; and,
- 14. Post adoption services offered or required by the agency as found at Section 7.710.62 and the cost of such services; and,

15. The right to seek legal counsel to further understand Colorado adoption laws; and,
 16. Process of record storage and maintenance in the event of closure of the agency; and,
 17. Any service if provided or offered by the CPA is provided by another agency, entity, or individual; and,
 18. That applicants who reside in Colorado and whose adoptions will be finalized in the State of Colorado must apply for and be certified as foster care parents until the finalization of the adoption; and,
 19. The agency's policy on concurrent adoptions; and,
 20. That only pregnancy related expenses may be paid to a birth parent and all payments made on behalf of a birth parent must be processed through the agency; and,
 21. The requirement of the agency to collect fees and submit court documents after the completion of an intercountry adoption to validate such adoption in the United States court.
- B. Prior to the provision of services, or for some items during the birth parent or legal parent counseling, each agency must provide the following information in writing to any parent(s) approaching the agency with an interest in the possible relinquishment of a child(ren) for the purpose of adoption. The written information must be easily understandable to any parents in language or manner they can understand. Disclosure items 1 through 8, below, must be provided prior to service. Disclosure items 9 through 18 may be provided to the parent(s) prior to service and/or during the parents' counseling process.
1. Information as to whether the birth or legal parent(s) is required to sign a contract or agreement with the agency.
 2. A list of birth or legal parent rights under current Colorado law including, but not limited to:
 - a. All of the legal options related to the relinquishment procedures presented in an accurate, competent, unbiased manner; and,
 - b. A fair and balanced picture of adoption presented; and,
 - c. Decision-making related to the possible relinquishment of a child in an atmosphere free from coercion and undue pressure; and,
 - d. Option, by law, to change the decision about relinquishment and to choose to parent the child. The decision can be changed at any

- time prior to the final order of relinquishment being signed by the court, unless otherwise limited by a court order if a court hearing is held; and,
 - e. Written grievance procedure for the agency; and,
 - f. Philosophy and/or religious affiliation of the agency; and,
 - g. Referral to helping organizations in the community if a decision is made to parent the child; and,
 - h. Termination of the relationship with the agency parent counselor at any point during the counseling relationship; and,
 - i. To seek independent counsel on any issue related to the relinquishment of a child(ren), including the right to seek independent counseling for an expedited relinquishment procedure.
3. The following must also be provided to birth parents considering relinquishment of a newborn infant:
- a. The right to receive a copy of any document signed by the birth parent(s) and to receive a copy of the original birth certificate; and,
 - b. The right to have all of the legal options related to a pregnancy presented in an unbiased manner; and,
 - c. The right to receive, in writing, the process by which an expedited relinquishment affidavit is withdrawn, including a personal meeting with a representative of the agency and the signing of a statement of withdrawal which is witnessed and signed by an agency representative; a copy of the statement with original signatures must be provided to the birth parent(s); and,
 - d. The right to withdraw an affidavit for expedited relinquishment at any time before the affidavit is filed with the court. In the case of expedited relinquishment, the petition for relinquishment may not be filed until at least four (4) calendar days after the birth of the child.
4. Range of available adoptive families, to include religion, race and ethnicity.
5. Information as to where any parent considering relinquishing a child for adoption may obtain:
- a. A list of all licensed adoption agencies in the State of Colorado.

- b. Information as to how to review the official licensing file for the agency.
 - c. Information as to how to file a complaint regarding the agency with the Colorado Department of Human Services.
 - d. Information as to how to obtain the rules regulating child placement agencies in the State of Colorado.
- 6. Copy of the most current annual report submitted to the State Department as described in Section 7.710.74.
- 7. Policy regarding the payment of pregnancy related expenses and that only pregnancy related expenses may be paid to a birth parent. All payments made on behalf of a birth parent must be processed through the child placement agency.
- 8. Information about whether the birth parent(s) have the right to choose the adoptive family.
- 9. Requirement for birth parent counseling; topics covered.
- 10. Information about the different types of adoption services the agency provides.
- 11. Types of communication between birth parents and adoptive parents, including a statement that the adoption agency cannot enforce any voluntary agreements written or unwritten entered into between birth parent(s) and adoptive parent(s).
- 12. How parental rights are terminated in Colorado; legal process; mother and father, including the option of expedited relinquishment and administrative notice to the presumed birth father.
- 13. The process for identifying and serving legal, alleged, and/or presumed birth father(s).
- 14. Any policy or requirement that would preclude the agency placing a child for adoption.
- 15. Provide information on the Birth Parent Contact Preference Form and Medical History Statement that can be filed with the State Registrar.
- 16. Process of record storage and maintenance in the event of closure of the agency.

17. The process of sharing available information regarding the child and the birth family as provided in accordance with Colorado statute, Section 19-5-301, C.R.S., pertaining to identifying and non-identifying information.
 18. The grievance and/or appeal process of the agency.
- C. The agency provides to all applicants, prior to application, a written schedule of expected itemized total fees and itemized total expenses, along with a written explanation of the conditions under which fees or expenses may be charged, waived, reduced, or refunded, and when and how the fees and expenses must be paid.
1. For intercountry adoptions, the agency must provide, in writing, a detailed listing of the following expenses:
 - a. Home study fee; and,
 - b. Adoption expenses in the United States; and,
 - c. The foreign country program expenses; and,
 - d. The care for the child in the foreign country expenses; and,
 - e. Translation and document expenses; and,
 - f. Contributions to humanitarian or other welfare services programs in the foreign country; and,
 - g. Post-placement and post-adoption report expenses; and,
 - h. A written explanation of when the fees may be charged, waived, reduced, or refunded; and,
 - i. To whom the fees are paid, and when must also be provided to the applicant.
 2. For intercountry adoptions, the agency may not customarily charge any additional fees and expenses beyond those disclosed in the adoption services contract and must have a written policy to this effect. In the event that unforeseen additional fees and expenses are incurred in the foreign country, the agency may charge such additional fees and expenses only when it notifies the prospective parents of such. The agency shall obtain specific written consent from the prospective parents prior to expending funds in excess of one thousand dollars (\$1,000) for which the agency will hold the adoptive parents responsible. The agency must provide written receipts to the prospective parents for fees and expenses paid directly by the agency, on behalf of the prospective parents, in the foreign country.

- D. The agency returns any funds to which the prospective adoptive parents may be entitled within sixty (60) days of the completion of the delivery of services.
- E. For intercountry adoptions, the agency must set up an escrow or other pass-through account for all monies that prospective adoptive applicants are required to pay directly to a foreign country to complete an intercountry adoption. These fees may not be deposited into any agency account or used for any purpose other than the foreign country fees. If the applicants request a refund of the monies to be paid to the foreign country, the agency must refund all monies paid by the applicants into the escrow or pass-through account, but not yet transferred to the foreign country, within thirty days of the applicant(s)' request for refund.
- F. For intercountry adoptions, the agency may only collect fees from the applicants for the actual services being rendered. Such fees may not be collected more than thirty (30) days prior to the event date of the service being initiated. All fees, scheduled payments, and anticipated completion dates of service must be in writing and provided to applicants at each time fees are paid.
- G. If the agency requires the prospective adoptive parents to sign a waiver of liability, the waiver shall be limited and specific, based on the risks that have been discussed with and explained in writing to the client in the adoption services contract or other written agreement for adoption services.
- H. The agency shall provide to all applicants for the foreign visiting children's program the following information:
 - 1. The total cost of the program, including all fees for the background checks, home assessment, and care of the children while in care of the applicants.
 - 2. The child(ren) in the applicant's care may not be available for adoption, and if the child(ren) are available for adoption, the applicants may not be the family approved to adopt the children.
 - 3. The foreign visiting children's program is not an adoption program and if the applicants desire to adopt a child, either from the visiting children's program, or other program, they must apply and complete the adoption process, which includes a full home study and assessment.
 - 4. The child(ren) in the applicant's care must be cared for by the applicants and any emergency care must be approved by the placement supervisor.
 - 5. The applicants must be certified as foster parents for the duration of the time the children are in care.

7.710.54 Adoption Procedure

- A. Adoptions Finalized in Colorado

For all adoptions to be finalized in Colorado in which a child is placed in Colorado with an adoptive applicant(s) residing in Colorado, the agency must:

1. Complete an assessment of each adoptive family in which a child is to be placed for adoption as detailed at Section 7.710.56.
2. Provide training to each adoptive family as detailed at Section 7.710.55.
3. Provide birth or legal parent counseling to any parent prior to a decision regarding whether or not to relinquish a child as detailed at Section 7.710.57.
4. Complete a study of the child for adoption as detailed at Section 7.710.58.
5. Provide all necessary services for the adoptive family while the family is present in the foreign country as detailed at Section 7.710.64.
6. Complete all requirements that must occur at the time of placement of a child for adoption as detailed at Section 7.710.59.
7. Provide post placement services to the adoptive family and child until a decree of adoption is granted as detailed at Section 7.710.6.
8. Receive a completed and signed State-approved foster care application from the adoptive family.
9. Submit a completed State-approved foster care certification form to the Division of Child Care prior to the child being placed in such adoptive home. A completed State-approved closure form must be submitted to the Division of Child Care upon finalization of the adoption.

B. Adoptions Finalized in a Foreign Country

For all adoptions of foreign nationals that will be finalized in the child's country of origin and jurisdiction, the agency must:

1. Complete an assessment of each adoptive family in which a child is to be placed for adoption as detailed at Section 7.710.56.
2. Provide training to each adoptive family as detailed at Section 7.710.55.
3. Complete a study of the child for adoption as detailed at Section 7.710.58.
4. Complete all requirements for United States Citizen and Immigration Services (USCIS) as detailed at Section 7.710.94.
5. Provide all necessary services related to the adoption while the family is present in the foreign country as detailed at Section 7.710.64. Such services may be provided by another licensed child placement agency or

by a contract staff member or approved entity in the foreign country. If the services are provided by another licensed child placement agency or approved entity in the foreign country, a signed agreement detailing the services and costs of such services must be signed by both the Colorado agency and the other agency or approved entity prior to the adoptive family traveling to the foreign country. Such services must be disclosed in writing to the adoptive family as listed at 7.710.53, A, 17.

6. Provide post adoption services to the adoptive family and child, if required by the foreign country, as detailed at Section 7.710.62.
7. Notify the selected agency when the family returns to Colorado with the child adopted in the foreign country.
8. Submit validation documentation to the Colorado court of jurisdiction after completion of intercountry adoption within thirty (30) days of the child arriving in Colorado.

C. United States Children Emigrating to a Foreign Country

For all adoptions in outgoing cases, the agency, if acting as the placing agency, must:

1. Be Hague accredited if the child is emigrating to a convention country.
2. Complete a child background study in compliance with Section 7.710.58 and with Federal Regulation 22 CFR Part 96 Section 96.53.
3. Ensure that all consents have been obtained in compliance with Federal Regulations 22 CFR Part 96, Section 96.53, and Colorado statutes; and meets all rules pertaining to birth parents and birth parent counseling as found in Section 7.710.57.
4. Except in the case of adoption by relatives or in cases in which the birth parents have identified specific prospective parents, make reasonable efforts to find a timely adoptive placement for the child in the U.S.
5. Take all appropriate measures to determine whether the placement will be in the best interest of the child.
6. Ensure that the home study on the prospective adoptive parent(s) is prepared in accordance with the laws of the receiving country, which includes:
 - a. Information on the prospective adoptive parent(s)' identity, eligibility, and suitability to adopt; and,
 - b. Background; and,

- c. History; and,
- d. Social environment; and,
- e. Reasons for adoption; and,
- f. Ability to undertake an intercountry adoption; and,
- g. The characteristics of the children for whom they would be qualified to care; and,
- h. The results of a criminal background check.

7.710.55 Training

- A. The adoptive agency shall verify and document that all adoptive applicants have completed training provided or approved by the agency as a part of the adoption process.
 - 1. All adoptive applicants shall complete sixteen (16) core hours of training provided in face-to-face format.
 - 2. Reasonable effort must be made to complete all required hours of training prior to the placement of a child(ren)/youth.
 - 3. All training must be completed by each adoptive applicant prior to the finalization of the adoption.
 - 4. Applicants adopting children over twelve (12) months of age or completing an intercountry adoption must complete training topics as listed at Section 7.710.55.C.
 - 5. Training must be separate from and in addition to the family assessment.
 - 6. If an adoptive applicant(s) is completing a subsequent adoption through the same agency or has complete, documented training from their previous agency, the core training need not be repeated if documentation of the prior training is on record with the current agency.
 - 7. All training must be documented in writing, including dates, number of hours and topics covered.
- B. Core training must include all of the following topics:
 - 1. Attachment/bonding issues.
 - 2. Loss and grief issues, including infertility, as applicable for all parties to the adoption.

3. Adoption as a lifelong issue as it pertains to all parties to the adoption.
4. Key concepts of child growth and development.
5. Boundary setting and discipline.
6. Parenting a child of a different cultural or racial background infancy through adulthood, if applicable.
7. Disclosure issues including the accuracy of family history information regarding the child and birth parent(s)' family, discussion with the child and sharing information with others.
8. Understanding adoption laws and procedures, including termination of parental rights and the expedited relinquishment process pursuant to Section 19-5-103.5, C.R.S., and the administrative notice for any other birth parent or possible birth parent pursuant to Section 19-5-103.7, C.R.S., if applicable.
9. On-going contact and/or communication of child and adoptive family with biological family and/or significant individuals, if applicable.
10. Possible current and/or future use of community resources, including help with parenting techniques.
11. Medical and health issues including, but not limited to, shaken baby syndrome, parental substance abuse, relevant environmental issues, and genetic risk factors.
12. Expectations of adoption and adoptive process.
13. Basic core and supervision appropriate to the age of the child.
14. Reasonable and Prudent Parent Standard.

C. Additional Areas of Training

1. For families that will be adopting a child who is either medically fragile, over twelve (12) months of age or any intercountry adoption, four (4) additional hours from the following topics must be completed as applicable and appropriate for the age(s) of the child(ren) being adopted for a total of twenty (20) hours. These four (4) hours of training may be approved in formats other than face-to-face training at the agency's discretion.
 - a. Parenting a child that has been abused or neglected.
 - b. Parenting the physically, mentally, developmentally, or emotionally delayed child.

- c. The impact of frequent moves and multiple caregivers on the development of a child.
 - d. Possible physical, mental, emotional, and behavioral issues pursuant to Section 19-5.5-303, C.R.S. concerning:
 - 1. Identity, loss, and trauma that a child/youth may experience before, during, or after adoption;
 - 2. A child/youth leaving familiar ties and surroundings;
 - 3. The effect that the access to resources, including health insurance, may have on the ability of an adoptive parent to meet the needs of a child/youth;
 - 4. Causes of disruption of an adoptive placement or dissolution of an adoption and resources available to help avoid disruption or dissolution.
- 2. For families that are only completing an intercountry adoption, an additional four (4) hours of training on all topic areas listed below must be completed as applicable for a total of twenty-four (24) hours. These four (4) hours of training may be provided in formats other than face-to-face at the discretion of the agency and must be completed prior to travel.
 - a. U.S. immigration requirements and the laws and procedures of the foreign country, including reporting requirements such as any post-placement or post adoption reports required by the expected country of origin.
 - b. Cultural heritage of the child, including available community resources.
 - c. Information on the long-term implications for a family that has become multicultural through intercountry adoption, including:
 - 1. The child's history and cultural, racial, religious, ethnic and linguistic background; and,
 - 2. The known health risks in the specific region or country where the child resides. Any general characteristics and needs of children awaiting adoption and the in-country conditions that affect children from the expected country of origin.
 - d. Any other medical, social and other data known about the particular child.

- e. The impact of a child leaving familiar surroundings as appropriate to the expected age of child.
 - f. The impact of frequent moves and multiple caregivers on the development of a child and data on institutionalized children and the impact of institutionalization on children, including the effect on children depending on the length of time spent in an institution and of the type of care provided in the expected country of origin.
- D. Applicants participating in the foreign children's visiting program must complete training in the following areas prior to a child/youth being cared for in the home:
 - 1. CPR and first aid training for all adult applicants in the home.
 - 2. Mandatory reporter training.
 - 3. Any known health risks in the specific region or country where the child/youth resides.
 - 4. The child/youth's history, cultural, racial, religious, ethnic and linguistic background.

7.710.56 Assessment of the Adoptive Family and Report

- A. Any individual, couple or family who desires to be considered to have a child(ren) placed for the purpose of adoption or second parent adoption must participate in an assessment regarding his/her suitability, appropriateness, and readiness for an adoptive placement.
- B. Any individual that is planning a second parent adoption must notify the agency and include the individual involved with the second parent adoption to participate in the home study, if the home study will be used in the next six months for a second parent adoption.
- C. An agency must complete the assessment without regard to race, color, religion, sex, sexual orientation, gender identity, or national origin of the person or the child/youth involved in the adoption, except as provided in 7.710.12, C, and 19-5-206, C.R.S.
- D. The agency must include in the assessment all members of the household.
- E. The agency shall not conduct an assessment for any paid or volunteer member of its staff or any member of the board of directors for the agency.
- F. As part of the assessment, the agency must conduct a minimum of three face-to-face joint interviews with a couple, one face-to-face individual interview with each adult member in the household, and a face-to-face age/developmentally

appropriate interview with all children residing in the home. For single applicants a minimum of three face-to-face interviews will be required.

- G. The agency must conduct at least one interview in the applicant's home.
- H. A family assessment shall be completed using the Structured Analysis Family Evaluation (SAFE) home study format. The SAFE home study must be completed by using all required tools and processes required by the SAFE format. Persons completing the home studies must be qualified, at a minimum, as a placement worker, with a Bachelor's degree in the social or behavioral sciences, and must complete the Department-required training prior to performing the home studies. Persons approving the home studies must be qualified as a placement supervisor and must comply with the Department-required training prior to reviewing and approving the home studies.
 - 1. Joint interviews of the adoptive applicants must be held on separate days.
 - 2. The second interview, and any subsequent interviews of the adults, shall be conducted during weekly meetings at least three (3) calendar days apart
- I. The assessment for a domestic adoption shall be updated annually and the assessment for an international adoption shall be updated as required by United States Citizenship and Immigration Services regulations. If the assessment is not required to be updated by USCIS, the intercountry assessment shall be updated every 24 months from the time of initial approval. An update shall include at least one home visit and a review of the current medical status. During each subsequent addendum applicants shall be questioned regarding any child abuse investigations during the previous year. The update shall be completed using the prescribed format.
- J. If there are changes in adults or children to the household, changes in the residence, marital or civil union status, criminal history, finances, or the age, sex, and special characteristics of child(ren) which will be considered for placement with the adoptive family, a re-evaluation of the family must be completed and the family assessment revised in the form of an addendum. Such addendums shall be signed by the applicants or a notice provided to the applicants to inform them of their option to review such addendum.
- K. As a part of the family assessment the agency must obtain:
 - 1. A completed, dated and signed formal application for adoption, including a signed fee agreement and a disclosure agreement as outlined at Section 7.710.53, A.
 - 2. A current photograph of all members of the household.
 - 3. A copy of a current marriage license or civil union license, if applicable.

4. A copy of a divorce decree(s) or civil union dissolution, if applicable.
5. The results of a current fingerprint-based criminal history record check through both the Colorado Bureau of Investigation (CBI) and Federal Bureau of Investigation (FBI) on all adult members of the household, processed through the current Colorado child placement agency license number. The results of the National Sex Offender Registry check on all adults in the household. The results of the child abuse or neglect records checks from the State Department and appropriate entity in each state in which the adult(s) in the household resided in the five years preceding the date of the application to adopt.
 - A. If adoptive applicants transfer from one Colorado agency to another, the new agency must process fingerprints through the applicant's license number prior to any certification or placement of children into a home.
 - B. The agency shall report to the court any case in which a fingerprint-based criminal history record check reveals that the prospective adoptive parent was convicted at any time of a felony or misdemeanor in one of the following areas:
 - 1) Child abuse or neglect; or,
 - 2) Spousal abuse; or,
 - 3) Any crime against a child, including child pornography; or,
 - 4) Any crime, the underlying factual basis of which has been found by the court on the record to include an act of domestic violence, as defined in Section 18-6-800.3, C.R.S.; or,
 - 5) Violation of a protective order, as described in Section 18-6-803.5, C.R.S.;
 - 6) Any crime involving violence, rape, sexual assault, or homicide;
 - 7) Any felony physical assault or battery conviction or felony drug-related conviction within, at a minimum, the past five years.
 - C. No person convicted of a felony offense specified in items Section 7.710.56, K, 5, d, a shall be allowed to adopt a child, except a person may be allowed to adopt a child if:

- 1) The applicant has had no further arrests or convictions subsequent to the original conviction; and,
 - 2) The applicant has not been convicted of a pattern of misdemeanors, as defined by rule of the State Board of Human Services at Section 7.701.33, D, 7, f, 1-3; and,
 - 3) The court enters a finding consistent with Section 19-5-210(2)(d), C.R.S., that the adoption is in the best interest of the child.
6. A minimum of three (3) personal references from individuals, unrelated to the applicant(s), that have knowledge of the applicant(s) character and suitability to parent a child. The references must be obtained in writing using the prescribed SAFE format instrument for the type of adoption the applicant has requested.
7. Verification of current health insurance coverage or a statement of intent that health insurance coverage will be obtained for the child upon placement.
8. A dated physician's statement current within one year from the time of application for adoption for each member of the household, certifying that the applicant(s) and all members of the household are free from communicable disease or a physical condition that would impair his/her/their ability to care for children. Subsequent medical statements shall be required upon the written direction of the physician or within one year of the last dated and signed statement if not specified by a physician.

No physical examination shall be required of any person who in good faith relies upon spiritual means or prayer in the free exercise of religion to prevent or cure disease unless there is a reason to believe such person's physical condition is such that he/she would be unable to care for a child.
- L. A family assessment using the Structured Analysis Family Evaluation (SAFE) instrument(s) shall be completed using the current and appropriate SAFE format for the type of adoption for which the applicant(s) has requested.
- M. If an applicant desires to be considered for another adoption with the same agency:
 1. If the period of time since the last family assessment is within three (3) years of the original approval date of the original family assessment, a subsequent update using the SAFE instrument shall be completed. The assessment update shall include at least one joint interview, coupled with one documented individual interview with each adult member of the household, and an age/developmentally appropriate interview with all children. At least one interview with the family must be conducted in the

- home. The individual interviews with the adoptive applicants must be completed on the same date.
2. If the period of time since the approval of the original family assessment is longer than three (3) years, a full and complete family assessment using the SAFE instrument shall be completed.
- N. The agency must update the previously completed family assessment in order to address significant changes in the time period subsequent to the original family assessment, including, but not limited to:
1. Criminal history charges or convictions; and,
 2. Child abuse investigations; and,
 3. Marital or civil union status; and,
 4. Changes in family structure; and,
 5. Employment/financial resources; and,
 6. Health changes/status; and,
 7. Place of residence; and,
 8. Other significant changes within the family.
- O. If an applicant desires to be considered for an adoption through a different licensed child placement agency, and the full SAFE assessment is received directly from the originating agency and along with an update with an approval date of less than one year old, the new agency may either complete:
1. An update of the original family assessment using the SAFE instrument that addresses all categories as listed in Section 7.710. 56,K; or,
 2. A full SAFE assessment. If the assessment or update approval date is more than one year old, a complete assessment using the SAFE instrument must be completed.
- P. Following the completion of the family assessment, a narrative report must be completed according to the SAFE instrument and training guidelines.
- Q. For families applying for an intercountry adoption, the agency must complete the adoptive family assessment using the international SAFE template and must:
1. Maintain on file at the agency a written copy of the home study requirements for each foreign country for which a child placement agency approves a family.

2. Ensure that the family assessment submitted to the foreign country:
 - a. Is a reasonable, true, and responsible assessment of the family concerning the requirements of the foreign country; and,
 - b. Is provided to the foreign country, including any and all information obtained during the assessment process; and,
 - c. Does not knowingly provide false or inaccurate information or knowingly withhold or exclude information from the foreign country, which is grounds for suspension of the child placement agency's license.
- R. At the conclusion of the family assessment process:
 1. The family shall be offered the opportunity to review the assessment with an agency staff member and submit corrections to the agency of any inaccuracies to the data contained in the assessment.
 2. The applicant(S) must either:
 - a. Sign a statement that they have been given the opportunity to read and review the final draft of the family assessment, excluding the confidential references, or
 - b. Sign the SAFE home study assessment section indicating they have read and reviewed the final draft of the home study.
- S. If at any time after the receipt of a formal application the agency becomes aware that it will not be able to continue to provide services or to recommend the applicant(s) for approval for an adoptive placement, or the adoptive family withdraws from the service provided by the agency, the agency must:
 1. Notify the applicant(s) in writing within five business days with the reason(s) for the discontinuation of services or confirmation of the family's request for withdrawal from services.
 - a. If the agency is discontinuing the services, include with the notification letter a copy of the agency's process for filing a grievance with the adoption process or assessment as required at Section 7.710.71; and,
 - b. That the agency shall submit a copy of the denial or withdrawal confirmation letter to the Division of Child Welfare at the same time it is submitted to the applicants.

2. In the case of an intercountry adoption with an approved I600A or I800A, the agency shall notify USCIS, the selected agency and the State Department.
- T. The approved SAFE family assessment, signed by the placement supervisor and notarized, and supporting documentation is a part of the official adoption record of the agency and shall be released upon the written authorization by the applicant(s):
1. To another licensed adoption agency, a Colorado county department of social/human services, or an entity licensed in other states to provide adoptive placement services or,
 2. When required by a foreign country to complete an intercountry adoption, to the adoptive applicant(s), to accompany them when traveling to the foreign country, if each page of the family assessment contains the agency raised seal and the pages are numbered according to the full number of pages in the document; and,
 3. Shall be transmitted within thirty (30) calendar days of the written request or of when the original agency receives full payment of all fees for services rendered by the agency.
- U. If agency policy allows for prospective adoptive parent(s) to apply for concurrent adoptions within their agency, or through both their agency and another Colorado licensed CPA or county department of social/human services, the following applies:
1. At least one of the concurrent adoptions does not require the adoptive applicant(s) to be certified for foster care; and,
 2. If two CPAs are providing services to the adoptive applicant(s), the applicant(s) shall sign a release of information for both CPAs prior to the agencies exchanging any information; and,
 3. The family assessment completed for the initial adoption must be updated to address the reason(s) for the concurrent adoption; and,
 4. All agencies involved are aware of the concurrent adoptions; and,
 5. The assessment or update(s) shall be completed in the correct format for the type of concurrent adoption being pursued; and,
 6. The assessment or update(s) completed for the second adoption shall:
 - a. Address the reason for the concurrent adoption; and,

- b. The type of child the adoptive applicant(s) are approved for in the second adoption; and,
 - c. If more than one agency is involved, that both agencies are aware of the concurrent adoptions; and,
 - d. That the original assessment/update(s) may be shared with the second agency, with written approval from the adoptive applicants.
 - 7. Updates to both assessments must be completed within one year of the original assessment if no adoptive placement has occurred.
 - 8. Both assessments must be submitted separately for ICPC or USCIS approval, if applicable.
 - 9. If one agency places a child(ren) for adoption, either foreign or domestic, the second agency shall:
 - a. Wait a minimum of six (6) months before placing another child(ren) into the home; and,
 - b. Complete a home study update to assess the appropriateness of placing an additional child(ren) into the home.
 - 10. The agency placing the child(ren) into the home shall also complete a home study update to assess the ability of the family to adopt an additional child(ren).
 - 11. The adoptive applicant(s) must sign a statement for each potential adoption agreeing to notify the other agency of when a placement of a child(ren) occurs.
- V. The agency shall complete a home and safety check on all applicants for the foreign children's visiting program, which shall include, at a minimum:
- 1. An interview with all adults and children who will reside in the Home during the time the child is in care.
 - 2. Background checks as listed at 7.710.56, K, 5.
 - 3. A review of the house and grounds of the property to ensure there are no safety hazards.
 - 4. A determination that all weapons, including guns, are stored separately from ammunition. Both the weapons and ammunition must be locked and inaccessible at all times the children are in care.
 - 5. All medications must be stored in areas that are inaccessible to children.

7.710.57 Birth Parent Counseling and Report [Rev. eff. 5/1/10]

- A. Any person who plans to petition the court for relinquishment of his/her child must obtain counseling about the relinquishment from a licensed Colorado child placement agency or a Colorado county department of social/human services. A court may refer a petitioner to a licensed child placement agency for counseling.
- B. An agency must provide counseling regarding possible relinquishment to birth parent(s) residing in Colorado in face to face interviews. In providing counseling services to an expectant parent(s), the agency must provide counseling prior to the birth of a child(ren) and following the birth of the child(ren).
- C. An agency must provide counseling to birth or legal parents without regard to age, race, color, national origin, religion, sex, or disability of either birth parent.
- D. An agency shall ensure that only pregnancy related expenses are paid on behalf of birth parent(s) by the CPA, the prospective adoptive parents, or agent acting on behalf of the prospective adoptive parents. All expenses paid on behalf of birth parents shall be processed through the agency. An agency cannot require birth parent(s) to repay pregnancy related or cradle care expenses except in the case of criminal fraud.
- E. The agency must conduct a comprehensive and diligent search for the legal father and/or the presumed birth father, or the birth mother if applicable. The agency must take seriously and promptly investigate any claims of paternity prior to the final orders of relinquishment or termination. Efforts to locate the birth father or birth mother must be documented.
- F. The agency must include in the counseling the legal, alleged and/or presumed birth father(s) if he can be located and is willing and interested in participating in the counseling.
- G. At the beginning of the counseling, the agency must ascertain if the birth parent(s) is receiving counseling from any other agency in Colorado.
- H. At the beginning of the counseling, the agency must provide, in writing, to the birth parent(s) a list of birth parents rights as found at Section 7.710.53, B.
- I. The content of the counseling must include at least the following:
 - 1. Discussion of the birth parent(s) motivation to consider relinquishment of the child(ren).
 - 2. Discussion as to whether the birth parent(s) have been pressured or coerced to relinquish the child.

3. Discussion as to whether the birth parent(s) have been promised or received money, goods, or services or anything of value to motivate the relinquishment of the child(ren).
4. Discussion of alternatives to relinquishment and the resources available in the community if the birth parent(s) choose to parent the child(ren).
5. Discussion of the permanency of the decision to relinquish a child(ren).
6. Discussion of life long loss and grief issues that include, but are not limited to, helping the birth parent(s) identify and understand the present emotional impact of the relinquishment decision and gain an understanding of possible future grief-related emotions and behaviors.
7. Discussion of the life changing effect of pregnancy and birth of a child.
8. Discussion of the birth parent(s) and birth families social developmental history and medical history, including the gathering of information regarding previous losses and life stability.
9. An assessment of the birth parent(s) ability to understand the consequences of the relinquishment decision and her/his ability to intellectually and emotionally understand the options.
10. Discussion regarding the identification of the presumed birth father(s), the serious ramifications of failing to provide known information and the possible impact to the relinquishment and/or finalization of the adoption.
11. Discussion of whether the birth mother or the presumed birth father is a member of or are eligible to be a member of a Native American Tribe and any applicable ramifications of such information.
12. Discussion that the birth mother may only be reimbursed by the agency for verified and documented pregnancy related expenses.
13. Discussion of the legal relinquishment options available, including being present at the termination or relinquishment hearing or the expedited relinquishment procedure of filing an affidavit with the court without the possible necessity of a court hearing.
14. Discussion of the time frame for withdrawal of the expedited relinquishment petition and affidavit, including that the petition for relinquishment may not be filed with the court until at least four calendar days after the birth of the child.
15. Discussion of the administrative procedure that the birth parent(s) information will be released to a presumed birth father(s) or published in

an official publication of the last known address of the presumed birth father(s).

16. Discussion that the filing of the administrative procedure notice to the presumed birth father(s) does not obligate the birth mother to file a petition for relinquishment.
 17. Discussion of the right of the birth parent to complete the contact preference form and the right to change the form at any time.
 18. Discussion of the right of the birth parent to complete a medical history statement form to be filed with the State Registrar and the right to update this information every three years.
- J. The agency must submit an affidavit that includes a thorough written report of the counseling, directly to the court. The report must contain at least the following information:
1. Identifying information of the birth parent(s) including the legal and/or the alleged or presumed birth father(s), which shall include at least full name, address, and birth date of parent(s).
 2. The name, address and qualifications, including the level of education and the number of years of adoption experience, of the individual that provided the counseling.
 3. Confirmation that venue is appropriate.
 4. The dates and hours on each date that significant face to face counseling occurred, excluding labor and delivery. The agency must document the number of counseling hours provided pre and post delivery.
 5. The total number of hours of counseling. If under extenuating circumstances some counseling was done in a manner other than face to face, the agency must specify how the other counseling was provided and the number of hours of the other counseling.
 6. An individualized narrative description of the specific content of the counseling covering required topic areas listed at Section 7.710.57, I, that were discussed during the counseling.
 7. If the birth parent(s) choose the expedited relinquishment procedure, the counseling report and affidavit must include a statement that all legal options for relinquishment were presented including both the benefits and the detriments of attending the court relinquishment hearing or filing the expedited relinquishment petition with the court, including the time frame for withdrawal of the petition and affidavit if the birth parent(s) changes his/her mind.

8. Summary of social and medical history of the birth parent(s) and the reason for relinquishment.
9. Description of and documentation to support the diligent efforts of the agency to locate the alleged, legal, and/or presumed birth father or birth mother and the cooperation or lack of cooperation of that parent, including the results of the anticipated notice procedure to the presumed birth father(s) pursuant to 19-5-103.7(8), C.R.S.
10. The results of contact and counseling with the alleged, legal, and/or presumed father(s).
11. Whether the Indian Child Welfare Act as stated in Section 7.309 applies to the results of the agency's investigation, including the documentation of efforts made to locate or contact the tribe and the tribe's response.
12. An itemized account of all expenses, including financial and material aid, the agency paid to the birth parent(s); the agency must also report any financial or material aid if they became aware it was paid to the birth parent(s) by another individual. Documentation must include justification as to how each expense is pregnancy related.
13. The specific recommendation as to whether the relinquishment and/or termination should be granted by the court.

7.710.58 The Child Available for Adoption

- A. Any CPA involved in assisting relinquishing parents and/or prospective adoptive parents must obtain all available information regarding each child available for adoption. Such information must be obtained from the relinquishing parent, and/or the CPA or county department involved in an original adoption, where applicable. Documentation must include but need not be limited to:
 1. A report of a physical examination performed by a medical professional licensed pursuant to Sections 12-240-101, C.R.S. through 12-240-145, C.R.S. conducted within the six (6) months prior to adoptive placement.
 2. A history of the child/youth including as much of the following information, including available records, as can be obtained:
 - a. Physical appearance;
 - b. Physical, mental, emotional, and behavioral health, and cognitive history;
 - c. The child's medical history, including immunization, medication and birth records when available;

- d. Family background; including, but not limited to, racial or ethnic, cultural, religious, and linguistic backgrounds;
 - e. Gender identity and background;
 - f. Educational background;
 - g. Interest/hobbies/talents;
 - h. Developmental history;
 - i. Region of origin;
 - j. Personal qualities;
 - k. Any previous written assessments;
 - l. United States immigration status;
 - m. Information available from the federal government on the process for the child/youth to acquire United States citizenship where applicable;
 - n. Whether the child has been separated from a sibling or other significant relation;
 - o. Previous adoption(s) or out-of-home placement(s) and the reason(s) the adoption(s) or placement(s) ended;
 - p. Trauma and attachment history; and
 - q. History of abuse or neglect, including adverse childhood/youth experiences such as malnutrition or drug/alcohol exposure.
- 3. Any other matter a child placement agency considers material to the adoption.
 - 4. A statement documenting the chronological history of a child's family background in as much detail as available, including verification of the child/youth's birth date and place and reasons for relinquishment or termination of the parent-child legal relationships, as listed on the petition of relinquishment or international equivalent and signed by the birth or legal parent(s), including a personal description written by the birth or legal parent(s), if available.
 - 5. All non-identifying information that has been obtained by the agency or provided to the agency regarding the child/youth or child/youth's birth family.

6. All available information regarding the child/youth's birth family including birth parents, extended family, and siblings as applicable, including as much of the following information as can be obtained:
 - a. Physical appearance;
 - b. Health/medical history;
 - c. Race or ethnic origin;
 - d. Religious background;
 - e. Cultural background;
 - f. Linguistic background;
 - g. Educational background;
 - h. Occupation;
 - i. Hobbies/interests/talents;
 - j. Mental health;
 - k. Substance use/misuse; and
 - l. Personal qualities.
 7. Medical, therapeutic, and educational resources, including language acquisition training, available to the adoptive parent and child after placement for adoption or adoption to assist in responding effectively to physical, mental, emotional, or behavioral health issues and available records relevant to the information in Subsection 7.710.58.A.2.a-q of this Section.
- B. Any CPA involved in assisting relinquishing parent(s) and/or prospective adoptive parent(s) must provide all information obtained specific to the child/youth to the prospective adoptive parent(s) to provide historical context for the child/youth's experience and to support the adoptive parents with resources to effectively respond to the needs of the child/youth. Such information must include, but may not be limited to:
1. The potential effect on the child of:
 - a. A previous adoption or out-of-home placement;
 - b. Multiple previous adoptions or out-of-home placements;

- c. Trauma, insecure attachment, fetal alcohol exposure, or malnutrition;
 - d. Neglect, abuse, drug exposure, or similar adversity;
 - e. Separation from a sibling or significant caregiver;
 - f. A difference in ethnicity, race, or cultural identity between the child and the prospective adoptive parent or other child of the parent;
- 2. Information available from the federal government on the process for the child to acquire United States citizenship; and
- 3. Any other matter a child placement agency considers material to the adoption.
- 4. The guidance and instruction must be provided:
 - a. For adoption of a child/youth residing in the United States within seventy-two (72) hours of receipt of the information before the adoption is finalized; or
 - b. For an intercountry adoption, in accordance with the federal law.
 - c. The CPA must further provide the adoptive parents with resources to effectively respond to the needs of the child/youth after placement for adoption or upon the finalization of an adoption. Such information must include, but may not be limited to:
 - 1. Medical, therapeutic, and educational resources;
 - 2. Language acquisition training to assist in responding effectively to physical, mental, emotional, or behavioral health issues; and
 - 3. Available records relevant to the child/youth.

7.710.59 Placement [Rev. eff. 5/1/10]

- A. The assessment of the adoptive applicant(s), as required at Section 7.710.56, K, must be completed prior to placement.
- B. As permitted under Colorado law, prior to placement as much information as possible shall be obtained and disclosed in writing to the adoptive parent regarding the child(ren) as required at Section 7.710.58, B.
- C. An effort shall be made to place siblings with the same adoptive applicant(s). If the county department locates an appropriate, capable, willing, and available joint placement for all of the children in the sibling group, there should be a

rebuttable presumption that placement of the entire sibling group in the joint placement is in the best interests of the child(ren). Such presumption may be rebutted by the county by a preponderance of evidence that placement of the entire sibling group in the joint placement is not in the best interest of a child or the children. If this cannot, or should not, be done in the best interests of the children involved, the record shall contain the reasons and supporting evidence for such separate placements.

- D. The agency shall not place a child in an adoptive home that would be detrimental to the child's well-being.
- E. The agency shall immediately inform the adoptive applicant(s) in writing of the relinquishing parent's decision to discontinue the adoption plan and the plan for returning the child to the birth or legal parent.
- F. A written contract between the placement agency and the adoptive applicant(s) shall be executed at the time of placement. The contract must include an agreement that the adoptive applicant(s) will:
 - 1. File an adoption petition in accordance with Colorado law.
 - 2. Participate in supervision by the placement agency of the child(ren) in the adoptive home until such time as a decree of adoption becomes final.
 - 3. Agree to allow the placement agency to complete and submit a report to the court regarding the findings of the post-placement visits.
 - 4. Prior to finalization, the adoptive parent(s) must obtain permission of the placement agency or birth parent, as appropriate, prior to removing the child from the state.
 - 5. Agree that the child may be removed from the adoptive placement at the discretion of either the agency or the adoptive parent(s) with good cause before the finalization of the adoption.

7.710.6 RESPONSIBILITY OF THE AGENCY AFTER PLACEMENT

- A. The placement agency having legal custody is responsible for the welfare of the child until a decree of adoption is granted and shall:
 - 1. Assume financial and planning responsibility for the child in the event that a child's adoptive placement is disrupted.
 - 2. Assume financial and planning responsibility for the child where a court refuses to grant a decree of adoption.
 - 3. Maintain a copy of the final decree of adoption from the court in the agency file on each child.

- B. Placements shall be supervised from the time a child(ren) is placed until court finalization to ensure:
 - 1. The physical and emotional well-being of the child(ren);
 - 2. Successful attachment between the child and parents;
 - 3. Positive adjustment of all family members; and,
 - 4. Adoption related issues are resolved as necessary.
- C. For adoptions to be finalized in Colorado the agency must provide post placement services until court finalization, and must provide at least a six (6) month period of post placement services between the time of placement and the finalization of the adoption and shall maintain at least monthly contact with the adoptive family and child(ren) until court finalization of the adoption. The supervision shall include:
 - 1. For children of less than one (1) year of age at time of placement, no less than three (3) face-to-face supervisory visits, including at least two (2) visits to the adoptive home, one (1) visit to the adoptive home must be within the first two (2) weeks after placement. Visits in the adoptive home should include all adoptive family members.
 - 2. For children of one (1) year of age or older at time of placement, at least one (1) visit per month with no less than half (1/2) of the visits as face-to-face visits in the adoptive home. The first (1st) visit must be within two (2) weeks after placement of the child(ren). Visits in the adoptive home should include all adoptive family members.
- D. During the period of supervision the placement agency shall maintain records that include:
 - 1. Legible, well organized case/contact notes of all contacts. These must be placed in the adoptive applicant's case file in a timely manner and must be available for review by the State Department.
 - 2. Two (2) formal post placement reports shall be completed for the court, one (1) at three (3) months after placement and the second (2nd) at the time of finalization, or more frequently if required by the court, and must include information regarding:
 - a. The physical health and emotional well-being of the child.
 - b. Adjustments of the family and child to the adoption.
 - c. Financial changes.

- d. Changes in family composition.
 - e. Other adoption issues that have arisen.
 - f. Recommendation on the continued placement of the child.
 - g. Recommendation on the finalization of the adoption.
- E. The agency may obtain a progress report, if appropriate or as necessary, from a school official or child care provider for each adoptive child that is attending child care or school.
- F. The placement supervisor of the Colorado CPA shall be responsible for removing the child from the home, at the request of the adoptive family, or in accordance with state law, if the placement is not in the best interests of the child.
- G. If any information becomes known or can be obtained before an adoption is finalized that had not already been reported, the CPA must provide any and all information to the prospective adoptive parent(s).
- H. On request of a child who was placed for adoption or the child's adoptive parent, the CPA or the county department, as referenced in Section 7.306.55, shall provide information about how to obtain adoption assistance and resources:
 - 1. To assist the child or parent to respond effectively to adjustment, behavioral health, and other challenges; and
 - 2. To help preserve the placement or adoption.

7.710.61 Finalization [Rev. eff. 5/1/10]

- A. The agency that has custody of the child(ren) must provide written consent to the court for the adoption if it is in the best interest of the child.
- B. A certified copy of the final decree must be placed in the adoptive applicant's file at the placement agency.
- C. Send written notification of the final decree of adoption to the court in which relinquishment took place. Such notification shall not disclose the name or address of the adopting parents.

7.710.62 Post Adoption

- A. Any and all previously issued certificates of birth, to include certificates of foreign birth, must be provided to the child's adoptive parent(s) upon finalization of adoption, where applicable.

- B. If the CPA does not provide post adoption services, the agency must provide resource and referral information regarding post adoption services available to the birth parent(s), adoptive family, and child, if appropriate.
- C. Post adoption services may include, but not be limited to:
 - 1. Consultation.
 - 2. On-going support and education.
 - 3. Resources for counseling or medical needs.
 - 4. Search or reunification resources.
 - 5. Networking with other adoptive families.
 - 6. Adoption education and support groups.
 - 7. Providing to the adoptive family additional social and/or medical background information obtained after placement that could significantly impact the child.
- D. For intercountry adoptions, the agency must provide post adoption services including, but not limited to, completion of written reports on the adoptive family and child if required by the foreign country.
- E. If any information becomes known or can be obtained after an adoption is finalized that had not already been reported, the child placement agency must make reasonable efforts to provide any and all information to the adoptive parent(s).

7.710.63 Interstate Adoptions [Rev. eff. 5/1/10]

- A. An agency may participate in an Interstate adoption under at least one (1) of the following conditions:
 - 1. The adoptive placement is arranged and carried out by a child placement agency licensed by the state of residence of the child in accordance with Section 7.710.1, A, 1.
 - 2. The adoptive services are performed pursuant to an order of the court of jurisdiction of the child.
 - 3. The adoptive services for public adoptions are performed at the request of the State Department through the ICPC of children located at the State Department.

4. The adoptive services for non-public adoptions are performed at the request of non-public agencies through the ICPC at the non-public agency selected and monitored by the State Department.
- B. An agency which participates in an interstate adoption shall perform at least the following:
1. Send all studies or reports for public adoptions through the ICPC at the State Department unless otherwise authorized.
 2. Send all studies or reports for non-public adoptions through the ICPC at the non-public agency selected and monitored by the State Department (see Interstate Non-Public Adoptions, Section 7.710.93).
 3. Comply with all rules and laws of the Interstate Compact on Placement of Children, as found at Section 24-60-1801, C.R.S. et seq., and Section 7.307 of the Program Area 7 rules (12 CCR 2509-4).
- C. Any agency which participates in an interstate adoption must have direct knowledge of and comply with all applicable laws and procedures of the other state in which they are working at all times.

7.710.64 Intercountry Adoptions [Rev. eff. 5/1/10]

- A. An agency that wants to provide intercountry adoption services must apply for, and be approved by the State Department, to provide intercountry adoptions prior to initiating such service with adoptive applicants.
- B. Agencies approved for intercountry adoptions must have a current, dated, written policy detailing:
1. What services will be provided to or coordinated for the adoptive family while in the foreign country.
 2. Whether the adoptive family will owe additional fees to be paid in country for services.
 3. Whether the services will be provided or coordinated by the agency or by another United States licensed placement agency pursuant to an interagency contract.
- C. For each foreign country, agencies approved for intercountry adoptions must either:
1. Have direct knowledge of and be able to comply with all applicable laws and procedures of the foreign national child's country of origin, and be able to provide services listed in Section 7.710.54, B; or,

2. Have an interagency contract in writing with another United States licensed placement agency that:
 - a. States that the other agency is licensed as a placement agency in accordance with the laws of the state of their principle place of business, and attaches a copy of the license.
 - b. States that the other agency has direct knowledge of and is able to comply with all applicable laws and/or procedures of the child's country of origin.
 - c. States that the other agency is authorized to conduct adoptions in the foreign country and, if applicable, attaches a copy of any license or accreditation issued by the foreign country.
 - d. Lists in detail what services will be provided or coordinated by the other agency to the adoptive families while in the foreign country.
 - e. Lists what projected fees and expenses the adoptive family will owe for services provided to them while in the foreign country.
- D. For each country in which the agency is directly providing or coordinating all adoption services, the agency shall keep on file:
 1. An English language translation of any agreements entered into with the foreign government.
 2. An English language translation of any document that evidences approval, accreditation or certification by the foreign government to conduct adoptions, if required by the laws of the foreign country.
- E. For each country in which the agency is providing or coordinating services through an interagency agreement with a foreign provider, the agency shall keep on file the original signed interagency agreement and applicable attachments. The interagency agreement must include:
 1. If the foreign provider is providing adoption services, it states that the foreign provider is authorized to provide adoption services in the foreign country, and, if applicable, attaches a copy of documents to verify authorization.
 2. List in detail what services will be provided or coordinated by the foreign provider.
 3. List what projected fees and expenses the adoptive family will owe for services provided to them by the foreign provider.

- F. Each agency that provides intercountry adoption services must have a separate pass-through or escrow account for which required foreign country fees paid by parents are maintained and available for refund if fees are not paid in the foreign country on behalf of the adoptive parents. Foreign country fees may not be co-mingled or otherwise used by the agency for any purpose other than direct payment to the foreign entities on behalf of the adoptive parents.
- G. Agencies providing adoption services for intercountry adoptions in Hague Convention countries, in either incoming or outgoing cases, must meet the federal regulations pertaining to intercountry adoptions with Hague countries.

7.710.650 Unregulated Child Custody Transfer - Applicability

- A. The transfer of custody of a child/youth by a parent, guardian, or custodian of the child/youth, or an individual with whom a child/youth has been placed for adoption, to any of the following individuals is not unregulated child custody transfer:
 - 1. A parent of the child/youth;
 - 2. A stepparent of the child/youth;
 - 3. An adult who is related to the child/youth by blood, marriage, adoption, or other relationship recognized by other law of this state;
 - 4. An adult who, at the time of the transfer, had a meaningful and safe relationship with the child/youth for a period of at least six (6) months prior to the transfer, and whom the parent, guardian, or custodian reasonably believes, at the time of the transfer, to be a fit custodian of the child/youth; or
 - 5. An Indian custodian as outlined in the Indian Child Welfare Act (ICWA) and its implementing regulations, incorporated by reference in Section 7.701.14.
- B. The transfer of custody of a child/youth by a parent, guardian, or custodian of the child/youth, or an individual with whom a child/youth has been placed for adoption, to any individual other than those listed under 7.710.650.A, is unregulated child custody transfer unless the transfer occurs through one of the following legal processes as outlined in 19-5.5-203(2), C.R.S.:
 - 1. A legal adoption or guardianship proceeding;
 - 2. Judicial award of custody;
 - 3. Other judicial or tribal action; or
 - 4. Colorado's safe haven law as described in Section 19-3-304.5, C.R.S.

7.710.651 Prohibited Unregulated Child Custody Transfers

Unregulated child custody transfers are prohibited, as follows:

- A. A parent, guardian, or custodian of a child/youth or an individual with whom a child/youth has been placed for adoption may not transfer custody of the child/youth to another person with the intent, at the time of the transfer, to permanently abandon the rights and responsibilities concerning the child/youth.
- B. A person may not receive permanent custody of a child/youth or act as an intermediary if permitted under other law of this state, in a transfer of custody of a child/youth if the person knows or reasonably should know the transfer violates Section 7.710.651.A. This prohibition does not apply if the person notifies the state department of the transfer or takes appropriate action to establish custody under Section 7.710.650.B within seventy-two (72) business hours.

7.710.652 Prohibited Solicitation or Advertising

- A. A person may not solicit or advertise for the purpose of:
 - 1. Identifying a person to whom to make a transfer of custody in violation of Section 19-5.5-203 (1), C.R.S.;
 - 2. Identifying a child/youth for a transfer of custody in violation of Section 19-5.5-203(3), C.R.S.; or
 - 3. Acting as an intermediary or facilitator, if permitted under other law of this state, in a transfer of custody in violation of Section 19-5.5-203(3), C.R.S.;
- B. Engaging in prohibited solicitation or advertising is a class six (6) felony as identified.

7.710.653 Suspected Unregulated Custody Transfer

- A. When a CPA suspects an unregulated transfer of custody of a child/youth has occurred, the CPA must report all known information regarding the suspected transfer to the appropriate county department of human/social services. The county department of human/social services will determine appropriate action to protect the welfare of the child/youth as provided in other areas of rule, regardless of any additional concern of abuse or neglect. If an assessment is completed regarding concerns of an unregulated custody transfer of an adopted child/youth, and it is determined that a person has transferred or intends to transfer custody in violation of Section 19-5.5-203, C.R.S., then the county department of human/social services shall:
 - 1. Provide notice of the assessment to the Colorado Department of Human Services when it involves an unregulated custody transfer of an intercountry adopted child/youth; and

2. In any circumstance where there is an allegation of unregulated custody transfer, the county department of human/social services shall send the referral to the appropriate law enforcement agency or agencies.

7.710.7 GRIEVANCE PROCESS, CONFIDENTIALITY, RECORDS AND REPORTS

7.710.71 Grievance Process [Rev. eff. 5/1/10]

- A. Each agency must have a written grievance and appeal process for adoptive applicants and for birth parents that provides adequate due process. This information must be provided as required in the Disclosure and Fee Information found at Section 7.710.53, A-B.
- B. Reports of grievances filed and the outcome must be available to the State Department for review upon request.
- C. The agency must follow the grievance procedure without alteration, interference, or unreasonable delay.
- D. If a grievance is filed with the agency, the grievance shall be recorded in the adoptive family or birth parent file with the investigation findings and resulting action taken by the agency.

7.710.72 Confidentiality and Disposition of Adoptive Records [Rev. eff. 5/1/10]

- A. All records required by the foregoing sections shall be treated as confidential and shall be protected from unauthorized examination. They shall be immediately available to the staff of the State Department, and for non-public adoptions, they shall be available to the appropriate staff of the selected non-public agency upon request.
- B. When a child is legally free for adoption, legal documents and records must be maintained in accordance with Title 19, Article 5, C.R.S., "Relinquishment and Adoption".
- C. The records and papers in relinquishment and adoption proceedings are open for inspection only upon order of the court for good cause shown. The court and the agency are required to act in a manner so as to preserve the anonymity of birth parents, child, and adoptive parents except in the case of a designated adoption.
- D. An agency engaged in adoption placement of children shall not be licensed unless they possess adequate, secure storage facilities for records. Records shall be maintained in a locked, secured room.
- E. The agency shall develop and implement a written policy and procedure which assures that records are protected from destruction, loss and unauthorized removal or access.

- F. The agency shall develop and implement a written plan to ensure annual scanning of their adoption records, including birth parent, adoptive applicant, and children's records. The plan must address scanning any closed files already in storage. Scanned records must be maintained in a current State Department approved technological format for the purpose of transferring those records to the State Department upon closure of the agency.
- G. The agency record for any Colorado adoptive family shall contain as a minimum: the signed formal application for adoption; signed fee and disclosure agreement; medical examinations for all household members; a copy of a current marriage license; a copy of a divorce decree where applicable; the results of the State Department's automated abuse and neglect background system check for all adults residing in the household; the original home study assessment with updates as appropriate; any subsequent family assessments; and, all signed and dated adoption agreements or contracts.
 - 1. For adoptions finalized in the United States, the agency record shall also contain: signed petition for domestic adoption; post placement supervisory reports; post adoption correspondence, including any correspondence from the birth parent(s); the type of adoption and communication agreed upon at time of finalization; final decree of adoption; post-adoption reports if applicable; and, validation of foreign adoption if applicable.
 - 2. For adoptions finalized in a foreign country, the agency record shall also contain: all legal documents; all medical and social history information; all foreign documents; all post adoption reports; arrival notification to state-selected agency; consent of termination of parental rights; notification to central authority; USCIS Forms I800, I600, I600A, and I171H, as applicable; and, validation of foreign adoption if applicable.
 - 3. For all adoptions, the results of the CBI and FBI background checks for all adults, eighteen (18) years of age and older residing in the household. For intercountry adoptions finalized in the foreign country, a copy of the approval issued by USCIS may substitute for the FBI background check.
 - 4. The agency record for an adoptee shall contain at a minimum: social and medical histories; birth record; certified birth certificate; birth parent consent for placement; ICPC files; record of admission or intake summary of the child to be adopted.
 - 5. The agency record for a parent(s) relinquishing a child(ren) shall contain at a minimum: signed disclosure agreement and birth parent rights; intake forms; birth parent counseling report and/or counseling affidavit; case notes; the type of adoption completed and the communication agreed upon at time of finalization between the birth parents and adoptive parents, and a copy of such contract if available, including the preference

form completed by the birth parent at the time of relinquishment; and, all legal documents related to the relinquishment.

6. For Colorado child placement agencies that provide Intercountry adoption services for families who reside outside of Colorado, the agency must maintain a record that contains, at a minimum: the name, address, and phone number of the adoptive family; the name, address, phone number, and copy of the license of the agency or agencies used to complete the adoption; a copy of the USCIS form granting approval of the home study; a copy of the final decree of adoption; the birth name, adoptive name and the date of birth of the adopted child.
7. Files must be indexed and cross referenced in the following sections:
 - a. Adoptive parent name; and,
 - b. Adoptive child name; and,
 - c. Adoptive child birth name; and,
 - d. Birth parent name; and,
 - e. Child's date of birth; and,
 - f. Year adoption is finalized.
- H. In the event that a child placement agency ceases operations, two (2) copies of all adoption records shall be forwarded to the State Department in a current State Department approved technological format of not less than 100 years storage capability.
- I. The records of birth parents, adoptive applicants and children, that do not result in an adoptive placement shall be maintained for a period of at least three (3) years.
- J. The agency shall have an appropriate secure procedure for confidential record destruction.
- K. The Colorado agency shall be responsible for maintaining a case record for each approved adoptive family, each parent who relinquishes a child for adoption, and each adoptee. The record for each client must be kept current from the point of intake to termination of completion of services. All correspondence and written communications including, but not limited to, e-mails, memos, and letters must be maintained in the file.
- L. The entire case file, including all worker notes or other documents, is the property of the agency and the entire case must be maintained in secure storage at the main office. Any and all duplicate files maintained by contract or staff

worker(s) during the completion of an adoption must be maintained in locked secured storage at all times and all items, including worker notes or other documents, must be returned to the main office at the completion of services. Contract or staff workers may not keep duplicate copies of any documents.

7.710.74 Required Reports [Rev. eff. 5/1/10]

An annual report shall be made to the State Department regarding the adoption services provided by the agency. The report shall be submitted on the State prescribed form within sixty (60) days following the end of the calendar year and shall provide the following information:

- A. The types of adoptions, as defined at Section 7.710.52, in which the agency was involved, including the total number of:
 - 1. Agency adoptions:
 - a. Domestic (non-relative) adoptions finalized in Colorado.
 - b. Domestic adoptions finalized outside of Colorado.
 - c. Foreign national finalized outside United States and placed in Colorado.
 - d. Foreign nationals finalized in Colorado.
 - e. Foreign nationals finalized outside United States and placed outside Colorado.
 - f. U.S. national finalized outside United States and placed outside United States.
 - g. Total number of adoptions involving Hague countries, incoming and outgoing.
 - h. Number of boys in domestic adoption.
 - i. Number of boys in foreign national adoption.
 - j. Number of girls in domestic adoption.
 - k. Number of girls in foreign national adoption.
 - 2. Non-agency adoptions as required by the courts.
 - 3. Domestic relative adoptions.
- B. The number of children whose domestic adoption was finalized within the previous calendar year as delineated by (do not duplicate numbers):

1. White, not-Hispanic or Latino children.
 2. Hispanic or Latino children.
 3. Black/African American children, not Hispanic or Latino.
 4. Asian children.
 5. Native Hawaiian/Pacific Islander children.
 6. Alaskan/Native American children.
 7. Children that were of two (2) or more races.
- C. The total number of birth parent counseling cases completed by the agency in a calendar year delineated by:
1. The total number of birth mothers counseled.
 2. The total number of presumed birth fathers counseled.
- D. The total number of birth mother counseling cases completed by the agency that resulted in relinquishment of a child(ren).
- E. The total number of approved SAFE home study assessments completed by the agency within the calendar year, including those completed for (do not duplicate number of applicants):
1. White, not Hispanic or Latino applicant(s).
 2. Applicant(s) of Hispanic or Latino ethnicity.
 3. Black/African American, not Hispanic or Latino applicant(s).
 4. Native Hawaiian/Pacific Islander applicant(s).
 5. Asian applicants
 6. Alaskan/Native American applicant(s).
 7. Applicants of tow (2) or more races (please specify races).
- F. The total number of adoptive placements made by the Colorado agency within a calendar year which resulted in:
1. Disruption of the placement.
 2. Dissolution of the adoption.

7.710.8 IDENTIFYING AND NON-IDENTIFYING INFORMATION [Rev. eff. 5/1/10]

Release of identifying information may occur only as listed in current statute or by court order.

- A. Non-identifying information is defined as information which does not disclose the name, address, place of employment or any other material information which would lead to the identification of the birth parents. Non-identifying information includes only the following:
 - 1. The physical description of the birth parents.
 - 2. The educational level achieved by the birth parents.
 - 3. The occupation of the birth parents at the time of the child's birth.
 - 4. Genetic information about the birth families.
 - 5. Medical information about the adult adoptee's birth.
- B. Child placement agencies authorized by the Colorado Department of Human Services, Division of Child Welfare, to release information shall be qualified as follows:
 - 1. Any agency selected shall be a non-profit agency which has been licensed for at least five years by the State Department to provide adoptive services and is in good standing with the State Department.
 - 2. The agency will submit a written policy which includes the following:
 - a. A written statement, signed by the Executive Director of the agency, stating the agency shall maintain all information which identifies members of the birth family strictly confidential. In addition, the agency shall identify by name all staff involved in the provision of this service and shall furnish copies of statements agreeing to preserve confidentiality signed by members of the staff who will be handling the material.
 - b. Agreements for the physical security of any material.
 - c. Fees to be charged and a sliding fee schedule for low income persons.
 - d. Policy for releasing the information and type of material that is to be included. Information must be released in written form by placement workers on approval of the placement supervisor.

- e. Time frame for response to requests.
 - f. Development of written information to be given to the persons seeking non-identifying information prior to providing the service which outlines the agency's procedures and fees for these services.
- C. The agencies .selected shall agree to participate in any training provided by the State Department.
- D. The agencies selected will return to the State Department the record and a copy of the material released within thirty (30) calendar days of release of the information.

7.710.9 NON-PUBLIC INTERSTATE AND INTERCOUNTRY ADOPTIONS

7.710.91 Non-Public Agency

Pursuant to Section 19-5-205.5, C.R.S., the State Department is authorized to select a non-public agency to perform the administrative review and approval or denial functions required by the Interstate Compact on the Placement of Children (ICPC) and statutes governing foreign adoptions. The agency selected by the State Department to provide these functions shall hereafter be referred to as "selected agency".

Agencies that perform or assist in obtaining adoptive placements shall hereafter be referred to as "placing agencies." The Interstate Compact defines the persons and agencies who, when they place a child from one state into another state, shall follow ICPC procedures. These persons and agencies are referred to as "sending agencies" (per Section 7.307.2, 12 CCR 2509-4)

7.710.92 Qualifying Criteria for Selection of a Non-Public Agency

- A. The agency selected to perform this administrative function shall be either a licensed child placement agency designated to facilitate adoptions or a non-public agency that meets the criteria to be a licensed child placement agency that facilitate adoptions.
- B. A designated placement supervisor or an individual who meets the criteria to be a designated placement supervisor shall sign documents related to interstate or intercountry adoptions (see Section 7.710.22, A, 6).
- C. Supervision of the position(s) responsible for performing this administrative function shall be provided from within the selected agency by a position(s) that is a designated placement supervisor or meets the criteria to be a designated placement supervisor.
- D. The selected agency shall have been a licensed child placement agency in good standing with the State Department or have met the criteria to be a licensed child placement agency for at least five (5) years.

- E. The selected agency shall not perform this administrative function regarding cases in which it is the placing agency or has any other vested interest in the outcome of the administrative review since such action would constitute a conflict of interest.

The selected agency shall submit such cases to a non-public agency with which it subcontracts to perform the administrative function. The agency with which the selected agency subcontracts must meet the same qualifying criteria as a selected agency pursuant to this section.

7.710.93 Interstate Non-Public Adoptions

A. Where and How to Send Adoptive Placement Request Packets

Adoptive placement request packets shall be sent in triplicate by the sending agency or other state's ICPC office to the selected agency. The name and address of such agency shall be distributed by the State Department.

B. Time Frame for Processing Adoptive Placement Request Packets

1. Upon receipt of a complete request packet for an adoptive placement from another state ICPC office including the fee for the administrative services of the selected agency, the selected agency shall review the request packets and shall grant or deny permission for the placement to occur within three business days, excluding weekends and holidays.
2. Upon receipt of a complete set of request packets from a Colorado local agency for an adoptive placement into another state, including the fee for the administrative services of the selected agency, the selected agency shall review the request packets for compliance with Colorado laws and procedures, and the packets, including the accompanying 100-A, signed and dated by the authorized signer or the designee of that authorized signer, shall be sent on to the receiving state ICPC office within three business days, excluding weekends and holidays.
3. Permission for the placement to occur or denial of the placement request shall be signified by the dated signature of the authorized signer or the designee of that authorized signer on the Form 100-A that accompanied the request packets. If verbal permission for placement of the child who is the subject of the placement request is initially given to the sending state ICPC office, it shall only be considered a valid form of permission for placement if such verbal permission is immediately followed by the properly signed 100-A being sent by facsimile process to the sending state ICPC office. Within twenty-four (24) hours of such facsimile being sent, copies of the original 100-A and an accompanying memo from the authorized signer of the 100-A that is written on the letterhead stationary of the selected agency shall be mailed to the sending state ICPC office and its local sending agency.

4. The authorized staff in the selected agency shall work with the sending state ICPC office, and/or with Colorado's local sending agency, and with other parties involved with a requested adoptive placement to assist the sending party in bringing the request packets into compliance with applicable statutes and/or rules and regulations, as soon as possible. While such compliance issues are being clarified and potentially resolved, the selected agency may either retain the submitted documents until compliance is achieved or return incomplete packets to the sending state ICPC office.
- C. Materials Required in Adoptive Request Packets for Children for Whom an Adoptive Placement into Another State is Requested
1. Court documents showing the child is free for adoption. All birth parents who are residents of Colorado shall relinquish in a Colorado court and fulfill all Colorado statutory requirements for relinquishment. Adoption consent forms from other states signed by Colorado residents shall not be considered legal in Colorado.
 2. Birth and discharge medical information on the child from the hospital where the child was born.
 3. Birth Parent counseling report and affidavit from a licensed child placement agency in Colorado regarding the birth parents.
 4. Genetic/medical/social background information regarding the birth parents
 5. Written study regarding the adoptive parent(s) that has been completed and approved by a Colorado licensed child placement agency or individual legally approved by the receiving state to perform such studies. Adoptive studies or their updates shall be no more than one year old.
 6. Itemized list of monies paid to attorneys, agencies and birth parents, including a statement that no money has been paid to locate for adoption the child who is the subject of the 100-A. This list shall separately specify all fees and costs charged for services associated with the review and approval of interstate adoptions.
 7. Documentation of compliance with the Indian Child Welfare Act.
 8. The name of any Colorado and/or out-of-state attorney involved in the requested placement.
 9. If legal rights have not been terminated or legally relinquished by both birth parents, a legal risk statement shall be signed by the adoptive parents, accompanied by documentation certifying the initiation of relinquishment or termination court procedures.

D. Materials Required in Request Packets for Children to be Placed into Colorado from Another State

The materials required in request packets for children to be placed into Colorado from another state are identical to the materials described in Section 7.710.93, C, with the following exceptions. If the sending state does not require proof of relinquishment counseling, Colorado shall not require this in order to approve an adoptive placement into this state. The fulfillment of the sending state's requirements regarding consent forms shall satisfy Colorado's requirements for purposes of approval of the adoptive placement.

7.710.94 Non-Public Intercountry Adoptions

A. Requests for Approval of United States Citizenship and Immigration Services Recommendation

For each adoptive family assessed for placement of a foreign national, the agency shall submit the following information to the selected agency:

1. Three (3) copies of the USCIS recommendation form with Section I completed, signed and dated.
2. Applicable fees.
3. An adoptive family assessment completed in accordance with Section 7.710.56.
4. Medical and/or legal documents of the foreign national, if available.
5. A statement of affirmation signed by the Executive Director that the Colorado licensed child placement agency has documentation as required in Section 7.710.64, C, 2.
6. Copies of any and all agreements that the Colorado licensed child placement agency has to verify compliance with Section 7.710.64, C and D.
7. A copy of the foreign country's adoption eligibility requirements.

B. Time Frames and Procedures for Processing Requests for Recommendations

The following case materials with the appropriate fees shall be forwarded to the selected agency for review. The name and address of the agency shall be distributed by the State Department.

1. Upon receipt of a properly completed recommendation form and attachments in accordance with Section 7.710.94, A, the selected agency

shall review and grant or deny the approval within three (3) business days of receipt, excluding weekends and holidays.

2. In the event of an unresolved concern or dispute between the sending Colorado agency and the selected agency regarding the role of the selected agency, the selected agency may refer the case to the Colorado State Department for review and resolution after reasonable attempts to obtain needed clarifications or additional information have been unsuccessful.
3. After approving the recommendation, the original plus one copy shall be returned to the Colorado agency.
4. Authorized staff in the selected agency shall work with the Colorado sending agency, the foreign country authorities, and other parties involved with a requested adoptive placement to assist with bringing the request packets into compliance as soon as possible. While such compliance issues are being resolved, the selected agency may either retain the submitted documents until compliance is achieved or return incomplete packets to the Colorado local sending agency.

C. Notice of Arrival

1. As soon as possible, but no later than six (6) months after arrival in the U.S., the Colorado agency shall send to the selected agency a notice of arrival which contains the following information:
 - a. The adoptive parent(s)' names and addresses.
 - b. The child's birth name, adoptive name, sex and date of birth.
 - c. If at any time in the process the licensed agency or prospective adoptive family becomes aware that the child's adoption will not be finalized in the foreign country, the family and agency are to notify the state in writing of the changes with an explanation as to the reasons for the change in status. If the child returns to the state and was not legally adopted in the foreign country, all documents concerning the child's legal status and the type of visa which was issued allowing entrance into the United States shall be submitted to the Colorado Department of Human Services, Division of Child Care, for review by appropriate entities. The licensed adoption agency shall ensure that the home is certified as a foster home.
 - d. The name of the county in which the adoption will be either finalized or validated.
2. The agency shall send the selected agency the following documents with the notice of arrival:

- a. An English translation copy of the child's original birth certificate with child's birth name.
 - b. An English translation copy of the foreign adoption decree or, if the adoption was not finalized in the foreign country, translated documents transferring guardianship to the agency and/or prospective adoptive parents.
3. Pursuant to Colorado statute, the selected agency shall generate a letter to the appropriate District Court and return the letter to the licensed agency for submission to the court.

7.710.95 Confidential Case Files and Data System

The selected agency and the agencies with which they subcontract are prohibited from engaging in conflict of interest in the manner in which they conduct the administrative function associated with non-public interstate or intercountry adoptions and authorized by statute and rules. Conflict of interest is defined in Section EE of the Administrative Information that accompanies the contract between the Colorado Department of Human Services and the selected agency.

7.710.96 Conflict of Interest

The selected agency and the agencies with which they subcontract are prohibited from engaging in conflict of interest in the manner in which they conduct the administrative function associated with non-public interstate or intercountry adoptions and authorized by statute and rules. Conflict of interest is defined in Section EE of the Administrative Information that accompanies the contract between the Colorado Department of Human Services and the selected agency.

7.710.97 Guidelines for Fees Charged by the Selected Agency to Provide the Administrative Function Associated with Non-Public Interstate or Intercountry Adoptions

- A. The selected agency shall charge a fee to provide the administrative function associated with non-public interstate and intercountry adoptions as authorized by statute and rules.
 1. For interstate placement requests, the fee shall include the processing of all required materials and providing procedures necessary to process the request.
 2. For intercountry adoptive requests the fee shall include authorization review, USCIS recommendation, processing the arrival notice and the court letter. If an approved family changes to another country after initial approval, there may be an additional charge.

- B. The fee charged for this service shall reflect the cost to the selected agency of direct and indirect expenses associated with the provision of administrative services required by statute and rules for the review and approval of interstate and intercountry adoptive requests.
- C. The fee covers indirect costs associated with initial inquiries prior to the establishment of an adoptive placement request or other inquiries about interstate or intercountry requests.

7.710.98 Standards by which the Colorado Department of Human Services Shall Evaluate the Delivery of Services by the Selected Non-Public Agency

The Colorado Department of Human Services shall monitor the selected agency to determine compliance with Sections 7.710.91 - 7.710.97, in accordance with Section 19-5-205.5(2)(b), (I-X), C.R.S.

- A. The State Department shall conduct a site visit to the selected agency and review interstate and intercountry adoption files that are a representative sample of pending, open and closed files.
- B. The State Department shall conduct a site visit on at least a semi-annual basis.
 - 1. Within thirty (30) calendar days following each site visit, the State Department will provide the selected agency with a written evaluation that indicates the following:
 - a. Whether or not the agency is in compliance with rules.
 - b. What corrections, if any, are necessary in order to be in compliance.
 - c. If there are corrections to be made, the time frame within which these corrections are to be made.
 - 2. Failure by the selected agency to make corrections that have been indicated in the written evaluation of the site visit shall be grounds for the Colorado Department of Human Services to terminate the contract between it and the selected agency.

7.711 RULES REGULATING CHILDREN’S RESIDENT CAMPS [Repealed eff. 03/05/2026]

7.712 RULES REGULATING SCHOOL-AGE CHILD CARE CENTERS [Repealed eff. 03/05/2026]

7.713 MINIMUM RULES AND REGULATIONS FOR SECURE RESIDENTIAL TREATMENT CENTERS [Rev. eff. 6/1/12]

All secure residential treatment centers must comply with the “General Rules for Child Care Facilities” found at Section 7.701, et seq., Section 7.714.53, et seq., and the applicable definitions in Section 7.714.1, and these “Rules Regulating Secure Residential Treatment Centers.”

7.713.1 DEFINITIONS [Rev. eff. 6/1/12]

The definition of a secure residential treatment center facility is found at Section 26-6-102(9) of the Colorado Revised Statutes.

7.713.11 Governing Body

- A. The governing body is the individual(s), partnership, corporation, or association which holds the ultimate authority and legal responsibility for the conduct of the secure residential treatment center. The governing body shall be legally organized and authorized to do business in Colorado.
- B. The governing body shall be identified by its legal name. The names and addresses of individuals who hold primary financial control, members of the board of directors, and officers of the governing body shall be disclosed fully to the Colorado Department of Human Services. The department shall be informed immediately of the names and addresses of the new individuals.
- C. The Governing Body shall have by-laws which include but are not limited to the following:
 - 1. Qualifications, rights, and duties of membership
 - 2. Size of the governing body
 - 3. Method of selection
 - 4. Term of office of members and officers
 - 5. Duties and responsibilities of officers
 - 6. Quorum
 - 7. Parliamentary procedures

8. Recording of minutes
 9. Method of amending the by-laws
 10. Conflict of interest provisions
 11. Specification of the relationship of the chief executive to the governing body.
- D. When the governing body does not include a board of directors, there shall be an advisory committee of at least two individuals who act in an advisory capacity to the governing body. The names of the advisory committee members shall be disclosed to the department. The advisory committee shall meet at regularly-stated intervals.
- E. The minutes of the Advisory Committee or the Board of Directors shall be maintained. The minutes shall be available to the Department upon request, except that the minutes containing confidential personnel information need not be shared with the department.
- F. The functions of the governing body shall include but not be limited to:
1. The appointment of an administrator who shall be responsible, according to established performance criteria, to the governing body, which shall delegate to him/her the executive authority and responsibility for the administration of the secure residential treatment center according to its defined purpose.
 2. The formulation and regularly-planned review of policies and procedures to be followed by the center.
 3. The provision of necessary facilities, adequate financing, qualified personnel, services, and program functions for the welfare and safety of children in accordance with these standards.
 4. The adoption of a written description and organizational chart which reflects the current structure of authority, responsibility, and accountability within the center.

7.713.12 Purpose

A secure residential treatment center shall have a written statement specifying its philosophy, purposes, and program orientation. The statement shall identify the types of services provided, the characteristics of the youth to be served by the program and the geographic area from which youth are accepted. The statement of purpose shall be available to the public on request.

7.713.13 Fiscal Management

- A. A secure Residential Treatment Center shall demonstrate that it is financially sound and manages its financial affairs prudently. All funds disbursed by the center shall be expended in accordance with the program objectives as specified by the governing body.
- B. There shall be a written policy of fiscal management which includes an annual budget, collection, safeguarding and disbursement of monies, internal controls, petty cash, check signatures, and fiscal system accounts for all income and expenditures on an ongoing basis.
- C. There shall be an annual financial audit conducted independent of the center.
- D. If a center has a juvenile benefit fund, there shall be written policy and procedure which govern its operation.
- E. There shall be a written policy and procedure for the management of personal funds of the resident which include accounting procedures to assure that the youth's funds are secured and the resident will have access to his/her funds at the time of discharge.
- F. There shall be a written policy and procedure to regulate the operation of a youth's commissary, which includes but is not limited to inventory and accounting procedures for the commissary.

7.713.14 Insurance

- A. Every center shall carry public liability insurance. The applicant or licensee shall submit to the Department of Human Services the amount of the insurance and the name and the address of the insurance company providing the insurance for the facility. Information about the insurance should be maintained at the facility.
- B. If a center operates its own transportation vehicles, it shall carry insurance in compliance with the minimum limits required by the Colorado Revised Statutes, Title 10, Article 4.
- C. The center shall carry workman's compensation and unemployment insurance as required by law.

7.713.2 CHILD CARE SERVICES

7.713.21 Admission Policy and Procedures [Rev. eff. 6/1/12]

- A. Admissions shall be in keeping with the stated purpose of the Secure Residential Treatment Center and shall be limited to youth who are adjudicated delinquent and who are guilty of an offense which would be a crime if committed by an adult.
- B. The secure residential treatment center shall have a written admission policy which shall include but not be limited to the following:

1. Policies and procedures related to intake.
 2. The age range and sex of the youth.
 3. The youth's needs, problems, circumstances, or patterns of behavior best addressed by the center's program.
- C. The written description of admission policies and criteria shall be provided to referring agencies upon request and shall be available to the parent(s) or guardian of any youth referred for placement.
- D. A Secure Residential Treatment Center shall accept a youth into care only after an evaluation of presenting problems in areas such as social, physical health, mental health, education, and psychological concerns.
- E. A secure residential treatment center shall obtain an account of the legal aspects of the youth's case, summary of the offense history, social, health and family history, psychological evaluation, developmental assessment, mental health evaluation. Educational records shall be obtained. As much of this information as possible shall be obtained prior to admission, but the total evaluation shall be completed within one month after admission.
- F. There shall be a placement agreement with the placement agency. The placement agreement shall include but not be limited to the following:
1. A delineation of the respective roles and responsibilities of all agencies and persons involved with the youth and his/her family.
 2. Commitment order of the court.
 3. Written authorization from the parent or custodian to obtain medical care for the youth.
 4. Description of mutual expectations regarding program, records, financial agreements, general contractual agreements and reporting requirements.
- G. Prior to placement, approval shall be obtained from the Colorado Department of Human Services, the Colorado Deputy Compact Administrator of the Interstate Compact on the Placement of Children (ICPC) for any youth, whose legal jurisdiction rests in a state other than Colorado.
- H. There shall be a written procedure for classifying youth which includes the level of risk presented, the type of housing required, participation in facility and community programs, and the youth's special needs. The procedure shall be reviewed annually.
- I. When a new resident arrives at the facility, the following shall occur

1. A search shall be completed of the youth and his/her possessions. There shall be a disposition of the youth's personal property. Any items held by the center's administration shall be recorded, with a copy of the record maintained in the youth's file and a copy given to the youth.
2. Each youth shall shower; each youth shall receive clothing, personal hygiene articles, and hair care services as necessary. The youth's personal clothing shall be washed or cleaned as appropriate and returned to the youth as soon as possible.
3. Each youth shall be assigned an identification number and personal data shall be recorded.
4. A medical screening shall be performed by a health trainee or qualified health care personnel on each youth upon arrival pursuant to Section 7.713.31, C.
5. The youth shall be classified according to the written procedure and placed in an appropriate situation within two weeks of admission. Information gathered during classification shall be shared with staff members who must make determinations for the child.
6. There shall be a program, which is carried out with the youth, during the days while classification is occurring.
7. The youth shall be given an orientation. If the youth does not understand English, the orientation is to be in the youth's own language. Completion of orientation is documented by a statement signed and dated by the youth. Orientation shall include but is not limited to the following:
 - a. A tour of the facility including fire escape routes and exits.
 - b. A copy of written rules and regulations of the facility which include but are not limited to daily schedule, medical services, discipline, mail, visitation, grievance, and communication procedures.
 - c. A discussion about the procedures to assure the youth's understanding.
 - d. Assistance in notifying family members as to his/her arrival at the center and procedure for mail and visiting.
 - e. Notifying the individual, legal custodian, and if appropriate, the individual's family and obtaining authorizations from the legal custodian to perform physical management, restraint and seclusion as delineated in Section 7.714.53.

- J. The total number of residents admitted to the Secure Residential Treatment Center shall not exceed the licensed capacity.

7.713.22 Youth Rights and Grievance Procedures

- A. Each licensed center shall have written policy and procedure which addresses and insures the availability of each of the following rights for residents:
1. No youth shall be subject to discrimination based on race, religion, national origin, sex, or physical handicap.
 2. There is equal access to programs and services for male and female youth in co-correctional centers.
 3. Each youth has the right to reasonable enjoyment of privacy.
 4. Each youth has the right to receive appropriate and reasonable adult guidance, support, and supervision.
 5. No youth shall be subjected to corporal or unusual punishment, humiliation, mental abuse, or punitive interference with the daily functions of living, such as eating or sleeping.
 6. Each youth has the right to be protected from all forms of sexual exploitation.
 7. Each youth has the right to receive adequate and appropriate medical care.
 8. Each youth has the right to receive adequate and appropriate food, clothing and housing.
 9. Each youth has the right to live in clean, safe surroundings.
 10. Each youth has the right to participate in an educational program which will maximize his/her potential.
 11. Each youth shall have the right to communicate or correspond with persons or organizations subject only to the limitations necessary to maintain facility order and security.
 12. Each youth shall have the right to participate in religious services and religious counseling on a voluntary basis, subject only to the limitations necessary to maintain order and security.
 13. Each youth shall have reasonable access to the general public through the communications media, subject only to the limitations necessary to

maintain order and security and protect the juvenile's rights. Media requests for interviews and juvenile consents shall be in writing.

14. No youth shall be required to participate in uncompensated work assignments unless the work is related to housekeeping, maintenance of the facility or grounds, personal, hygienic needs, or the work is part of an approved vocational or training program.
 15. Each youth shall have access to recreational opportunities and equipment, including, when the climate permits, outdoor exercise.
 16. Each youth has the right of access to the courts.
 17. Each youth has the right to assistance in making confidential contact with attorneys and the attorneys' authorized representatives; such contact includes, but is not limited to, telephone communications, uncensored correspondence, and visits.
 18. Each youth has the right to determine the length and style of hair, except in individual cases where such restrictions are necessary for reasons of health and safety.
 19. Each youth has the right to keep facial hair, if desired, except in individual cases where such restrictions are necessary for reasons of health and safety.
- B. If the secure residential treatment center enforces any restrictions upon the youth's rights, the center shall:
1. Inform the youth of the conditions of and reasons for restriction or termination of his/her rights.
 2. Place a written report summarizing the conditions of and reasons for restriction or termination of the youth's rights in that youth's case record.
- C. A center shall not bar a youth's attorney, clergyman, or an authorized representative of the responsible placing agency from visiting, corresponding with, or telephoning the child.
- D. Written policies and procedures pertaining to visiting, mail and other forms of communication shall be established and implemented to encourage and maintain family and other relationships while ensuring the protection of the youth, staff and program from unreasonable and unnecessary intrusions and disruptions. Policies and procedures shall address, but not be limited to, the following:
1. Visits of the youth with relatives, friends, or others interested in his/her welfare, unless in the judgment of treatment staff and placement agency it would be detrimental to the youth and/or his family.

2. Reasonable access to the telephone to make and receive personal calls by youth.
 3. The forwarding of first class letters and packages after transfer or release.
 4. Reasonable access to publications by youth.
 5. No limit on the volume of mail a youth may send or receive, except when the center provides postage or when there is clear and convincing evidence to justify such limitations.
 6. Youth's letters, both incoming and outgoing, are not read, except where there is clear and convincing evidence to justify such actions; if correspondence is read, the youth is informed in advance and is present when the letter is opened; and the action is documented.
 7. Inspection of youth's letters or packages for money or contraband.
 8. All cash received through the mail is held for the youth in accordance with the procedures of the center.
 9. Incoming and outgoing mail is forwarded within 24-hours and packages are forwarded within 48-hours, excluding weekends and holidays
 10. Youth are permitted to send sealed letters to a specified class of persons and organizations, including but not limited to courts, counsel, officials of the confining authority, administrators of grievance systems, and officials of the placing agency.
 11. The center shall provide postage for the mailing of a minimum of two letters per week for each youth, if requested, excluding legal correspondence.
- E. The secure residential treatment center shall establish a written grievance procedure which provides adequate due process safeguards, spells out an appeal process of at least one level of appeal, and assures that youth are entitled to report any grievance and shall not be subject to any adverse actions as a result of filing the grievance.
1. Grievance procedures shall be processed without alteration, interference, or unreasonable delay.
 2. This grievance procedure shall be made available and explained to each resident as provided for in the admission procedures.
 3. If a youth files a grievance, it shall be recorded in the youth's record along with the investigation findings and resulting action taken by the center.

7.713.23 Program Description and Individual Treatment Plan [Rev. eff. 6/1/12]

- A. A secure residential treatment center shall have a written overall program description which is submitted to the Colorado Department of Human Services for review prior to original licensing. Any significant change in this description shall be submitted to the licensing authority for review prior to implementation. The written description shall include the following:
 - 1. The position title and qualifications of the person who has overall responsibility for the treatment program.
 - 2. Staff responsibility for planning and implementation of the treatment procedures and techniques.
 - 3. Staff competencies and qualifications.
 - 4. The range of services and techniques which shall include at least modes of therapy, behavior management, physical management, restraint and seclusion, education, medical and recreation.
- B. Within thirty (30) days of admitting a youth into care, a secure residential treatment center shall conduct a comprehensive assessment of the youth.
 - 1. The assessment shall be conducted by a planning team. This team shall include persons responsible for implementing the treatment plan on a daily basis. At least one member of the team shall have a graduate degree in psychology, psychiatry, social work, or counseling plus two years of treatment-oriented experience.
 - 2. The planning team shall complete an assessment in at least the following areas:
 - a. Social History
 - b. Medical and Dental status
 - c. Education
 - d. Personal/Social development
 - e. Family relationships
 - f. Vocational training
 - g. Recreation
 - h. Life skills development
 - i. Religious interests

- j. Mental health
 - k. Delinquency history
 - 3. All methods and procedures used in this assessment shall be appropriate to the age, cultural background, and dominant language or mode of communication of the youth.
- C. On the basis of this assessment, a secure residential treatment center shall develop a written, time-limited, goal-oriented individual treatment plan.
- 1. A secure residential treatment center shall provide an opportunity for the following persons, in addition to staff members, to participate in the planning process:
 - a. The youth;
 - b. His/her parent(s) or guardian, if available and unless contraindicated;
 - c. Representative(s) of the placing agency;
 - d. Other persons significant in the youth's life.
 - 2. Based on the assessment, the individual treatment plan shall include the following components:
 - a. A statement of long-term and short-term goals to be achieved by the youth and the method to be used for evaluating the youth's progress.
 - b. Strategies for strengthening positive family relationships.
 - c. Specification of the daily activities, including education and recreation, to be pursued by the program staff and the child in order to attempt to achieve the stated goals.
 - d. Specification of therapeutic and/or any specialized services that will be provided directly or arranged for, frequency of services, and measures for ensuring their proper integration with the child's ongoing program activities.
 - e. Goals and preliminary plans for discharge and aftercare.
 - f. Identification of all persons responsible for implementing or coordinating implementation of the plan.
 - 3. The completed treatment plan shall be signed by the youth and the chief administrator of the center or his/her designee.

4. The treatment plan and any subsequent revisions shall be explained to the youth and documented by signature of youth and staff.
 5. Each treatment plan shall be reviewed at least monthly to evaluate the degree to which the goals have been achieved. The treatment plan shall be revised as appropriate to the needs of the youth.
- D. If the assessment process or the treatment plan requires the services of a specialist, such as a psychiatrist, psychologist, speech therapist or physical therapist, the specialist shall be currently certified or licensed according to state law.
- E. If an individual treatment plan requires the individual to be placed in seclusion for more than twenty-four (24) hours, the individual:
1. Shall be afforded living conditions and rights approximating those available to the general population, such as one hour of large muscle activity every twenty-four hours, the use of toilet and shower, the receipt and sending of mail, and the same meals as the general population.
 2. Shall receive a counseling visit as soon as possible and a visit at least once every twenty-four hours, and an administrative review of the use of seclusion by the facility director or designee, who was not involved in the incident every twenty-four hours.

A record shall be kept pursuant to Section 7.714.53 and include documenting the actions taken while operating under this provision.

7.713.24 Discipline, Physical Management, Restraint, and Seclusion [Rev. eff. 6/1/12]

- A. Discipline shall be constructive or educational in nature and may include diversion, withholding of privileges, separation from problem situation, talk with the youth about the situation, praise for appropriate behavior, physical management, and seclusion. Youth shall not be subjected to physical harm or humiliation.
- B. A secure residential treatment center shall have written policies and procedures regarding discipline and control, and pursuant to Section 7.714.53, written policies and procedures regarding physical management, restraint and seclusion, which shall be explained to all youth, families, staff, and placing agencies. These policies shall include measures for positive responses to appropriate behavior.
- C. A secure treatment center shall prohibit all cruel and unusual punishments including, but not limited to, the following:

1. Punishments including any type of physical hitting or any type of physical punishment inflicted in any manner upon the body such as punching, shaking, biting, or roughly handling a child.
 2. Physical exercises such as running laps or push-ups, when used solely as a means of punishment, and when such activities are not approved in the treatment plan.
 3. Requiring or forcing the youth to take an uncomfortable position, such as squatting or bending, or requiring or forcing the youth to repeat physical movements when used solely as a means of punishment.
 4. Group punishments for misbehaviors of individuals except in accordance with the center's written policy.
 5. Verbal abuse or derogatory remarks about the youth, his family, his race, religion or cultural background.
 6. Excessive denial of on-grounds program services or denial of any essential program service solely for disciplinary purposes.
 7. Deprivation of meals, although scheduled meals may be provided individually.
 8. Denial of visiting or communication privileges with family solely as a means of punishment.
 9. Denial of sufficient sleep.
 10. Requiring the youth to remain silent for long periods of time.
 11. Denial of shelter, clothing or bedding.
 12. Extensive withholding of emotional response or stimulation.
 13. Physical management, restraint and seclusion used as sanctions.
 14. Assignment of physically strenuous or harsh work which could result in harm to the youth.
- D. The secure residential treatment center shall have written rules for resident conduct which specify prohibited acts and the sanctions which may be imposed. The written rules are given to each resident and are to be fully understood by all staff.
- E. The secure residential treatment center shall have written guidelines for the informal resolution of minor behavior infractions. These guidelines shall include the opportunity for youths to have input into the problem solving and decision

making that relate to their participation in the program and to the consequences for their minor behavior infractions.

F. Minor Rule Violations

1. Prior to privilege suspension or a room restriction sanction, the reasons for the sanction shall be explained and the youth shall have the opportunity to explain the behavior leading to the sanction.
 - a. Whenever possible, the discussion should take place in a private setting. When a private setting is not available, the youth shall be afforded as much privacy as is possible.
 - b. Staff shall advise the youth of the expectations of the program and of the youth's responsibilities in the situation.
 - c. Staff shall assist the youth in developing solutions and/or strategies for correcting the problematic behavior.
2. Room restriction or time-out in an unlocked room or area away from the group for minor misbehaviors shall not exceed sixty (60) minutes, to be continuously reviewed in fifteen (15) minute intervals. At the time that the restriction is imposed, the youth shall be informed that the restriction period will last between fifteen and sixty minutes, depending on his/her interaction with staff and participation in the counseling process during the restriction period.
3. Reports are prepared on each incident of a youth's behavior infractions resulting in room restriction or loss of privileges.
4. During room restriction or time-out in or out of an unlocked room, direct staff communication shall occur at least every fifteen minutes or more frequently, depending on the youth's emotional state. During this interaction with the staff, the youth should participate with staff in determining the end of the restriction or time-out period.

G. Major Rule Violations

1. When a youth is charged with violating a major rule of the facility which could result in a disciplinary action, the youth shall be insured the right to due process prior to any disciplinary sanction being imposed.
2. In an emergency, an individual may be placed in seclusion if less restrictive alternatives have failed. Seclusion shall occur in compliance with Section 7.714.53, et seq.

3. A youth charged with a major rule violation shall be given a written notice of charge(s) of the alleged violation as soon as possible, but at least within twenty-four (24) hours of the time that the infraction is discovered.
 - a. A written notice of the scheduled disciplinary hearing shall be provided to the youth at least twenty-four hours in advance of the hearing.
 - b. Although notice of the time and place of the hearing must be provided to the youth at least twenty-four hours in advance, the youth may consent in writing to waive the twenty-four hour notice and to proceed with an earlier hearing time.
4. A disciplinary hearing shall be conducted within seventy-two (72) hours of the discovery of an alleged violation.
5. The youth shall be present at the disciplinary hearing except when his/her behavior prior to or during the hearing justifies exclusion or he/she has waived the right to be present.
 - a. The youth may be temporarily excluded during the testimony of youths who wish their testimony to be given in confidence.
 - b. Reasons for the youth's absence or exclusion shall be documented in the youth's file.
6. The disciplinary hearing shall be conducted by an impartial person or committee.
7. A youth may request the assistance of a staff member to represent him/her at a disciplinary hearing. A staff member or another resident shall be appointed when it is apparent that a youth is not capable of presenting evidence on his/her own behalf.
8. The youth shall have the opportunity to make a statement, present evidence, and call witnesses. Any exceptions shall be clearly documented in the youth's file.
9. The disciplinary officer's/committee's decision shall be based solely on the information obtained in the hearing process, including staff reports, statements of witnesses, and evidence. Once it has been determined that a youth has violated a rule and prior to any disciplinary action being taken, the reasons for the disciplinary action shall be explained to the youth. The youth shall have an opportunity to explain the behavior leading to the violation.

10. If the youth is found not guilty of the alleged violation, the disciplinary report shall be removed from all files, except the director may retain his/her copy for administrative review purposes.
 11. There shall be a written record of the findings of the hearing. A copy of the record shall be given to the youth, one placed in his/her file, and one provided to the facility director or designee. The facility director or designee shall retain copies of all proceedings findings for a minimum of six months.
 12. The facility director or designee shall review all disciplinary hearings and dispositions to ensure conformity with policy and procedures.
 13. The youth shall be informed of his/her right to appeal the decision of the disciplinary hearing officer/committee to the facility director or his/her designee at the time he/she is provided with the decision. The appeal shall be made in writing stating the basis of the appeal and shall be made within five calendar days of the receipt of the decision. The appeal shall be decided within 10 calendar days and the youth promptly notified in writing of the results of the appeal.
 14. If a youth has allegedly violated multiple major rules at the same time, one disciplinary hearing shall be scheduled to hear all the charges. If the youth is found guilty of one or more of those violations, a disciplinary sanction shall be determined according to the facility/program's behavior management program. The hearing officer/committee cannot issue more than one disciplinary sanction for each hearing. Completion of program assignments can reduce the time of a disciplinary sanction. If the youth fails to complete a program assignment within 14 calendar days, an administrative staffing may be held to review the disciplinary sanction
- H. When a youth is alleged to have committed a criminal act covered by criminal law, the center should refer the matter to an appropriate law enforcement agency or court-officials.
- I. Youth placed in a secure residential center shall not punish other youth except as part of an organized therapeutic self-government program that is conducted in accordance with written policy and is supervised directly by staff. All restrictions of cruel and unusual punishment as found at Section 7.713.24, C, shall apply.
- J. Disregard of any of the foregoing disciplinary rules or any disciplinary measure resulting in physical injury or abuse of any child may be grounds for the denial, revocation or making probationary of the license.

7.713.25 Security, Control, and Supervision [Rev. eff. 6/1/12]

- A. Youth shall be under the supervision of qualified and trained staff members or volunteers at all times.

- B. The door of the bedroom may be locked during sleeping hours for the safety of youth and/or staff and the security of the center.
- C. Staff shall conduct visual checks of youth at least every five minutes when youth are in their bedrooms and the door is locked.
- D. The center's perimeter shall be controlled by an appropriate means to ensure that youth remain within the facility and to prevent access by the general public without authorization.
- E. Residents of the secure residential treatment center shall be physically counted according to a system established by the facility. Any changes in the number of residents shall be reported to the appropriate staff member on a shift by shift basis.
- F. Staff shall inspect the center's security system and devices on a weekly basis and shall take appropriate corrective actions.
- G. Except in emergency situations, firearms are not permitted in the secure residential treatment center.
- H. The use of physical management and restraint shall comply with Section 7.714.53, et seq.
- I. The use of mechanical restraints made of metal, soft leather, rubber, plastic or cloth is limited to cases of emergency and prevention of escape and after the failure of less restrictive alternatives. Only the facility director or designee may authorize the use of mechanical restraints. Any mechanical restraint will comply with Section 7.714.53, et seq. The facility shall establish written policy and shall train all staff in the established written policy. The written policy shall include the following elements, at the minimum:
 - 1. Handcuffs shall be applied wrist to wrist in the front or back, or may be attached to a waist belt in the front only. Soft elbow restraints may also be utilized when necessary and shall be applied in the rear only. Thin, hard, portable plastic wrist restraints shall not be used on youth, except under emergency conditions identified in the facility's written policy.
 - 2. Shackles shall be applied on one person's ankles only and shall not be used to connect two persons' ankles together.
 - 3. Handcuffs and shackles shall never be intertwined directly together in such a manner as to interfere with a person's ability to maintain his or her spinal column in an erect or straight position.
 - 4. Youth shall never be handcuffed or otherwise attached to a vehicle.

5. Youth placed in restraints shall not be undressed or intentionally made uncomfortable.
 6. Youth shall be immediately removed from restraints and evacuated into a safer area or separate smoke compartment whenever a fire alarm, set off because of concern of a fire, results in the evacuation of other youth from the building or smoke compartment where the restrained youth is located. In the event that a fire alarm sounds which does not result in evacuation, the restrained youth shall not be left unattended during the alarm/emergency/drill.
 7. Appropriate allowances shall be made to assure the safety, comfort, and dignity of the youth. Normal bodily functions shall be attended to, including elimination and respiration. The room shall be maintained at a normal room temperature and shall meet all state and local safety, sanitation, and health standards.
 8. Because of the vulnerability of the youth during a physical management, precautions shall be taken to assure that the youth is protected from mistreatment, antagonism, and harm from another person.
 9. Hard metal restraint may be used for transporting youth from one location to another.
 10. When the only equipment immediately available to a staff member is hard restraint equipment and the equipment must be used to restrain a youth who poses a serious, probable, imminent threat of bodily harm to self or others, the equipment shall be exchanged for soft restraint equipment as soon as it is safely possible.
 11. Following application, all restraint equipment shall be checked by the supervisor on duty to assure that the equipment has been properly applied, is of the proper type to assure the youth's safety, and is not likely to cause injury or undue discomfort.
- J. All special incidents and emergency situations shall be reported to the director of the center.
- K. The secure residential treatment center shall maintain a control center to provide order and security. A manual of all the written procedures for the center's security and control with detailed instructions for the implementation of the procedures shall be maintained at the control center. At least the following procedures shall be written and maintained:
1. A procedure for dealing with escapes.
 2. A procedure to govern the supervision of all youth outside the facility and movement of youth within the facility.

3. A procedure regarding searches for control of contraband. The procedure shall be explained to both youth and staff.
4. A procedure to govern the control and use of keys, tools, culinary, and medical equipment.
5. A procedure for the use of physical management, and necessary reporting of their use pursuant to Section 7.714.53, et seq. and Section 7.713.25, et seq.

7.713.26 Educational Programming

- A. A comprehensive educational program shall be developed and provided for all youth who are residents of the secure residential treatment center.
1. Such programs shall be developed cooperatively by the facility and Local Education Agency (LEA) or State Education Agency based on applicable auricular requirements.
 2. The center shall develop assurances that the educational program is an integral part of the total treatment plan. Such assurances shall include procedures for information sharing, joint planning and follow through.
 3. The educational program allows for flexible scheduling that permits the youth to enter at any time and to proceed at his/her own learning pace. The youth shall attend classes appropriate to his/her educational level.
 4. There shall be a written policy and procedure which provides that each youth is assessed in terms of academic, vocational, and personal needs.
 5. Educational and vocational supervisors and instructors shall be licensed or certified by the state or as required by law.
 6. Formal educational and vocational programs have a minimum of one teacher for every 15 students.
 7. Educational and vocational training opportunities are available to each youth except when there is substantial evidence to justify otherwise.
 8. Provision is made to meet the educational and vocational needs of youth who require special placement because of physical, mental, or emotional handicaps or learning disabilities.
 9. Educational and vocational counseling are provided so that youth are placed in that phase of an educational or vocational program most suited to his/her needs and abilities.

10. Pre-vocational training programs are integrated with academic programs and are relevant to the vocational needs of the youth and to employment opportunities in the community.
 11. There is an annual evaluation to measure the effectiveness of the educational and vocational training programs against stated performance objectives.
 12. There is a system whereby the educational and vocational training programs are assessed against stated objectives by qualified individuals, professional groups and trade associations; this assessment is done at least every three years.
- B. The center shall provide space, staff, equipment, and educational materials for the educational program, which is approved by the Colorado Department of Education.

7.713.27 Library Services

- A. Library services shall be provided and shall be available to all youth.
- B. There shall be a qualified person who coordinates and supervises library services.
- C. Library services which are provided shall include but not be limited to the following:
1. Planned and continuous acquisition of materials to meet the needs of users.
 2. Logical organization of materials for convenient use.
 3. Circulation of materials to satisfy the needs of users.
 4. Information services to locate facts as needed.
 5. A reader's advisory service that helps provide users suitable materials.
 6. Promotion of the uses of library materials.
 7. A congenial library atmosphere.

7.713.28 Religious Services

- A. There shall be a written policy and procedure which provides for youth to participate in religious services and counseling on a voluntary basis.
- B. A staff member shall coordinate the center's religious programs.

- C. The facility shall provide space and equipment for the conduct of religious programs for residents.

7.713.29 Recreation Program

- A. There shall be a written policy and procedure which assures the provision of a recreation program with schedules and a plan for constructive leisure time activities, which includes both indoor and outdoor activities.
- B. A variety of fixed and movable equipment shall be provided for each outdoor play area.
- C. A center licensed for 50 or more youth shall have a full-time, qualified recreation director who plans and supervises all recreation programs. Facilities licensed for less than 50 youth shall have a staff member, who is trained in recreation or a related field and assigned to the responsibilities of the recreation director.

7.713.3 PERSONAL CARE OF THE CHILD

7.713.31 Medical and Health Services [Rev. eff. 6/1/12]

- A. A secure residential treatment center shall ensure the availability of a comprehensive policy and procedures for the provision of preventive, routine, and emergency medical, mental health, and dental care for each youth in care. A primary physician, licensed to practice medicine in Colorado, shall advise the facility about establishment and implementation of the medical policy and procedures which shall include, but not be limited to:
 - 1. Ongoing appraisal of the general health of each youth, including immunizations in accordance with state law and regulations.
 - 2. Diagnostic services, emergency care, corrective care, recuperative care, and immunization updates.
 - 3. Provision of medical examination of any youth suspected of having a communicable disease.
 - 4. Provision of health education and sex education as appropriate including information about Acquired Immune Deficiency (AIDS).
 - 5. Provision that any medical treatment administered will be explained to the youth in a language understandable to him/her.
 - 6. Provision of dental care by a dentist, licensed in Colorado, who is available to the center.

7. Provision of mental health treatment by a mental health practitioner who is licensed or certified according to state law.
 8. Availability of a physician and an emergency medical facility on a 24-hour, seven-day-a-week basis for treatment of the youth.
 9. Procedures for dispensing medication, storage of medication, documentation of administration of all medication, disposition of medications, and notification of primary physician in cases of medication errors and/or drug reactions.
 10. Provision of medical and dental prosthesis when the health of the youth would otherwise be adversely affected, as determined by the responsible physician.
 11. Assurance that youth are informed both orally and in writing of procedures required for gaining access to medical services.
 12. Assurance that program staff are informed appropriately of a youth's special medical problems. At the time of admission, staff are informed of any physical problems which might require medical attention.
 13. Provision for the management of serious and infectious diseases which are updated as new information becomes available.
- B. The center shall prepare and maintain a quarterly report on the health delivery system and health environment. An annual statistical summary of health care provided to residents shall be maintained.
- C. A medical screening shall be performed by health-trained or qualified health care personnel on each youth upon arrival at the facility; all findings shall be recorded on a printed screening form approved by the primary physician.
- D. A general medical examination for each child shall be completed by a physician or a qualified nurse practitioner within thirty days after admission unless one was completed within sixty (60) calendar days before admission. A statement form signed by the examiner shall be retained in the child's file. This exam shall include the following:
1. An examination for physical injury and disease.
 2. Vision and hearing screening.
 3. A current assessment of the child's health, including immunizations.
- E. Sick call for non-emergency medical service, conducted by a physician and/or other qualified medical personnel, shall be available to each youth weekly.

- F. Whenever indicated, a youth shall be referred to an appropriate specialist for either future assessment or treatment.
- G. Subsequent physical and other examinations shall be done annually or as directed by the physician.
- H. The facility shall ensure that the youth receive annual dental examinations.
- I. There shall be first aid supplies readily available.
- J. Youth care staff and other personnel shall be trained to respond to emergency health-related situations within a four minute response time. A training program shall be established which includes the following:
 - 1. Recognition of signs and symptoms, and knowledge of action required in potential emergency situations.
 - 2. Administration of first aid and cardiopulmonary resuscitation (DPR).
 - 3. Methods of obtaining assistance.
 - 4. Signs and symptoms of mental illness, retardation, and chemical dependency.
 - 5. Procedures for patient transfers to appropriate medical facilities or health care providers.
- K. The facility shall make every effort to ensure that a child needing corrective devices such as glasses, hearing aids, etc., is provided with the necessary equipment.
- L. If a youth wishes an exemption from a medical examination or medical treatment due to religious beliefs, the youth shall submit a written statement signed by his/her parents or guardian which states the reasons for such an exemption. A secure residential treatment center has the right to request a statement regarding general health from a medical examiner. In a potential life-threatening situation, the center shall refer the youth's care to the appropriate medical and legal authority. A center does have the right to refuse admission to a youth whose parent or guardian refuses medical treatment or examination.
- M. Medications shall be administered and stored in the following manner:
 - 1. Any prescriptive or non-prescriptive medication shall be administered by staff members of the Secure Residential Treatment Center only on the written prescription of a physician for each youth.
 - 2. Medication shall be administered only by a staff member of the secure residential treatment center who is a licensed physician, licensed

registered nurse, or a staff member who has passed a competency evaluation, which is authorized by the Colorado Department of Public Health and Environment.

3. The secure residential treatment center shall obtain written authorization from the prescribing physician to administer any prescriptive or non-prescriptive medication.
 4. Medication shall be stored in a safe, locked, clean container or cabinet.
 5. The center shall have a written medication schedule for each youth for whom medication is prescribed, a copy of which shall be available to appropriate staff.
 6. The center shall maintain for each youth a cumulative record of all medication, both prescriptive and non-prescriptive, dispensed to that youth including:
 - a. The name of the youth,
 - b. The name and dosage of medication,
 - c. The reason for prescribing the medication,
 - d. The time and date the medication is dispensed,
 - e. The name and position of the dispensing person,
 - f. The name of the prescribing physician.
- N. Under no circumstances is a stimulant, tranquilizer or psychotropic drug administered for purposes of behavior management and control, or for purposes of experimentation and research.
- O. When a youth first comes into care, the center shall ascertain all medication the youth is currently taking. At this time the center's physician shall carefully review all medication the youth is using and make plans to either continue the medication or to reconsider the medication needs of the youth.
- P. All informed consent standards in the Colorado shall be observed and documented for medical care. The informed consent of parent, guardian, or legal custodian applies when required by law. When health care is rendered against the patient's will, it shall be in accord with State and Federal laws and regulations.

7.713.32 Food and Nutrition

- A. Meals shall be served under conditions that minimize regimentation. The dining area shall provide normal group eating facilities and conversation shall be permitted during dining room hours.
- B. The center shall provide nutritious foods in the variety and amounts to meet the recommended "National Research Council's Recommended Daily Dietary Allowances" as adjusted for age, sex and activity of each youth in care.
- C. At least three meals, of which two are hot meals, shall be provided at regular meal times during each 24-hour period, with no more than 14 hours between the evening meal and breakfast. If basic nutritional goals are met, variations may be allowed during weekend and holidays.
- D. Menus shall be planned at least one week at a time, shall be dated, posted and filed for at least 12 months.
- E. If menus are not prepared by a qualified nutritionist or dietitian, there shall be review and approval by a qualified nutritionist or dietitian at least quarterly.
- F. Youth shall be encouraged to eat a variety of food served but shall not be subjected to undue coercion, including forced feeding or other punishment for refusal to eat.
- G. All food shall be from sources approved or considered satisfactory by the health authority. All foods shall be stored, prepared, and served in such a manner as to be clean, wholesome, free from spoilage, and safe for human consumption. Home-canned vegetables and meats shall not be served. Only pasteurized milk shall be served.
- H. Special diets as prescribed by appropriate medical, dental or religious personnel shall be prepared for the youth. A record of the diet shall be maintained with the youth's record of medication.
- I. Water from an approved source shall be readily accessible to youth by means of an approved water fountain or single service cups.
- J. There shall be a weekly inspection of all food service areas, including but not limited to the following:
 - 1. Dining and food preparation areas and equipment.
 - 2. Sanitary, temperature-controlled storage facilities for all foods
- K. There shall be daily checks of refrigerator and water temperatures by administrative, medical, or dietary personnel.

7.713.33 Personal Hygiene and Daily Routine

- A. The center shall establish procedures to ensure that youth receive training in good habits of personal care, hygiene and grooming appropriate to their age, sex, race and culture.
 - 1. There shall be personal supervision by staff to provide for proper grooming and physical cleanliness of the youth.
 - 2. Hair care services shall be available to youth.
 - 3. The center shall insure that youth are provided with all necessary toiletry items, including clean, individual towels and washcloths, toothbrush, toothpaste, comb and shampoo.
- B. A secure residential treatment center shall have a written plan of basic daily routines which shall be available to all personnel. This plan shall be revised as necessary.
 - 1. Youth shall be provided activities outside his/her bedroom at least 14 hours per day.
 - 2. Daily routines shall not be allowed to conflict with the implementation of a youth's treatment plan.
 - 3. Daily routines shall be established for mealtimes, waking, and bedtimes.
 - 4. Opportunity for physical exercise shall be planned for each youth to include at least one hour daily of large muscle activity.

7.713.34 Clothing and Personal Belongings

- A. A residential facility shall ensure that each child in care has adequate clean, well-fitting, attractive and seasonable clothing as required for health, comfort and physical well-being and as appropriate to age, sex, and individual needs.
 - 1. Each youth shall have clean socks, underwear and towels on a daily basis and other clean clothing at least twice a week.
 - 2. At time of admission the center shall provide for the thorough cleaning and, when necessary, disinfecting of the personal clothing of a new youth before storage or before allowing the youth to keep and wear personal clothing.
 - 3. Each youth's personal clothing shall be identified.
 - 4. A youth's clothing shall be kept clean and in good repair. The youth shall be involved, as appropriate, in the care and maintenance of his/her clothing. As appropriate, laundering, ironing, and sewing facilities shall be accessible the youth.

- B. A secure residential treatment center shall ensure that discharge plans make provisions for clothing needs at time of discharge. The wardrobe for each youth shall go with him/her at time of discharge.
- C. The center shall allow a youth in care to bring his/her personal belongings to the center as defined by the center's policy, and to acquire belongings of his/her own in accordance with the youth's treatment plan. However, the center shall, as necessary, limit or supervise the use of these items while the youth is in care. Where limitations are imposed, the youth shall be informed of the reasons by staff; and the decision and reasons shall be recorded in the youth's case record. Provisions shall be made for the protection of the youth's property.
- D. Youth assigned to food service, hospital, farm, garage, institution physical plant maintenance shops, and other special work shall be issued special and, when appropriate, protective clothing and equipment.

7.713.4 HUMAN RESOURCES

7.713.41 Personnel Policy, Orientation and Training [Rev. eff. 6/1/12]

- A. The center shall have a written statement of personnel policies which include but are not limited to:
 - 1. Job descriptions for all positions required. The descriptions shall describe duties of the job, qualifications for performance, and supervision to be provided.
 - 2. Salary range and provision for increments.
 - 3. Hours of work, holidays, vacations, sick leave, and other leaves.
 - 4. Conditions of employment, tenure, and promotion.
 - 5. Employment benefits, including retirement plan, social security, hospitalization, and other insurances.
 - 6. Employee evaluation procedure.
 - 7. Termination and sanction procedures including but not limited to child abuse and the use and/or sale of an illegal substance.
 - 8. Grievance procedures which may be used by staff.
- B. A copy of the personnel policy shall be given to each staff member at the time of his/her employment.
- C. The center shall have a comprehensive written plan for the orientation ongoing training and development of staff members.

1. All new full-time employees shall receive 40 hours of orientation/training prior to being independently assigned to a particular job. This orientation/training is to include, at a minimum, orientation to the purpose, goals, policies and procedures of the center; working conditions and regulations; responsibilities and rights of employees; and an overview of the juvenile justice and correctional field. Depending upon the employee(s) and the requirements of the particular job, the orientation/training may include some preparatory instruction related to the particular job. Provisions may be made for acknowledging and giving credit for prior training received.
2. All clerical/support employees who have minimal contact with youth shall receive an additional 16 hours of training during the first year of employment and 16 hours of training each year thereafter.
3. Professional specialists employees who have contact with youth and all support employees who have regular or daily contact with youth shall receive an additional 40 hours of training during the first year of employment and 40 hours of training each subsequent year of employment.
4. Training for clerical, support and professional specialists employees shall include such topics as security procedures, rights and responsibility of youth, fire and emergency procedures, interpersonal relations, social/cultural lifestyles of the youth population, communication skills, First Aid and CPR.
5. All new youth care/supervisory staff shall receive an additional 120 hours of training during the first year of employment and 40 hours of training each subsequent year of employment. At a minimum this training covers the following areas:
 - a. Security Procedures,
 - b. Supervision of Youth,
 - c. The use of physical management, restraint and seclusion pursuant to the requirements of Section 7.714.53, et seq.
 - d. Report Writing,
 - e. Youth Rules and Regulations,
 - f. Rights and Responsibilities of Youth,
 - g. Fire and Emergency Procedures,
 - h. Key Control,

- i. Interpersonal Relations,
 - j. Social/Cultural Lifestyles of the Youth Population,
 - k. Child Growth and Development.
 - l. Communication Skills,
 - m. First Aid and CPR.
- 6. All part-time staff and volunteers working less than 40 hours per week shall receive training appropriate to their assignments; volunteers working the same schedule as full-time, paid staff shall receive the same training as full-time staff.
 - 7. Personnel who work with youth confined separately from the total population shall receive specialized training.
 - 8. All administrative and managerial staff shall receive 80 hours of training during their first year of employment, and 40 hours of training each subsequent year of employment. This training shall cover the following areas, at a minimum: General Management and Related Subjects, Labor Law, Employee-Management Relations, The Interaction of Elements of the Criminal and Juvenile Justice Systems, and Relationships with Other Service Agencies.
 - 9. The center shall maintain written documentation of training held, the participation of individual staff members, the hours involved, and/or other in-service training activities in which each staff member was involved. Activities related to supervision of the staff members' routine tasks shall not be considered training activities for the purpose of this requirement.
- D. All training programs shall be presented by persons who are qualified in the areas in which they are conducting training.
 - E. A staff member shall be designated as Training Director and shall plan and implement staff training programs.

7.713.42 General Requirements for All Personnel

- A. The Secure Residential Treatment Center shall provide professional staff and service personnel necessary to assure the health, safety, proper care, and treatment of the youth under care.
- B. All personnel in the center shall evidence an interest in and a knowledge of youth and a concern for their proper care and well-being.

- C. The center shall have written screening and hiring procedures and make reasonable efforts to evaluate the overall emotional health and stability of each applicant. Procedures shall include exploring for history of child battering, child abuse, child molestation, child neglect, previous criminal convictions, and drug or alcohol abuse. (See Section 7.701.36).
- D. The center shall not hire or continue to employ any person whose health, educational achievement, emotional, or psychological makeup impairs his/her ability to properly protect the health and safety of the youth, or is such that it would endanger the physical or psychological well-being of the youth.
- E. Each member of the staff, including part-time and substitutes, students, and volunteers whose assignment is required to meet staff qualifications or staff ratio shall have a medical examination within six months prior to employment and thereafter as required, in writing, by a physician, nurse practitioner, or physician's assistant (see Section 7.713.48 for volunteers). The written reports of the medical examinations, which shall be on file at the facility, shall be dated and signed by the examining medical personnel. Reports shall include:
 - 1. Statement of evaluation of the person's physical condition and his/her suitability for employment in a secure facility caring for youth.
 - 2. A list of known immunizations he/she has had and the most recent dates when immunized.
 - 3. Tuberculin status. If the staff member has a certificate of previous negative tuberculin testing, the testing need not be repeated. If there is no certificate, the testing needs to be completed prior to employment.
- F. In addition to a physical examination, food handlers, or those who prepare food for youth, shall have special tests as may be required by local ordinances or by the physician's recommendation.
- G. An employee who, upon examination or as a result of tests, shows indication of a physical condition which could be hazardous to a youth, other staff, or self, or which would prevent performance of duties, shall not be assigned or returned to his/her position until the condition is corrected to the satisfaction of the examining physician.
- H. Any employee suspected of a communicable disease shall have a medical examination.

7.713.43 Administrative Staff

- A. Administrator

The administrator of a secure residential treatment center shall be qualified as follows:

1. The administrator shall have received a bachelor's degree from an accredited college or university and have five years of verified experience in the human services field with youth; three years of experience shall be in a supervisory and/or administrative position.
2. The administrator shall assume the following duties:
 - a. Overall direction and responsibility for the youth program, facility and fiscal management.
 - b. Overall direction and responsibility for supervision of adequate staff.
 - c. The selection and training of a capable staff member who can assume responsibility for management of the center in the administrator's absence.
 - d. The establishment and maintenance of relationships with allied agencies, services and resources within the community.

B. Assistant or Acting Administrator

1. In each Secure Residential Treatment Center, there shall be a specifically designated staff member, age 21 or over, capable of acting as a substitute for the administrator during his/her absence. The duties and responsibilities of the substitute administrator shall be clearly defined in order to avoid confusion and conflict among other staff and youth.
2. If the administrator is regularly absent from the facility more than 50 percent of his/her working hours, an assistant administrator shall be appointed who meets the same qualifications as the administrator found at Section 7.713.43, A.

C. Administrative Coverage

1. When there is a change in administrator, or when he/she has left the center permanently without a replacement, the State Department of Human Services shall be notified within 30 calendar days; or when a possible change in administrator is anticipated, it is preferable to notify the state department prior to the change.
2. The administrator or the staff member to whom the administrator has delegated responsibility shall be available at all times.

7.713.44 Medical and Health Staff

- A. A secure residential treatment center shall have a primary physician, licensed to practice medicine in Colorado, available to establish and maintain the health and medical policy and procedures of the facility as found at Section 7.713.31.
 - 1. If the physician is not a full-time employee, the description of the physician's consultative services or other duties to be performed shall be set forth clearly in a written, signed agreement with the facility.
 - 2. Any medical personnel, who is an employee, a volunteer, or whose services are purchased by the center, shall hold appropriate state and federal license, certification or registration and be responsible to the primary physician for the medical aspects of his/her job. A copy of the credentials shall be maintained at the center.

7.713.45 Youth Care Staff

- A. Each youth care staff member shall be at least 21 years of age and have completed two years of college education. A high school diploma or equivalent and one year's experience in the human services field may be substituted for the required college.
- B. Youth care staff aides shall be at least 18 years old, shall work under the direct supervision of a youth care staff member in activities specified by the youth care staff member, or with the approval of the director.
- C. Relief staff shall have the same qualifications as the regularly assigned youth care staff.

7.713.46 Youth Care Staff-to-Youth Ratios

- A. The center shall know the intended whereabouts of each youth in care at all times. Youth shall be supervised at all times (Section 7.713.25).
- B. There shall be a minimum of one (1) adult qualified as a youth care staff member on duty and one (1) adult on call who can be summoned at all times when there is one or more youth at the center.
- C. At night there shall be at least one awake youth care staff member within each physically separate building and within hearing of youth, and a second person who can be summoned in case of emergency.
- D. The ratio of youth care staff members to youth in care shall not be less than the following schedule except when transporting youth in vehicles (see Section 7.713.57):

Waking Hours

1 youth care staff member to 10 youth

Sleeping Hours

1 awake youth care staff member to 20 youth

7.713.47 Case Management Staff and Other Professional Staff

- A. Case management shall be provided by:
1. A qualified professional having a master's degree in social work, psychology, or related fields from an accredited college or university. This professional shall have two years of treatment-oriented experience; or,
 2. A designated member of the staff, who shall have a bachelor's degree from an accredited college or university with a major in behavioral science, human services or related fields, and three years experience in working with youth, and for whom there is an effective arrangement for consultation from a qualified professional as described above at Section 7.713.47, A, 1.
 3. The ratio of case management staff to youth shall be at least one full-time case management staff member for every twenty (20) youth, or a part-time staff member assigned for a fraction thereof.
- B. The case management staff shall be responsible for implementing the individual treatment plan as stated at Section 7.713.23.
- C. Psychiatrists used by the center shall be qualified and licensed to practice, and shall provide, as needed, diagnosis, treatment and consultation services.
- D. Psychologists who perform testing and diagnostic services shall have a master's degree in psychology, or shall have a bachelor's degree in a human services field and receive supervision from a psychologist who has a master's degree or a Ph.D. in psychology.
- E. Other professionals providing specific therapy shall be licensed professionals as designated by Colorado state law.

7.713.48 Volunteers and Students

- A. If volunteers or students are used by a center, the administration shall define specifically the services to be given by that individual.
- B. A volunteer shall perform professional services only when certified or licensed to do so.
- C. Volunteers and students who are assigned to work directly with the youth shall:
1. Be subject to reference and criminal record checks similar to those performed for applicants for employment.
 2. Be in good general health. The center has the right to contact the individual's physician.

- D. Volunteers and students shall be:
 - 1. Directly supervised by a paid staff member.
 - 2. Oriented and trained as required at Section 7.713.4, C.

7.713.49 Food Services and Maintenance Staff Members

- A. All staff members shall comply with the requirements for all personnel as specified in Sections 7.713.41 and 7.713.42.
- B. Food service staff of the center shall meet requirements of the state or local health requirements for food handlers.
- C. There shall be one food service staff member who has basic knowledge and understanding of nutrition, food purchasing, menu planning, and food preparation. If the staff member is not qualified as a dietitian or nutritionist, there shall be regular consultation from a specialist in the field.
- D. Maintenance staff shall be in sufficient numbers to maintain an adequate physical plant.

7.713.5 BUILDING AND FACILITIES

7.713.51 Building Site

- A. A secure residential treatment center shall be located in an area that is readily accessible to health resources, public and private utilities, adequate and safe water supplies, sewage disposal, fire and police protection.
- B. The site shall be approved by the local zoning department.
- C. If the secure residential treatment center is located in the same building as, or immediately adjacent to, other residential facilities, such as a residential child care facility or an adult treatment center, it shall be so arranged that the care and activities of the youth residing in the secure residential treatment center are completely separate and independent from the other residential facility. A secure residential treatment center may not be operated on the premises of a business of a nature which might be hazardous to the health, safety, morals, welfare of youth, and the operation of the secure residential treatment center. The center shall only care for youth of the age stated on the license. The center shall not be used for the care of persons over the age of 21 years old.

7.713.52 Building Approvals

- A. Each licensed center shall meet the requirements of the State Department of Public Health and Environment or its local unit, and the local fire department, and shall be inspected at least annually for compliance with current sanitation and life

safety code regulations. All health and life safety hazards shall be corrected as required by the appropriate regulatory agency.

- B. A center staff member, who is trained in and is familiar with the safety and sanitation requirements, shall conduct weekly inspections of the center and assure that any items of non-compliance with safety and sanitation regulations are corrected immediately. A record of each inspection shall be maintained for 12 months.
- C. Licensed centers shall comply with applicable state or local building code regulations.
- D. Prior to construction, architectural plans for new buildings, or for extensive remodeling of existing buildings, shall be submitted for review and approval by the State Department of Public Health and Environment or its local unit, the local fire department, and local building department. Plans shall be submitted and reviewed by the State Department of Human Services as to appropriateness, general adequacy, and suitability for youth care.

7.713.53 Living Spaces and Equipment Necessary in a Residential Treatment Care Facility

- A. There shall be separate sleeping rooms for boys and girls. In sleeping rooms that accommodate two or more youth, 60 square feet of floor space per youth shall be provided. There shall be no more than six youth in any bedroom. Each bedroom for single occupancy shall have a minimum of 70 square feet of floor space. Closet and/or drawer space for storage of personal items sufficient for the occupants in each sleeping room shall be provided.

Sleeping rooms above or below the floor of exit travel shall not be used for sleeping purposes for youth who are non-ambulatory.

- B. Each youth shall be provided suitable sleeping facilities consisting of individual beds or bunks complete with mattresses in good repair and constructed so as to facilitate cleaning while in use by residents and upon each change of occupancy. Triple-deck bunks are prohibited. Beds being used by youth shall have a mattress, clean linens, pillows, pillowcases, and blankets.
- C. Each room of occupancy shall have natural light, be well-lighted and adequately ventilated by exterior windows or by an approved air-conditioning system. If a mechanical ventilation system is provided, it shall meet requirements of local building codes and fire regulations and a backup system to assure that ventilation is available in the event of power failure.
- D. Each sleeping room has, at a minimum, the following facilities and conditions:
 - 1. Toilet facilities available for use 24 hours a day.

2. A hand washing sink with hot and cold running water.
 3. A desk, chair or stool.
 4. Temperatures which are appropriate to the summer and winter comfort zones.
- E. Staff rooms shall be located on the same floor or in the general area of youth's sleeping rooms so that the youth care worker can supervise youth and be readily accessible when needed.
- F. There shall be a minimum of 35 square feet of space for each youth for informal individual or group activities. The area shall be adequately and appropriately furnished to accommodate social and recreational activities associated with such living areas.
- G. There shall be a designated space distinct from youth's living areas to serve as an administrative office for such activity as secretarial work and bookkeeping.
- H. There shall be a designated space to allow private discussions and counseling sessions.
- I. Each center shall have a telephone. Each separate living unit within a center shall have 24-hour telephone service or an intercom system connected with an outside telephone service. Emergency telephone numbers, including fire, police, physician, poison control, health agency and ambulance, shall be conspicuously posted adjacent to the telephone.

7.713.54 Outdoor Space and Equipment

- A. All structures on the grounds of the center shall be maintained in good repair and free from any danger to health or safety.
- B. The grounds of the center shall be maintained in an acceptable manner and shall be free from any hazard to health or safety.
1. Garbage and rubbish which is stored outside shall be stored securely in noncombustible, covered containers and shall be removed at least once every week, or more frequently if necessary.
 2. Garbage and rubbish containers and incinerators shall be located separate from play areas.
 3. Fences shall be in good repair.
 4. Areas determined to be unsafe, including steep grades, cliffs, open pits, swimming pools, high-voltage boosters, or high-speed roads, shall be fenced off or have natural barriers to protect youth.

- 5. Playground equipment shall be so located, installed and maintained as to ensure the safety of youth.
- C. A secure residential center shall have access to outdoor recreational space and suitable recreational equipment.
- D. When a swimming pool is provided, it shall meet the requirements of the Colorado Department of Public Health and Environment. Safety precautions shall include protective fencing, winter coverage, which shall exclude plastic or inflatable-type domes, and a non-skid surface area of at least four (4) feet adjoining poolsides.

7.713.55 Dining, Kitchen, Laundry, Bathroom Facilities

- A. The dining area, whether located in the living unit or in a separate, central dining facility, shall meet the following requirements:
 - 1. It shall be clean, well-lighted, properly heated, and ventilated.
 - 2. Fifteen (15) square feet per person be provided to accommodate the youth and staff.
 - 3. Floors shall be constructed and maintained with a non-skid surface.
 - 4. Tables and chairs shall be of sturdy construction, scaled or adjusted to the proper height and size so that youth can be comfortably served.
 - 5. Table service and settings shall be of the type, size, and design appropriate to the security of the facility.
 - 6. All dishes, cups, and glasses used by youth in care shall be free from chips, cracks, or other defects.
 - 7. Walls and floors shall be of materials that are easy to keep clean.
 - 8. Dining and recreation areas may be combined if regulations for dining areas are consistently met.
- B. The kitchen shall be designed and equipped to meet the requirements of sanitation, fire safety, and comfortable working conditions for the staff. There shall be:
 - 1. Adequate space for receiving, storage, refrigeration, and preparation of food. Storage space shall be clean and well-ventilated; and containers of food shall be covered and stored above the floor on shelves or other clean surfaces.

2. Provision for daily disposal of garbage and other refuse. Food waste grinders shall be installed in compliance with applicable laws and regulations.
 3. Separate storage of poisonous and toxic materials from food. Such materials shall be labeled and used only in ways that will neither contaminate food nor be hazardous to employees.
 4. Mechanical dish washing equipment or other approved methods of dish washing in accordance with requirements of the State Department of Public Health and Environment.
 5. Provision for a CO2 or dry powder fire extinguisher(s) in kitchen. If a commercial-type range is used, a hood with a filter must be installed.
 6. Hand washing and toilet facilities for use of kitchen staff shall be readily accessible.
- C. When a center has a central laundry facility, it shall be located in an area separate from areas occupied by youth. Laundry facilities with ample space for sorting, drying, and ironing shall be made available to youth old enough and capable of handling their personal laundry. These facilities shall be in an area supervised by a responsible adult.
- D. Laundry trays or slop sinks shall be available and located conveniently for purposes of cleanliness and sanitation.
- E. There shall be a ratio of at least one toilet, lavatory, and bathtub or shower for every six (6) youth, readily accessible and kept in sanitary condition. Separate toilet and bath facilities shall be available for boys, girls, and staff. Toilets and bath facilities shall have doors and partitions. Urinals may be substituted for up to one-third of the required number of toilets for boys in facilities which accept boys. Toilets and bath facilities shall be accessible from a common hallway and be on the same floor with sleeping rooms.
- F. Bath and toilet rooms shall be constructed of easily cleanable, nonabsorbent materials. Floors shall have an impermeable, nonskid surface. Walls shall have a finished surface extending to a height of four (4) feet in toilet rooms and six (6) feet in shower rooms. All surfaces shall be maintained in good repair.
- G. Hot and cold water under pressure shall be supplied to all required plumbing fixtures except toilets. Water temperature control valves shall be inaccessible to youth, and water temperature shall be controlled by employees. Hot water in lavatories and bathing facilities shall not be above 110 degrees Fahrenheit.
- H. If drinking fountains are provided, they shall be approved, angle-jet type with adequate water pressure at all times.

7.713.56 Building Safety [Rev. eff. 6/1/12]

A. Maintenance

1. Buildings shall be kept in good repair and maintained in a safe, clean, and sanitary condition. Good housekeeping shall be observed in all areas at all times. A housekeeping plan shall be written and implemented.
2. All areas available for youth's activities shall be maintained in safe condition, including elimination of debris and hazardous items of all kinds, and removal of broken play and recreational equipment and any other devices which are in poor repair.
3. There shall be a written procedure, which shall be implemented, to provide for the control of vermin and pests.
4. Closets, attics, basements, cellars, furnace rooms, and exit routes shall be kept free from accumulation of extraneous materials such as discarded furniture, furnishings, newspapers, or magazines. Combustibles such as cleaning rags, mops, and cleaning compounds shall be stored in well-ventilated areas. Solutions, cleaning compounds, and other hazardous substances will be properly labeled and stored in areas inaccessible to youth.
5. Storage of gasoline, kerosene, fuel oil, and other flammable material shall meet requirements of safety and fire codes.
6. There shall be a written policy to govern the control and use of all flammable, toxic, and caustic materials. The policy shall be implemented.
7. Provision shall be made for collection, storage, and disposal of refuse in an approved manner to prevent nuisance conditions.
8. Storage shall not be permitted around or near water heaters and furnaces.
9. There shall be ample closets for cleaning supplies and equipment. Closets shall have good ventilation and be located in each principal area.

B. Exits

1. Every building or structure, new or old, shall be provided with exits sufficient to permit the prompt escape of occupants in case of fire or other emergency. Additional safeguards shall be provided for life safety in case any single safeguard is ineffective due to some human or mechanical failure.

2. There shall be at least two approved, alternate, remotely-located means of egress from each floor of the building to safe and open space at ground level.
3. Egress from each dwelling unit or sleeping room shall be directly available without passage through another dwelling or room unit to the outside or to a common hallway leading to the outside.
4. Exits from bedrooms and other interior rooms, and exits to the outside of the building shall have the capability to be locked to limit the freedom of the youth in residence. Because of this capability the local fire department has the right to require safeguards not commonly found in less restrictive settings.
5. Every exit shall be clearly visible, or the route to reach it shall be conspicuously indicated in such a manner that every occupant of every building or structure who is physically and mentally capable will readily know the direction of escape from any point. Each path of escape shall be so arranged or marked that the way to a place of safety outside is unmistakable.
6. Pathways or hallways which lead to an exit shall not be cluttered or hazardous thus resulting in the obstruction of access to the exit.
7. In every building or structure, adequate and reliable illumination shall be provided for all exit facilities. Every building or structure shall be so constructed, arranged, equipped, maintained, and operated as to avoid undue danger to the lives and safety of its occupants from fire, smoke, fumes, or resulting panic during the period of time reasonably necessary for escape from the building or structure in case of fire or other emergency.
8. Compliance with this rule shall not be construed as eliminating or reducing the necessity for other provisions for safety of persons using a structure under normal occupancy conditions, nor shall any provision of this rule be construed as requiring or permitting any condition that may be hazardous under normal occupancy conditions.
9. The local fire department shall determine the adequacy of exits and other measures for life safety in accordance with the requirements of the Uniform Building Code and the National Fire Protection Codes. In cases of practical difficulty or unnecessary hardship, the local fire department may grant exceptions from the Uniform Building Code or the National Fire Protection Codes, but only when it is clearly evident that reasonable safety is thereby secured.

C. Heating and Electrical Equipment

1. All heating units, gas or electric, shall be installed and maintained with safety devices to prevent fire, explosions, and other hazards. No open-flame gas or oil stoves, hot plates or unvented heaters shall be used for heating purposes.
2. Electrical wiring systems in all buildings shall conform to the requirements of the state electrical board. Electrical appliances shall be examined frequently for worn or otherwise defective wiring.
3. Heating devices such as radiators, registers, fireplaces, and steam and hot water pipes within the reach of youth shall be screened or otherwise protected as deemed necessary for the youth in residence at the center.

D. General Safety Practices

1. A secure residential treatment center shall immediately notify the responsible agency or department of fire or other disaster which might endanger residents or require their removal for reasons of health and safety. The Licensing Section of the Colorado Department of Human Services shall be notified of a fire or other disaster.
2. A secure residential treatment center shall not maintain any firearm on the grounds or within the structures of the facility.
 - a. A facility shall not permit any staff member or youth to be in possession of any firearm on the grounds or within the structures of the center. If chemical weapons are carried by a staff member for personal protection, the weapons shall be locked when the staff member is in the center.
 - b. Security guards patrolling the grounds and structures of a center, whether employed by the center or by a security service under contract to the center, shall not be permitted to be in possession of any firearm or chemical weapon on the grounds or within the structures of the facility except in emergency situations which are detailed in written policy.
3. Porches, elevated walkways and elevated play areas within the center shall have barriers to prevent falls.
4. Power-driven equipment used by the center shall be kept in safe and good repair. Such equipment shall be used by youth only under the direct supervision of a staff member and according to state law.
5. All stairways containing more than four steps shall be equipped with a handrail.

6. Staff and youth shall be trained to report fires and other emergencies appropriately and shall be trained in fire prevention.
7. Power generators shall be tested at least every two weeks, and other emergency equipment and systems tested at least quarterly for effectiveness and shall be repaired or replaced if necessary.
8. The center shall have access to an alternate power source to maintain essential services in an emergency.
9. There shall be a procedure which provides for a communications system in emergency situations within the center and between the center and the community.

E. Evacuation and Fire Drills

1. There shall be a written evacuation plan prepared in the event of fire or major emergency which is approved by the local fire protection authorities pursuant to national fire safety codes. The plan shall be reviewed annually, updated if necessary, and reissued to the local fire jurisdiction. The plan shall include but not limited to the following:
 - a. Location of building/room floor plans.
 - b. Use of exit signs and directional arrows for traffic flow.
 - c. Location of publicly posted plan.
 - d. At least quarterly drills or simulated drills on all shifts throughout the center.
 - e. Simulation of drills for evacuating extremely dangerous youth.
 - f. The means for the release of youth from locked areas and provision for a back-up system of security and control of youth.
2. All center personnel shall be trained in the implementation of the written emergency plans including fire, riot, hostage, medical, evacuation, and natural disasters. These plans are made available to all personnel. The plan and amendments shall be reviewed by staff at least annually.
3. In every building or structure, fire alarms shall be provided to warn occupants of the existence of fire so that they may escape, or to facilitate the orderly conduct of fire exit drills.
4. Responsibility for the planning and conduct of fire drills shall be assigned to competent persons qualified to exercise leadership.

5. Drills shall be held at unexpected times and under varying conditions to simulate the unusual conditions prevailing in case of fire or other disasters.
 6. In the conduct of drills, emphasis shall be placed upon orderly evacuation under proper discipline rather than upon speed. No running or horseplay shall be permitted.
 7. Drills shall include suitable procedures to make sure that all persons in the building, or all persons subject to the drill, actually participate.
 8. Fire alarms shall be regularly used in the conduct of fire exit drills.
 9. The center shall make special provisions for the evacuation of any physically handicapped youth.
 10. The center shall take special care to help emotionally disturbed or perceptually handicapped youth understand the nature of such drills.
- F. A secure residential treatment center shall maintain an active safety program, including investigation of all accidents and recommendations for prevention.

7.713.57 Transportation

- A. A secure residential treatment center shall ensure that each youth is provided with the transportation necessary for implementing the youth's treatment plan.
- B. A Secure Residential Treatment Center shall have means of transporting youth in cases of emergency.
- C. There shall be a written procedure to govern safety and security precautions pertaining to center and staff vehicles.
- D. Any vehicle used in transporting youth in care of a Secure Residential Treatment Center, whether such vehicle is operated by the center, a staff member or any other person acting on behalf of the center, shall be properly licensed, and shall be maintained in conformity with all applicable motor vehicle laws. The vehicle shall be enclosed, provided with door locks, and shall be equipped with a first aid kit and fire extinguisher.
- E. Any staff member of a secure residential treatment center or other person acting on behalf of the center operating a vehicle for the purpose of transporting youth shall be properly licensed to operate that class of vehicle in accordance with applicable laws of the Department of Revenue.
- F. A secure residential treatment center shall not allow the number of persons in any vehicle used to transport youth to exceed the capacity established by the manufacturer for the vehicle.

- G. In addition to the driver, there shall be sufficient number of supervisors traveling in any vehicle to meet the required staff-youth ratio as stated at Section 7.713.46 when transporting youth.
- H. A secure residential treatment center shall ascertain the nature of any need or problem of a youth which might cause difficulties during transportation, such as seizures, a tendency toward motion sickness or a disability. The center shall communicate such information to the operator of any vehicle transporting youth in care.

7.713.6 RECORDS

7.713.61 Confidentiality

- A. There shall be a written procedure to govern record management which includes but is not limited to the following: Establishment and utilization, content, privacy, security, preservation, and a schedule for retiring or destroying inactive records. These policies and procedures shall be reviewed annually.
 - 1. Records shall be the property of the secure residential treatment center; and the center, as custodian, shall secure records against loss, tampering, or unauthorized use. The center shall designate who is to supervise the maintenance of records and to whom records may be released.
 - 2. Employees of the center shall not disclose or knowingly permit the disclosure of any information concerning a youth or his/her family, directly or indirectly, to any unauthorized person except in case of medical emergency.
 - 3. Information concerning a youth in care shall not be released without the voluntary, written consent of the parent(s) or guardian except to the youth, his/her parents(s) or guardian, their respective legal counsel, the court having jurisdiction over a youth's case, or an authorized public official in the performance of his/her mandated duties.
 - 4. A secure residential treatment center may make available information in the case record to the youth, his parent(s) or guardian and their respective legal counsel if the information being released does not contain material which violates the right of privacy of another individual and/or material that must be withheld from release according to other laws or by order of the court. If, in the professional judgment of the administration of the center, it is believed that information contained in the record would be damaging to a youth, that information may be withheld except under court order. Educational records shall be governed by federal and state laws.
 - 5. It is acceptable to use material from case records for teaching or research purposes, development of the governing body's understanding and knowledge of the center's services or similar educational purposes, when

names are deleted and other identifying information is disguised or deleted.

6. It is necessary to obtain an employee's permission or court order for the release of information from a personnel file.
 7. The contents of records shall be identified and separated according to an established format.
 8. All entries in the master file are dated and identified.
 9. Personnel and records of youth shall be available, upon request, to authorized personnel of the Colorado Department of Human Services. All records regarding youth and all facts learned about youth and their relatives shall be kept confidential by the staff of the Colorado Department of Human Services pursuant to the state law.
- B. There shall be a procedure which upholds the principle of confidentiality of the health record and includes the following requirements:
1. The active health record is maintained separately from the confinement record.
 2. Access to the health record is controlled by the center's primary physician and the medical policy and procedure.
 3. The center's primary physician or his/her designee shares with the center administrator information regarding a youth's medical management, security, and ability to participate in programs.

7.713.62 Necessary Records and Their Retention

- A. The secure residential treatment center shall maintain complete records as required for licensing the center in accordance with the Minimum Rules and Regulations for a Secure Residential Treatment Center.
- B. A permanent register shall be maintained which contains:
1. Identifying information, such as name, sex, birth date, race, on each youth who has been in care at the center including a youth admitted in an emergency.
 2. Name and address of referring agency. Name of referring personnel.
 3. Date of admission.
 4. Discharge date and name and address of person or agency to whom the youth is discharged.

- C. Records for youth shall be retained for at least seven years. Retention of records for a longer period may be desirable when they reflect an accident, injury, or other unusual circumstance.
- D. Records for personnel shall be retained for at least three years.
- E. The following records shall be on file at the center or its administrative office:
 - 1. Documents regarding the governing body including the charter, articles of incorporation, by-laws or other legal basis for existence, and names and addresses of current board of directors and officers of primary workers of the governing body.
 - 2. Current health department inspection report.
 - 3. Current fire department inspection report.
 - 4. Weekly safety and sanitation inspection reports.
 - 5. A list of current staff members, substitutes, and staffing pattern
 - 6. Insurance coverage.
 - 7. Annual audit.

7.713.63 Case Record for Youth

- A. A record for each youth in care shall be maintained in a master file at the center where the youth resides which shall contain the following:
 - 1. Name, age, sex, place of birth.
 - 2. Initial intake information form and study;
 - 3. Documented legal authority to accept juvenile;
 - 4. Information on referral source;
 - 5. Social history;
 - 6. Medical consent forms;
 - 7. Name, relationship, address, and phone number of parent(s) and/or guardian(s) and person(s) juvenile resides with at the time of admission;
 - 8. Driver's license number, social security number, and Medicaid number, when applicable;
 - 9. Court and disposition;

10. Signed release of information forms, when required;
 11. Progress reports on program involvement;
 12. Program rules and disciplinary policy signed by juvenile;
 13. Grievance and disciplinary record, if applicable;
 14. Referrals to other agencies; and
 15. Written agreements signed by parents or legal guardians pertaining to permission to care for youth, financial responsibility, medical services, and other permits. This may include the placement agreement signed by the center and the placement agency or individual placing youth.
 16. The treatment plan, a summary of the periodic evaluations of the child's progress, and resultant changes in treatment plan.
 17. Educational records and reports;
 18. Incident reports;
 19. Summary recording of significant contacts with parents or guardians and other agencies involved.
 20. A summary of the discharge of the youth from the center which includes, but is not limited to the following:
 - a. The date of the discharge of the youth from the center,
 - b. To whom the youth was released,
 - c. The physical condition of the youth at time of discharge
 - d. A summary of services provided during residence at the center.
 - e. A summary of the youth's response to treatment, accomplishments during residence and psychosocial status at the time of discharge.
 - f. The assessed needs which remain to be met and alternate service possibilities which might meet those needs.
 - g. A statement of an aftercare plan and identification of agency responsible for follow-up services and after care.
 - h. Circumstances which led to an unplanned discharge.
- B. There shall be a health record maintained for each youth. The form and format of the health record, and the procedures for its maintenance and safekeeping shall

be approved by the center's primary physician. The health record shall include but not be limited to the following:

1. The completed receiving screen form.
2. Health appraisal data forms.
3. All findings, diagnoses, treatments, dispositions.
4. Prescribed medications and their administration.
5. Laboratory, x-ray, and diagnostic studies.
6. Signature and title of documenter.
7. Consent and refusal forms.
8. Release of information forms.
9. Place, date, and time of health encounters.
10. Health service reports, e.g., dental, mental health and consultations.
11. Treatment plan, including nursing care plan.
12. Progress reports.
13. Discharge summary of hospitalization and other termination summaries.

7.713.64 Personnel Records

A personnel record for each employee shall include: name, address, birth date, names and telephone numbers of persons to be notified in event of an emergency; date of employment, and date and reason for separation; documents verifying education, training, and work experience pre-employment references; physical examination at the time of employment and subsequent health examinations; and the indication of awareness of agency policies

7.713.65 Incident Reports and Logs [Rev. eff. 6/1/12]

- A. There shall be maintained a permanent log in which is reported a summary of situations involving individual or groups of youth for use by supervisory and treatment staff. Each shift of staff members shall prepare shift reports that record routine and emergency situations.
- B. All special incidents such as emergency situations, injuries, physical management or major rule violations shall be recorded and reported as required in these rules, Section 7.714.53, et seq., and reported to the director of the

facility or his/her designee. A copy of the record shall be maintained in the youth's case record.

7.713.66 Reports

- A. A residential center shall notify immediately the youth's parent(s), guardian, and/or the placing agency of any illness, injury, or severe psychiatric episode resulting in medical treatment, hospitalization, or death.
- B. Critical incidents shall be reported as outlined in Section 7.701.52.

7.714 QUALITY STANDARDS FOR TWENTY-FOUR (24)-HOUR CARE FOR CHILDREN AND YOUTH

All rules in Section 7.714 will be known and hereinafter referred to as the Quality Standards for Twenty-Four (24)-Hour Care for Children and Youth and will apply to all applicants and licensees subject to licensing as a specialized group facility, a child placement agency (CPA) or county that certifies specialized group facilities, or a residential child care facility (RCCF), including facilities operating under a RCCF license. Facilities operating under a RCCF license include: qualified residential treatment programs (QRTP), psychiatric residential treatment facilities (PRTF), shelter care service programs, and runaway and homeless youth shelter care facilities. Secure residential treatment centers (SRTC) must comply with rules for physical restraint found in Section 7.713.24.B and Sections 7.714.53 through 7.714.536. Day treatment centers must comply with applicable rules for physical restraint found in Sections 7.706.17 and 7.714.53 through 7.714.536.

7.714.1 Program Description

- A. The facility must have a written overall program description for the facility. The written description must include the following:
1. The population served, including, but not limited to, the following: ages of children/youth accepted, special populations served, and exclusionary criteria that conforms to the Americans with Disabilities Act of 1990 (ADA), 42 U.S.C. §§ 12101-12213 (2009), as incorporated by reference in Section 7.701.14;
 2. Treatment modalities and programming utilized by the facility;
 3. The title of the person who has overall responsibility for the development, implementation, and coordination of the treatment program;
 4. Staff responsibility for planning, changes, and implementation of the treatment procedures and therapeutic interventions;
 5. The range of procedures and interventions to be used and the anticipated range or types of behavior or conditions for which such procedures and interventions are to be used, including philosophy of treatment, modes of therapy, trauma-informed practices, positive behavior intervention, skill-building strategies, physical restraint, and seclusion where allowed and approved by the department; and
 6. The facility's responsibility for monitoring the safety of children/youth during routine care and treatment, including when the program may utilize emergency interventions.

7.714.2 GENERAL REQUIREMENTS FOR ADMISSION

- A. Admission of a child/youth shall be in keeping with the stated purpose of the facility. Admission must be limited to those children/youth for whom the facility is qualified and licensed to care for. Care must be provided in a manner consistent with the child/youth rights outlined in Sections 7.714.31 through 7.714.32 and in the least-restrictive, most appropriate setting in order to meet the individualized child's/youth's needs. Additionally, for children/youth in foster care or participating in the foster youth in transition program, care must be provided in a manner consistent with the child/youth rights outlined in Section 19-7-101(2), C.R.S. and Sections 7.714.33 through 7.714.34 and in the least-restrictive, most appropriate setting in order to meet the individualized child's/youth's needs. A facility that accepts youth in foster care or participating in a foster care transition program who are eighteen (18) years of age and older but under twenty-one (21) years of age must have a written policy to address how the facility will ensure the youth's adult rights and duties will be accommodated, including, but not limited to, registering to vote and registering for selective service, in addition to their rights as identified in Section 7.714.33.

7.714.21 Admission Procedures

Each facility must develop and adhere to written admission procedures. such procedures must be provided to the referring agencies and parent(s) and/or guardian(s) of the child/youth referred for placement. Procedures must include:

- A. The written description of the program, as stated in Section 7.714.A.
- B. The facility may only accept a child/youth into care after receiving all necessary information from the referral source, to include:
1. The name, date of birth, sex assigned at birth, gender identity, and physical description of the child/youth;
 2. Reason for referral and identified risk and safety triggers;
 3. Any known medical needs, including any allergies, injuries, prescribed medications, or necessary treatments;
 4. Current presenting problems and areas of concern;
 5. Anticipated length of stay;
 6. Current functioning;
 7. Required accommodations, including sleeping/roommate accommodations;

8. The name and contact information of the placing authority, which must include an emergency contact phone number for access to the placing authority twenty-four (24) hours a day. Verification of the authority of the individual placing the child/youth at the facility must be provided and such information documented in the child's/youth's admission record; and
 9. A placement agreement, including provisions for discharge pending necessary information on child/youth.
- C. The facility must observe and document the physical condition of the child/youth at the time of admission, as outlined in the facility's procedures, to include marks or tattoos, signs of abuse or neglect, or signs of illness. This process must not violate the child's/youth's right to privacy. Any signs of potential abuse or neglect must be reported to the child abuse and neglect hotline immediately. If there is reasonable cause to believe or suspect that the child/youth has physical signs of abuse or neglect in bodily areas not readily visible, then the facility must have the child/youth immediately seen by the facility nurse or outside medical staff.
- D. For any child/youth admitted to the program, the facility must request and make every diligent effort, no less than three (3) documented requests within the first month of admission, to obtain, utilize, and keep in the child's/youth's file current, relevant information on the child/youth, to include:
1. Expected goals to be achieved by the child/youth while in care;
 2. Required level of supervision for the child/youth, including during times of transportation and when off facility grounds;
 3. Social, emotional, and developmental history;
 4. History of trauma, abuse/neglect;
 5. A mental health assessment and source of the assessment information, if obtained from an outside source such as a prior placement or mental health provider;
 6. current family structure, relationship dynamics, and family functioning;
 7. Behavioral health history or observable behaviors and/or actions that impact overall physical and psychological health, including, but not limited to, lifestyle choices like diet or exercise;
 8. Physical conditions and health, to include allergies, chronic conditions, and current medications;
 9. Mental and psychological health, to include emotional regulation, coping strategies, and current psychotropic medications;

10. Immediate supports available, to include cultural, relational, religious, spiritual, or emotional;
 11. Substance use;
 12. Previously successful and unsuccessful interventions;
 13. Skills, assets, and strengths;
 14. Previous placements;
 15. The family services plan, if applicable and available; and
 16. Educational records for the current and past year in accordance with Section 7.701.63.B.8. Such records must be evaluated by the child's/youth's educational team or administrative unit to determine the child's/youth's educational needs for purposes of enrollment in school and ensuring a child/youth has access to required services listed in school plans, such as an individualized education plan (IEP) or 504 plan.
- E. Authorizations must be obtained from the parent(s), legal guardian(s), and/or custodian(s) within one (1) week of admission, unless otherwise noted below, authorizations must include the following:
1. Consent for routine and emergency medical, dental, and vision care, obtained prior to or on day of admission in accordance with Section 13-22-103(1), C.R.S. youth eighteen (18) years or older may give consent for medical care, dental care, and related care;
 2. Consent for physical restraint and/or seclusion, obtained prior to utilizing and when applicable, as described in Section 7.714.531;
 3. Consent to treatment for mental health services in accordance with Sections 27-65-103, C.R.S and 27-65-104, C.R.S. Children fifteen (15) years or older may give consent to receive mental health services;
 4. Consent to release confidential information as required by law or court order;
 5. Consent to treat for other behavioral health services and all treatment modalities the facility may utilize in the child's/youth's course of treatment;
 6. Specific to QRTPs, consent for the BHA to release the state-approved assessment in accordance with Section 7.705.208; and
 7. Consent for search practices conducted by the facility;

- F. For an emergency admission of a child/youth to the facility, facilities may request additional information as needed, but at least the following must be obtained prior to or on the day of admission:
1. The name, date of birth, sex assigned at birth, gender identity, and physical description of the child/youth;
 2. Any known medical needs, including any allergies, injuries, prescribed medications, or necessary treatments;
 3. The name and contact information of the placing authority, which must include an emergency contact phone number for access to the placing authority twenty-four (24) hours a day. Verification of the authority of the individual placing the child/youth at the facility must be provided and such information documented in the child's/youth's admission record;
 4. Written authorization allowing routine and emergency medical, dental, vision, and mental health care to be provided to the child/youth; and
 5. A placement agreement, including provisions for discharge pending necessary information on child/youth.
- G. The facility must request and make every diligent effort, no less than three (3) documented requests within the first month of admission, to obtain, utilize, and document the following for each child/youth admitted to the program:
1. All persons allowed to have contact with the child/youth, including the manner and frequency of contact.
 2. All documentation from referral sources of persons not allowed to have contact with the child/youth, including court orders, protection orders, and reasons for such restrictions if applicable and made available.
 3. The name(s) and title(s) of the individual(s) to whom the child/youth may be released at the time of discharge from the facility, if made available; and
 4. The legal status, including probation or criminal adjudications, and custody of the child/youth.
- H. When a child/youth is placed by a sending agency out-of-state, the interstate compact on the placement of children (ICPC) procedures must be followed as detailed in Part 18 of Article 60 of Title 24, C.R.S., including:
1. In accordance with Section 24-60-1802, C.R.S., the sending agency includes a party state, officer, or employee thereof; a subdivision of a party state, or officer, or employee thereof; a court of a party state; a person, corporation, association, charitable agency, or other entity which sends,

brings, or causes to be sent or brought any child/youth to another party state ultimately responsible for the planning, financing, and placement of the child/youth.

2. Requests must be sent by the sending state directly to the Colorado Deputy Compact Administrator on federal form ICPC-100A (2021) prior to the placement of the child/youth in the Colorado facility herein incorporated by reference. No later editions or amendments are incorporated. The federal form ICPC-100A can be found at <https://drive.google.com/drive/folders/1ZIRMFGQCETXSWY1KGXWBOS6ROK61J-PM>. Copies are also available for public inspection and copying at the Colorado Department of Human Services, 1575 Sherman Street, Denver, CO 80203, during regular business hours; and
3. No child/youth residing out of state may be admitted to a Colorado facility until an ICPC approval is granted by the Colorado Compact or Deputy Compact Administrator.

7.714.22 Child/Youth Orientation

- A. Within twenty-four (24) hours of arrival at the facility, the child/youth must be given an orientation to the facility. If the orientation cannot happen within the twenty-four (24) hour timeframe for child/youth-specific reasons or due to the need to acquire language or interpretation services, this must be documented in the child's/youth's file. The orientation must be conducted in the child's/youth's preferred language. The orientation must be consistent with the child's/youth's age, reported developmental and/or cognitive level, and ability to participate. If the child/youth is not able to participate in the orientation, the individual placing the child/youth in the facility must be given the orientation on behalf of the child/youth. The child/youth who received the orientation or their placing authority must sign and date the orientation form upon completion of the orientation. The orientation must include:
 1. A tour of the facility to include where the child/youth will sleep, instructions on fire alarm and fire evacuation procedures, escape routes, and exits;
 2. An overview of the rules/regulations and policies of the facility;
 3. If applicable, a discussion as to when physical restraint may be used and the necessary permissions and an explanation of the debriefing process, as required by Sections 7.714.53 through 7.714.536;
 4. For children/youth residing in foster care or participating in the foster youth in transition program, the location of the posted written notice of child/youth rights required by Section 19-7-101(5), C.R.S. The written notice must be posted in a place where children/youth have access.

For other children/youth, an explanation of child/youth rights, as identified in Section 7.714.31.B, and where the rights are posted in the facility;

5. For children/youth residing in foster care or participating in the foster youth in transition program, a notice must be provided containing contact information for the child's/youth's appointed attorney; the Office of the Child Protection Ombudsman; the appropriate county department of human/social services; and the Department of Human Services.
 - a. The notice must include an explanation that such children/youth may speak with the parties listed in Section 7.714.22.A.5 if they have any questions, concerns, or grievances related to the child/youth rights.
 6. For children/youth who are not residing in foster care or not participating in the foster youth in transition program: an explanation of the facility's grievance policies and procedures, the name and contact information for the designated client representative, and where the grievance forms and grievance box are located. The child/youth must be advised that they can request staff assistance with submission of a grievance.
 - a. Such children/youth must also be provided notice that they may contact the Office of the Child Protection Ombudsman, the appropriate county department of human/social services, and the Department about any questions, concerns, or grievances related to child/youth rights;
 7. An introduction to staff and peers; and
 8. An overview of the anticipated daily schedule for the child/youth.
- B. A form listing the orientation requirements must be signed by the staff member and the child/youth, or the individual placing the child/youth, verifying the date the orientation occurred. A copy of such form must be provided to the child/youth and the placing authority of the child/youth, and a copy must be maintained in the child's/youth's file.
- C. A copy of the rules/regulations and policies of the facility, a copy of the written notice form of child/youth rights, an explanation of the facility's grievance policies and procedures, a name and number of a staff member to be contacted with questions and/or concerns, the contact information for filing a complaint with the Department, the Child Protection Ombudsman, and if applicable, information about when physical restraint may be used, must be sent to the child's/youth's parent(s) and/or guardian(s) and custodian(s) within one (1) week of admission and documentation that this information was provided to the parent(s) and/or legal guardian(s) and custodian(s) must be maintained in the child's/youth's file.

7.714.3 RELIGION, RIGHTS, AND GRIEVANCE PROCEDURES

7.714.31 Children's/Youth's Rights

- A. The facility shall have written policies and procedures that address and ensure the availability of each of the following rights for children/youth in residence. A list of children's/youth's rights and the grievance procedure described in Section 7.714.31 and 7.714.32 may be posted in all facilities in areas frequented by children/youth, parent(s), and legal guardian(s) and custodian(s).
- B. The following rights apply to all children/youth who are placed in facilities. These rights may not be limited or denied by the facility unless limited or denied in accordance with Section 7.714.31.C through 7.713.31.E. These rights include:
1. Every child/youth has the right to enjoy freedom of thought, conscience, cultural and ethnic practice, and religion;
 2. Every child/youth has the right to a reasonable degree of privacy;
 3. Every child/youth has the right to have their opinions heard and considered, to the greatest extent possible, when any decisions are being made affecting their life;
 4. Every child/youth has the right to receive appropriate and reasonable adult guidance, support, and supervision;
 5. Every child/youth has the right to be free from physical abuse or neglect and inhumane treatment. Every child/youth has the right to be protected from all forms of exploitation;
 6. Every child/youth has the right to receive adequate and appropriate medical, mental health, and behavioral health care in the least restrictive setting possible, suited to meet their individual needs;
 7. Every child/youth has the right to receive adequate and appropriate food, clothing, and housing;
 8. Every child/youth has the right to live in clean, safe surroundings;
 9. Every child/youth has the right to participate in an educational program that will maximize their potential in accordance with existing law;
 10. Every child/youth has the right to communicate with a parent or legal guardian or custodian, caseworker, attorney, guardian ad litem, counsel for youth, therapist, physician, religious advisor, probation officer, other significant members of the child's/youth's team outside of the facility, and/or other significant approved connections;
 11. No child/youth may be fingerprinted for the purpose of a criminal background check unless required by law enforcement;

12. A child/youth may be photographed upon admission for identification and administrative purposes of the facility. Such photographs must be confidential and must not be released by the facility except pursuant to court order. No other non-medical photographs or videotaping may be taken except in the case of a child abuse or police investigation or for use by a medical professional;
13. Every child/youth has the right to the same consideration for care and treatment as anyone regardless of race, color, national origin, religion, age, sex, gender identity, gender expression, political affiliation, sexual orientation, financial status, or disability;
14. Every child/youth has the right to be given the names and professional status of the staff members responsible for their care;
15. Every child/youth has the right to receive assistance from the child's/youth's client representative in filing a grievance or making complaints and to receive copies of the grievance or complaint procedure;
16. Every child/youth fifteen (15) years of age and older has the right to request their own medical records, to see the records at reasonable times, and to be given written reasons if the request is denied;
17. Notwithstanding any other provision of law, a child who is fifteen (15) years of age or older, whether with or without the consent of a parent or legal guardian or custodian, may consent to receive mental health services rendered by a facility, a professional person, or mental health professional licensed pursuant to Part 3, 4, 5, 6, or 8 of Article 245 of Title 12 in any practice setting. A licensed mental health professional may provide psychotherapy services, as defined in Section 12-245-202(14)(A), C.R.S., to a child who is twelve (12) years of age or older, without the consent of a parent or legal guardian or custodian, if the mental health professional determines that the minor is knowingly and voluntarily seeking such services and the provision of psychotherapy services is clinically indicated and necessary to the child's/youth's well being;
18. Every child/youth has the right to receive culturally responsive care, treatment, and services;
19. Every child/youth has the right to receive trauma-responsive care, treatment, and services; and
20. Every child/youth has the right to have contact and correspondence with their siblings, unless restricted by a court order.
21. Every child has the right to consent to substance use treatment in accordance with Section 13-22-102, C.R.S. All youth over the age of

eighteen (18) years old have the right to consent to substance use treatment pursuant to Section 13-22-101, C.R.S.

- C. The following children's/youth's rights may be limited to reasonable periods during the day or restricted according to written policies of the facility to ensure the protection of the children/youth, staff, and program from unreasonable and unnecessary intrusions and disruptions and from health and safety hazards. All facilities are responsible to adhere to the rights of the children/youth and must also take reasonable steps to ensure safety of all involved, as identified in written policies of the facility.
1. Every child/youth has the right to have access to letter-writing materials, including postage, and to have staff members of the facility assist them if unable to write, prepare, and mail correspondence. Every child/youth has the right to receive and send sealed correspondence, including email if made available. No incoming or outgoing correspondence shall be opened, delayed, held, or censored by the personnel of the facility.
 2. Every child/youth has the right to have access to telephones to both make and receive calls in privacy.
 3. Every child/youth has the right to have convenient opportunities to meet with visitors.
 4. Every child/youth has the right to wear their own clothes, including shoes, keep and use their own personal possessions, and keep and be allowed to spend a reasonable sum of their own money.
- D. If the facility enforces any restrictions upon the child's/youth's rights as listed at Section 7.714.31.C, the facility must, in compliance with the written policy and procedure of the facility:
1. Inform the child/youth, the child's/youth's parent(s), custodian(s), or legal guardian(s), in a language or mode of communication the child/youth, parent(s), legal guardian(s), and their custodian(s) can understand, of the conditions of and reasons for limitations or denials of their rights and document the notification in the child's/youth's file.
 2. Place a written report summarizing the conditions of and reasons for the limitations or denials of the child's/youth's rights in that child's/youth's file. Information pertaining to limitations or denials contained in the child's/youth's file must be made available, upon request, to the child/youth or the child's/youth's guardian ad litem (GAL) or counsel for youth (CFY).
 3. When a limitation or denial of a child's/youth's rights affects another individual, the individual must be informed, in a language or mode of communication the individual can understand, of the conditions of and

reasons for the action. All limitations or denials must be documented in the child/youth file and made available to the department upon request.

4. Ensure an administrator, case manager or licensed mental health professional conducts a documented review of the restriction at least every seven (7) days. Written documentation of the reason for continuing the restriction must be maintained in the child's/youth's case file. If a review can not be obtained within the stated time frame, then the restriction must be discontinued.

7.714.32 Children's/Youth's Grievance Procedure

- A. The facility must designate a client representative and establish a written grievance procedure that provides adequate due process safeguards, spells out the appeal process, and assures that children/youth, their parent(s), custodian(s), legal guardian(s) are entitled to report any grievance and shall not be subject to any adverse action as a result of filing a grievance. If requested, a child/youth shall have a support person to help them write and file a grievance.
 1. The facility must follow grievance procedures without alteration or interference and must respond in writing to any grievance filed within seventy-two (72) hours. The facility's documented written response must include whether the grievance was approved or denied and the specific reasons for the decision. The response must also be reviewed and communicated with the child/youth in their preferred language or mode of communication.
 2. This grievance procedure shall be made available to all children/youth.
 3. If a grievance is filed with the facility, the grievance shall be recorded in the child's/youth's file along with the investigation findings and resulting action taken by the facility. Information regarding the grievance must be sent to the individual or agency holding legal custody of the child/youth within seven (7) business days. A copy of the child's/youth's grievance must be sent to the parent or legal guardian(s) or custodian(s) and the placing authority. Upon request, a copy must be provided to the child's/youth's guardian ad litem or counsel for youth.
 4. A list of the children's/youth's rights may be prominently posted in all facilities in areas frequented by children/youth and legal guardians or custodian(s). The child/youth must also receive the grievance procedure; the name, address, and telephone number of their designated client representative; as well as a list of agencies where complaints may be filed. Grievance forms must be available to children/youth without requiring assistance from a staff member to access and submit the form.

- B. A list of the children's rights and the grievance procedures must be provided and explained to the child and the parent or guardian in a language or manner of communication that they can understand.
- C. The child/youth has the right to file a complaint with the Child Protection Ombudsman's (CPO's) office, the Department, the placing authority, and, if applicable, the appointed legal counsel.

7.714.33 Children's/Youth's Rights Pursuant to Section 19-7-101, C.R.S.

- A. In addition to the rights described in Section 7.714.31, children/youth in foster care or participating in the foster youth in transition program must receive the following rights described in Section 19-7-101,(2), C.R.S.:
 - 1. Fair and equal access, including:
 - a. Freedom from discrimination or harassment on the basis of actual or perceived race or ancestry, ethnic group, national origin, religion, sex, sexual orientation, gender identity, gender expression, disability, marital status, familial status, source of income, military status, or hiv status;
 - b. Freedom of thought, conscience, cultural and ethnic practice, and religion, including the right to attend or refuse to attend cultural, ethnic, and religious services and activities;
 - c. Expression of the child's or youth's gender identity and to be referred to by the child's or youth's preferred name or gender pronouns;
 - d. Freedom from threats, punishment, or retaliation for asking questions, stating concerns, or making complaints about a violation of the rights and protections established in Article 7 of Title 19, C.R.S.;
 - e. Access to services, placement, and state and federal programs and services for which the child or youth is eligible;
 - f. Developmentally appropriate notification of any benefits for which the child or youth is eligible or receiving, including written notification of who is serving as a representative payee, when applicable, and;
 - g. If there is a representative payee who accepts payments for a child or youth, an accounting of the use of those payments when requested by the child or youth and on an annual basis.
 - 2. Appropriate placement and care, including:

- a. Appropriate adult guidance, support, and supervision in a safe, healthy, and comfortable environment in which the child or youth is treated with fairness, respect, and dignity;
- b. Consideration of the child's or youth's preference regarding the child's or youth's placement;
- c. Freedom from physical, sexual, emotional, or other abuse; corporal punishment; neglect; or any other form of inhumane treatment, such as exploitation or trafficking;
- d. Placement in the least restrictive setting that is appropriate to the child's or youth's needs, and receipt of the services and supports necessary to maintain that placement;
- e. Freedom from abandonment or being locked in a room, building, or premises, or being subjected to isolation or other physical or medical restraint, unless otherwise authorized by statute;
- f. Placement with a foster care provider who is aware of and understands the child's or youth's unique history as it relates to the child's or youth's care and who possesses the appropriate knowledge and skills to provide for the child's or youth's needs;
- g. Application of the reasonable and prudent parent standard as required by federal law;
- h. Return to the custody of a parent or legal guardian, consistent with applicable legal standards and considering the preferences of the child or youth;
- i. Timely court proceedings and determinations about the child's or youth's placement;
- j. Effective case management and planning that prioritizes the safe return of the child or youth to the child's or youth's parents, legal guardians, relatives, or kin, or moving the child or youth to other forms of permanent placement, if necessary;
- k. A requirement that the representative payee for the child or youth, including the department of human services if it is acting as the representative payee, use any benefits the child or youth receives to meet the child's or youth's individual needs after the representative payee meets with the child or youth to ascertain the child's or youth's current and foreseeable needs; and
- l. Timely notification to the social security administration to initiate the transfer of benefits from a representative payee when a child or

youth who is receiving benefits leaves the custody of the department of human services.

3. Access and communication, in a setting that provides privacy for in-person or other methods of contact, with professionals and other supports, including:
 - a. Persons working on the child's or youth's behalf, including, but not limited to, caseworkers, the child's or youth's guardian ad litem or counsel for youth and their legal teams, mental health professionals, tribe, foster youth advocates and supporters, court-appointed special advocates, education rights holders, and probation officers. The child's or youth's contact with the child's or youth's guardian ad litem, counsel for youth, other attorneys, and members of the child's or youth's legal team must protect the applicable legal privileges and confidentiality;
 - b. The child's or youth's guardian ad litem or counsel for youth, the Child Protection Ombudsman, county departments of human/social services, or the Department of Human Services regarding any questions, concerns, or violations of the rights and protections established in Article 7 of Title 19, C.R.S.; and
 - c. Connections to the child's or youth's community and supports, including communication and contact with people outside the foster care system who are important to the child or youth, unless otherwise limited by law or court order.
4. Confidentiality and privacy, including:
 - a. Confidentiality of all juvenile court records, consistent with existing law;
 - b. Freedom from unreasonable searches, limitation of use, or taking of personal belongings or other unreasonable invasions of privacy; and
 - c. Reasonable expectations of privacy to make and receive telephone calls and send and receive text messages, e-mail, and postal mail.
5. Education, including:
 - A. Receipt of a free and appropriate education, access to transportation to educational institutions, and an opportunity to participate in sports and extracurricular, cultural, personal enrichment, and social activities consistent with the child's or youth's age and developmental level, including access to computer

- technology and the internet as necessary for the child's or youth's education; and
 - B. School stability that presumes the child or youth will remain in the school of origin, as defined in Section 22-32-138, C.R.S., in which the child or youth is enrolled at the time of placement, unless remaining in that school is not in the child's or youth's best interests.
6. Basic essentials, including:
- a. Adequate and gender and culturally affirming food, clothing, hygiene products, and necessary hygiene services;
 - b. Personal allowance or employment opportunities appropriate to the child's or youth's age and developmental level, including appropriate opportunities to experience the use and value of money by making purchases for items according to the child's or youth's choice; and
 - c. Personal belongings, including wearing the child's or youth's own clothing, having a safe place to store personal belongings, and having personal belongings transported in appropriate luggage with the child or youth during or shortly after a change of placement. Personal belongings stored and transported after a change of placement must be maintained in the same or better condition.
7. Health care, including:
- a. Adequate and appropriate medical, dental, vision, mental health, and substance use disorder services;
 - b. Reproductive and sexual health care consistent with Colorado state law;
 - c. Freedom from the administration of prescription medication or other chemical substances, unless authorized by a physician or, when necessary, court order, after an individualized assessment of the child or youth, including a consultation with the child or youth, and with the child's or youth's consent, consistent with Colorado state law;
 - d. Access to prescribed medications during a placement change or similar transition so the child or youth does not experience a disruption in the availability of necessary prescription medication; and

- e. Notification of the medications that were prescribed to the child or youth and the purpose for each medication for the child or youth.
- 8. Participation in legal and case planning, including:
 - a. Appointment of a guardian ad litem or counsel for youth pursuant to Sections 19-3-203 and 19-7-308, C.R.S.
 - b. Attending and fully participating in all hearings related to the child's or youth's case, including access to appropriate transportation for the child's or youth's court proceedings, and the opportunity to be heard separately when deemed necessary by the court pursuant to Section 19-1-106(5), C.R.S.;
 - c. At the child's or youth's request, allowing certain people to be present for the child's or youth's court proceedings that take place pursuant to Section 19-1-106, C.R.S.;
 - d. Consulting with the court regarding the child's or youth's permanency goals pursuant to Section 19-3-702(1)(a), C.R.S.;
 - e. As appropriate to a child's or youth's age and developmental level, being informed of and participating in the development of the child's or youth's case plan, consistent with state and federal law; and
 - f. As appropriate to a child's or youth's age and developmental level, participation in and accompaniment of support to meetings about the child's or youth's court proceedings.
- 9. The necessities to be self-sufficient during the transition to adulthood, including:
 - a. Consistent with the child's or youth's developmental level, assistance with establishing a bank account, including identifying institutions where a child or youth may open a bank account, obtaining documents necessary to open an account, learning how to spend a reasonable amount of money, and managing personal income. This does not create an obligation to fund the child's or youth's bank account if one is created;
 - b. Having access to information regarding the work and educational options available to the child or youth, including, but not limited to, information regarding available financial aid and postsecondary education support consistent with state law;
 - c. Being allowed to work and develop job skills, consistent with state law and the child's or youth's age and developmental level;

- d. Obtaining or receiving a free annual credit report from the Department of Human Services or a county department of human/social services and the additional credit-related and identity theft protections provided in Section 19-7-102, C.R.S. if the child or youth is in the legal custody of a county department of human/social services and is fourteen (14) years of age or older;
 - e. When the youth is eighteen (18) years of age or older and leaving foster care, being provided with the youth's birth certificate, social security card, immigration documents, health insurance information, medical records, education records, either a driver's license or a state-issued identification card, written information concerning the youth's family history and contact information for siblings, if appropriate, and proof of foster care pursuant to Section 19-3-702(4)(d), C.R.S.;
 - f. Notifying a youth who is turning sixteen (16) years of age of the youth's eligibility for the foster youth in transition program;
 - g. Assisting a youth in applying for benefits the youth is eligible for or currently receiving, to ensure that benefits continue once the youth turns eighteen (18) years of age or transitions out of foster care, including redetermination for the purposes of social security benefits; and
 - h. Driving instruction for youth fifteen (15) years of age and older pursuant to Section 26-5-115, C.R.S.
- B. A juvenile court shall not limit or deny the rights enumerated in Section 19-7-101(2), C.R.S. unless the court finds by clear and convincing evidence that there are extraordinary circumstances and the limitation or denial is necessary for the safety of the child/youth in foster care.
 - 1. This is applicable to only the rights enumerated in Section 19-7-101(2), C.R.S. and does not alter the hearing requirements or standards otherwise established in Title 19, C.R.S.

7.714.34 Children's/Youth's Grievance Procedure Pursuant to Section 19-7-101, C.R.S.

- A. According to 19-7-101(5), C.R.S., the county department of human/social services shall provide each child or youth who is five (5) years of age or older with the written notice developed pursuant to Subsection 3 of Title 19 Part 7 at the time of the child's or youth's initial placement in foster care, at each placement change, and at least annually. The notice must be in the child's or youth's primary language.

- B. The notice form required by 19-7-101(3), C.R.S., must be provided to children/youth residing in foster care or participating in the foster youth in transition program.

7.714.35 Religious and Spiritual Practices

The facility shall demonstrate consideration for, and sensitivity to, the religious and/or spiritual backgrounds of children/youth in care. The facility shall assist a child's/youth's involvement in religious and/or spiritual activities appropriate to the child's/youth's religious and/or spiritual background, based upon the needs and interests of the child/youth.

- A. A child/youth in care at the facility shall be allowed and encouraged to celebrate their religious and/or spiritual holidays.
- B. Opportunity and assistance shall be provided for each child/youth to practice their chosen/preferred religious and/or spiritual beliefs. This includes, but is not limited to, making necessary arrangements for attendance of children/youth at the appropriate religious institution, spiritual ceremony, or at a study group for religious instruction.
- C. A child/youth may be invited to participate in the religious or spiritual activities of the facility/agency. Participation in any religious and/or spiritual activities of a facility are optional and not required.
- D. A child/youth shall not be coerced or forced to participate in the religious and/or spiritual activities of the facility or to attend religious and/or spiritual services.
- E. Prior to placement of the child/youth at the facility, the child/youth, their parent(s), legal guardian(s), custodian(s), and placing agency shall be notified of the practices, philosophy, and religious or spiritual affiliation of the facility. Documentation of such notification must be maintained in the child's/youth's file.
- F. Religious or spiritual intervention shall not be used by the facility in any form to control or change a child's/youth's behavior, or treat or heal a medical condition.
- G. A facility cannot deny medical, dental, behavioral, or mental health care to a child/youth because of the religious and/or spiritual affiliations of the facility.

7.714.4 Comprehensive Assessment and Individual Child's/Youth's Plan

The following section applies to the following license types: certifying child placement agencies or counties for specialized group facilities, residential child care facilities (RCCFs), psychiatric residential treatment facilities (PRTFs), and qualified residential treatment programs (QRTPs). QRTPs must also complete an assessment as required in Sections 7.705.208, 7.705.210, 7.705.211, and 7.705.212.

- A. Comprehensive Assessment

1. Within seven (7) calendar days of admitting a child/youth into a facility, the facility or provider shall begin a comprehensive assessment/evaluation of the child/youth.
2. The comprehensive assessment shall be conducted by a multidisciplinary team. This team shall include persons responsible for implementing the plan on a daily basis, which includes persons who supervise the child/youth, persons who document the outcomes of the assessment or evaluation and provide treatment interventions, and persons who have had direct interaction with and observation of the child/youth. For those facilities that provide clinical services, the multidisciplinary team shall include licensed or provisionally licensed clinical professionals and/or medical professionals where required and as outlined in Sections 7.705.204 through 7.705.206.
3. The comprehensive assessment should assess for the need of a specialist to include, but not limited to, the following: psychiatric services, speech therapy services, and/or physical therapy services.
4. The comprehensive assessment must be completed by the fourteenth (14th) calendar day after admission.
5. The multidisciplinary team shall gather historical and current information to assess and evaluate the child/youth in at least the following areas:
 - a. Name, birthdate, and demographic data;
 - b. Reason for referral;
 - c. Immediate safety concerns;
 - d. Legal guardianship and custody;
 - e. Cultural factors that may impact placement or treatment, including age, ethnicity, linguistic/communication needs, gender identity, sexual orientation, relational roles, spiritual beliefs, socio-economic status, personal values, level of acculturation and/or assimilation, and coping skills;
 - f. Medical, health, and dental care, including allergies and a health history of the child/youth and family, and if applicable, health information regarding speech therapy, occupational therapy, and physical therapy needs of the child/youth;
 - g. Mental health and treatment history, including all current and past medications, historical and current information on diagnoses, where applicable; and previous placements in treatment facilities;

- h. Trauma history;
 - i. Education/vocation;
 - j. Personal/social development;
 - k. Family structure and community relationships;
 - l. Vocational training, if applicable;
 - m. Recreation;
 - n. Capacity for self-care and activities of daily living, including life skills development;
 - o. Independent living skills, if applicable;
 - p. Legal issues and history to include all adjudications and criminal charges and convictions;
 - q. Out-of-home placement history;
 - r. Substance use history;
 - s. Strengths, abilities, skills, and interests; and
 - t. The needs of the child/youth that must be resolved for the child/youth to move to a less restrictive setting and/or successfully discharge from the facility.
 - 6. All direct assessments of the child/youth shall be conducted in the child's/youth's dominant language or mode of communication, including augmented or facilitated communication, if necessary. The child's/youth's age, developmental stage and ability to understand, and cultural and religious or spiritual background must be taken into consideration.
- B. Individual Child's/Youth's Plan**
- 1. On the basis of the comprehensive assessment, and within fourteen (14) calendar days of admission, the facility shall develop an individual plan that is written, time-limited, strength-focused, outcome-based, and goal-oriented. The plan must support the family services plan, if applicable.
 - 2. The facility must provide an opportunity for the following persons to participate in the planning/evaluation process:
 - a. The primary caregiver for the child/youth;

- b. The child/youth, consistent with state and federal laws and as appropriate to the child's/youth's age and developmental level;
 - c. Parent(s), legal guardian(s), or custodian(s) unless contraindicated or prohibited by law or court order;
 - d. Representatives of the placing agency;
 - e. School personnel;
 - f. Other persons significant in the child's/youth's life, such as a guardian ad litem, counsel for youth, respondent attorney, religious advisor, court appointed special advocate (CASA), therapist, or others identified by the child/youth and/or family, unless contraindicated;
 - g. When any of the above persons do not participate, the facility must have documentation of its efforts to involve the person(s). When the involvement of the child/youth, parent(s) legal guardian(s), or custodian(s) of the child/youth is deemed contraindicated or inappropriate by the agency or individual holding legal custody of the child/youth, the reasons must be documented. Any decision to not involve the child/youth, parent(s), legal guardian(s), or custodian(s) must be approved by the referral/placing agency.
- 3. The individual child's/youth's plan must include the following components:
 - a. The findings of the comprehensive assessment, including the strengths and needs of the child/youth.
 - b. Discharge criteria for the child/youth to successfully move to a less restrictive setting.
 - c. Documentation supporting the need for the current level of care.
 - d. Goals for the child/youth and family that address the identified areas of concern related to the current diagnosis, where applicable; and address the current areas of focus identified in the comprehensive assessment.
 - e. Objectives that are specific, measurable, achievable, relevant, and time-limited. The objectives must support and measure the child's/youth's progress toward meeting the goals.
 - f. Prior to placement and incorporated into the child's/youth's plan, a statement of the plan for supervision of the child/youth during their placement at the facility when the individual supervision plan

deviates from the overall supervision policies as stated in Sections 7.705.47 and 7.709.25.

- g. A plan for fostering, maintaining, and enhancing positive family relationships with the child/youth and their family, including siblings, other individuals considered like family, or guardian(s) and custodian(s), and including goals for the development of a permanent home for the child/youth.
- h. A plan for fostering, maintaining, and enhancing active community involvement for the child/youth.
- i. Specification of therapeutic services, specialized services, and strategies for positive behavior intervention that will be provided directly or arranged for, frequency of services, and method for ensuring their proper integration with the child's/youth's ongoing program activities.
- j. A plan for behavior support of the child/youth. Staff members who are responsible for the care of the child/youth must be familiar with the behavior support and/or intervention plan for each child/youth.
- k. A plan for discharge, aftercare, medication management, referrals to community resources, and moving to a less restrictive setting.
- l. Identification of all persons responsible for implementing or coordinating implementation of the plan.
- m. Signature, or written documentation of participation and review, by a representative of the facility, a representative of the placing agency, the child/youth, and the child's/youth's parent(s), legal guardian(s) and custodian(s). If an individual child's/youth's plan is not reviewed and/or signed by a member of the multidisciplinary team, an explanation as to why the member did not review and/or sign the plan must be documented in the child's/youth's file.
- n. Written documentation by the facility that the plan and any subsequent revisions are developed in consultation with the child/youth when that child/youth has attained fourteen (14) years of age, explained to the child/youth in care and their parent(s), legal guardian(s), and custodian(s) in a language or manner of communication the child/youth, parent(s), legal guardian(s), and custodian(s) can understand.
- o. Review of each plan must be completed every thirty (30) days to evaluate the progress toward achieving the goals and objectives. The parties listed in Section 7.714.4.B.2 must have a documented

opportunity to participate in the reviews. The plan must support the family services plan, when available.

- p. The individual child's/youth's plan must include a family support goal addressing services and/or interventions to be provided by the program, as informed by the state-approved assessment. The individual child's/youth's plan must incorporate the findings of the comprehensive assessment, to include any goals as determined in the assessment, if available or appropriate.
4. If the individual child's/youth's plan requires the services of a specialist, such as a psychiatrist, psychologist, speech therapist, occupational therapist, or physical therapist, the specialist must be currently certified or licensed according to state law.

7.714.5 CONTRABAND, PHYSICAL RESTRAINT, AND SECLUSION

7.714.51 Searches and Control of Contraband

- A. The facility must have a written policy and procedures for conducting searches and control of contraband regarding children/youth, staff members, or visitors. Searches involving children/youth must be completed upon admission to the facility or when it is suspected that a child/youth has contraband in their possession. Searches must be conducted in the least intrusive manner possible for the reason the search was being conducted. All contraband must be disposed of in accordance with the facility's written policy and procedures. All searches must be documented, including the reason for the search, the person(s) conducting the search, and the results. The policies and procedures at a minimum must include:
1. Person searches of children/youth may only be conducted when the facility deems it necessary to discourage introduction of contraband or to promote the safety and security of other children/youth, staff members, and the facility. Person searches shall be conducted as follows:
 - a. The child/youth must be told they are about to be searched in a language or mode of communication the child/youth understands and prefers.
 - b. The child/youth must remove outerwear (e.g. sweater, jacket, coat, gloves, hat, and shoes) and empty all pockets.
 - c. Person searches must be performed in an area and in a manner that maintains the dignity and privacy of the child/youth.
 - d. Staff members must be trained in proper search techniques prior to conducting searches.
 - e. When conducting a search of a child/youth, staff may only physically touch children/youth when it is reasonable to suspect the child/youth possesses an item(s) that poses a safety risk to the child/youth, other children/youth, or staff. When a safety risk is not suspected, staff conducting searches of children/youth must not physically touch the child/youth, but may search the child's/youth's property and clothing. All searches, whether it involves physical touch or not, must be conducted in a manner that protects and respects the child's/youth's privacy.
 - f. Whenever possible, person searches should be performed by two (2) staff members with at least one (1) staff member conducting the person search of the same gender identity the child/youth being searched identifies with.

- g. Staff members must search the child/youth's removed outer layer of clothing and return it to the child/youth or notify the legal guardian(s), custodian(s), and guardian ad litem and/or counsel for youth (if any), if contraband is found and confiscated.
 - h. If a staff member detects anything that appears to be drug paraphernalia, contraband, or a weapon, the staff member must contact law enforcement's non-emergency line to confiscate the item(s). The child's/youth's name and other identifying information must not be disclosed during the call to law enforcement. However, if a weapon is detected or suspected, the child's/youth's name and other identifying information may be disclosed during the call to law enforcement in accordance with HIPAA, as incorporated by reference in Section 7.701.14.
 - i. Searches of a person that include the removal of clothing to inspect a child/youth's private areas of their body, strip searches, body cavity searches, or frisk/pat searches are prohibited. If a facility has reasonable cause to suspect a child/youth may have hidden an object on their body or in their body cavity, facilities must contact medical personnel.
 - j. The use of metal detection wands may be used unless contraindicated.
 - k. If the child/youth refuses to comply, the administrator or designee must be notified immediately and be responsible for resolving the matter.
 - l. The parent(s), legal guardian(s), custodian(s), the child/youth, and placement agency must be notified in writing and provide authorization for search practices conducted by the facility.
 - m. Any additional steps regarding searches must be added to the child/youth's individual plan. The child/youth, parent(s), legal guardian(s), custodian(s), and placement agency must be notified of such steps during quarterly updates to the individual child/youth plan or as needed.
- 2. If the presence of contraband poses an imminent risk to the children/youth in care, the use of law enforcement and/or drug sniffing animals may be used after safety has been assessed.
- 3. If drugs, drug paraphernalia, or other illegal substances are found on the child/youth at the time of the search, the facility must confiscate and turn the contraband into law enforcement and document if law enforcement was not able to pick them up. A notification must then be made to the child's/youth's team, including the child's/youth's guardian and litem

and/or counsel for youth (if any), and the Department. If the situation rises to the need of a critical incident report, one must be filed in accordance with Section 7.701.52.

7.714.53 Physical Restraint and Seclusion

- A. In accordance with the following rules, only facilities authorized by the Department may use physical restraint or seclusion as outlined by specific facility/license type. The facility shall use physical restraint only in accordance with the following rules.
- B. All facilities must comply with all applicable statutes referenced in and including Section 26-20-103, C.R.S. regarding Protection of Persons From Restraint.
- C. Foster care providers may not utilize physical restraint unless approved by appeal waiver in accordance with Section 7.701.13.

7.714.531 Authorization of Physical Restraint

- A. At the time of admission to the facility, the parent(s), legal guardian(s) or custodian(s) of the child/youth must be notified that physical restraint may be performed in accordance with Section 26-20-103(1), C.R.S. For a facility to perform physical restraint, the legal guardian(s) or custodian(s) must give written consent for physical restraint to be performed on the child/youth. No physical restraint may be performed on an individual without the specific written permission of the child's/youth's legal guardian(s) or custodian(s). Youth eighteen (18) years of age or older may consent for themselves.
- B. Upon a child's/youth's admission to a facility and at least annually thereafter, the facility or provider must obtain information and documents relevant to the use of reactive strategies from a variety of sources for the child's/youth's records. Appropriate sources include the child/youth, their family members, treating medical professionals, and others familiar with the child/youth.
 - 1. The child's/youth's records must include the following documentation:
 - a. A physician's report of medical conditions or physical limitations that would place them at risk of physical injury during restraint or seclusion or otherwise preclude the use of one (1) or more emergency strategies;
 - b. Documentation of any history of trauma, such as a history of sexual or physical abuse that the child/youth, facility, or providers believe to be relevant to the use of emergency strategies;
 - c. Medical conditions or physical limitations that might create a risk to the individual, including, but not limited to:

1. Obesity;
 2. Cardiac conditions;
 3. Pregnancy;
 4. Asthma or other respiratory conditions;
 5. Back conditions or spinal problems;
 6. Seizure disorders;
 7. Deafness;
 8. Blindness;
 9. Limitations on range of motion;
 10. Osteoporosis;
 11. Osteopenia; and
 12. Hemophilia.
2. In addition to the annual review:
- a. The child's/youth's file information must be updated whenever there is a change in the child's/youth's physical or psychological condition that might affect their tolerance of physical restraint techniques.
 - b. The child's/youth's file must also be updated in compliance with any state-approved assessments.

7.714.532 Use of Physical Restraint

- A. Pursuant to Section 26-20-103, C.R.S., facility staff may only use restraint in cases of emergency, as defined in Section 26-20-102(3), C.R.S., and after a determination that such alternatives would be inappropriate or ineffective.
- B. Prior to the use of restraint, the facility must have tried all positive and constructive methods of managing the child's/youth's behavior, based on policies and procedures of the facility and components of the individual child's/youth's plan, including but not limited to, the implementation of a structured and consistent behavior management program, physical structuring of the environment, talking with the child/youth, praise for appropriate behavior, skill training and development, assisting the child/youth with the expression of feelings, and de-escalation of the situation.

- C. Only facility staff that have been appropriately trained or certified in accordance with a restraint model that includes nationally-recognized criteria that is evidence, performance, and competency based may use restraint.
- D. Restraint must only be used for the period of time necessary to accomplish its purpose. The child/youth must be released from a restraint within fifteen (15) minutes after the initiation of the restraint, except when precluded for safety reasons pursuant to Section 26-20-104(4), C.R.S., which must be documented in the individual file. Repeated application and release of the restraint in order to circumvent the fifteen (15) minutes is prohibited.
- E. The following reactive strategies are prohibited:
 - 1. Physical restraint or restrictive procedures involving noxious or painful stimuli;
 - 2. Untested or experimental procedures;
 - 3. Any physical crisis management technique that might restrict or obstruct a child's/youth's airway or impair breathing, including techniques whereby staff persons use their hands or body to place pressure on the child's/youth's head, neck, back, chest, abdomen, or joints;
 - 4. Restraint of a child's/youth's hands with or without a mechanical device, behind their back must only be used in accordance with the approved restraint model used by the facility;
 - 5. Physical holds relying on the inducement of pain for behavioral control;
 - 6. Movement that results in hyperextension or twisting of body parts;
 - 7. Any maneuver that causes a loss of balance without physical support (such as tripping or pushing) for the purpose of containment;
 - 8. Any physical restraint procedure in which a pillow, blanket, or other item is used to cover the child's/youth's face as a part of the restraint process;
 - 9. Any physical restraint procedure that may exacerbate a known medical or physical condition or endanger the child's/youth's life;
 - 10. Use of any restraint technique medically contraindicated for a child/youth;
 - 11. Restraint without continuous monitoring and documentation of vital signs and status with respect to release criteria;
 - 12. Restraint should never place any weight on the chest, abdomen, head, neck, back, or on any joints of a child/youth;

13. If there are any circumstances that may pose a threat to the health and safety of a child/youth on the grounds of the licensed facility, including, but not limited to, soft, pliable surfaces, concrete, asphalt, foreign objects, or areas including broken glass, or outside in inclement weather, staff must refrain from restraining the child/youth. Staff may only utilize restraint if the child/youth is able to be positioned away from the circumstances that pose a health/safety hazard and only in cases of emergency, as defined in Section 26-20-102(3), C.R.S.
14. Staff must make every effort to not use physical restraint in any setting outside of the licensed facility. If there is no other alternative and the situation cannot be controlled by the staff member(s) present through the use of restraint techniques that are described within the facility's approved restraint model, staff must call law enforcement. Physical restraint shall be used only in accordance with an emergency situation, as defined in Section 26-20-102(3), C.R.S.
15. Restraint must never be used as punishment or disciplinary sanction, as part of a treatment plan or behavior modification plan, for the purpose of retaliation by staff or for the purpose of protection unless ordered by the court or in an emergency.

7.714.533 Implementation of Physical Restraint

Facility procedures shall, at a minimum, include and comply with the following:

- A. The use of prone restraint is prohibited. Momentary (as quickly as possible, but not to exceed five (5) minutes) utilization of a prone position is permissible only during the transitional measure portion of a physical restraint. Repeated application and release of the prone position in order to circumvent the five (5) minute criteria is prohibited.
- B. In the course of a physical restraint, trained or and certified staff must be positioned within arm's length of the child/youth and continuously monitor to assure that the child/youth is properly positioned, that the child/youth's blood circulation is not restricted, that the child/youth's breathing and/or airway is not hindered or obstructed, and that the child/youth's other physical needs are met. Staff must not place weight or pressure on the chest, abdomen or back of a child/youth or inhibit or impede the child/youth's ability to breathe. Staff must continuously monitor to ensure that the breathing of the child/youth in such restraint is not compromised. If the child/youth is exceedingly agitated, staff may move farther from the child/youth, but must still be able to effectively observe, assess, and respond as necessary, to the child/youth's physical condition. If breathing is restricted or compromised in any way, the restraint must be discontinued immediately and a physical assessment must occur to determine if medical attention is needed.

1. A transitional measure may be used during an episode of physical restraint to effectively gain initial physical control of an individual in order to prevent harm to self or others. A transitional measure may result in a restraint to maintain prolonged physical control or containment of an individual.
 2. The facility shall provide adequate numbers of staff to assure the health and safety and the proper care and treatment of the children/youth in care as outlined in Section 7.705.46 and Section 7.714.91.A.1.
- C. The restraint must be used only for the purpose of preventing the continuation or renewal of an emergency; only for the period of time necessary to accomplish its purpose; and in the case of physical restraint, only if no more force than is necessary to limit the individual's freedom of movement is used.
- D. Restraint may be utilized only if certified staff persons are available in sufficient numbers to ensure its safe implementation and that there are enough staff to meet the individual needs of each child/youth in care. The staff must use no more force than is necessary to limit the child's/youth's freedom of movement.
- E. The physical restraint requires continuous staff monitoring to include, but need not be limited to, ensuring breathing is not restricted or compromised and the need to terminate the restraint if there is a medical emergency for the safety of the child/youth and staff involved.

7.714.534 Staff Training

- A. Staff utilizing any physical restraint in facilities or programs shall be trained in the appropriate use and implementation of a model that includes nationally-recognized criteria and a culturally-sensitive, trauma-informed care component that is evidence, performance, and competency based, prior to any staff being approved to use physical restraint. The model must include, at a minimum, the following:
1. Staff training and/or certification, to include training upon hire, and ongoing (at least every six (6) months) refresher training or practice exercises for each staff member trained or certified in restraint, to review and refresh skills related to early warning signs, prevention strategies, positive behavior intervention, de-escalation, and physical restraint, in accordance with the model. Verification of competency (e.g. passing post test score, certificate, etc.) must be kept in the staff file and made available to the department upon request;
 - a. Staff will be observed quarterly when performing physical restraint by a supervisor, trained professional, or medical staff of the facility who has been trained in physical restraint and will be involved in a debriefing process to identify what changes in practice can occur to reduce the use of physical restraint. If a supervisor of the facility

determines a staff member did not correctly perform a physical restraint, the staff member must be immediately retrained or be restricted from performing further physical restraint until retraining can occur.

2. The health and behavioral effects of restraint on children/youth, including those with behavioral or mental health disorders or intellectual and developmental disabilities;
3. How to assess the signs of physical distress in a person in restraint;
4. A restraint prevention and de-escalation component, to include identifying antecedents including environmental factors that may cause an individual to escalate, non-physical crisis intervention techniques, and/or development of behavior management plans that are in alignment with individual treatment plans if necessary, to include the value of positive reinforcement over punishment in working with children/youth and methods for implementing positive behavior incentives prior to restraint being utilized;
5. A physical restraint process that prohibits or provides alternatives to a prone position, and includes identifying primary control techniques that emphasize utilizing only the minimum amount of force necessary to gain control and keep the child/youth safe; and
6. A debriefing process which includes a review of physical restraint, to determine the appropriateness and effectiveness of preventive/de-escalation interventions used, the appropriateness of physical restraint, awareness of intentional and unintentional staff provocation, situational or environmental factors, and how, or if, physical restraints are preventable. The debriefing process must also include the neutral reviewer who was not involved in the physical restraint.

7.714.535 Documentation Following Utilization of Physical Restraint

If a physical restraint occurs, the following must be documented:

- A. A description of the incident, including the name of the child/youth, date and time of the incident, the name of all witnesses to the incident, staff members involved, their position at the facility and their involvement in the physical restraint, the location of the restraint, and the date and exact time the restraint was implemented and the exact time the restraint was terminated.
- B. A description of the observed precipitating incident(s) and/or the reported antecedent by the child/youth and the child's/youth's behavior before the physical restraint occurred.

- C. A description of what specific prevention strategies and trauma-informed interventions were attempted and/or taken to prevent or de-escalate the situation and the effect of any such intervention on the child/youth. Documentation of the individualized preventive and de-escalation interventions, trauma-informed practices, or interventions considered or utilized and positive behavioral intervention that must be attempted by staff prior to the use of physical restraint.
- D. A detailed description of the physical restraint must include:
 - 1. A clear rationale for the decision to initiate the physical restraint.
 - 2. The specific emergency, unsafe situation, or imminent threat that led to the staff's utilization of physical restraint.
 - 3. A description of the physical restraint including the child's/youth's physical, emotional and behavioral condition prior to, during, and after the physical restraint, including, but not limited to, breathing, pulse, change in skin coloring, and signs of choking or respiratory distress. If the restraint lasts more than fifteen (15) minutes due to safety reasons, the justification must be documented within the report.
 - 4. Staff must also document any negative outcomes of the physical restraint and any observations of behaviors or procedures that were not conducted in accordance with proper use of physical restraint training.
- E. A description of the debriefing and evaluation with the child/youth and staff following the physical restraint and/or to address other options that may have been successful in de-escalating the child/youth.
 - 1. The debrief must be informed by the child's/youth's safety plan to determine appropriate staff, location, and manner of debrief.
 - 2. Staff involved in the physical restraint must be involved in the debrief with the child/youth, except when the presence of a particular staff may jeopardize the wellbeing of the child/youth.
 - 3. A reasonable amount of time must be permitted to allow the child/youth to self-regulate after the physical restraint and the beginning of the debrief. Efforts to debrief with the child/youth must be documented and kept in the child's/youth's file.
 - 4. The debrief must take into consideration the needs of the child/youth, prioritize clear, simple language, allow ample time for responses, focus on positive reinforcement, with questions tailored to the child's/youth's individual understanding, and must ensure a supportive and non-judgmental environment that respects the child's/youth's agency and ability to contribute to the discussion. The debrief must also be in alignment with Section 7.714.531.B.

- F. Documentation of review by the neutral reviewer, as defined in Section 7.701.2, of whether the physical restraint was in accordance with the facility or model training, rules and regulations, agency policy, and in the best interests of safety of the child/youth and others.
- G. Verification that notification of the use of physical restraint was made to the legal guardian(s), custodian(s), parent(s), and guardian ad litem or counsel for youth (if any) of the child/youth as soon as possible.

7.714.536 Review

- A. Each facility shall include physical restraint in its critical incident review process and/or quality management program.
 - 1. Review the use of physical restraint
 - a. Records of each physical restraint shall be reviewed by a supervisor of the facility within forty-eight (48) hours of each restraint.
 - b. Every month, the facility must review in detail all restraints that occurred at the facility the month prior to identify trends that will be used by the facility to determine how to reduce the use of restraints, based on empirically-validated variables, such as, but not limited to, staff involved in the restraint, whether the restraint was justified in accordance with applicable regulations, and location. The reviews must incorporate quality assurance recommendations and safety measures to be implemented within policies and staff training. Reviews of the restraints must be documented and made available to the department upon request.
 - c. According to the policies and procedures of the facility, the entire child's/youth's behavior management or the individual child's/youth's plan must be reviewed if it appears that the child/youth is being physically managed more than two (2) times in seventy-two (72) hours, more than three (3) times in seven (7) days, or more than three (3) times by the same staff member in fourteen (14) days. The child's/youth's individual plan must be evaluated to reconsider new behaviors, triggers, rewards, and de-escalation techniques to reduce acting out behaviors.
 - d. If any particular de-escalation technique appears to be causing an escalation in the behavior of a child/youth or a group of children/youth, the use of the technique must be evaluated for its effectiveness. de-escalation techniques that are not effective or are counter-productive must be terminated at the earliest opportunity.

- e. If either the child/youth or a staff member was seriously injured or died during a physical restraint, a thorough review of the physical restraint and injuries must be instituted immediately. Additionally, reports must be made to the state child abuse and neglect hotline, and notification must be sent to the Department in accordance with Section 7.701.52. Based on the findings of the review, the staff members involved in the physical restraint must be retrained, be restricted from performing further physical restraint, and/or corrective personnel action must be taken.
 - f. If a staff member appears to be involved in a larger number of physical restraints due to intentional or unintentional provocation than other staff members or is unsuccessful at using de-escalation effectively, the facility must conduct a thorough review of the staff member's interactions with children/youth in care, prior physical restraint training, and need for further training or corrective personnel action as required by the facility's policies.
- B. Pursuant to Section 26-6-914(2)(k), C.R.S., a license can be suspended or revoked for a willful or deliberate failure to comply with the rules governing restraint.

7.714.54 Seclusion

These rules are specific to psychiatric residential treatment facilities (PRTFs) and any other facilities authorized to utilize seclusion through the appeal/waiver process in accordance with Section 7.701.13. The purpose of seclusion is to temporarily isolate a child/youth in a designated room, during an emergency, to manage potentially dangerous or aggressive behavior with the goal of preventing harm to self or others and to de-escalate a crisis situation when less-restrictive measures have failed.

7.714.541 Authorization for Seclusion

- A. At the time of admission, the child/youth's legal guardian(s) and custodian(s) must be notified of the facility's use of seclusion and must give written consent for the child/youth to be secluded in conjunction with facility policy. No child/youth may be secluded without the specific written permission of their legal guardian(s) and custodian(s). If the legal guardian(s) and custodian(s) do not give authorization for seclusion, then seclusion cannot be utilized in any form. Youth eighteen (18) years of age or older may consent for themselves.
- B. All facilities must ensure that staff are trained to explain, at time of admission, the use of seclusion to the child/youth, legal guardian(s), custodian(s), and if appropriate, to the child's/youth's parent(s). The explanation provided to the legal guardian(s), custodian(s), and child/youth must occur in a language or communication understandable to them and will include the purpose of seclusion and the emergency circumstances when a seclusion may occur.

7.714.542 Use of Seclusion

- A. Facility staff must only use seclusion in cases of emergency, as defined in Section 26-20-102(3), C.R.S.; and after the failure of less restrictive alternatives or after a determination that such alternatives would be inappropriate or ineffective under the circumstances.
- B. A child/youth who is placed in seclusion must be experiencing an emergency, meaning a serious, probable, imminent threat of bodily harm to self or others where there is the present ability to effect such bodily harm.
- C. The child/youth may be physically guided during a transport to the seclusion room for up to two (2) minutes. If it requires more than two (2) minutes, this is considered a physical restraint and must be documented in accordance with Section 7.714.535.
- D. Prior to the use of seclusion, the facility must have tried all positive and constructive methods of de-escalating a child/youth, including but not limited to, implementation of a structured and consistent behavior management program, physical structuring of the environment, talking with the child/youth, praise for appropriate behavior, skill training and development, assisting the child/youth with the expression of feelings, and de-escalation of the situation.
- E. Facility staff may only use seclusion if, prior to the use of placing the child/youth in seclusion, the staff has been appropriately trained in accordance with Section 7.714.544 and 42 CFR Section 483.376 of the Conditions of Participation for the Use of Restraint or Seclusion, as incorporated by reference in Section 7.705.101.D.
- F. Seclusion must never be used as punishment or disciplinary sanction, as part of a treatment plan or behavior modification plan, for the purpose of retaliation by staff, for the purpose of protection unless ordered by the court or in an emergency.
- G. Seclusion shall only be used only for the purpose of preventing the continuation or renewal of an emergency and only for the period of time necessary to accomplish its purpose.

7.714.543 Facility Procedures When Utilizing Seclusion

- A. When seclusion is utilized, relief periods must be provided for reasonable access to toilet facilities. While in seclusion, staff must be physically present outside of the room for continuous observation purposes, and staff must document those observations of children/youth no less than every fifteen (15) minutes. A staff member must be physically present within hearing and sight distance of the seclusion room at all times when a child/youth is locked inside the room.

- B. Seclusion must only occur for the period of time necessary to accomplish its purpose. The child/youth must be released from seclusion when the state of emergency has ceased. Seclusion must not exceed two (2) hours per incident for children/youth ages nine (9) and older and must not exceed one (1) hour for children/youth under age nine (9).
- C. Orders for seclusion must be by a physician or other licensed practitioner in the facility permitted to order seclusion and trained in the use of safety interventions, according to 42 CFR 483.358, as incorporated by reference in Section 7.705.101.D. If the physician is available, only they can order the seclusion. If the physician is unavailable, a verbal order must be received by a registered nurse or other licensed practitioner while the emergency safety intervention is being initiated by staff or immediately after the emergency safety intervention ends. The physician must verify the verbal order in a signed written form in the child's/youth's file. The physician must be available to staff for consultation, at least by telephone, throughout the period of the emergency safety intervention.
- D. There must be notification to another staff member on duty, that a child/youth has been placed in seclusion. If the seclusion room supervisor is not immediately available, the staff must continue to maintain safety and make all attempts to get a hold of the supervisor as soon as the situation is safely managed.

7.714.544 Required Seclusion Training for Staff

- A. Staff utilizing seclusion must have ongoing training and supervision to include at least the following:
 - 1. The health and behavioral effects of restraint and seclusion on children/youth, including those with behavioral or mental health disorders or intellectual and developmental disabilities;
 - 2. Emergency procedures including First Aid and fire protection;
 - 3. The facility's purpose, policies, and procedures for seclusion;
 - 4. Any legal or clinical ramifications of placing the child/youth in seclusion;
 - 5. The role of the neutral reviewer;
 - 6. The dynamics of the behavior of children/youth when in seclusion;
 - 7. Safe methods of moving the child/youth to the seclusion room in alignment with Sections 7.714.532, 7.714.533, and 7.714.542;
 - 8. Methods of searching a child/youth when placing the child/youth in the seclusion room, only if in alignment with Section 7.714.51;
 - 9. The protection of keys for the seclusion room, if applicable;

10. Trauma-informed care;
11. Effective de-escalation techniques for children/youth in crisis;
12. The value of positive over negative reinforcement in working with children/youth;
13. Methods for implementing positive behavior incentives prior to seclusion being utilized in an attempt to reduce the emergency;
14. Consideration of and how to minimize traumatization or retraumatization.

7.714.545 Facility Staff Requirements for Seclusion

- A. There must be a seclusion room supervisor who is a full-time facility staff member and holds one of the following license types in Colorado: licensed clinical social worker (LCSW), licensed professional counselor (LPC), licensed marriage and family therapist (LMFT), licensed psychologist, or a board-eligible psychiatrist licensed to practice medicine in Colorado. This staff member must be designated and trained to be responsible for the use of the seclusion room. If the seclusion room supervisor is not a psychiatrist or a licensed psychologist, there must be such a person contracted to provide consultation with the seclusion room supervisor and staff. Seclusion room supervisors must be present and able to be available at the facility at all times.
- B. The residential child care facility which operates a seclusion room must appoint a neutral reviewer. This neutral reviewer must not be the seclusion room supervisor or the person who placed the child/youth in the seclusion room. The reviewer must determine if the situation resulting in the confinement of a child/youth in a seclusion room merits such a decision. The neutral reviewer must be a staff member of the facility or a professional contracted by the facility in one (1) of the following positions: administrator, assistant administrator, social worker, psychologist, psychiatrist, nurse, or lawyer.
- C. There must be sufficient staff on the milieu in compliance with Section 7.705.46 when a child/youth is placed in seclusion. At the time of placement of the child/youth in the seclusion room, the neutral reviewer must be notified in person or by telephone. Leaving a message is not notification, and if the neutral reviewer cannot be contacted in person or by telephone and present for observation purposes, the child/youth must not be placed in a seclusion room.
- D. The facility must designate in its statement of purpose and policy for the operation of the seclusion room, as identified in Section 7.705.68.A, which staff members may be able to place a child/youth within the seclusion room. Identified staff members must be employed or contracted as one (1) or more of the following positions: administrator, assistant administrator, child care staff, social worker, teacher, psychologist, psychiatrist, or nurse. Designated staff members must be trained and supervised in the use of the seclusion room.

7.714.546 Documentation Following Utilization of Seclusion

The following information must be documented:

- A. A written report which states the child's/youth's name, the time the child/youth was placed in the seclusion room, the staff member who placed the child/youth in the seclusion room, the staff member who was notified, and describes the following: the precipitating incident and the child's/youth's behavior before placement in the seclusion room, and actions taken by staff members of a less restrictive nature to try to control, calm, or contain the child/youth and a clear rationale for the decision to initiate the seclusion.
- B. During seclusion, the following must be documented, if applicable:
 - 1. An indication that the child/youth was checked continuously for observation purposes and documentation of those observations of children/youth no less than every fifteen (15) minutes, with the following notations: time, one-line description of what the child/youth was doing, and initials of the person supervising;
 - 2. When the child/youth was last given access to restroom facilities;
 - 3. When the child/youth had opportunity for exercise, if applicable;
 - 4. When and what type of medications were given and by whom; and
 - 5. When the child's/youth's last staff contact occurred.
- C. Description of the resolution between the staff member and the child/youth at the termination of the use of the seclusion room and the behavior of the child/youth as a result of being in the seclusion room.
- D. The observable physical condition of the child/youth when entering the seclusion room and any change in the observable physical condition of the child/youth when leaving the seclusion room.
- E. An indication of review by the neutral reviewer as to the appropriateness of confinement of the child/youth in the seclusion room.
- F. Verification that notification was made to the placing authority by the next day of the seclusion and the appropriateness of the seclusion.
- G. Notification to the parent(s), legal guardian(s), and custodian(s) of each use of the seclusion, no later than the end of the day that the seclusion occurred, with a written report completed and given to the legal guardian(s), custodian(s), and guardian ad litem or counsel for youth by the next day.

7.714.547 Review of the Use of Seclusion

- A. The record of use of the seclusion room must be reviewed daily by the seclusion room supervisor and weekly by the facility administrator. If one (1) child/youth is placed in the seclusion room more than three (3) times in seventy-two (72) hours, the entire plan for the child/youth must be reviewed. A physician must evaluate and authorize any further use of the seclusion room or other treatment for the child/youth.
- B. If the same staff member places a child/youth in the seclusion room three (3) times in a thirty (30) day period, this must be investigated by the seclusion room supervisor and evaluated in the child's/youth's plan.
- C. All agencies must have an administrative oversight component, to include, at a minimum, tracking and reviewing episodes of seclusion and physical restraint data such as through a quality assurance or performance improvement process.
- D. Pursuant to Section 26-6-914(2)(k), C.R.S., a license can be suspended or revoked for a willful or deliberate failure to comply with the rules governing seclusion.

7.714.548 Physical Requirements for Seclusion

- A. The seclusion room must be located in reasonable proximity to the living unit or other areas of activity. An adult staff member must stay directly outside the seclusion room at all times when a child/youth is locked inside the room.
- B. The seclusion room must be a minimum of eighty (80) square feet in size.
- C. The seclusion room must be kept in a clean and sanitary condition.
- D. All switches for light, heat, and ventilation, as well as other electrical outlets, must be outside the room. All switches must be available only to the staff.
- E. There must be no features by which a child/youth might injure themselves within the seclusion room such as utility pipes, cleaning equipment and materials, or mirrors.
- F. Exterior windows are not recommended, but if there are window panes they must be of shatter-resistant material.
- G. There must be an observation window on the door from which all parts of the room are visible for purposes of supervision. The window must be made of shatter-resistant materials.
- H. There must be an approved ventilating system.
- I. There must be an approved system which detects products of combustion and is wired into a no switch electrical outlet and is out of the reach of children/youth.

- J. The seclusion room must be constructed of one (1) hour fire-resistant material. The door must be of one (1) hour fire-resistant material.
- K. The seclusion room must have a lighted, soothing environment. The child/youth must not be subjected to glaring lights. All lights must be recessed into the ceiling and must be covered with a shatter-resistant guard which is flush with the ceiling.
- L. There must be no more than one (1) locked door between the child/youth and the staff member.
- M. If the seclusion room is soundproof, there must be an intercom system which is activated when a child/youth is in the room.
- N. There must be a light over the entrance of the seclusion room which is lighted any time a child/youth is locked in the room.
- O. Seclusion rooms must be examined after each use to determine any damage to the room or identified safety issues needing to be corrected.

7.714.549 Approvals Necessary to Operate a Seclusion Room

- A. The written approval of the local fire department and the department must be received prior to the initial use of the seclusion room.
- B. The licensee must request an inspection by the fire department at least annually. The licensee must retain a copy of the inspection report in the facility file.
- C. The records of the use of the seclusion room, the policy for operation of the room, the children/youth's records, staff records, and the room must be open to staff members of the department of human services for inspection at any time.

7.714.6 Educational/Vocational Programs

- A. Educational/vocational programs shall be developed and provided for all school-age children/youth who are residents of the facility in accordance with the individual child's/youth's plan, the Public School Finance Act outlined in Article 54.5 of Title 22, C.R.S., and as required by the Exceptional Children's Educational Act outlined in Article 20 of Title 22, C.R.S. and Rules and Regulations for the Administration of the Exceptional Children's Educational Act (1 CCR 301-8) (June 30, 2024), herein incorporated by reference. No later editions or amendments are incorporated. The rules and regulations for the Administration of the Exceptional Children's Act can be found at <https://www.sos.state.co.us/CCR/GenerateRulePdf.do?ruleVersionId=6624>. Copies are also available for public inspection and copying at the Colorado Department of Human Services, 1575 Sherman Street, Denver, CO 80203, during regular business hours.

1. Children/youth shall attend educational/vocational programs in the most appropriate and least restrictive educational setting for the child/youth and following the rules of a best interest determination (BID) meeting as outlined in Section 7.301.241. Facility schools must not require students to attend their educational facility unless and until the county has determined that it is in the child's/youth's best interest to change schools and has specifically requested a change in enrollment in accordance with Section 22-32-138(1)(g), C.R.S. and Section 7.301.241.D. Any school attended within the community must be accredited by the state of colorado. online education or home-based learning programs must:
 - a. Be accredited or approved by the state of Colorado.
 - b. be taught by a teacher licensed in the state of Colorado.
 - c. Be included in the individual child's/youth's plan and approved by the placing agency.
 - d. Include a written plan for recreation, socialization, and community involvement for the child/youth.
 - e. Include a written plan for supervision while the child/youth is in the online education program.
 2. Homeschooling is prohibited.
 3. Children/youth attending school must be permitted to participate in school extracurricular activities to the extent of their interests and abilities and in accordance with each individual child's/youth's plan.
 4. The facility must develop assurances that the educational/vocational program is an integral part of the child's/youth's plan.
- B. In order to ensure that all students who may have disabilities are provided an appropriate education, the facility must ensure that adequate "child find" procedures are utilized. Such procedures must be developed cooperatively with local education agencies (LEA) in accordance with Exceptional Children's Educational Act rules and regulations and LEA procedures and in accordance with the Individuals with Disabilities Education Act (IDEA) (October 2010), 20 U.S.C. Sections 1400 through 1482, and Section 504 of the Rehabilitation Act of 1973 (October 2016), 29 U.S.C. Section 794, as incorporated by reference in Section 7.701.14. Child find includes a process for screening, referring, assessing, and staffing students suspected of having a disabling condition.
- C. The facility that operates its own educational program must provide adequate space, staff, equipment, and educational materials as required in the Exceptional Children's Educational Act Rules and Regulations outlined in 1 CCR 301-8, as incorporated by reference in Section 7.714.6.A.

- D. School records must be transmitted according to state law, pursuant to Section 24-72-204, C.R.S., Section 504 of the Rehabilitation Act of 1973, (October 2016), 29 U.S.C. Section 794, and the Individuals with Disabilities Education Act (IDEA) (October 2010), 20 U.S.C. Sections 1400 through 1482, as incorporated by reference in Section 7.701.14.

7.714.65 REQUIREMENTS OF A SPECIALIZED PROGRAM FOR CHILDREN/YOUTH AT RISK AND SURVIVORS OF SEX AND LABOR TRAFFICKING

- A. The facility must have written policies and procedures that describe the admission requirements of the facility, an overview of the facility and its programming, the transitional planning services provided, and how to support the long-term impact of trauma on survivors of human trafficking.
- B. The written policies and procedures of the facility must address child/youth unapproved leave from the campus, peer recruitment for trafficking, external safety threats, and any other known safety risk to children/youth or staff.
- C. All staff members must complete training in the following areas within ninety (90) days of beginning employment at the facility and renew the training on an annual basis.
1. Indicators of human trafficking;
 2. The different types of trafficking;
 3. The impact of trauma on survivors of human trafficking;
 4. The service needs of human trafficking survivors;
 5. Identifying secondary trauma;
 6. Survivor-informed active listening;
 7. Individualized needs assessment; and
 8. Gender identity, spiritual, cultural, intellectual, physical, and emotional dynamics resulting from their trafficking experience.
- D. In addition to the requirements in Section 7.714.65.C, all staff members must complete eight (8) additional hours of annual training in human trafficking subject areas.
- E. The facility must complete an assessment of service needs with every child/youth in the facility. The needs assessment must be conducted at the time of intake and on a minimum of a quarterly basis, in order to develop and maintain a

comprehensive individual plan. The comprehensive individual plan must include, but is not limited to:

1. Safety planning including online safety, harm reduction, and safe/approved contacts;
2. Long-term housing;
3. Identifying and building a natural support network;
4. Identifying and building healthy relationships;
5. Spiritual/religious support;
6. Cultural support;
7. Identifying and accessing legal services;
8. Medical and mental health services;
9. Accessing transportation;
10. Educational, vocational, and job skill training services;
11. Job search, employment, and budgeting services;
12. The impact of trauma on survivors of human trafficking; and
13. Discharge planning to include safe community re-entry, identified supports outside of the program, and ongoing case management services.

7.714.7 Community Participation

Participation in community activities must be encouraged, supported, and a vital part of each child's/youth's life and choice, and must be in accordance with each individual child's/youth's plan.

- A. The facility must reflect consideration for, and sensitivity to, the racial, cultural, ethnic, and religious backgrounds of children/youth in care. The facility must involve a child/youth in cultural and/or ethnic activities appropriate to their cultural and/or ethnic background and involve the child/youth in determining appropriate activities.
- B. The facility must utilize available services, facilities, and activity programs of the community, and children/youth must be given opportunities to participate as individuals or as a group in agency-sponsored recreational and cultural programs such as provided by YMCA, YWCA, scouting organizations, schools, and churches or other religious institutions.

- C. Children/youth in foster care or participating in a foster care transition program must be provided access to information regarding available work options consistent with applicable laws and Section 7.714.33.
- D. Facilities must establish written policies regarding monies earned and spent. Such policies must include:
 - 1. Money earned, received as a gift, or received as allowance by a child/youth in care must be deemed to be that child's/youth's personal property.
 - 2. Facilities must document all monies earned and received by children/youth, the procedure for requesting money, the time frame in which requests will be completed, and accounting for monies received from and distributed to or on behalf of the child/youth. Documentation of such transactions must be made available to the child/youth and their legal guardian(s) and custodian(s) upon request and at time of discharge. Any monies remaining in a child's/youth's account must be given to the child's/youth's legal guardian(s) and custodian(s) at time of discharge, unless the child/youth is eighteen (18) years or older, at which time the money must be transferred to them directly.
 - 3. For other children/youth not residing in foster care or not participating in the foster youth in transition program, limitations may be placed on the amount of money a child/youth may possess or have access to at one time when such limitations are in the child's/youth's best interests as documented in the child's/youth's file.
 - 4. Restitution, or court-ordered payment made by a criminal offender to a victim to compensate for losses caused by a crime, cannot be used as a means of discipline unless it is ordered by a court based on a criminal offense that was issued prior to placement in the facility. Monies received as gifts must not be used toward restitution. A written record of any restitution paid by a child/youth must be maintained in the case file. An accounting of all payments made must be made available to child/youth or their legal guardian(s) and custodian(s) upon request and at time of discharge.
- E. Chores at the facility are considered part of the participatory responsibility of living together. All chores must provide constructive experiences in accordance with the age and ability of the child/youth and must not substitute for the work of staff.
- F. All chores must be scheduled so as not to conflict with other essential scheduled activities.
- G. The facility shall comply with all child labor laws and regulations in making work assignments.

- H. A child/youth must not be exploited in any manner. Facilities may not permit a child/youth to participate in a solicitation on behalf of the facility for a fundraising activity without the written permission of the parent(s) legal guardian(s), and custodian(s) for each specific activity, and the child/youth must be willing to participate in the activity.

7.714.8 PERSONAL CARE OF THE CHILD/YOUTH

7.714.81 Preventative, Supportive, and Corrective Feedback and Discipline

- A. Preventative and supportive discipline is designed to be constructive and educational and may include diversion, counseling, predetermined consequences such as temporary loss of privileges, and/or interventions from the trauma behavior support plan developed by the child/youth and their therapist.
- B. Rights must not be denied as a disciplinary measure.
- C. Separation when used as discipline must be properly assessed if appropriate to the child's/youth's age, developmental level, history of trauma, and circumstances. The child/youth must always be within hearing of an adult in a safe, clean, well-lit, well-ventilated room in the facility that contains at least eighty (80) square feet of floor space. Exit routes from any room which a child/youth is being separated must be kept free of barriers to escape. No lock or other mechanism preventing egress from the room may be installed. No child/youth may be isolated in a bathroom, closet, attic, pantry, or garage as a means of discipline.
- D. Children/youth in care at the facility must not discipline other children/youth. This does not prohibit a facility from operating an organized, therapeutic, self-government program or positive peer culture that is conducted in accordance with the written policies of the facility and these rules and is directly supervised by a staff member.
- E. A facility shall prohibit all corporal, inhumane, cruel, and unusual discipline including, but not limited to, the following:
1. Any type of physical punishment inflicted in any manner upon the body of the child/youth such as spanking, striking, swatting, punching, shaking, biting, hair pulling, rough handling, striking with an inanimate object, or discipline that is designed to, or likely to, cause physical pain.
 2. Any shaming, humiliating, or frightening method of discipline to control the actions of any child/youth or group of children/youth.
 3. Physical exercise such as running or walking laps, push-ups, or carrying or stacking heavy rocks, bricks, or lumber when used solely as a means of punishment.

4. Assignment of physically strenuous or hard work that could result in harm to the child/youth.
5. Requiring or forcing a child/youth to take an uncomfortable position such as squatting or bending or requiring a child/youth to stay in a position for an extended length of time such as standing with nose to the wall, holding hands over head, or sitting in a cross-legged position on the floor, or requiring or forcing a child/youth to repeat physical movements when used solely as a means of punishment.
6. Group discipline except in accordance with the facility's written policy and these rules.
7. Verbal abuse, derogatory remarks about the child/youth, their family, their race, ancestry, ethnic group, national origin, religion, sex, sexual orientation, gender identity, gender expression, disability, marital status, familial status, source of income, military status, hiv status, cultural background, or any other component of the child's/youth's identity, or any intervention or type of treatment that subjects a child/youth to verbal abuse, ridicule, humiliation, or that can be expected to cause excessive emotional trauma.
8. Denial of any essential, basic program service solely for disciplinary purposes.
9. Deprivation of meals or snacks, although scheduled meals or snacks may be provided individually.
10. Denial of visits or communication privileges with family, clergy, attorney, guardian ad litem (GAL), counsel for youth (CFY), mental health professionals, court-appointment special advocates, education rights holders, probation officers, and/or caseworker solely as a means of punishment.
11. Releasing noxious, toxic, or otherwise unpleasant sprays, mists, or aerosol substances in proximity to the child's/youth's face.
12. Any intervention that denies a child/youth sleep, food, water, shelter, access to bathroom facilities, adequate bedding, or appropriate physical comfort.
13. Requiring the child/youth to remain silent for a period of time as a form of punishment.
14. Denial of shelter, clothing, or bedding.
15. Withholding of emotional response or stimulation.

16. Discipline associated with toileting, toileting accidents, or lapses in toilet training.
17. Sending a child/youth to bed as punishment. This does not prohibit a facility from setting individual bedtimes for children/youth.
18. Force feeding a child/youth.
19. Use of emergency interventions (e.g. physical restraint or seclusion) as discipline for a child/youth, as a part of therapy, or as a threat to control or gain compliance of a child's/youth's behavior.
20. Interventions that use a device, material, or object that is designed to immobilize the child/youth.
21. Any intervention that deprives a child/youth of the use of their senses, including sight, hearing, touch, taste, or smell.
22. Use of "rebirthing therapy" or any therapy technique that may be considered similar to rebirthing therapy as a therapeutic treatment, as defined in Section 12-245-224(1)(t)(iv), C.R.S.
23. Use of "conversion therapy" or any therapy technique aimed at changing a person's gender identity, sexual orientation, or gender expression in opposition to the child's/youth's documented requests. Conversion therapy does not include practices or treatments that provide acceptance, support, and understanding for the facilitation of an individual's coping, social support, and identity exploration and development, including sexual orientation-neutral interventions to prevent or address unlawful conduct or unsafe sexual practices, as long as the counseling does not seek to change sexual orientation or gender identity in accordance with Section 12-245-224(1)(t)(v), C.R.S.

7.714.82 Medical and Health Services

- A. The facility shall ensure the availability and make every reasonable effort to obtain a comprehensive program of preventive, routine, and emergency medical, mental health, vision, and dental care for each child/youth in care. The facility shall have a written plan for providing such care. This plan shall include at a minimum:
 1. Assignment of one (1) person to be responsible for the coordination of medical care for the children/youth residing in the facility.
 2. Ongoing appraisal of the general health of each child/youth, including documentation of immunization status or exemption as required by the Colorado Department of Public Health and Environment (CDPHE). Proof of immunization must be provided prior to or on the first day of admission.

3. Procedures for obtaining diagnostic services; emergency care, including the availability of a physician or emergency medical facility on a twenty-four (24)-hour, seven (7)-day-a-week basis; corrective care; recuperative care; and immunization updates.
 4. Provision of age and developmentally appropriate health education, which includes: sexual health, including the risk of sexually transmitted disease, and birth control information.
 5. Provision that any medical treatment administered will be explained to the child/youth in a language or manner of communication understandable to them.
 6. The provision of dental care by a Colorado-licensed dentist, who is available to the facility.
 7. Procedure for obtaining the verbal consent by a physician, if in an emergency situation, and written consent from the child's/youth's legal guardian(s) or custodian(s) prior to any new psychotropic medication being administered.
 - a. Verbal consent by a physician shall only be utilized in an emergency situation. Verbal consent must be communicated only through the physician and the nurse on duty and documented in the child/youth individual file.
 - b. Written consent shall be obtained after the emergency has subsided and within forty-eight (48) hours of administration of any new psychotropic medication.
 - c. The child's/youth's parent(s), legal guardian(s), and custodian(s) shall be notified of any change of dosage in psychotropic medications prior to the new dosage being administered, including the reason for the change and the new dosage ordered by the prescribing provider.
 8. Procedures for notifying children/youth about prescribed medication and their purposes; administration of medication; storage of medication; documentation of administration of all medication; disposing of medications when not needed or no longer in use; documenting current or unused medications at time of discharge; ensuring children's/youth's access to prescribed medications during placement changes and similar transitions; and notification to a primary physician in cases of medication errors and/or drug reactions.
- B. A general medical examination for each child/youth must be completed or scheduled with a physician, physician's assistant, or a nurse practitioner prior to or within fourteen (14) calendar days following admission. A statement from the

examiner shall be retained in the child's/youth's file. This exam shall include the following:

1. A review of the child's/youth's medical history, when available, at time of examination.
 2. A review of current medications the child/youth is taking and medications the child/youth has been taking within thirty (30) days prior to placement.
 3. An examination for physical injury and ongoing medical conditions.
 4. Vision and hearing screening.
 5. A current assessment of the child's/youth's health, including immunizations.
- C. Whenever indicated, a child/youth must be referred to an appropriate specialist for either further assessment or treatment.
- D. Subsequent physical and other examinations shall be done annually or as directed, in writing, by the physician or other licensed health care provider. Ongoing medical conditions must be considered before implementation of a treatment strategy.
- E. Dental examinations for all children/youth three (3) years of age or older, or at a younger age if recommended by a physician or dentist, must have been completed within four (4) months prior to placement or within eight (8) weeks following placement. The facility must ensure that each child/youth receives a dental examination every six (6) months or as required in writing by a dentist.
- F. At all times there shall be First Aid supplies readily available at the facility. For every twenty (20) children/youth, at least one (1) person supervising the children/youth must be certified in First Aid or the equivalent, CPR, and AED for all ages of children/youth in care. When children/youth are with a staff member away from the facility, the staff member must have a current certification(s) in First Aid, CPR, and AED. The First Aid, CPR, and AED course must be from a nationally recognized organization and include a face-to-face demonstration of competency with a certified instructor in both child/youth and adult competencies.
- G. The facility, in conjunction with the parent(s), legal guardian(s), and custodian(s) must make every effort to ensure that a child/youth needing corrective devices such as glasses, hearing aids, etc., is provided with the necessary equipment.
- H. The facility has the right to request a statement regarding the child's/youth's general health from a licensed health care provider. In a potentially life-threatening situation, the facility must refer the child's/youth's care to the appropriate medical and legal authority. If a child/youth wishes an exemption from a medical examination or medical treatment due to religious beliefs, the

child/youth must submit a written statement signed by their parent(s), legal guardian(s), and custodian(s) at the time of admission which states the reasons for such an exemption. The facility has the right to refuse admission to a child/youth whose parent(s), legal guardian(s), and custodian(s) refuses medical treatment or examination based upon religious convictions.

- I. The facility must regularly maintain and update a summary of the medical care received by the child/youth while at the facility, including the dates of service; the name and contact information of the medical professional who examined the child/youth; the results of the exam; and any medication that was prescribed or had a change of frequency or dosage. A copy of the document must be given to the caseworker upon the child's/youth's discharge so that it can be given to the facility where the child/youth is being admitted or to the child's/youth's parent(s), legal guardian(s), custodian(s), or family member(s) with whom the child/youth is placed. When the facility has received a summary of medical care from a previous placement, this information must be included in the discharge summary, as outlined in Section 7.714.86, to provide a cumulative health history to subsequent caregivers.
- J. Medications shall be administered and stored in accordance with the manner described within this Subsection J and CDPHE Regulations Regarding Medication Administration, Storage, and Disposal (July 1, 2017) which are herein incorporated by reference. No later editions or amendments are incorporated. The regulations can be found at 6 CCR 1011-1, Chapter 24 or <https://www.sos.state.co.us/CCR/GenerateRulePdf.do?ruleVersionId=7567&fileName=6%20CCR%201011-1>. These regulations are also available for public inspection at the Colorado Department of Human Services, 1575 Sherman Street, Denver, CO 80203, during regular business hours:
 1. When a child/youth first comes into care, the facility must ascertain all medication the child/youth is currently taking and the medications that arrived with the child/youth. Medications arriving with the child/youth must only be accepted if stored in the original pharmacy-labeled container with the child's/youth's name and prescriber. Multiple medications combined in one (1) container must not be accepted. Medications currently prescribed for the child/youth that did not arrive with the child/youth must be obtained within twenty-four (24) hours after admission. When a medication is required to sustain life, the medication must be obtained by the next scheduled dose, even if the next dose is due in less than twenty-four (24) hours.
 2. All medication must be kept in a clean, locked storage area inaccessible to children/youth and stored according to pharmacy instructions.
 - a. Medications must be stored in their original container with a pharmacy label at all times. The use of "pill boxes" or "reminder

- boxes” or pre-pouring of medications is prohibited unless prepared and administered by an approved pharmacy.
- b. Medications that require refrigeration must be stored separate from food. If medications that require refrigeration are stored in the same refrigerator as food, they must be in a sealed container, away from food, and not on a shelf over any other food in the refrigerator.
 - c. Containers of over-the-counter medications used occasionally as a “pro re nata” (PRN) and not prescribed as an ongoing treatment program for an individual child/youth, do not have to be labeled with the name of an individual child/youth for use.
 - d. Medication must not be stored with disinfectant, insecticides, bleaches, household cleaning solutions, or poisons.
 - e. Controlled medications must be double locked, and the keys to the locks must be under the control of the staff member responsible for administering medications at all times.
3. All prescriptive medications shall be administered only upon the written prescription of a licensed health care provider. The facility must also obtain written authorization from the prescribing provider to administer any non-prescription, pro re nata (PRN), or over-the-counter medication, including dietary supplements and vitamins used to treat an identified symptom.
- a. All medications and supplements authorized for use by the child/youth must be labeled with the child’s/youth’s name.
 - b. The facility may maintain on site and use epinephrine injectors in accordance with Section 25-47-103, C.R.S.
 - c. Staff authorized to administer epinephrine must be trained as required in Section 25-47-104, C.R.S. Any staff member who administers epinephrine must immediately ensure the injected individual is transported to an emergency room.
4. The facility must obtain consent from the child’s/youth’s physician, parent(s), and legal guardian(s) or custodian(s) for any changes in the type or dose of medication the child/youth is taking in accordance with Section 7.714.82.A.7. Verbal consent by a physician is only allowed in an emergency situation. In non-emergency situations, written consent from the child’s/youth’s legal guardian(s) and custodian(s) must be obtained prior to any new psychotropic medication being administered. The facility must identify a procedure for obtaining verbal consent from a physician in an emergency situation prior to any new psychotropic medication being administered consistent with Section 7.714.82.A.7.

5. The facility must obtain a minimum of verbal authorization from the child's/youth's physician, parent(s), and legal guardian(s) or custodian(s) for any medication prescribed for each acute illness or injury. This must be documented in the child/youth file.
6. The facility must have a written medication schedule for each child/youth to whom medication is prescribed, including non-prescriptive medications or procedures ordered by a prescribing provider, a copy of which must be available to staff. The information on the medication schedule and the medication administration record must include the reason the medication is being prescribed and must be consistent with the written prescribing provider order and the pharmacy label for each medication and/or procedure ordered.
7. In an emergency situation, non-prescriptive medication may be administered on the verbal authorization of a prescribing provider. Written confirmation must then be obtained for the verbal authorization within forty-eight (48) hours excluding weekends and federal holidays.
8. The facility must maintain for each child/youth a cumulative record of all medication, both prescriptive and non-prescriptive, administered to that child/youth and any procedures ordered by the prescribing provider including:
 - a. The name of the child/youth;
 - b. The name of the prescribing provider;
 - c. The reason the medication or procedure was ordered;
 - d. The name and dosage of medication;
 - e. A description of the procedure administered and the results, along with any medical follow up required;
 - f. The time and date the medication was administered or procedure was conducted; and
 - g. The name of the person administering the medication or conducting the procedure.
9. The facility must maintain a record of all controlled substances on site, including:
 - a. The name of the licensed health care provider prescribing the medication provider;
 - b. The reason the medication was prescribed;

- c. The name of the medication;
 - d. The dosage of the medication;
 - e. The number of pills brought into the facility;
 - f. The name of the child/youth for which the medication is prescribed;
 - g. A starting count and ending count for each shift conducted by two (2) different staff, one (1) on each shift;
 - h. The initials of the individual(s) who conducted the count; and
 - i. A printed name/identifier for the initials.
- 10. All controlled substances shall be stored under double lock at all times. Keys that open the locks to the controlled substances must be under the control of the person(s) responsible for administering the medication or secured in an additional locked area at all times.
- 11. The facility shall document the following information as it relates to handling administration of, return of, and disposal of medication:
 - a. Documentation of:
 - 1. All medication errors, including follow-up with the staff member who was responsible for the error and/or the prescribing provider, if necessary; and
 - 2. Adverse reactions caused by the medication error or after the administration of the medication.
 - b. Documentation of all discontinued medication, medications that are refused or wasted, and disposing of medications. Medications must be disposed of in accordance with CDPHE Regulations Regarding Medication Administration, Storage, and Disposal, as incorporated by reference in Section 7.714.82.J.
- 12. Only staff members who are qualified managers, as defined in Section 25-1.5-301(4), C.R.S. or authorized by passing the Qualified Medication Administration Personnel (QMAP) training and assessment through the Colorado Department of Public Health and Environment within the last five (5) years are approved to administer medications. The approved training entity (ate) must also include in its course content the expectations regarding medication administration set forth in Section 7.714.82.J. Verification of current QMAP training must confirm the date the training was given. The staff member must also receive additional training specific to the program as found at Section 7.714.82.J. Verification of passing the

QMAP evaluation and any other additional program-specific medication administration training must be maintained in the staff member's personnel file.

- a. A facility must require each QMAP or qualified manager, as a condition of employment or promotion to a position for which the individual has access to medications, to sign a disclosure statement, under penalty of perjury, stating that they have never had a professional license to practice nursing, medicine, or pharmacy revoked in Colorado or any other state for reasons directly related to the administration of medications.
 - b. A minimum of one (1) staff currently authorized to administer medications must be on site and available during all operational hours.
 - c. Staff currently trained as a QMAP must follow all requirements covered in the QMAP certification except where these practices are prohibited or do not apply to children or youth in care. Practices must take into consideration the cognitive and developmental level of the child/youth and not invade a child's/youth's privacy.
 - d. Any medications that require the QMAP to use an administration route other than topical or oral, including but not limited to, gastrostomy tube or administration of insulin by syringe, must be specifically authorized for administration per Section 12-255-131, C.R.S. of the Nurse and Nurse Aide Practice Act.
 - e. In addition to the CDPHE QMAP training, each facility must provide on-the-job, evidence-based training that focuses on the unique medication needs of the population of the facility.
 - f. All direct care staff must be trained in the recognition of common side effects of medication.
13. Staff currently trained as a QMAP must follow the seven (7) rights of the administration of medications as follows:
- a. Right person;
 - b. Right medication;
 - c. Right dose;
 - d. Right time;
 - e. Right route;

- f. Right documentation; and
 - g. Right to refuse.
- 14. Medications prepared for children/youth that will be away from the facility during the time of scheduled administration, must be prepared in compliance with the Pharmaceutical Practice Act, Sections 12-280-101 through 12-280-604, C.R.S. (2024); the Uniform Controlled Substances Act of 2013, Sections 18-18-101 through 18-18-607, C.R.S. (2024); and the State Board of Pharmacy Rules and Regulations at 3 CCR 719-1 (November 30, 2024), herein incorporated by reference. No later editions or amendments are incorporated. These regulations are available at no cost from the Colorado State Pharmacy Board at 1560 Broadway, Denver, CO 80202 or at <https://www.sos.state.co.us/CCR/GenerateRulePdf.do?ruleVersionId=11189&fileName=3%20CCR%20719-1>. These regulations are also available for public inspection and copying at the Colorado Department of Human Services, Provider Services Unit, 1575 Sherman St., Denver, CO 80203, during regular business hours.
- 15. When a child's/youth's individual plan includes a provision for the child/youth to learn about and/or manage an ongoing health condition, per prescribing provider's orders, for the purpose of continued life skills, and self care, a QMAP staff may monitor the youth self-administering the medication. Monitoring means:
 - a. Reminding the child/youth to take medication(s) at the time ordered by the prescribing provider.
 - b. Handing the child/youth a container or package of medication that was lawfully labeled previously by a pharmacist for the individual child/youth.
 - c. Visual observation of the child/youth self-administering the medication to ensure compliance.
 - d. Making a written record of the child's/youth's compliance with regard to each medication, including the time taken.
 - e. Notifying the prescribing provider if the child/youth refuses or is unable to comply with the prescribing provider's instructions regarding the medication.
- K. In all instances where a new prescription medication is to be ordered as a part of an ongoing treatment program or due to an acute illness or injury, the following information must be provided to the child/youth, their parent(s), custodian(s), and legal guardian(s):

1. The name of the prescribing provider that ordered the medication;
 2. The name of the medication being prescribed;
 3. The proper use of the medication;
 4. The reasons for ordering the medication for this child/youth;
 5. A description of the benefits expected;
 6. The common side effects, if any;
 7. The major risks, if any;
 8. The probable consequences of not taking the medication explained in a language or mode of communication that is easily understood by the child/youth, their parent(s), and/or legal guardian(s) or custodian(s).
- L. No involuntary medications may be administered unless the facility is licensed as a PRTF and approved through facility policies and procedures to administer chemical restraints.
- M. The child/youth may refuse to take their medication at any time. The facility must document the refusal in the medication log. The facility must hold a staffing with the child's/youth's multidisciplinary team for a child/youth who has refused medication for more than five (5) days to determine possible solutions and evaluate continued placement. The staffing must occur within twenty-four (24) hours if the child/youth is refusing any life-saving medications.
- N. No child/youth shall be threatened with or experience any behavioral consequences or disciplinary action by staff due to refusal to take medication.
- O. The facility must have naloxone available onsite and in facility vehicles at all times. All staff must be trained on how to use naloxone. Certificates for naloxone training must be kept in each staff member's file.

7.714.83 Personal Hygiene and Daily Routine

- A. The facility shall establish procedures to ensure that children/youth receive training in good habits of personal care, hygiene, and grooming appropriate to their age, gender identity, race, and culture.
1. Facility staff must supervise children/youth to provide for proper grooming and physical cleanliness of the children/youth.
 2. The facility must ensure that children/youth have access to necessary and appropriately maintained toiletry items, individual towels and washcloths, toothbrush, toothpaste, comb, shampoo, and other gender and culturally

affirming hygiene products during daily hygiene routine times and other times hygiene is necessary for the child/youth.

- B. The facility must have a plan of basic daily routines for children/youth in care.
 - 1. Daily routines must not be allowed to conflict with the implementation of a child's/youth's plan.
 - 2. Daily routines must be established for mealtimes, waking, and bedtimes.
 - 3. Opportunity for physical activity must be planned for each child/youth.

7.714.84 Clothing and Personal Belongings

- A. The facility shall make provisions for the storage and protection of children's/youth's property when admitted into the facility. The facility/agency is responsible for creating a written policy or acknowledgement, as identified in Section 7.705.63.C, that identifies the length of time children's/youth's property will be stored at the facility/agency.
- B. The facility shall allow a child/youth in care to bring their personal belongings to the program and to acquire belongings of their own as appropriate to their age and developmental level. However, the facility shall, as necessary, supervise the use of these items while the child/youth is in care. Children/youth residing in foster care or participating in the foster youth in transition program have a right to basic essentials including personal belongings as outlined in Section 19-7-101(2)(f)(iii), C.R.S. (2024) that shall only be limited or denied by court order. For other children who are not residing in foster care or not participating in the foster youth in transition program, the facility shall, as necessary for the safety of the child/youth, limit the use of these items while the child/youth is in care. Where extraordinary limitations are imposed, the child/youth shall be informed of the reasons, in a language or manner of communication the child/youth can understand. The decision and reasons shall be recorded in the child's/youth's file. Provisions shall be made for the protection of the child's/youth's property.
- C. The facility shall ensure that each child/youth in care has adequate clean, proper-fitting, suitable, and seasonable clothing as required for health, comfort, and physical well-being and as appropriate to age, gender identity, individual needs, culture, and ethnicity.
 - 1. Each child's/youth's clothing shall be distinguished as their own in accordance with the facility's policies.
 - 2. A child's/youth's clothing shall be kept clean and in good repair. The child/youth shall be involved, as appropriate, in the care and maintenance of their clothing. As appropriate, laundering, ironing, and sewing facilities must be accessible to the child/youth and supervised by staff while in use.

- D. The facility shall ensure that discharge plans make provisions for clothing needs at time of discharge. The wardrobe for each child/youth must go with them at time of discharge in appropriate luggage.
- E. Should there be belongings left at the time of the child's/youth's discharge, the facility is responsible for following their policy for storage and disposal of belongings, as identified in Section 7.705.63.E.

7.714.85 Discharge and Aftercare Plan

- A. Within forty-five (45) calendar days of admitting a child/youth into care, the facility must develop a discharge and aftercare plan in collaboration with the county department of human/social services or placing entity, the child/youth, and the family engagement and permanency team.
- B. The aftercare plan must describe the services and resources needed by the subsequent care provider to meet the needs of the child/youth and how those services and resources will be provided.
- C. If a child/youth is discharged during the first seventy-two (72) hours of admission, the facility must assure that available mental health information is included in the discharge summary pursuant to Section 7.714.85.H.
- D. The plan must be reviewed and/or modified by the licensed clinician or provisionally licensed clinician with review and signature by the supervising licensed clinician no less than monthly during the child's/youth's placement and during aftercare services. Changes to the plan must be documented in the child's/youth's individual file.
- E. The facility must incorporate the plan into the discharge summary as outlined at Section 7.714.85.H.
- F. The facility may revise the terms of aftercare support and services with the placing entity of any child/youth not placed by a county department of human/social services or the Division of Youth Services (DYS) and must provide aftercare support in accordance with the contract established with the placing entity.
- G. Facilities may refer to other entities to provide aftercare services and must clearly document in the aftercare plan whom they referred to, contact information, and the specific services to be provided.
- H. Within five (5) business days of when a child/youth is removed from placement, the facility must complete and send to the placing agency, a summary of the child's/youth's discharge from the facility which includes, at a minimum:
 - 1. The date of the discharge of the child/youth from the facility;

2. Where the child/youth was placed following discharge;
 3. A summary of the services provided to the child/youth during care;
 4. Goals met/not met during treatment at the discharging facility;
 5. The treatment goals and assessed needs which remain to be met and alternate service recommendations which might meet those goals and needs;
 6. A statement of an aftercare plan and identification of who is responsible for follow-up services and aftercare;
 7. If the discharge was planned or unplanned; and
 8. The circumstances which led to an unplanned discharge;
- I. Copies of a youth's file, including discharge information but excluding all medical and/or educational information covered by HIPAA and FERPA, must be provided to parent(s), legal guardian(s), or custodian(s) upon request or to others by written consent pursuant to Section 7.701.61. Copies of a youth's file, including discharge information but excluding all medical and/or educational information covered by HIPAA and FERPA, may be provided upon request with written consent of the youth pursuant to Section 7.701.61. Releases of information shall be consistent with applicable federal and state law.

7.714.86 Required Notification

- A. The facility shall immediately notify the child's/youth's parent(s), legal guardian(s), custodian(s), guardian and litem and/or counsel for youth (if any), and/or the responsible placing authority of any serious illness or serious injury resulting in medical treatment away from the facility, hospitalization, or death involving a child/youth in care.
- B. The facility must notify the parent(s), legal guardian(s), custodian(s), guardian ad litem and or/counsel for youth (if any), and/or placing authority as soon as possible or at least within twenty-four (24) hours excluding weekends and state-observed holidays upon discovery that a child/youth has run away.
- C. Critical incidents must be reported as outlined in Section 7.701.52.
- D. A report about death must immediately be provided to the department licensing and monitoring specialists and include:
 1. The child's/youth's name, birthdate, address, and telephone number;

2. The names of the child's/youth's parent(s), legal guardian(s), custodian(s), and their address and telephone number if different from that of the child/youth;
 3. Date of the fatality;
 4. Brief description of the incident or illness leading to the death;
 5. Names and addresses of witnesses or persons who were with the child/youth at the time of death; and
 6. Name and address of the police department or authority to whom the report was made.
- E. Any change in the status of the facility, which includes, but is not limited to, moving locations, structural damage to the facility due to flooding or fire, or any instance that could potentially affect the safety and care of the children/youth in care, must be immediately reported to the Department.
- F. The facility must immediately notify the department of any court order or physician's order that violates these regulations.
- G. The facility must regularly notify the parent(s), legal guardian(s), custodian(s), and/or placing authority of the issues related to the care of the child/youth including changes in medication; discipline; treatment; behavior management; physical restraint and seclusion; and restriction of rights as outlined in Section 7.714.31.D.1.

7.714.9 PERSONNEL/POLICY REQUIREMENTS

7.714.91 General Requirements for All Staff

- A. The facility shall provide adequate numbers of staff to assure the health and safety and the proper care and treatment of the children/youth in care as outlined in Section 7.705.46.
- B. All staff in the facility shall demonstrate:
1. An interest in, and a knowledge of, children/youth and a concern for their proper care and well-being; and
 2. An ability to maintain appropriate physical boundaries and maintain a level of professionalism with children/youth in care.
- C. The facility shall have written screening procedures and make reasonable efforts to evaluate the overall emotional health and stability of each applicant and/or staff member.

- D. A facility shall not hire or continue to employ any person whose health, educational achievement, or emotional or psychological makeup impairs their ability to properly protect the health and safety of the children/youth in care, or who could endanger the physical or psychological well-being of the children/youth.
- E. A staff member who, upon examination or as a result of tests, shows indication of a physical condition which could be hazardous to a child/youth, other staff, or self, or which would prevent performance of duties, shall not be assigned or returned to their position until the condition has cleared to the satisfaction of the examining physician, physician's assistant, or nurse practitioner.

7.714.92 Personnel Policy, Orientation, and Training

- A. The facility must have a comprehensive written policy for the hiring, retention, orientation, initial and ongoing training, and professional development of staff.
 - 1. The facility must provide an introductory training and orientation program for all facility staff. This program must include orientation to emergency and safety procedures, professional boundaries, and the general and specific duties and responsibilities of the job. The introductory training and orientation training must be completed within thirty (30) days of hire and before working alone with children and youth in care.
 - a. The orientation training must include the facility's policy on required boundaries between staff members and the children/youth cared for or discharged from the facility.
 - b. The professional boundaries training must include:
 - 1. Physical and emotional boundaries;
 - 2. Staff social media, artificial intelligence (AI), and cell phone boundaries;
 - 3. Staff use of video or other electronic contact boundaries;
 - 4. The legal ramifications for inappropriate boundaries with children/youth in care; and
 - 5. The facility's employment and termination policy for employees having or suspected of having inappropriate boundaries with children/youth in care.
 - c. Each employee must review the facility's policy on boundaries and be trained in professional boundaries within thirty (30) days of hire and annually thereafter and sign a facility-created form stating that they have reviewed and agreed to the policy. Documentation must

be maintained in the staff file and made available to the department upon request.

- B. The facility must document that all staff who in the course of their job duties engage in unsupervised programming or activities with children/youth receive appropriate training in the following areas as applicable to their position:
1. Within ninety (90) days of hire, the following training must be completed and annually thereafter.
 - a. Developmentally appropriate behavior management practices including positive behavior supports and appropriate discipline strategies for the population served in the facility and according to the facility's trauma-informed care model.
 - b. The facility's administrative procedures and overall program goals. When the facility has a certifying agency, the administrative procedures of the certifying agency must also be included in the training.
 - c. De-escalation strategies to be used with children/youth, in alignment with the facility's trauma-informed care plan and/or the facility's physical management and de-escalation model. Training to the definition of emergency and imminent danger to self and others, and the responsibility of all staff to report suspicion of abuse and neglect if a child/youth is physically restrained without emergency criteria being met every six (6) months in accordance with Section 7.714.534.A.1.
 - d. Procedures for child/youth supervision based on the individual needs of children/youth, including the need for informed supervision of children/youth with inappropriate sexualized behaviors and/or suicidal ideation.
 - e. The development and implementation of the individual child's/youth's plan of care.
 - f. Training on the facility's approved trauma-informed care model.
 - g. Confidentiality as it relates to 42 C.F.R. parts 160, 162, and 164 of the Health Insurance Portability and Accountability Act (HIPAA) (2024); which are hereby incorporated by reference. No later editions or amendments are incorporated. These statutes and regulations are available for public inspection and copying at the

Colorado Department of Human Services 1575 Sherman St., Denver, Colorado 80203, during regular business hours. These statutes and regulations are also available for public inspection at no cost at the U.S. Department of Health and Human Services website <https://www.hhs.gov/hipaa> and to 20 U.S.C. 123G and 34 C.F.R. part 99 of the Family Educational Rights and Privacy Act (FERPA) (2024) which is hereby incorporated by reference. No later editions or amendments are incorporated. These statutes and regulations are available for public inspection and copying at the Colorado Department of Human Services 1575 Sherman St., Denver, Colorado 80203 during regular business hours. These statutes and regulations are also available for public inspection at no cost at the U.S. Department of Education website <https://studentprivacy.ed.gov/FERPA>.

- h. All facility staff must complete the state-approved mandatory reporting training.
 - i. All staff must complete the state-approved Volume 7 rules and regulations training according to the license type of the facility.
 - j. Training on how to recognize when children/youth have potentially been victims of labor or sex trafficking and reporting such to the Colorado child abuse and neglect hotline.
- 2. Every six (6) months and/or two (2) times per training year:
 - a. The facility's emergency and safety procedures, including but not limited to fire evacuation drills and disaster drills, a minimum of two (2) times per year, at least four (4) months apart.
 - b. Physical restraint of children/youth in accordance with facility policies and Section 7.714.534.A.1, including recognition of early warning signs, prevention strategies, positive behavior interventions, the ability to recognize and respond to signs of physical distress in children/youth who are subject to a physical restraint as required in Sections 7.714.53 through 7.714.536 for those staff authorized to perform physical restraint.
- 3. Additional training requirements:
 - a. QMAP training in accordance with 7.714.82.J.
 - b. CPR/First Aid training in accordance with 7.714.82.F.
 - c. In addition to the training requirements identified in Section 7.714.92.B, all staff must have a minimum of ten (10) hours annually of specialized training specific to the facility's stated purpose and population of children and youth being served at the facility.

- d. Staff at facilities serving children/youth funded by the Children's Habilitation Residential Program (CHRP) must complete thirty-two (32) hours of annual training, with sixteen (16) hours dedicated to specialized training that specifically address the needs of the children/youth being served at the facility.
- 4. The facility must maintain written documentation of specific in-service training including:
 - a. The name of the training;
 - b. The date, time, and length of the training;
 - c. A description or summary of the topics covered in the training;
 - d. The name and the title of the trainer;
 - e. Names, titles, and signatures of staff who completed the training.
 - f. The individual staff members' job duties do not meet the requirement of initial and annual training.

7.714.93 Colorado Provider Training Academy

This applies to residential child care facilities, qualified residential treatment programs, psychiatric residential treatment facilities, secure residential treatment centers, and homeless youth shelters. The Department will provide a ninety (90) day notice to specialized group facilities and day treatment facilities prior to the implementation of staff being required to participate in the training.

- A. Facility direct child care staff, supervisors, clinical staff, case management staff, management, administrative staff, and directors must complete the certification process through the Colorado Provider Training Academy.
 - 1. The facility must ensure that each newly hired staff member is enrolled in the Colorado Provider Training Academy within ten (10) days of hire and successfully completes the forty (40) hour, initial, foundational certification within ninety (90) days of employment.
 - 2. The facility must ensure that all staff have access to the necessary technology to complete the required training.
 - 3. Staff hired before the implementation of the Colorado Provider Training Academy must complete the forty (40) hour foundational certification within six (6) months of the implementation of these rules.

4. The facility must ensure supervisors complete the additional six (6) hours of the supervisor certification program provided by the Colorado Provider Training Academy.
5. Staff enrolled in the Colorado Provider Training Academy must complete all required transfer of learning exercises with the assistance of a supervisor or supervisor designee.
6. All staff must demonstrate an emerging level of competence through reflection, pre- and post-tests, trainer review, observation, and verification by the Colorado Provider Training Academy as outlined in the trainee certification requirements.
7. Certification is considered current as long as the staff member maintains successful completion of all ongoing training requirements. If staff do not meet their training requirements by the designated timeframe, client contact must not be permitted until the training requirements are met.
8. The forty (40) hour foundational training through the Colorado Provider Training Academy, mandatory reporting training, and Volume 7 rules and regulations training may be transferred if a person becomes employed through another facility. QMAP and CPR/First Aid may also transfer if they are not expired.

7.715 STANDARDS FOR HOMELESS YOUTH SHELTERS

7.715.1 INTRODUCTION [Rev. eff. 9/15/12]

All rules in Section 7.715 will be known and hereinafter referred to as the Standards for Homeless Youth Shelters. All homeless youth shelters shall comply with the General Rules for Child Care Facilities located at Section 7.701.

7.715.11 Definitions [Rev. eff. 9/15/12]

“Homeless youth” is defined in the general rules at Section 7.701.21.

“Homeless youth shelter” is defined in the general rules at Section 7.701.21.

“Religion” when used in these rules includes traditional religious beliefs and spiritual beliefs such as those of Native Americans.

“Staff member” of the shelter, as used in these rules, includes staff that work time-limited work shifts and do not live at the shelter on a permanent basis.

7.715.2 MANAGEMENT REQUIREMENTS

7.715.21 Governing Body

- A. The governing body is the individual, partnership, corporation or association in whom the ultimate authority and legal responsibility is vested for the conduct of the shelter.
- B. When the governing body does not include a board of directors, there shall be an advisory committee of at least two other individuals who act in an advisory capacity to the governing body. The names of the advisory committee members shall be disclosed to the Department. The advisory committee shall meet at regularly- stated intervals.
- C. Minutes of the advisory committee or the board of directors shall be maintained. The minutes shall be available to the Department upon request, except that the minutes containing confidential personnel information need not be shared with the Department.
- D. The functions of the governing body shall include:
 - 1. The appointment of an administrator who shall be responsible, according to established performance criteria, to the governing body, and shall delegate to him/her the executive authority and responsibility for the administration of the shelter according to its defined purpose.
 - 2. The formulation and regularly-planned review of policies and procedures to be followed by the shelter.

3. The provision of necessary facilities, adequate financing, qualified personnel, services, and program functions for the welfare and safety of youth in accordance with these standards.

7.715.22 Fiscal Management

- A. A shelter shall demonstrate that it is financially sound and manages its financial affairs prudently. All funds disbursed by the shelter shall be expended in accordance with the program objectives as specified by the governing body.
- B. A shelter shall demonstrate fiscal accountability through regular recording of its finances.

7.715.23 Insurance

- A. Every shelter shall carry public liability insurance. The applicant or licensee shall submit the amount of the insurance and the name and the address of the insurance agency providing the insurance for the shelter. Information about the insurance should be maintained at the shelter.
- B. Facilities operating their own transportation vehicles shall carry insurance in compliance with the minimum limits as required by the Colorado Revised Statutes.
- C. The shelter shall carry workman's compensation and unemployment insurance as required by law.

7.715.3 HUMAN RESOURCES

7.715.31 Administrative Staff

- A. Administrator

The on-site administrator of a shelter shall be qualified as follows:

1. The administrator shall have received a bachelor's degree from an accredited college or university and have three years of verified experience in the human services field, two of which were in a supervisory or administrative position; or

The administrator shall have received a master's degree and have two years of verified experience in the human services field, one of which was in a supervisory or administrative position.

2. The administrator shall assume the following duties:
 - a. Overall direction and responsibility for the youth, program, shelter, and fiscal management.

- b. Overall direction and responsibility for supervision of adequate staff.
- c. The selection and training of a capable staff member who can assume responsibility for management of the shelter in the administrator's absence.
- d. The establishment of relationships and contact maintenance with allied agencies, services, and resources within the community.

B. Assistant or Acting Administrator

- 1. In each youth care shelter, there shall be a specifically designated staff member, age 21 or over, capable of acting as a substitute for the administrator during his/her absence. The duties and responsibilities of the substitute administrator shall be clearly defined in order to avoid confusion and conflict among other staff and youth.
- 2. If the administrator is regularly absent from the shelter more than 50 percent of his/her working hours, an assistant administrator shall be appointed who meets the same qualifications as the administrator found at Section 7.715.31, A.

C. Administrative Coverage

- 1. The Department shall be notified in writing when a possible change in administrator is anticipated or at the time there is a change in administrator. Documents verifying the qualifications of the acting/interim or new administrator, including official transcripts and documents verifying work experience, shall be submitted to the Department.
- 2. The administrator or the staff member to whom the administrator has delegated responsibility shall be available at all times.

7.715.32 Youth Care Staff

- A. Each youth care staff shall be at least 21 years of age and have completed two years of college education. A high school diploma or equivalent and one year's experience in the human services field may be substituted for the required college.
- B. Each peer counselor shall be at least 17 years old and shall work directly under the supervision of a youth care staff in activities specified by the youth care staff or with the approval of the shelter director.
- C. Relief staff shall have the same qualifications as regularly assigned youth care staff.

- D. There shall be one staff member that has basic knowledge and understanding of nutrition, food purchasing, menu planning, food handling, preparation, and storage.

7.715.33 Youth Care Staff-to-Youth Ratios [Rev. eff. 9/15/12]

- A. There shall be a minimum of one (1) adult qualified as a youth care staff on duty and one (1) adult on call who can be summoned at all times when there is one youth or more at the shelter.
- B. At night there shall be at least one awake staff person within each physically separate building, and within hearing of youth, and a second person who can be immediately summoned in case of emergency.
- C. The ratio of youth care staff to youth in care shall be dependent on the needs of the youth. The ratio of on duty staff shall not be less than the following schedule except when transporting youth in vehicles (see Section 7.715.87):

<u>Age of Youth</u>	<u>Waking Hours</u>	<u>Sleeping Hours</u>
18 years and over	1 staff member: 20 youth	1 staff member: 20 youth
11 to 18	1 staff member: 20 youth	1 staff member: 20 youth

For facilities that serve youth under the age of eighteen, and youth over the age of eighteen, a separate staff member is required to serve each population.

The ratio of youth care staff to youth in care does not apply to the adult on call identified in provision A, above. A single individual can serve as the adult on call when a facility size or age range requires more than one on duty adult staff member.

7.715.34 Volunteers and Students

- A. If volunteers or students are used by a shelter, the administration shall define specifically the services to be given by that individual.
- B. Volunteers and students who are assigned to work directly with the youth shall:
1. Be subject to reference checks similar to those performed for employment applicants.
 2. Be in good general health. The shelter has the right to contact the individual's physician.
- C. Volunteers and students shall be:
1. Directly supervised by a paid staff member.

2. Oriented and trained in the philosophy of the shelter, confidential nature of their work, and the specific job which they are to do, prior to assignment.

7.715.35 Medical and Health Staff

A shelter shall have a primary physician or Family Nurse Practitioner (FNP) licensed in Colorado, available to establish and maintain the health and medical plan and procedures of the shelter. This person must be present on-site at least weekly. All medical staff, whether employed by the shelter, or whether their services are purchased or volunteered, shall be licensed and responsible to the primary physician or FNP for the medical aspects of their job.

7.715.36 Case Management Staff and Other Professional Staff

A. Case management shall be provided by:

1. A qualified professional having a master's degree in social work, psychology, or child development from an accredited college or university. This professional shall have two years of treatment-oriented experience, or
2. A designated member of the staff, who shall have a bachelor's degree from an accredited college or university with a major in behavioral science, human services or related fields, and three years experience in working with youth, or a designated member of the staff, who shall have at least five years verified full-time experience working with homeless or runaway youth; and for whom there is an effective arrangement for consultation from a qualified professional as described above.
3. The qualified professional, as specified in Section 7.715.36, A, 1, may be a full-time or part-time employee of the shelter, a staff member of a recognized agency, or an individual with which the shelter has a contract to provide the necessary services and/or consultation.
4. The ratio of case management staff to youth shall be dependent on the type of shelter and the needs of the youth therein, but there shall be at least one full-time case management staff member for every twenty (20) youth, or a part-time staff member assigned for a fraction thereof.

B. The case management staff shall be responsible for providing case management services as stated at Section 7.715.42, B.

C. Youth in need of specific individual therapy shall be referred to an appropriate treatment provider.

7.715.37 General Requirements for All Staff

- A. The shelter shall provide adequate numbers of staff to assure the health and safety and the proper care and treatment of the youth in care.
- B. All staff in the shelter shall demonstrate an interest in, and a knowledge of, youth and a concern for their proper care and well-being.
- C. The shelter shall have written screening procedures and make reasonable efforts to evaluate the overall emotional health and stability of each applicant and/or staff member. Procedures should include exploring for any history of youth battering, youth abuse, youth molestation, youth neglect, or previous criminal convictions.
- D. Each staff member, including part-time staff and substitute staff, shall have on file at the shelter verification of tuberculin status. If the staff member has a certificate of previous negative testing, the testing does not need to be repeated. If there is no certificate, the testing needs to be completed prior to employment or certification.
- E. A staff member who, upon examination or as a result of tests, shows indication of a physical condition which could be hazardous to a youth, other staff, or self, or which would prevent performance of duties, shall not be assigned or returned to his/her position until the condition has cleared to the satisfaction of the examining physician or nurse practitioner.

7.715.38 Personnel Policy, Orientation, and Training

- A. The shelter shall have a comprehensive written plan for the recruitment, hiring or certification, orientation, ongoing training, and professional development of staff.
 - 1. The shelter shall have an introductory training and orientation program for all staff. This program shall include orientation to emergency and safety procedures and the general and specific duties and responsibilities of the job.
 - 2. The shelter shall maintain written documentation of specific in-service training held, staff participating, the hours involved, and/or other on-going training activities in which staff were involved. Activities related to supervision of the staff members' routine tasks shall not be considered training activities for the purpose of this requirement.
- B. The shelter shall document that all staff receive appropriate training in the following areas:
 - 1. The shelter's emergency and safety procedures, including but not limited to fire evacuation drills, tornado drills, where appropriate, and flood evacuation drills, where appropriate, on at least a semiannual basis.

2. Universal precautions regarding safe work practices to prevent exposure to blood and bodily fluids.
3. The principles and practices of working with youth.
4. The shelter's administrative procedures and overall program goals.
5. Acceptable behavior management techniques, including appropriate discipline in accordance with shelter policies and these rules.
6. Appropriate professional boundaries (both physical and emotional) between staff and youth while in placement at the shelter and after discharge.
7. Positive and constructive methods of dealing with the youth including but not limited to physical structuring of the environment and de-escalation of crisis situations.
8. Annual review of these regulations by all appropriate staff members of the shelter.
9. All staff must have twenty-clock hours of on-going job specific training a year. Training may include areas listed above.

7.715.4 PROGRAM

7.715.41 Program Description

- A. The shelter shall have a written overall program description for the shelter.
- B. The written description shall address at least the following areas:
 1. Case Management Provided
 2. Family counseling
 3. Education
 4. Shelter provided (housing, food, clothing)

7.715.42 Case Management Services

- A. A staff member qualified as stated at Section 7.715.36, A, shall be assigned the responsibility for case management for each youth.
- B. Each individual assigned the responsibility of providing case management services for a youth shall:

1. Participate actively in the treatment planning, discharge and after care planning.
2. Assure that appropriate information concerning the youth and her/his background is shared with other staff members who deal with the youth regularly and to maintain communication with parents, guardians, if available, and with the individual or agency responsible for discharge planning and follow-up care of the youth, if applicable.
3. Assure that the plan developed for each youth is implemented, and documented in the youth's file.

7.715.43 Admission Policy and Procedures [Rev. eff. 9/15/12]

- A. Admission of a youth shall be in keeping with the stated purpose of the homeless youth shelter and shall be limited to those youth for whom the shelter is qualified by staff, program, equipment, and needs of youth already in residence to provide care deemed necessary. Care must be provided in the least restrictive, most appropriate setting in order to meet the youth's needs.
- B. Each shelter shall have a written admission policy which at a minimum must include:
 1. The policies and procedures related to intake.
 2. The age range and sex of youth accepted for care.
 3. The needs, problems, situations or patterns best addressed by the shelter's program.
 4. The anticipated criteria, problems, situations, and patterns that would result in the shelter requesting removal of a youth.
 5. A statement regarding the religious orientation or affiliation of the shelter, and of the religious activities at the shelter, if any.
 6. A statement regarding any charges or costs for services that may be expected from the youth, the youth's family or others who may be responsible for the youth.
- C. The shelter shall accept a youth into care only after a preliminary assessment and screening of presenting problems in areas such as social, physical health, mental health.
- D. Upon admission of a youth to a shelter:
 1. The shelter shall provide all necessary services pursuant to section 26-5.7-106(2), C.R.S.

2. A youth who has reached the age of 15 or older may consent to shelter and services for a period not to exceed 21 days when the following criteria are met:
 - A. The youth shall voluntarily and knowingly sign a consent form that includes:
 1. All services the youth may receive during their stay at the shelter and the total number of days the youth may consent.
 2. Legal limits to confidentiality.
 3. How and when parent(s), legal guardian(s), and/or child welfare agencies will be notified of the youth's stay.
 4. An explanation that services provided by the shelter are provided free of charge to the youth without exchange of any goods, services, or sexual acts.
 - B. The consent form shall be explained in the child's dominant language or mode of communication, including augmented or facilitated communication, if necessary, and shall take into consideration the child's age, disability, and cultural and religious background.
 - C. The consent form shall be signed within 12 hours of admission.
- E. Within 24 hours of arrival at the shelter, a youth shall be given an orientation to the shelter, consistent with the youth's age and ability to participate, which includes at least the following:
 1. Tour of the shelter and instruction on fire alarm and fire evacuation procedures, escape routes and exits.
 2. The rules/regulations of the shelter.
 3. Procedures affecting the youth's behavior, including limiting or restricting a youth's rights where allowed, the type of discipline used in the shelter, and consequences for certain behaviors.
 4. The complete youth's rights and youth's grievance procedures as developed by the shelter or by the certifying authority.
- F. For youth under the age of eighteen (18), if reconciliation with the youth's family has not occurred within seventy-two (72) hours following admission to the shelter, and the director of the shelter or other person in charge does not anticipate that reconciliation will be achieved within twenty-one (21) consecutive

days, the director or other person in charge shall provide the youth and the youth's parent(s) or legal guardian(s) with a written statement identifying:

1. The availability of counseling services;
2. The availability of longer term residential arrangements; and
3. The possibility of referral to the county department.

- G. Youth may reside at a shelter for a period not to exceed twenty-one (21) consecutive days unless the youth is placed in a voluntary alternative residential placement pursuant to Section 26-5.7-107 or 26-5.7-108, C.R.S.

For youth under the age of eighteen (18), a voluntary residential agreement shall be developed with the involvement of the youth and, if possible, the youth's parent or the legal guardian(s). Where the involvement of any of these is not feasible or desirable, the reasons for the exclusion shall be recorded by the shelter. If the youth and the youth's parent cannot agree on an initial voluntary alternative residence within twenty-one (21) days, the shelter may make a referral to the county department. If an agreement can be reached, the placement agreement shall include at least the following information:

1. Discussion of the youth's and the parent's or guardian's expectations regarding: family contact, reconciliation and involvement; how family contact and involvement are to occur, the nature and goals of care, the anticipated planned discharge date and the plan for the youth following discharge.
2. A delineation of the respective roles and responsibilities of all agencies and persons involved with the youth and his/her family.
3. Legal status or custody of the youth.
4. If a youth is placed by a Colorado county department of social services, the appropriate state form or contract shall be completed. This form or contract may provide some of the required authorizations.

For youth between the age of eighteen (18) and twenty-one (21), the voluntary residential agreement shall be developed with the involvement of the youth. Other individuals may participate in the development of the agreement at the youth and shelter's discretion. The agreement shall include at least the following information: the nature and goals of care, the anticipated planned discharge date, and the plan for the youth following discharge.

- H. Pursuant to Section 26-5.7-105(5), C.R.S., if the facility determines that a referral for additional services needs to be made, it shall make the referral to the appropriate county department of human/social services, notify the county department of the facility's relationship to the youth pursuant to section 19-1-307

(2)(e.5)(I), and notify the county department of the date when the twenty-one-day shelter time period will expire.

For youth under the age of eighteen (18), the shelter will contact the county department of residence of the parents(S) of the youth for the limited purpose of determining whether a county department is serving the youth.

- I. Pursuant to Section 26-5.7-105(4), C.R.S., when a youth under fifteen years of age is admitted to a licensed homeless youth shelter, the director of the shelter or other person in charge shall notify the county department of residence of the parents of the youth within seventy-two (72) hours of the youth's admission.
- J. Pursuant to Section 26-5.7-105(7), C.R.S., if a youth who is at least eleven (11) years of age but less than fifteen (15) years of age has been served up to twenty-one (21) consecutive days and returns again to the licensed homeless youth shelter after leaving the shelter, the director of the shelter or other person in charge shall notify the county department of residence of the parents of the youth within seventy-two (72) hours of the youth's admission.
- K. If the shelter staff know the youth is away from home without permission, the shelter shall notify the youth's parent or law enforcement pursuant to Section 26-5.7-106, C.R.S.

7.715.44 Youths Rights

- A. The shelter shall have written policies and procedures that address and ensure the availability of each of the following core rights for youth in residence. These rights may not be restricted or denied by the shelter.
 - 1. Every youth has the right to enjoy freedom of thought, conscience, cultural and ethnic practice, and religion.
 - 2. Every youth has the right to a reasonable degree of privacy.
 - 3. Every youth has the right to have his or her opinions heard and considered, to the greatest extent possible, when any decisions are being made affecting his/her life.
 - 4. Every youth has the right to receive appropriate and reasonable adult guidance, support and supervision.
 - 5. Every youth has the right to be free from physical abuse or neglect and inhumane treatment. Every youth has the right to be protected from all forms of sexual exploitation.
 - 6. Every youth has the right to receive adequate, appropriate, and timely emergency medical care.

7. Every youth has the right to receive adequate and appropriate food, clothing, and housing.
 8. Every youth has the right to live in clean, safe surroundings.
 9. Every youth has the right to participate in an educational program that will maximize his/her potential in accordance with existing law.
 10. Every youth has the right to communicate with others outside the shelter, such as a parent or guardian, caseworker, attorney or guardian ad litem and/ or counsel for youth, current therapist, physician, religious advisor, and, if appropriate, probation officer.
- B. The following youth's rights may be limited to reasonable periods during the day or restricted according to written policies of the shelter to ensure the protection of the youth, staff, and program from unreasonable and unnecessary intrusions and disruptions and from health and safety hazards.
1. Every youth has the right to have access to letter-writing materials, including postage, and to have staff members of the shelter assist him/her if unable to write, prepare, and mail correspondence.
 2. Every youth has the right to have access to telephones to both make and receive local emergency telephone calls.
- C. Written policies that restrict or limit a youth's rights as listed at Section 7.715.44, B, must include at a minimum the requirement that the shelter notify the youth at the time of admission of any policy that would limit or restrict a youth's rights. The notification must be communicated in a language or mode of communication the youth can understand and, if possible, be signed by the youth.
- D. If the shelter enforces any restrictions upon the youth's rights as listed at Section 7.715.44, B, the shelter must, in compliance with the written policy and procedure of the shelter
1. Inform the youth in a language or mode of communication the youth can understand, of the conditions of and reasons for restriction or termination of his/her rights.
 2. Place a written report summarizing the conditions of and reasons for restriction, denial, or termination of the youth's rights in that youth's case record along with information about if or when the youth's right(s) can be restored. This Information must be made available to the youth upon request.
 3. When a restriction of a youth's rights affects another youth at the shelter, the youth shall be informed, in a language or mode of communication the individual can understand, of the conditions of and reasons for the action.

7.715.45 Youth's Grievance Procedure

- A. The shelter must establish a written grievance procedure that provides adequate due process safeguards, spells out the appeal process, and assures that youth and parent(s) or guardian(s) are entitled to report any grievance and shall not be subject to any adverse action as a result of filing the grievance.
 - 1. The shelter must follow grievance procedures without alteration, interference, or unreasonable delay.
 - 2. If a grievance is filed with the shelter, the grievance shall be recorded in the youth's record along with the investigation findings and resulting action taken by the shelter.
- B. A list of the youth's rights and the grievance procedures must be provided and explained to the youth and the parent or guardian in a language or manner of communication that they can understand.

7.715.46 Discipline [Rev. eff. 6/1/12]

- A. The shelter shall have written policies and procedures regarding discipline that must be explained to all youth, parent(s), guardian(s), staff, and placing agencies. These policies must include positive responses to a youth's appropriate behavior.
- B. Discipline shall be constructive or educational in nature and may include talking with the youth about the situation, praise for appropriate behavior, diversion, separation from the problem situation, and withholding privileges.
- C. Basic rights shall not be denied as a disciplinary measure.
- D. Youth in care at the shelter shall not discipline other residents.
- E. A shelter shall prohibit all cruel and unusual discipline including, but not limited to, the following:
 - 1. Any type of physical hitting or any type of physical punishment inflicted in any manner upon the body of the youth such as spanking, striking, swatting, punching, shaking, biting, hair pulling, roughly handling a youth, striking with an inanimate object, or any humiliating or frightening method of discipline to control the actions of any youth or group of youth.
 - 2. Discipline that is designed to, or likely to, cause physical pain.
 - 3. Physical exercises such as running laps, push-ups, or carrying heavy rocks, bricks, lumber or other heavy items when used solely as a means of punishment.

4. Assignment of physically strenuous or harsh work that could result in harm to the youth.
5. Requiring or forcing a youth to take an uncomfortable position such as squatting or bending, or requiring a youth to stay in a position for an extended length of time such as standing with nose to the wall, holding hands over head, or sitting in a cross-legged position on the floor, or requiring or forcing a youth to repeat physical movements when used solely as a means of punishment.
6. Group discipline except in accordance with the shelter's written policy and these rules.
7. Verbal abuse or derogatory remarks about the youth, his/her family, his/her race, religion, or cultural background.
8. Denial of any essential/basic program service solely for disciplinary purposes. This does not prohibit the shelter from requesting the youth to leave the shelter because of inappropriate behavior.
9. Deprivation of meals or snacks to a youth that has not been requested to leave the shelter.
10. Releasing noxious, toxic, or otherwise unpleasant sprays, mists, or aerosol substances in proximity to the youth's face.
11. Denial of sleep.
12. Requiring the youth to remain silent for a period of time inconsistent with the youth's age, developmental level, or medical condition.
13. Denial of shelter, clothing or bedding to a youth that has not been requested to leave the shelter.
14. Withholding of emotional response or stimulation.
15. Use of physical management, seclusion or restraint as discipline for a youth.

7.715.47 Non-Discrimination

In compliance with Section 7.701.14, the shelter shall not discriminate against a youth based upon race, religion, age, gender or disability.

7.715.48 RELIGION AND CULTURE

- A. The shelter shall demonstrate consideration for, and sensitivity to, the cultural and religious backgrounds of youth in care.

- B. Youth in care at the shelter shall be allowed and encouraged to celebrate their cultural and religious holidays.
- C. A youth may be invited to participate in religious and cultural activities of the shelter.
- D. A youth shall not be coerced or forced to participate in the religious activities of a shelter or to attend religious services.
- E. Any form of religious or cultural intervention used by the shelter to control or change a youth's behavior, or treat or heal a medical condition, must be approved, in writing, by the legal guardian(s) of the youth prior to the use of the intervention.
- F. A shelter cannot deny medical care to a youth because of religious beliefs.

7.715.5 PERSONAL CARE OF THE YOUTH

7.715.51 Medical and Health Services

- A. The shelter shall ensure the availability of emergency medical care for each youth in care.
- B. Whenever indicated, a youth shall be referred to an appropriate specialist for either further assessment or treatment.
- C. At all times there shall be first aid supplies readily available at the shelter and at least one (1) person present at the shelter or with the youth who is certified in first aid, or the equivalent, and CPR for all ages of youth in care.
- D. The shelter, in conjunction with the parent(s) or guardian(s), shall make every effort to ensure that a youth needing corrective devices such as glasses, hearing aids, etc., is provided with the necessary equipment.
- E. In a potentially life.-threatening situation, the shelter shall refer the youth's care to the appropriate medical and legal authority.
- F. Administration of any medication at the shelter shall be done only by a person licensed and authorized by law and administered according to the written policies of the shelter.
- G. All medication to be administered must be kept in a clean storage area inaccessible to the youth and stored according to pharmacy instructions.
- H. All prescribed medications shall be administered only upon the written prescription of a physician.

**7.715.6 FOOD, NUTRITION, PERSONAL HYGIENE, DAILY ROUTINE
CLOTHING AND PERSONAL BELONGINGS**

7.715.61 Food and Nutrition

- A. The shelter shall provide nutritious foods in the variety and amounts as appropriate for the age, appetite, and activity of each youth in care.
- B. At least three nourishing, wholesome, well-balanced meals a day shall be offered at regular intervals.
- C. Menus shall be planned at least one week at a time, shall be dated, posted and filed for at least one month.
- D. Youth shall be encouraged to eat a variety of the food served but shall not be subjected to undue coercion, including forced feeding, or punished for refusal to eat.
- E. All food shall be from sources considered satisfactory by the health authority. All foods shall be stored, prepared, and served in such a manner as to be clean, and wholesome.
- F. Youth must not be given foods that are contrary to their religious beliefs, or of their family, or are known to cause an allergic reaction or a health hazard.
- G. Water shall be readily accessible to youth by means of an approved water fountain or single-service cups.
- H. Common drinking cups shall not be permitted.

7.715.62 Personal Hygiene and Daily Routine

- A. The shelter shall establish procedures to ensure that youth receive training in good habits of personal care, hygiene, and grooming appropriate to their age, gender, race and culture.
- B. There shall be supervision by staff to provide for proper grooming and physical cleanliness of the youth.
- C. The shelter shall ensure that youth are provided with necessary and appropriate toiletry items, including clean, individual towels and washcloths, toothbrush, toothpaste, comb, and shampoo.

7.715.63 Clothing and Personal Belongings

- A. The shelter shall allow a youth in care to bring his/her personal belongings to the program, as defined by the shelter policy, and to acquire belongings of his/her own in accordance with the youth's plan. However, the shelter shall, as

necessary, limit or supervise the use of these items while the youth is in care. Where extraordinary limitations are imposed, the youth shall be informed of the reasons, in a language or manner of communication the youth can understand. The decision and reasons shall be recorded in the youth's case record.

- B. Provisions shall be made for the protection of the youth's property while the youth is residing at the shelter. If the shelter safeguards any money, valuable: or personal property for a youth, the shelter shall maintain an inventory of such possessions. A full return or accounting of such possessions shall be made upon discharge of the youth from the shelter.
- C. The shelter shall ensure that each youth in care has adequate clean, proper-fitting, and seasonable clothing as required for health, comfort, and physical well-being and as appropriate to age, gender, individual needs, culture, and ethnicity.
- D. The wardrobe for each youth shall go with him/her at time of discharge.

7.715.7 LIVING SPACES, EQUIPMENT, AND NECESSARY FACILITIES

7.715.71 Living Spaces and Equipment Necessary in a Shelter [Rev. eff. 9/15/12]

- A. Youth eleven to eighteen years of age may be housed in the same area in a shelter. Youth eighteen (18) to twenty-one may be housed in the same area in a shelter. Youth under the age of eighteen may not be housed in the same area as youth over the age of eighteen.
- B. There shall be separate sleeping rooms for boys and girls. Sleeping rooms above or below the floor of exit travel shall not be used for sleeping purposes for youth who are non-ambulatory.
- C. Each youth shall be provided suitable sleeping equipment consisting of individual beds, or bunks, complete with mattresses in good repair, or individual cots. All sleeping equipment shall be constructed so as to facilitate cleaning while in use by youth and upon each change of occupancy. Triple-deck bunks are prohibited. Sleeping equipment being used by youth shall have, clean sheets, pillows, pillowcases and blankets.
- D. Each room of occupancy shall be well-lighted and adequately ventilated.
- E. Staff rooms shall be located on the same floor or in the general area of youth's sleeping rooms so that the youth care staff can supervise youth and be readily accessible when needed.
- F. There shall be a designated space distinct from youth's living areas to serve as an administrative office.

- G. There shall be a designated space distinct from youth's living areas to allow for private discussions and/or counseling sessions.
- H. Each shelter shall have a telephone. Each separate living unit within a shelter shall have 24-hour telephone service or an intercom system connected with an outside telephone service. Emergency telephone numbers, including fire, police, physician, poison control, health agency, and ambulance shall be conspicuously posted adjacent to each telephone.

7.715.72 Dining, Kitchen, Laundry, Bathroom Facilities

- A. The dining area, whether located in the living unit or in a separate central dining room in the shelter, shall meet the following requirements:
 - 1. It shall be clean, well-lighted, properly heated, and ventilated.
 - 2. Walls and floors shall be of materials that are easy to keep dean.
- B. The kitchen shall be designed and equipped to meet the requirements of efficiency, sanitation, fire safety, and comfortable working conditions. There shall be:
 - 1. Adequate space for receiving, storage, refrigeration, and preparation of food. Storage space shall be dean and well-ventilated, and containers of food shall be covered and stored above the floor on shelves or other clean surfaces.
 - 2. Provision for daily disposal of garbage and other refuse.
 - 3. Separate storage of poisonous and toxic materials from food. Such materials shall be labeled and used only in ways that will neither contaminate food nor be hazardous to staff or youth.
 - 4. Mechanical dish washing equipment or other approved methods of dish washing in accordance with requirements of the State Department of Public Health and Environment.
 - 5. Hand washing and toilet facilities for use of kitchen staff shall be readily accessible.
- C. Bath and toilet rooms shall be constructed of easily cleanable material. All surfaces shall be maintained in good repair. Toilet and bathing facilities shall have doors or partitions.

7.715.8 BUILDING, FIRE SAFETY AND TRANSPORTATION

7.715.81 Building Site

- A. The shelter must be located in an area that is readily accessible to health resources, public and private utilities, adequate and safe water supplies, sewage disposal, and fire and police protection.
- B. The shelter site shall be located in accordance with local zoning department requirements.
- C. The entire premises of the shelter are subject to inspection for licensing purposes, including but not limited to the residence where care is to be provided, the grounds surrounding the shelter, the basement, the attic (if accessible), any storage buildings, and a garage or carport, if applicable.
- D. If the shelter is located in the same building as, or immediately adjacent to, other facilities, or an adult treatment center, or a nursing home, it shall be so arranged that the care and activities of the youth residing in the shelter are completely separate and independent from the other facility. The shelter may not be operated on the premises of a business that might be hazardous to the health, safety, morals, or welfare of youth and the operation of the shelter.
- E. The shelter, including indoor and outdoor space, shall be maintained in a dean and safe condition free from hazards to health and safety.
- F. There shall be outdoor space available to provide some recreation area for south.

7.715.82 Building Maintenance

- A. Buildings shall be Kept in good repair and maintained in a safe, dean, and sanitary condition. Good housekeeping must be observed in all areas at all times.
- B. All areas of the shelter available to youth's activities including equipment, materials and furnishings shall be of sturdy, safe construction, easy to dean, and free of hazards, such as sharp points or comers, splinters, protruding nails, broken play and recreational equipment or paint that contains lead or other poisonous materials and might be dangerous to the life or health of youth.
- C. Closets, attics, basements, cellars, and furnace rooms shall be kept free from accumulation of significant amounts of extraneous materials such as furnishings, newspapers, or magazines that could pose a fire or health hazard.
- D. Provision shall be made for collection, storage, and disposal of refuse in an approved manner to prevent nuisance conditions.

7.715.83 Building Safety

- A. Egress from each dwelling unit sleeping room, and dormitory shall be available directly to the outside or to a common hallway leading directly to the outside.

- B. In every building or structure, exits shall be so arranged and maintained as to provide free and unobstructed egress from all parts of the building or structure at all times when it is occupied. No lock or fastening to prevent free escape from the inside of any building shall be installed.
- C. Every exit shall be clearly visible or the route to reach it shall be conspicuously indicated in such a manner that every occupant of every building or structure who is physically and mentally capable will readily know the direction of escape from any point. Each path of escape shall be so arranged or marked that the way to a place of safety outside is unmistakable.
- D. In every building or structure adequate and reliable illumination shall be provided for all exit facilities.
- E. Every building or structure shall be so constructed, arranged, equipped, maintained, and operated as to avoid undue danger to the lives and safety of its occupants from fire, smoke, fumes, or resulting panic during the period of time reasonably necessary for escape from the building or structure in case of fire or other emergency.
- F. The local fire department shall determine the adequacy of exits and other measures for life safety.

7.715.84 Fire Safety [Rev. eff. 6/1/12]

- A. Fire hazards, such as defective electrical appliances and electric cords, dangerous or defective heating equipment or flammable material stored in such a manner as to create a risk of fire shall be corrected or eliminated.
- B. The shelter shall contain at least one U.L.-approved fire extinguisher, highly visible, easily accessible, and in working condition, weighing not less than five pounds, that has a rating of 2A, 10BC. This requirement may be waived where more extensive fire-control measures are required by a local fire department.
- C. A smoke detector, in working condition, must be installed on each level of the shelter where care occurs and near sleeping areas.
- D. No gas or electric space heaters, open-flame gas or oil stoves, hot plates, or unvented heaters shall be used in the shelter for heating purposes.
- E. Flammables, aerosol paints, insecticides, chemicals, and other dangerous materials shall be locked or stored so they are inaccessible to youth and must be stored in areas separate from sleeping or living areas. Flammables shall be stored in an approved container.
- F. Heating devices such as radiators, registers, fireplaces, and steam and hot water pipes that pose a fire or burn hazard to youth shall be screened or otherwise protected.

- G. Flammable material must not be stored near a furnace, hot water heater, or other heating device.
- H. There shall be no candles or other burnable objects permitted in sleeping areas.
- I. Exit doors shall be obvious. No lock or fastening to prevent free escape from the inside of any room used by the youth shall be permitted.
- J. Exit routes shall be kept free of discarded furniture, furnishings, laundry, and stacks of newspapers or magazines that could interfere with the prompt evacuation of the shelter.

7.715.85 Emergency Drills

- A. There shall be written procedures for staff and youth to follow in case of emergency or disaster. These procedures shall be developed by the shelter with the assistance of qualified fire and safety personnel and shall include provisions for staff roles and responsibilities during an emergency, evacuation of the shelter, and the assignment of a central meeting place where each individual may be accounted for.
- B. Fire exit drills must be held regularly.
- C. Drills must be held at unexpected times and under varying conditions to simulate the conditions of an actual fire.
- D. Drills must emphasize orderly evacuation under proper discipline rather than speed. Running or horseplay shall not be permitted.
- E. Drills must include suitable procedures for ensuring that all persons in the shelter actually participate.
- F. A record of fire drills held over the past 12 months must be maintained by the shelter.
- G. Fire alarm facilities shall be regularly used in the conduct of drills.
- H. The shelter shall make special provisions for the evacuation of any physically handicapped youth in the shelter.
- I. The shelter shall take special care to help emotionally disturbed or perceptually handicapped youth understand the nature of such drills.
- J. If appropriate to the location of the shelter, tornado drills must be held often enough so that all occupants are familiar with the drill procedure and conduct during a drill is a matter of established routine. A record of tornado drills held over the last 12 months must be maintained by the shelter.

7.715.86 General Comfort and Safety

- A. AH hazardous chemicals, tools, and other equipment, including matches, plastic bags, paints, gasoline, medicines, insecticides, and cleaning and laundry materials, shall be stored in a safe manner. Products which could cause poisoning or contamination shall not be stored in rooms or areas where food is stored or prepared.
- B. A shelter shall not maintain any firearms or air rifles on the grounds or within the structure of the shelter. A shelter shall not permit any staff member or youth to be in possession of any firearm on the grounds or within the structures of the shelter. Any other weapons such as bows, hunting knives and hunting sling shots must be unstrung at all times and must be locked and inaccessible to youth. Chemical weapons, even when carried by staff members to and from work for protection, shall be locked when present at the shelter. Weapons must not be transported in any vehicle in which youth are riding.
- C. Water from any source other than a regular municipal water supply shall be tested annually for compliance with water quality requirements. (Sterilized containers for free laboratory tests of drinking water may be secured from the county health department or by writing to the Colorado Department of Public Health and Environment, 4210 East 11th Avenue, Denver. Colorado 80220.)
- D. The shelter shall be equipped with adequate light, heat, ventilation, and plumbing for safe and comfortable occupancy.
- E. The shelter must be equipped with hot and cold running water.
- F. All garbage, refuse and other wastes shall be kept in a suitable receptacle and disposed of in such a manner as not to constitute a health hazard or nuisance.
- G. All stairways containing more than four steps shall be equipped with a handrail.
- H. The shelter shall have a telephone, and emergency numbers shall be posted near the telephone, including those related to medical care, fire, law enforcement and poison control where available.

7.715.87 Transportation

- A. A shelter shall ensure that each youth is provided with the transportation necessary for implementing the youth's individual plan.
- B. A shelter shall have means of transporting youth in cases of emergency.
- C. Any vehicle used by the shelter in transporting youth in care, whether such vehicle is operated by a staff member or any other person acting on behalf of the shelter, shall be properly licensed, and the vehicle shall be maintained in accordance with Colorado law.

- D. Any staff member or other person acting on behalf of the shelter operating a vehicle for purpose of transporting youth shall be properly licensed to operate the class of vehicle in accordance with Colorado law.
- E. Youth under 16 years of age must be properly fastened into a restraint system that conforms to Section 42-4-236, C.R.S.
- F. A shelter shall not allow the number of persons in any vehicle used to transport youth to exceed the number of available seats and seatbelts in the vehicle.
- G. The vehicle shall be enclosed.
- H. A shelter shall ascertain the nature of any need or problem of a youth which might cause difficulty during transportation, such as seizures or a tendency toward motion sickness. The shelter shall communicate this information to the driver of any vehicle transporting youth in care.

7.715.9 RECORDS, REPORTS AND CONFIDENTIALITY

7.715.91 Confidentiality

- A. The shelter shall have a policy as to the maintenance, storage and confidentiality of records.
- B. Records shall be the property of the shelter and shall be protected against loss, tampering, or unauthorized use.
- C. Facts learned about youth and their families shall be kept confidential, with the following exceptions:
 - 1. In medical emergencies, and then only when the assistance and/or expertise is required of that unauthorized person; or
 - 2. To the youth, his/her parent(s) or guardian(s) and their respective legal counsel(s), a court having jurisdiction over the youth, or an authorized public official, or licensing representative in performance of his/her mandated duties; or
 - 3. If the parent(s) or guardian(s) has given voluntary, written consent.
 - 4. Mandatory reporting of child abuse as required by law; and;
 - 5. Reporting to the appropriate authority a youth's attempt to harm his/herself or others.
 - 6. A youth's records may only be released upon the written consent of the youth.

7.715.92 Records

- A. The shelter shall maintain complete records as required for the licensing of the shelter in accordance with the rules regulating the shelter.
- B. Records for youth shall be retained for at least three years. Retention of records for a longer period may be desirable when they reflect an accident, injury or other unusual circumstance.
- C. A record of admission shall be completed for each youth in care at the time of admission. The admission record shall be maintained at the shelter where the youth resides and shall contain:
 - 1. Youth's name, date and place of birth (verified by a birth certificate when possible), gender, race, religious preferences, and date of admission.
 - 2. Psychiatric and psychological reports, when available.
 - 3. Summary recording of significant contacts with parent(s), guardian [s] and other involved agencies.
- D. When a youth is removed or discharged from placement, the shelter shall complete a summary of the youth's discharge from the shelter which includes at a minimum:
 - 1. The date of the discharge of the-youth from the shelter.
 - 2. The plan for the youth.
 - 3. Circumstances which led to an unplanned discharge.
- E. Copies of a youth's file, including discharge information, shall be provided to parent(s) or guardian(s) upon request or to others by written consent pursuant to Section 7.715.91.
- F. A copy of all policies developed by the shelter shall be maintained for at least three years.

7.715.93 Reports

- A. The shelter shall immediately attempt to notify the youth's parent(s), guardian(s), and/or the responsible agency of any serious illness or serious injury resulting in medical treatment away from the shelter, hospitalization or death involving a youth in care.
- B. The shelter must immediately report in writing to the licensing authority any accidents, injuries, serious illnesses, or fatalities that occurred at the shelter and that resulted in emergency medical treatment away from the shelter, hospitalization or death. The report must be made within 48 hours after the accident, illness, or death occurred.

- C. A report about a death must include:
 - 1. The youth's name, birth date, address, and telephone number.
 - 2. If known, the names of the youth's parent(s) or guardian(s) and their address and telephone number if different from that of the youth.
 - 3. Date of the fatality.
 - 4. Brief description of the incident or illness leading to the death.
 - 5. Names and addresses of witnesses or persons who were with the youth at the time of death.
 - 6. Name and address of police department or authority to whom the report was made.
- D. The shelter must immediately report in writing to the department any change in the status of the shelter that would affect care of youth.
- E. The facility must immediately report in writing to the department any legal action brought against a shelter regarding any youth, staff, or conduct of the shelter at the time of initiation of such legal action

7.716 COLORADO SCHOOL READINESS QUALITY IMPROVEMENT PROGRAM
[Repealed eff. 03/05/2026]

7.717 EARLY CHILDHOOD COUNCILS [Repealed eff. 03/05/2026]

7.718 (None) [Repealed eff. 03/05/2026]

7.719 RULES REGULATING SPECIAL ACTIVITIES [Rev. eff. 6/1/07]

These rules for Special Activities shall apply to School-Age Child Care Centers, Residential Child Care Facilities, and Children's Resident Camps.

7.719.1 GENERAL PROVISIONS [Rev. eff. 6/1/07]

- A. There shall be a written program that reflects the purpose of the child care facility, including a list of activities at the child care facility. The written program must be provided to parents.
- B. Parents shall be given the opportunity to indicate to child care facility staff whether they do not wish their child to participate in a special activity (see Section 7.711.61, A. 10).

- C. Each phase of the child care facility program shall be under the supervision of a resident qualified staff member who shall be responsible for health and safety precautions. Verification of experience and/or certification shall be in the staff members personnel files at the child care facility.
- D. If the child care facility participates in special activities other than those for which rules are found in this section, such as ballooning or winter camping, the child care facility shall develop and follow a written plan which includes at least the following:
 - 1. The qualifications of the supervisor of the activity.
 - 2. The qualifications of any other staff members necessary for proper supervision of the activity.
 - 3. The number of necessary staff members needed to supervise the activity.
 - 4. Conditions under which a child may participate in the activity, such as age or skill level of the child.
 - 5. Any special equipment necessary, its supply and condition.
 - 6. Access to medical treatment.
 - 7. Development of an emergency plan.
- E. Paint ball activities where children shoot paint balls at other children are prohibited at a child care facility.
- F. The staff member supervising special activities shall possess evidence of appropriate experience, training, and/or certification in the program specialty Said staff member shall be present at the site of the activity whenever the activity is being earned out unless other wise indicated in these rules.
- G. The qualified supervising staff member of special activities shall have the following duties:
 - 1. Direct training of other staff members working in the activity.
 - 2. Assign duties to staff members.
 - 3. Assure that all necessary equipment is complete, in good repair, and safe to use.
 - 4. Assure that environmental hazards are not sever enough to cause danger to children.
- H. Rules shall be reviewed with children at the beginning of each activity.

- I. First Aid supplies shall be available at each special activity site.
- J. The staff to child ratio for each type of facility must be followed according to rules for that facility regardless of activity unless the ratio is different for the specified activity, in which case the activity staff to child ratio should apply.

7.719.2 WATER ACTIVITIES

7.719.21 Swimming [Rev. eff. 6/1/07]

- A. There shall be a swimming supervisor who, as a minimum, holds a current Red Cross life guard training certificate or equivalent, such as a YMCA or Boy Scout aquatics instructor's certificate. If the child care facility is offering swimming instruction, the swimming supervisor must also hold a Red Cross water safety instructor certificate or equivalent.
- B. At any time the swimming area is open, there shall be at the swimming area a staff member who holds at least a current life guard training certificate or equivalent for each thirty campers in the water. There shall be present as least one staff member for each ten children in the water. The lifeguard does not count in the staff to child ratio for supervision of children.
- C. The swimming area shall be off limits when appropriate numbers of qualified staff members are not present.
- D. If the child care facility uses a pool for which the child care facility is not responsible, the child care facility need not provide a lifeguard if there is a qualified lifeguard provided by the pool. If the pool does not provide a qualified lifeguard, staff members meeting qualifications stated at Section 7.719.2, B. must be provided by the child care facility. There shall be at least one staff lookout counselor at the pool for each ten children in the water.
- E. Swimming area rules and emergency procedures shall be posted in a visible location at the swimming area.
- F. The swimming pool or swimming area shall meet the standards of the Colorado Department of Public Health and Environment.
- G. If children are permitted to swim in a lake or pond, swimming areas shall be clearly designated
- H. Before children are permitted to swim in deep water, swimming skills must be tested by property trained staff members.
- I. There shall be a system known to child and lookout staff for checking the children when children are in the water.

- J. The following equipment must be available for use at the pool side or the take shore in which swimming is permitted:
 - 1. A rescue tube;
 - 2. Reach pole; and,
 - 3. Backboard.
- K. Where the size of the body of water makes it impossible to reach victims by reach pole, rescue tube or other rescue device, a rescue boat must be available at all times.
- L. If a child care facility has shoreline activities such as wading, fishing, ecology or nature studies, the child care facility shall have a written policy which defines qualifications of persons accompanying the group and safety, factors to be followed. Staff members shall be acquainted with the policy.
- M. In the case of a travel-trip camp, there shall be a minimum of one staff member who holds at least current Red Cross life guard training certificate or equivalent who is responsible for all swimming activities.

7.719.22 Boating, Canoeing, Sailing, and Kayaking on Flatwater [Rev. eff. 6/1/07]

- A. The boating supervisor shall hold, at a minimum:
 - 1. A current Red Cross life guard training certificate or equivalent; or,
 - 2. Boy Scout certificate; or,
 - 3. Basic small craft instructor, small craft safety, or paddle safety certificate for the type of craft which is to be supervised; or,
 - 4. Documentation of experience indicating knowledge and skill in teaching and supervision specific to the watercraft activities to be conducted.
- B. The boating supervisor, or staff member equally qualified who has been trained by the boating supervisor, must be on site during the activity.
- C. Other staff members shall have appropriate experience and training for the type of craft to be utilized.
- D. Whenever children are on the water they shall be wearing a United States Coast Guard approved personal notation device appropriate to the weight of the child.
- E. There shall be a minimum of two lookout staff members at the shoreline and/or on the water at any time when children are on the water in boating, canoeing, kayaking or sailing activities. Hazards such as the size of the lake, the skill of the

children, the conditions of the water, and the temperature of the water, shall be taken into account by the supervisor of the activity when determining the number and location of lookout staff necessary with the children, but there shall never be fewer staff with the children than those required at Section 7.711.23, D.

- F. Except for kayaking, there shall be a staff member in any boat which holds one or more children under seven years old.
- G. At no time shall the occupancy of the craft exceed the capacity established for the craft by the United States Coast Guard standards.
- H. There shall be a warning device, such as a loud whistle, air horn, or other audible signal device, which can readily be heard by persons on the water that indicates the need for children and staff to return to the facility.
- I. Where the size and depth of the Body of water indicates, there shall be a rescue boat in close proximity to where the activity takes place. This rescue boat shall be in good repair and shall contain appropriate equipment, such as a rescue tube, reach pole, extra oar, or paddle.
- J. Water craft shall not enter a swimming area when swimmers are in the water.

7.719.23 Boating, Canoeing, Tubing, and Kayaking on Class I or II Moving Water [Rev. eff. 6/1/07]

- A. The boating supervisor shall hold, at a minimum:
 - 1. Current Red Cross life guard training certificate or equivalent; or,
 - 2. Boy Scout certificate; or,
 - 3. Basic small craft instructor certificate for the type of craft which is to be supervised; or,
 - 4. Documentation of experience indicating knowledge and skill in teaching and supervision specific to the watercraft activities to be conducted.
- B. The boating supervisor must be on site during the activity.
- C. Children shall only canoe, tube, or kayak on Class II or less water.
- D. Supervising staff must be experienced and knowledgeable about the river being used, including the height and speed of the river.
- E. The child care facility must have a written policy on evaluating the safety of the river. Supervising staff must be trained on the policy.
- F. Each child shall wear a United States Coast Guard approved personal flotation device whenever they are on the moving water.

- G. The supervisor of this activity shall be trained in Red Cross standard First Aid and safety, and CPR.
- H. The supervisor shall be familiar with rescue techniques with canoes, kayaks, and tubes on moving water and shall train children in these techniques.
- I. Rescue equipment appropriate to the activity shall be available, such as rope throw bag and rescue tubes.

7.719.24 White Water Rafting on Class III and IV Rivers (Classes of rivers are those as defined by the International Scale of River Difficulty) [Rev. eff. 6/1/07]

- A. If the child care facility operates white water rafting, the child care facility must be licensed by the Division of Parks and Outdoor Recreation as a river outfitter.
- B. If a child care facility provides a white water rafting experience by purchase from a river outfitter, the license of the outfitter must be valid.

7.719.3 ARCHERY AND RIFLERY

7.719.31 Archery [Rev. eff. 6/1/07]

- A. The archery supervisor shall have certification, documented training or experience from a recognized organization or certifying body for the type of activities offered.
- B. The archery range shall be free from hazards and well-marked. There shall be a clear path to the target which is not obstructed by such things as rocks, trees or branches. Traffic, trail, or other camp activities shall not be placed in the direction of the flight of the arrows.
- C. Equipment shall be maintained in safe condition. Bows and arrows shall be inspected for fractures, splinters or cracks before each use. Damaged bows and arrows shall not be utilized.
- D. Equipment shall be stored under lock and key when not in use. Bows and arrows shall be used only in the specified archery area.
- E. If the child care facility has field archery, a procedure shall be established and posted to provide for the safety of the archers, including issuance of arrows at check-in point of the archery trail, check in of archer at the beginning of the archery trail, and check out when archer has completed the trail.
- F. The archery supervisor or a staff person trained and authorized by the archery supervisor must be present at all times when children are present at the archery range or field.

- G. All archers shall use the same firing line. Arrows shall be issued only at the firing line.
- H. Arrows shall be nocked to bow string after shooters are on the firing line and after the signal to shoot has been given.
- I. Before arrows are released, shooters shall have a definite target.
- J. Movement must be controlled by a supervising staff member. All persons must stay behind the firing line until the signal to retrieve arrows is given. All arrows shall be retrieved at the same time.

7.719.32 Riflery [Rev. eff. 6/1/07]

- A. The riflery supervisor shall hold a National Rifle Association instructor's or assistance instructor's certification in rifle shooting or equivalent certification from a national organization or shall have verified experience equivalent to that necessary to obtain the National Rifle Association Fire Arm certification.
- B. If the riflery supervisor is not present at the rifle range whenever children are firing guns, the staff person(s) trained by the riflery supervisor must be present at all times when children are present.
- C. The rifle range shall be free from all hazards, away from other activities and traffic of any type; shall be well marked with danger signs or flags; all blind approaches shall be fenced or blocked off.
- D. The range shall be constructed with an appropriately designed bullet-stop so that all bullets will be stopped behind the targets. The bullet-stop shall be free of trees, rocks, boulders, or other objects which may cause a bullet to ricochet away from the bullet-stop.
- E. There shall be a well-defined firing line which shall be level with the targets and elevated off the ground. A minimum space of five feet between firing points shall be established or firing points separated by a permanent divider. Targets must be designed to minimize potential for ricochet. Targets cannot depict human form.
- F. Only the following types of guns shall be permitted:
 - 1. .22 caliber rimfire, single-shot, bolt-action rifles having no trigger modification other than the factory setting.
 - 2. Pneumatic spring-type and CO₂ air guns may be either .22 caliber or .177 (BB size).
- G. Proper condition of the firearms shall be maintained by inspection before and after usage, cleaning as necessary. Firearms that do not function properly shall be repaired and tested before usage.

- H. Instruction on the use of firearms shall be presented to the children prior to the use of the rifle range.
- I. No more than five cartridges at a time shall be distributed to a child by the responsible supervising staff member and issued only at the firing line.
- J. Firing shall be permitted at the firing line only. Observers shall remain behind firing line.
- K. Actions of uncased firearms shall be kept open except when on firing line ready to fire.
- L. All firearms shall be unloaded immediately upon the command "cease firing" regardless of when this command is given. Actions shall remain open until further commands are given.
- M. On ranges where shooters must go down range to change targets and score: movement must be controlled by the supervising staff member.
- N. All spent or unspent cartridges must be returned to the supervising staff member.

7.719.4 HORSEBACK RIDING [Rev. eff. 6/1/07]

- A. The horseback riding supervisor shall have completed at least one of the following:
 - 1. Certificate from nationally recognized organization or riding school.
 - 2. Written verification of successful experience in formal horseback riding instruction.
- B. The horseback riding supervisor shall train a sufficient number of child care facility riding staff members in the supervision of children in the horseback riding program for the anticipated size of the riding program.
- C. Child care facility riding staff shall be trained by the horseback riding supervisor in emergency procedures appropriate to the horseback riding activity.
- D. At least two trained riding child care facility staff members, one of whom holds a current American Red Cross standard First Aid and safety certificate or equivalent, shall accompany each trail excursion. If the horseback ride is more than one hour from emergency medical services, at least one staff member shall be trained in wilderness first aid training. If the horseback ride is for seven or more nights and is more than one hour away from emergency medical services, there must be at least one staff member with each group of children with wilderness first responder training, CPR, and medication administration training. If more than twenty children participate in the trail excursion, there shall be a

- trained riding child care facility staff member assigned for each additional ten or fewer riders.
- E. First Aid supplies shall be carried on each trail excursion and available at each horseback riding ring/arena.
 - F. No person is allowed in the riding area unless the horseback riding supervisor or a trained riding child care facility staff member is present.
 - G. The riding supervisor shall determine the child's riding experience and level of skill and must take these into account in assigning which horse each child should ride and determining the type of riding activity in which each child should engage. Children shall be given instruction in basic safety, which shall include at least the following: riding rules in the ring and on the trail, how to approach, mount and dismount.
 - H. Children shall be appropriately dressed for riding, which shall include shoes or boots and long pants. The riding supervisor must evaluate the footgear of each child and make the stirrups safe for each child's shoe or boot.
 - I. Protective head gear/helmets are mandatory for children ring riding and on trail rides.
 - J. Parents must be notified in advance of what type of protective gear is used by the child care facility. If children bring helmets from home, they must be specifically designed for equestrian use, worn correctly, and in good condition.
 - K. The horseback riding equipment shall be in good condition, properly sized and adjusted for each rider.
 - L. The horse barn or stable, ring, and commonly used trail(s) shall be in good repair and free of dangerous obstructions.
 - M. Horses shall be cared for with evidence of an adequate feeding schedule and a means to care for sick horses.
 - N. Horses shall not be permitted in the other designated activity areas.

7.719.5 TRAMPOLINE [Rev. eff. 6/1/07]

- A. The trampoline supervisor shall have documented formal training and experience in use of trampoline and knowledge of safety and spotting techniques.
- B. Trampolines shall be equipped with pads along the sides and shall be kept in good repair.
- C. No person shall be on the trampoline unless a trampoline supervisor is present and spotters are present on all four sides of the trampoline.

- D. Trampolines shall be secured from unauthorized use by any person.
- E. The child shall dismount the trampoline by sitting on the edge and sliding off. No child shall jump off the trampoline.
- F. Spotters shall be posted on four sides of each trampoline at all times. Spotters shall not stand, sit, or lie on trampoline, but shall stand in a position of readiness, watching the jumper at all times.

7.719.6 CLIMBING ACTIVITIES

7.719.61 Rock Climbing and Ropes Courses [Rev. eff. 6/1/07]

- A. When a child care facility offers basic/single-pitch rock climbing or advanced/multi-pitched climbing, which includes such topics as the care and use of basic equipment, knots, anchors and belays, verbal signals, safety measures, basic climbing holds and moves, and techniques of rappelling, the following rules must be complied with:
 - 1. The climbing supervisor shall:
 - a. Be at least 18 years old;
 - b. Have certification or documented experience in knots, anchors, safety zones, verbal signals, belaying, rappelling, and safe tie-ins, or training or experience from a recognized organization, such as the Association for Challenge Course Technology or certifying body for the type of activities offered; and,
 - c. Have at least six weeks' experience in a management or supervisory capacity in similar types of programs.
 - 2. A climbing instructor shall have verified knowledge of technical climbing by completion of a course or climbing school, or a minimum of ten hours of instruction.
 - 3. At least two climbing instructors must be present at the climbing site at all times.
 - 4. There shall be one climbing instructor for each six (6) climbers or two climbing instructors for thirteen (13) children.
 - 5. There shall be a staff member who holds at least a current Red Cross standard First Aid and safety certificate or equivalent at the rock climbing site.

6. First Aid supplies, put together by a person knowledgeable in First Aid supplies needed for climbing activities and possible injuries, shall be present at the climbing site.
 7. No child shall be forced to participate in this activity.
 8. The climbing supervisor shall be responsible for the proper maintenance of all equipment used. Equipment shall be checked by the supervisor immediately prior to use.
 9. All rock climbing equipment shall meet industry standards and shall be maintained, visually and physically inspected, and replaced on a timely basis.
 10. Climbers must wear helmets at all times when in designated helmet zones.
 11. The child care facility shall not permit an unsupervised climb.
 12. The climbing supervisor must have knowledge of where the climb is to occur and must give approval on the day of the climb for the climb to occur.
 13. Each rock climber must be visually supervised.
 14. Children waiting to climb must be supervised by a staff member.
 15. All climbers and rappellers shall be belayed in a top rope manner by a belayer that has been instructed in proper procedures, and directly supervised until competency has been demonstrated.
- B. If the child care facility offers advanced/multi-pitched climbing, the following rules shall also be complied with:
1. The climbing supervisor accompanying participants shall:
 - a. Hold a current Red Cross standard First Aid and safety certificate or equivalent, and a current certificate for cardiopulmonary resuscitation;
 - b. Have been an instructor, under supervision, for two seasons with verifiable experience and a review of any serious accidents;
 - c. Have completed a technical climbing school or training in technical climbing with evidence by letter of such completion;

- d. Have led ten additional multi-pitched Class V climbs (the classification of the climbs as defined by the American Alpine Club) within the last two years; and,
 - e. Have knowledge of mountain rescue techniques. If the climb is more than sixty (60) minutes from emergency medical services, the climbing supervisor must hold a current wilderness First Aid training certificate or equivalent.
 - 2. The climbing instructor or the rope leader shall have:
 - a. The same training as the climbing supervisor;
 - b. Have been an instructor, under supervision, for one season with verifiable experience and a review of any serious accidents;
 - c. Completed a technical climbing school or training in technical climbing;
 - d. Led five additional multi- pitched climbs; and,
 - e. Knowledge of mountain rescue techniques. No instructor shall take campers on a climb he/she has not completed previously.
 - 3. No child will be the rope leader.
 - 4. A child who is permitted to participate in the climb must be at least thirteen (13) years old. The climbing supervisor shall assess the ability of the child as to the difficulty of the climb.
 - 5. The climbing instructor and climbing site must be approved by the climbing supervisor for each climb.
 - 6. The climbing supervisor, an equally qualified person, or two (2) equally qualified rope leaders shall be present at the climb site.
 - 7. There shall be one rope leader that is at least eighteen (18) years of age to each three climbers in an extended climb.
 - 8. First Aid equipment must be carried with the staff on each climb.
- C. If the child care facility offers high and/or low ropes courses or a climbing wall, the following rules must be complied with at all times:
 - 1. The rope supervisor must have training and experience on the type of rope course or climbing wall being used and must hold a current standard First Aid and safety certificate or, if the ropes course or climbing wall is

- more than sixty (60) minutes from definitive care, must hold a wilderness First Aid card.
2. The rope instructor must have training and experience on the type of rope course or climbing wall being used and must be supervised by the rope supervisor and must hold a current standard First Aid and safety certificate or, if the ropes course or climbing wall is more than sixty (60) minutes from definitive care, must hold a wilderness First Aid card.
 3. Ropes courses must have written evidence of annual inspection by qualified Association of Challenge Course Technology (ACCT) personnel of course elements for integrity of all hardware, materials, and equipment.
 4. Ropes courses must be inspected regularly before use by the rope supervisor or the rope instructor.
 5. All equipment and elements of a rope course or climbing wall must be safety checked prior to each use and have written records of regular inspection and maintenance of all equipment and elements utilized.
 6. Children must wear safety equipment appropriate to the size of the child and appropriate helmets when using the high ropes course or climbing wall.
 7. At all times, there must be a rope supervisor or rope instructor on the ropes course with children.
 8. Ropes courses and climbing walls must be off limits to children when a rope supervisor or rope instructor is not present.
 9. Access to ropes courses and climbing walls must be controlled by education, signs, and whatever other means are necessary to control unsupervised access.
 10. The child care facility must have written safety procedures for use of the ropes course(s) and climbing wall. Staff must be trained on the safety procedures.

7.719.7 HIKING, BACKPACKING, AND CAMPING

7.719.71 Hiking [Rev. eff. 6/1/07]

If the child care facility offers hiking activities, the following rules shall be complied with:

- A. The hiking supervisor must hold a current Red Cross standard First Aid and safety certificate or equivalent; shall have knowledge of outdoor experience and the symptoms and correct treatment procedures for hypothermia and

- dehydration; and, shall have verifiable experience in hiking and backpacking at the elevation where the hike is to take place.
- B. The staff members involved in hiking shall be trained by the supervisor and shall continually observe and monitor campers on the trail for early diagnosis and treatment of injury or illness.
 - C. When a group takes a hike within 60 minutes of definitive medical care, there must be at least one staff member currently qualified with Red Cross standard First Aid and safety training certificate or equivalent, current CPR certificate, and current training in the Department required and approved medication administration training.
 - D. When a group takes a hiking or backpacking trip where children are either more than 60 minutes away from definitive medical care, there must be at least one staff member with each group of children with current wilderness First Aid training, or equivalent, current CPR training, and current medication administration training.
 - E. At least two (2) staff members must accompany a group in hikes. From time to time, hiking groups may divide up as long as hikers are always with one staff member and staff members are in visual, verbal or electronic (radio or wireless communication) contact with each other.
 - F. In selecting the area for hiking, the hiking supervisor shall consider the hiker's age, physical condition and experience, as well as the season, weather trends, methods of evacuation, and communication.
 - G. Before participation, children must be instructed on:
 - 1. The fundamental safety procedures to follow on the trail;
 - 2. Procedures to follow if lost;
 - 3. Proper health and sanitation procedures on the trail;
 - 4. Rules governing land to be hiked;
 - 5. Potential high-risk areas; and,
 - 6. Fire precautions.
 - H. Each hiker shall be equipped with protective clothing against natural elements such as rain, snow, wind, cold, sun, and insects.
 - I. First Aid supplies, put together by a person knowledgeable in First Aid supplies needed for possible accidents and/or injuries, shall be present on each hike. The

contents of each kit shall be adequate for the number of children, the terrain, and the length of the hike.

- J. An itinerary of the hiking trip and a list of all people on the hike must be kept at the child care facility.
- K. The child care facility must have written safety procedures for hiking, including the written protocol for evacuating a child that becomes sick or injured on a hike. Staff and children must be trained on the safety procedures and protocol.

7.719.72 Backpacking and Camping [Rev. eff. 6/1/07]

- A. The backpacking and camping supervisor shall have knowledge and verifiable experience in camping and/or backpacking at the elevation where the backpacking or camping will take place.
- B. When a group is backpacking or camping within sixty (60) minutes of definitive medical care, there must be at least one staff member currently qualified with Red Cross standard First Aid training certificate or equivalent, current CPR training, and current training in the Department required and approved medication administration training.
- C. When a group is backpacking or camping where children are more than sixty (60) minutes away from definitive medical care, there must be at least one staff member with each group of children with current wilderness First Aid training or equivalent, current CPR training, and current medication administration training.
- D. If a child will require medication administration while away from the child care facility while backpacking or camping, there must be at least one staff member present with current medication administration training who has been delegated by a registered nurse to administer medication.
- E. The staff members involved in backpacking or camping shall be trained by the supervisor and shall continually observe and monitor children on the trail for early diagnosis and treatment of injuries or illness.
- F. The backpacking or camping supervisor shall consider the hiker's age, physical condition, and experience, as well as the season, weather trends, methods of evacuation and communication, and water quality and quantity in selecting the area for backpacking or camping.
- G. Children shall have a safety orientation and be instructed on the applicable precautions, such as:
 - 1. The fundamental safety procedures to follow on the trail;
 - 2. Procedures for a hiker if he/she becomes lost;

3. Proper health procedures, including the need for drinking fluids and eating appropriate foods;
 4. Sanitation procedures;
 5. Relevant rules and regulations;
 6. Potential high-risk areas which may be found on the trail;
 7. Fire danger precautions; flash floods; lightening dangers; and,
 8. Procedures when encountering wild animals.
- H. Children shall be oriented to minimum impact guidelines and techniques.
- I. Each child shall be equipped with protective clothing and equipment against anticipated natural elements such as rain, snow, wind, cold, sun, and insects.
- J. Appropriate first aid supplies shall be present on each trip. The contents of each kit shall be adequate for the number of children, the terrain, and the length of the trip.
- K. An itinerary of the trip with a list of participants must be available to parents, staff, local police jurisdictions and staff or contractors of the Colorado Department of Human Services.
- L. The child care facility must have written safety procedures for backpacking or camping, including the written protocol for evacuating a child that becomes sick or injured.

7.719.8 BIKING [Rev. eff. 6/1/07]

If a child care facility has bicycling trips either on a public road or on mountain trails, the following rules shall be complied with:

- A. The bicycling supervisor must be familiar with state laws about bicycling; be knowledgeable about the type of bicycling terrain where the bicycle trips will occur be knowledgeable about bicycling in the mountains, if applicable: shall know how to make simple bicycle repairs; and, shall hold at lest a current Red Cross standard First Aid and safety certificate or equivalent.
- B. At least two (2) staff members must accompany a group while biking. From time to time, biking groups may divide up as long as bikers are always with one staff member and staff members are in visual, verbal or electronic (radio or wireless communication) contact with each other. A bicycling supervisor or staff member equally qualified and another qualified staff member must accompany each bicycle trip. Correct staff to child ratios must be complied with at all times. There must be one staff member at the beginning and end of each bicycle group.

- C. Each bicycler shall wear head protection and the bicycle shall be equipped with brakes in good condition. Bicycles shall be in good condition, properly maintained, inspected prior to each bicycling trip, and adjusted to the size of the child riding the bicycle. Children using their own bicycles will be informed in advance, in writing, that their bicycles must be in good condition, properly maintained, inspected prior to each bicycling trip, and adjusted to the size of the child riding the bicycle.
- D. An appropriate bicycle repair kit and First Aid equipment must be taken on each trip. The First Aid supplies must be put together by a person knowledgeable in First Aid supplies needed for bike trips and possible accidents and/or injuries.
- E. The bicycling supervisor must instruct children as to emergency procedures, safe riding practices, and road and trail etiquette.
- F. The bicycling supervisor shall evaluate each child as to his/her physical capability to participate in the planned bicycling trip, keeping in mind the trip length, terrain, altitude of the trip, and weather conditions.
- G. Water/fluids must be taken on each bicycle trip.
- H. An itinerary of the biking trip and a list of all people on the biking trip must be kept at the child care facility.
- I. The child care facility must have written safety procedures of bike trips, including the written protocol for evacuating a child that becomes sick or injured on a bike trip. Staff and children must be trained on the safety procedures and protocol.

7.720 RULES REGULATING NEIGHBORHOOD YOUTH ORGANIZATIONS
[Repealed eff. 03/05/2026]

7.721 RULES REGULATING HOST FAMILY HOMES [Eff. 9/15/12]

Child Placement Agencies and county departments are authorized to certify Host Family Homes. These rules apply to those child placement agencies and county departments that choose to certify Host Family Homes.

7.721.1 DEFINITIONS [Eff. 9/15/12]

“Homeless youth” is defined in the General Rules at Section 7.701.21.

“Licensed Host Family Home” is defined in the General Rules at Section 7.701.21.

“Host home parent” is the responsible adult in the host family home that is participating in the host family home.

7.721.2 GENERAL REQUIREMENTS [Eff. 9/15/12]

- A. The certifying child placement agency or county shall be able to assure the health and safety and the proper care and treatment of the youth in the host family home.
- B. The host family home must be located in an area that is accessible to health resources, public and private utilities, adequate and safe water supplies, sewage disposal, and fire and police protection.
- C. The child placement agency or county shall have written screening procedures and make reasonable efforts to evaluate the overall emotional health and stability of each host family home applicant and/or staff member. Procedures shall include exploring for any history of youth battering, youth abuse, youth molestation, youth neglect, or previous criminal convictions.
- D. The child placement agency or county shall determine policies, procedures and case management structure that clearly defines duties and responsibilities for host home parents.
- E. The child placement agency or county shall develop an adequate emergency preparedness and management plan for host home parents.
- F. The child placement agency or county shall have on file the verification of tuberculin status of host home parents and other residents of the home. If the host family home residents have certificates of previous negative testing, the testing does not need to be repeated. If there is no certificate, the testing needs to be completed prior to host family home certification and annually thereafter.
- G. The child placement agency or county shall ensure that all staff, certified host family homes and program partners demonstrate an interest, receive training and have knowledge of positive youth development, homelessness and a concern for the proper care and well being of youth.

7.721.21 Management Requirements

7.721.211 Certifying Child Placement Agency or County [Eff. 9/15/12]

- A. The child placement agency or county is the individual, partnership, corporation or association in whom the ultimate authority and legal responsibility is vested for the conduct of the host family home.
- B. When the child placement agency or county does not include a board of directors, there shall be an advisory committee of at least two other individuals who act in an advisory capacity to the governing body. The names of the advisory committee members shall be disclosed to the Department. The advisory committee shall meet at regularly stated intervals.
- C. Minutes of the advisory committee or the board of directors shall be maintained. The minutes shall be available to the Department upon request, except that the

minutes containing confidential personnel information need not be shared with the Department.

- D. The functions of the child placement agency or county shall include:
1. The appointment of an administrator who shall be responsible to establish and implement, according to performance criteria, to the governing body, and shall delegate to him/her the executive authority and responsibility for the administration of Host family home certification according to its defined purpose.
 2. The certification of all host family homes.
 3. The formulation and regularly planned review of policies and procedures to be followed by the host family homes.
 4. As necessary, establish formal agreements with local agencies participating in the host family home. If a local agency delegates responsibility for any responsibility established by the host family home, the child placement agency or county must certify and document that the local agency meets requirements established in rule.
 5. Sixty-day notice must be given to State Department of any changes to formal agreements established by child placement agency or county.
 6. The provision of necessary emergency services and temporary shelter using host family homes; adequate financing; qualified personnel and host family home provider; and services and program functions for the welfare and safety of youth in accordance with these standards.

7.721.212 Fiscal Management [Eff. 9/15/12]

The child placement agency or county shall demonstrate that it is financially sound and manages its financial affairs prudently. All funds disbursed by the child placement agency or county shall be expended in accordance with the program objectives as specified by the governing body.

7.721.213 Insurance [Eff. 9/15/12]

- A. The child placement agency or county certifying the host family home shall determine policy and procedures on liability insurance that includes the host family home supervised by the agency.
- B. The host family home shall maintain adequate insurance and submit annually the amount of the insurance and the name and the address of the insurance agency providing the insurance for the host family home.

- C. Information about the insurance should be maintained at the child placement agency or county.
- D. Agencies and host family homes participating in the host family home operating their own transportation vehicles shall carry insurance in compliance with the minimum limits as required by the Colorado Revised Statutes.
- E. The child placement agency or county shall establish procedures to review and establish annually that all agencies and host family homes participating in the host family home meet program insurance requirements.

7.721.22 HUMAN RESOURCES

7.721.221 ADMINISTRATIVE STAFF [Eff. 9/15/12]

- A. Administrator of the child placement agency or county shall be qualified as follows:
 - 1. The administrator shall have received a Bachelor's degree from an accredited college or university and have two years of verified experience in the human services field, two of which were in a supervisory or administrative position; or the administrator shall have received a Master's degree and have two years of verified experience in the human services field, one of which was in a supervisory or administrative position.
 - 2. The administrator shall assume the following duties:
 - a. Overall direction and responsibility for the youth, program, host family home, and fiscal management.
 - b. Overall direction and responsibility of supervision of adequate staff.
 - c. The selection, roles, responsibilities and training of a capable staff member who can assume responsibility for management of host family homes in the administrator's absence.
 - d. The establishment of relationships, protocols, agreements and contact maintenance with allied agencies, services, and resources within the community.

- B. Case Manager

The child placement agency or county shall specifically designate a staff member, age twenty-one (21) or over, capable of acting as a substitute for the administrator during his/her absence. The duties and responsibilities of the substitute administrator shall be clearly defined in order to avoid confusion and conflict among other staff, provider within host family home and youth.

C. Administrative Coverage

1. The Department shall be notified in writing when a possible change in administrator is anticipated or at the time there is a change in administrator. Documents verifying the qualifications of the acting/interim or new administrator, including official transcripts and documents verifying work experience, shall be submitted to the Department.
2. The administrator or the staff member to whom the administrator has delegated responsibility shall be available at all times.

7.721.222 Case Management Staff and Other Professional Staff [Eff. 9/15/12]

- A. Case management shall be provided by the child placement agency or county.
1. A qualified professional having a Bachelor's degree in social work, psychology, or child development from an accredited college or university shall supervise the host family home. This professional shall have two years of treatment-oriented experience; or,
 2. A case manager who meets the requirements of the child placement agency or county.
- B. The case management staff shall be responsible for providing case management services as stated at Section 7.715.42, B.

7.721.23 Capacity [Eff. 9/15/12]

- A. When a certified home participating in the host family home only provides temporary emergency shelter for youth, the home may be certified for up to four (4) youth including siblings groups.
- B. Child placement agency or county shall instruct homes participating in the host family home in requirements in accommodations and required supervision placement of both male and female youth.

7.721.3 HOST FAMILY HOME CHARACTER, SUITABILITY, AND QUALIFICATIONS [Eff. 7/1/14]

- A. A certificate shall be denied in accordance with Section 7.710.34, D and E.
- B. A certificate may be denied or revoked if the host family home's own children have been placed in foster care or a residential treatment facility under circumstances which demonstrated that the provider(s) in host family home or another resident of the home was abusive, neglectful, or a danger to the health, safety, or well-being of children within the home.

- C. The providers in host family home shall be able to provide for a homeless youth's physical, mental and character development.
- D. Applicants shall demonstrate stability in family relationships within the host family home.
- E. The licensing or certifying authority must receive at least three written statements which describe the applicant's character, interpersonal relations, and ability to provide care for youth from references provided by the applicant, at least two from a source/person who are not related to the applicant and who have known the applicant one year or longer (references need not be residents of Colorado). Licensing or certifying representatives may contact others who may have knowledge or information regarding the applicant's character or suitability.
- F. The host family home provider shall possess basic knowledge of childcare, positive youth development, runaway and homeless youth issues, and good nutrition, and shall cooperate with the licensing or certifying agency in programs designed to increase such knowledge.
- G. Providers in a host family home shall protect youth receiving shelter in the home from exposure to second hand smoke.
 - 1. Smoking is prohibited inside the host family home at all times when homeless youth are present.
 - 2. Smoking is prohibited in motor vehicles used by providers in host family homes or case managers when a youth is present.
- H. The financial resources of a host family home shall be adequate to assure that the home where the shelter is provided is maintained in safe repair and in conformity with standards and that the requirements of these regulations can be fulfilled.
- I. Each host family home provider shall have a health assessment within one year prior to certification or within thirty (30) calendar days after certification and thereafter as required, in writing, by a licensed health care professional. The reports of the medical examinations shall be dated and signed by the examining physician or nurse practitioner and shall be provided to the certifying authority. Reports shall include a statement of the evaluation of the person's physical ability to care for youth.
- J. If, in the opinion of the licensed health care professional or the assessment worker, an emotional or psychological condition exists which would have a negative impact on the care of youth, the certification of a host family home shall be conditioned upon the satisfactory report of a licensed mental health practitioner.

- K. Other children of the providers of the host family home and any other persons not placed by the agency and living in the host family home shall obtain a medical statement from a licensed health care professional verifying that each such person suffers from no illness or communicable disease which would adversely affect youth receiving shelter in the host family home. This statement shall be obtained annually or as required in writing by an approved health care professional. A licensed health care professional is defined as a physician, nurse practitioner, or a physician's assistant. This statement shall have been signed within the twelve-month period preceding the certificate granted to the home.

7.721.31 Medical and Health [Eff. 9/15/12]

- A. The child placement agency or county shall provide referrals to local health care providers for any youth requiring medical attention while participating in a host family home.
- B. Child placement agency or county of the host family home will determine policies for storage and distribution of medications.

7.721.32 Personnel Policy, Orientation, and Training [Eff. 9/15/12]

- A. The child placement agency or county shall have a comprehensive written plan for the recruitment, hiring or certification, orientation, ongoing training, and professional development of providers in a host family home.
- B. The child placement agency or county shall have an introductory training and orientation program for all providers in a host family home. This program shall include orientation to emergency and safety procedures and the general and specific duties and responsibilities of staff/case managers and host family home providers.
- C. The child placement agency or county shall maintain written documentation of specific in-service training held, staff/providers participating, the hours involved, and/or other on-going training activities in which staff /providers were involved.
- D. The child placement agency or county shall document that all staff and providers in the host family home receive appropriate training in the following areas:
1. The child placement agency or county's administrative procedures and overall program goals.
 2. The emergency and safety procedures, including but not limited to fire evacuation drills, tornado drills, where appropriate, and flood evacuation drills, where appropriate, on at least a semiannual basis.
 3. Universal precautions regarding safe work practices to prevent exposure to blood and bodily fluids.

4. CPR and basic First Aid.
5. The principles of positive youth development and practices of working with youth.
6. Acceptable behavior management techniques, including appropriate discipline in accordance with host family home policies and these rules.
7. Appropriate boundaries (both physical and emotional) between case workers, the host home parent, and/or the youth while the youth is in the host family home and after the youth leaves the host family home.
8. Positive and constructive methods of dealing with the youth including, but not limited to, physical structuring of the environment and de-escalation of crisis situations.
9. Hazardous materials.
10. Storage and distribution of medications.
11. Mandated reporting.

7.721.4 PROGRAM

7.721.41 Program Description [Eff. 9/15/12]

- A. The child placement agency or county shall have a written overall program description for the host family home.
- B. The written description shall address at least the following areas:
 1. Provision of case management;
 2. Referral to emergency medical and/or dental services;
 3. Referral to education/vocational services;
 4. Housing, food, and clothing resources;
 5. Referral to medical, dental and substance abuse services;
 6. Recreation opportunities;
 7. Youth rights;
 8. Requirements of the host family home provider.
- C. Child placement agency or county will develop and provide services using a positive youth development approach that ensures a young person a sense of:

1. Safety and structure;
2. Belonging and membership;
3. Self-worth and social contribution;
4. Independence and control over one's life; and closeness in interpersonal relationships.

7.721.42 Case Management Services [Eff. 9/15/12]

- A. The child placement agency or county shall be assigned the responsibility for case management for each youth.
- B. Each individual assigned the responsibility of providing case management services for a youth shall:
 1. Participate actively with the youth, family, host family home provider, and other supports to create a service and transition plan and to address the provision of resources and/or identified services.
 2. Assure that appropriate information concerning the youth and her/his background is shared with other staff members and providers in host family home who deal with the youth regularly and to maintain communication with youth's parents, guardians, if available, and with the individual or agency responsible for discharge planning and follow-up care of the youth, if applicable.
 3. Assure that the plan developed for each youth is implemented, and documented in the youth's file.

7.721.43 Admission Policy and Procedures

- A. Admission of a runaway/homeless youth to a host family home shall be in keeping with the stated purpose of the program.
- B. Notification and contacts:
 1. Pursuant to Section 26-5.7-105(4), C.R.S., when a youth under fifteen years of age is admitted to a licensed host family home, the director of the facility, shelter, or other person in charge shall notify the county department of the county of residence of the parents of the youth within seventy-two (72) hours of the youth's admission.
 2. Pursuant to Section 26-5.7-105(5), C.R.S., if the facility determines that a referral for additional services needs to be made, it shall make the referral to THE APPROPRIATE county department of human/social services, notify the county department of the facility's relationship to the youth

pursuant to section 19-1-307 (2)(e.5)(I), and notify the county department of the date when the twenty-one-day shelter time period will expire.

3. Pursuant to Section 26-5.7-105(7), C.R.S., if a youth who is at least eleven (11) years of age but less than fifteen (15) years of age has been served up to twenty-one (21) days and returns again to the licensed host family home after leaving the host family home, the child placement agency or county certifying the host family home shall make a referral for services to the county of residence of the parents of the youth.
- C. Child placement agency or county shall place youth in host family homes where the provider of the host family home is qualified and taking into account the needs of children already in residence to provide the care necessary. Care must be provided in the least restrictive, most appropriate setting in order to meet the youth's needs.
 - D. Each host family home and its certifying authority shall have a written admission policy that at a minimum must include:
 1. The policies and procedures related to intake.
 2. The age range and sex of youth accepted/admitted for care.
 3. The needs, problems, situations or patterns best addressed by the host family home.
 - E. Any pre-placement requirements for the youth, the parent(s) or guardian, and/or the placing agency.
 - F. The anticipated problems or situations that would result in the host family home or certifying authority requesting removal of a youth from the host family home.
 - G. The written description of admission policies and criteria shall be provided to referring agencies.
 - H. The child placement agency or county shall accept a youth into temporary/emergency shelter only after a preliminary intake assessment and screening of immediate needs. Further assessments of areas such as social, physical health, mental health shall be conducted within three days of admission.
 - I. Pursuant to Section 26-5.7-106(2), C.R.S., upon admission of a youth to a host family home, the child placement agency or county shall:
 1. Notify the youth's parent(s) or county department of human/social services of the youth's whereabouts, physical and emotional condition, and the circumstances surrounding the youth's placement within 24 hours;

2. Notify the youth's parent that it is the paramount concern of the facility or shelter to achieve reconciliation between the parent and the youth, to reunify the family, and to inform the parent about the alternatives that are available;
 3. Arrange transportation for the youth to the residence of the youth's parent when the youth and the parent agree that the youth shall return to the home of the youth's parent. The parent shall reimburse the party who paid for the transportation costs to the extent of the parent's ability.
 4. Arrange transportation for the youth to an alternative residential placement facility when the youth and the youth's parent agree to such placement. The parent shall reimburse the appropriate person for transportation costs to the extent of the parent's ability.
- J. Within 24 hours of admission the child placement agency or county is responsible to contact parent(s)/legal guardians(S) of youth under the age of eighteen and document their permission to serve or attempts made to contact parents/legal guardians. If notifying the parent(s) or legal guardian(s) would not be in the youth's best interest due to an imminent risk of abuse or neglect by the parent(s) or legal guardian(s), the shelter shall notify the appropriate county department of human/social services instead of the parent(s)/legal guardian(s).
- K. Within 24 hours of arrival at the host family home, a youth shall be given an orientation to the host family home, consistent with the youth's age and ability to participate, which includes at least the following:
1. Tour of the host family home and instruction on fire alarm and fire evacuation procedures, escape routes and exits.
 2. The rules of the host family home.
 3. Procedures affecting the youth's behavior, including limiting or restricting a youth's rights where allowed, the type of discipline used in the host family home, and consequences for certain behaviors.
 4. The complete youth's rights and youth's grievance procedures as developed by the host family home or by the certifying authority.
- L. If reconciliation with the family and voluntary return of the youth has not been achieved within 48 hours following admission to the host family home, excluding Saturdays, Sundays and legal holidays, and the child placement agency or county department does not anticipate reconciliation occurring within twenty-one (21) consecutive days, the child placement agency or county shall provide the youth and the youth's parent with a written statement identifying:
1. The availability of counseling services;

2. The availability of longer term residential arrangements; and,
 3. The possibility of referral to the county department.
- M. The case plan shall be developed with the involvement of the youth, the parent(s) or guardian(s) of the youth, the representative of the child placement agency or county, and when possible, the host family home provider. Where the involvement of any of these is not feasible or desirable, the certifying authority shall record the reasons for the exclusion. The case plan shall include by reference or attachment at a minimum the following:
1. Discussion of the youth's and the parent's or guardian's expectations regarding: family contact and involvement; how family contact and involvement are to occur; the nature and goals of care, including any specialized services or specialized treatment to be provided; the religious orientation and practices of the host family home; and, the anticipated planned discharge date and plan for the youth following discharge.
 2. Written authorization for care and treatment of the youth.
- N. A youth who has reached the age of 15 or older may consent to shelter and services for a period not to exceed 21 days when the following criteria are met:
1. The youth shall voluntarily, knowingly, and willingly sign a consent form that includes:
 - A. All services the youth may receive during their stay at the shelter and the total number of days the youth may consent to stay in the host home.
 - B. Legal limits to confidentiality.
 - C. How and when parent(s), legal guardian(s), and/or child welfare agencies will be notified of the youth's stay.
 - D. An explanation that services provided by the shelter are provided free of charge to the youth without exchange of any goods, services, or sexual acts.
 2. The consent form shall be explained in the child's dominant language or mode of communication, including augmented or facilitated communication, if necessary, and shall take into consideration the child's age, disability, and cultural and religious background.
 3. The consent form shall be signed within 12 hours of admission.
- O. Documentation of all required notifications and consents shall be kept in the youth's file.

7.721.5 YOUTH RIGHTS

7.721.51 Youth Rights

- A. The child placement agency or county shall have written policies and procedures that address and ensure the availability of each of the following core rights for youth in host family home. These rights may not be restricted or denied by the host family home.
 - 1. Every youth has the right to enjoy freedom of thought, conscience, cultural and ethnic practice, and religion.
 - 2. Every youth has the right to receive adequate and appropriate food, clothing, and housing.
 - 3. Every youth has the right to live in clean, safe surroundings.
 - 4. Every youth has the right to participate in an educational program that will maximize his/her potential in accordance with existing law.
- B. Every youth has the right to communicate with others outside the host family home, such as a parent or guardian, caseworker, attorney or guardian ad litem and/ or counsel for youth, current therapist, physician, religious advisor, and, if appropriate, probation officer.
- C. Written policies that restrict or limit a youth's rights as listed at Section 7.721.5 must include at a minimum the requirement that the host family home notify the youth at the time of admission of any policy that would limit or restrict a youth's rights. The notification must be communicated in a language or mode of communication the youth can understand and, if possible, be signed by the youth.
- D. If the host family home enforces any restrictions upon the youth's rights as listed at Section 7.721.5, the host family home must, in compliance with the written policy and procedure of the host family home:
 - 1. Inform the youth in a language or mode of communication the youth can understand, of the conditions of and reasons for restriction or termination of his/her rights.
 - 2. Place a written report summarizing the conditions of and reasons for restriction, denial, or termination of the youth's rights in that youth's case record along with information about if or when the youth's right(s) can be restored. This Information must be made available to the youth upon request.
 - 3. When a restriction of a youth's rights affects another youth in the host family home, the youth shall be informed, in a language or mode of

communication the individual can understand, of the conditions of and reasons for the action.

7.721.51 Youth's Grievance Procedure [Eff. 9/15/12]

- A. The child placement agency or county of the host family home must establish a written grievance procedure that provides adequate due process safeguards, spells out the appeal process, and assures that youth and parent(s) or guardian(s) are entitled to report any grievance and shall not be subject to any adverse action as a result of filing the grievance.
 - 1. The host family home must follow grievance procedures without alteration, interference, or unreasonable delay.
 - 2. If a grievance is filed with the host family home, the grievance shall be recorded in the youth's record along with the investigation findings and resulting action taken by the child placement agency or county.
- B. A list of the youth's rights and the grievance procedures must be provided and explained to the youth and the parent or guardian in a language or manner of communication that they can understand.

7.721.6 DISCIPLINE [Eff. 9/15/12]

- A. The child placement agency or county shall have written policies and procedures regarding discipline that include:
 - 1. Discipline shall be constructive or educational in nature.
 - 2. Procedures for diversion or separation from the problem situation.
 - 3. Procedures for withholding privileges.
 - 4. Positive responses to youth's appropriate behavior.
- B. Policies and procedures must be explained to all youth, parent(s), guardian(s), staff, host family home provider and placing agencies. Basic rights shall not be denied as a disciplinary measure.
- C. Youth in care at the host family home shall not discipline other residents.
- D. Separation when used as discipline must be brief and appropriate to the youth's age and circumstances. The youth shall always be within hearing of an adult in a safe, clean, well-lighted, well-ventilated room in the host family home that contains at least 50 square feet of floor space.

- E. The child placement agency or county shall prohibit all cruel and unusual discipline and ensure providers of host family home and case managers are trained on established guidelines and appropriate discipline.

7.721.7 NON-DISCRIMINATION [Eff. 9/15/12]

In compliance with Section 7.701.14, the host family home shall not discriminate against a youth based upon race, religion, age, gender or disability.

7.721.71 Religion and Culture [Eff. 9/15/12]

- A. The host family home shall demonstrate consideration for, and sensitivity to, the cultural and religious backgrounds of youth in care.
- B. Youth in care of the host family home shall be allowed and encouraged to celebrate his/her cultural and religious holidays.
- C. A youth may be invited to participate in religious and cultural activities of the host family home but shall not be required to participate.
- D. Any form of religious or cultural intervention used by the host family home to control or change a youth's behavior, or treat or heal a medical condition, must be approved, in writing, by the legal guardian(s) of the youth prior to the use of the intervention.
- E. The host family home cannot deny medical care to a youth because of host family home's religious beliefs.
- F. Opportunity and assistance shall be provided for each youth to practice the chosen/preferred religious beliefs and faith of his/her family. If the family has no preference, the individual preference of the youth shall be respected. The child placement agency or county will attempt to make necessary arrangements for attendance of youth at the appropriate religious institution or at a study group for religious instruction.

7.721.8 EDUCATION AND COMMUNITY INVOLVEMENT

7.721.81 Education [Eff. 9/15/12]

Youth shall attend educational/vocational programs in the most appropriate and least restrictive educational setting for the youth, including, but not limited to, attending regular classes conducted in accredited secondary schools within the community and workforce programs to achieve outcomes such as decreasing secondary school dropout rates, increasing rates of attaining a secondary school diploma or its recognized equivalent, or increasing placement and retention in postsecondary education or advanced workforce training programs.

7.721.82 Community Participation [Eff. 9/15/12]

- A. Participation in community activities shall be encouraged, supported, and a vital part of each youth's life and choice.
- B. The child placement agency or county shall reflect consideration for, and sensitivity to, the racial, cultural, ethnic and/or religious backgrounds of youth in care. The host family home shall involve a youth in cultural and/or ethnic activities appropriate to his/her cultural and/or ethnic background. Other factors should include, but are not limited to, consideration of the child's family, community, neighborhood, faith or religious beliefs, school activities, friends, and child's and family's primary language.
- C. The host family home shall utilize available services, facilities, and activity programs of the community, and youth shall be given opportunities to participate as individuals or as a group in agency-sponsored recreational and cultural programs.
- D. Chores at the host family home are considered part of the participatory responsibility of living together. They shall provide constructive experiences in accordance with the age and ability of the youth. All chores shall be scheduled so as not to conflict with other essential scheduled activities. Youth participating in the host family home cannot provide independent care/supervision of other residents in the host family home.
- E. The child placement agency or county shall comply with all child labor laws and regulations in making work assignments, with consideration for agricultural work assignments in those communities.
- F. Case manager shall approve paid or voluntary work assignments outside of the host family home. Approval shall include knowledge of the employer, the specific type of work, and the conditions of employment/voluntary work.
- G. A youth shall not be exploited. A youth may not participate in solicitation on behalf of the child placement agency or county or certifying authority for a fund-raising activity without the written permission of the parent(s) or guardian(s) for each specific activity, and the youth must be willing to participate in the activity.

7.721.9 PERSONAL CARE OF THE YOUTH

7.721.91 Medical and Health Services [Eff. 9/15/12]

- A. The child placement agency or county of the host family home shall ensure the availability of emergency medical care for each youth in care.
- B. Whenever indicated, a youth shall be referred to an appropriate specialist for either further assessment or treatment.

- C. At all times there shall be first aid supplies readily available at each host family home and at least one (1) person present at the home or with the youth who is certified in First Aid, or the equivalent, and CPR for all ages of youth in care.
- D. The child placement agency or county, in conjunction with the parent(s) or guardian(s), shall make every effort to ensure that a youth needing corrective devices such as glasses, hearing aids, etc., is provided with the necessary equipment.
- E. In a potentially life-threatening situation, the child placement agency or county shall refer the youth's care to the appropriate medical and legal authority.
- F. Child placement agency or county of the host family home will determine policies for Medications which include the following:
 - 1. Required storage procedures in homes participating in the host family home including locked containment of all medication.
 - 2. During admission process case manager shall ascertain all medication the youth is currently taking.
 - 3. All medication must be kept in a clean storage area inaccessible to youth and stored according to pharmacy instructions.
 - 4. All prescriptive medications shall be administered only upon the written prescription of a physician. The child placement agency or county shall also obtain written authorization from the prescribing physician to administer any non-prescriptive medication.
 - 5. Child placement agency or county will provide providers of host family homes complete instructions to ensure proper administration of medication for each youth.
 - 6. For youth under 18 years old, in an emergency situation, non-prescriptive medication may be administered on the verbal authorization of a physician. Written confirmation must then be obtained for the verbal authorization.
 - 7. The child placement agency or county maintains for each youth a cumulative record of all medication, both prescriptive and non-prescriptive, dispensed to that youth, including:
 - a. The name of the youth.
 - b. The name and dosage of medication.
 - c. The time and date the medication was dispensed.

- d. The name or initials of the person administering the medication.
- 8. The child placement agency or county will establish guidelines for documenting an error of any sort with medication including:
 - a. Notification to poison control if medication is administered incorrectly.
 - b. Documentation of missing medication.

7.721.92 Food, Nutrition, Personal Hygiene, Daily Routine, Clothing, and Personal Belongings

7.721.921 Food and Nutrition

- A. The host family home shall provide nutritious foods in the variety and amounts as appropriate for the age, appetite, and activity of each youth.
- B. At least three nourishing, wholesome, well-balanced meals a day shall be offered at regular intervals. The host family home provider will document meals in a log to be audited by the child placement agency or county.
- C. Family meals including all children and adults present in the home shall be provided whenever possible.
- D. Youth shall be encouraged to eat a variety of the food served but shall not be subjected to undue coercion, including forced feeding, or punished for refusal to eat.
- E. All food shall be from sources approved or considered satisfactory by the health authority. All foods shall be stored, prepared, and served in such a manner as to be clean, wholesome, free from spoilage, and safe for human consumption. Only pasteurized milk shall be served. Fruits, vegetables and meats may be frozen.
- F. The child placement agency or county of the host family home shall inform providers of host family home of the special diets prescribed for youth including those that are contrary to their religious beliefs, or of their family, or are known to cause an allergic reaction or a health hazard.
- G. Water shall be readily accessible to youth by means of an approved water source. Youth will be provided clean single-service cup for administering medication and rinsing mouth in bathroom.

7.721.922 Personal Hygiene and Daily Routine [Eff. 9/15/12]

- A. The child placement agency or county of host family home shall establish procedures to ensure that youth receive training in good habits of personal care, hygiene, and grooming appropriate to their age, gender, race and culture.

- B. The host family home shall ensure that youth are provided with necessary and appropriate toiletry items, including clean, individual towels and washcloths, toothbrush, toothpaste, comb, shampoo and clean cups and dishes.

7.721.923 Clothing and Personal Belongings [Eff. 9/15/12]

- A. The host family home shall allow a youth in care to bring his/her personal belongings to the program, as defined by the child placement agency or county policy, and to acquire belongings of his/her own in accordance with the youth's plan. However, the host family home shall, as necessary, limit or supervise the use of these items while the youth is in the program. Where extraordinary limitations are imposed, the youth shall be informed of the reasons, in a language or manner of communication the youth can understand. The decision and reasons shall be recorded in the youth's case record.
- B. The child placement agency or county shall establish provisions for the protection of the youth's property while the youth is participating in the host family home. If the host family home safeguards any money, valuables or personal property for a youth, the child placement agency or county shall maintain and inventory all such possessions. A full return or accounting of such possessions shall be made upon discharge of the youth from the host family home.

7.721.93 Living Spaces, Equipment, and Necessary Facilities for a Host Family Home

7.721.931 Physical Requirements for a Safe and Adequate Host Family Home [Eff. 9/15/12]

- A. The entire premises of homes participating in the host family home are subject to inspection for licensing or certification purposes, including, but not limited to, the interior of the home, the grounds surrounding the home, the basement, the attic (if accessible), any storage buildings, and a garage or carport, if applicable.
- B. Homes participating in the host family home, including indoor and outdoor space, shall be maintained in a clean and safe condition free from hazards to health and safety.
- C. The following shall be required for all host family homes:
 - 1. Although the presence of firearms and ammunition is strongly discouraged in host family homes, if child placement agency or county policies allow the presence of any weapons such as firearms, air rifles, bows, hunting knives or hunting sling shots, they shall be unstrung and unloaded at all times when youth are in the home and shall be stored in locked containers out of the reach youth. Ammunition and arrows shall be stored in separate locked containers. Firearms that are solely ornamental can be excepted from the storage requirement. Weapons shall not be transported in any vehicle in which youth are riding. Law enforcement professionals are

exempted from the requirements of this section if conditions of their employment require them to carry weapons.

2. At least thirty-five (35) square feet of usable indoor space exclusive of halls, baths and sleeping area shall be available for each child. All floor space shall have carpets, tile or smooth finish that may be easily cleaned. Interior walls shall be constructed of solid material and be free from holes. Unfinished basements must be inaccessible to youth when safety hazards are present.
 3. Exterior doors shall be maintained in such a manner that would permit easy exit. Interior doors shall be designed to prevent youth from being trapped.
 4. A basement that will be used regularly in the care of youth in any manner shall be equipped with more than one exit; such exit may be an accessible window.
 5. The home shall be equipped with refrigeration, and provisions shall be made for the washing, rinsing and storing of dishes in a safe and sanitary manner.
 6. A comfortable bed in a well-ventilated room, which is not customarily used for other purposes, shall be available for each youth in the host family home. There shall be a minimum of forty (40) square feet of floor space for each youth's bed, and the beds should be placed at least two feet apart when arranged in parallel. Youth under the age of eighteen (18) may not be housed in the same living areas, including bedrooms, as youth over the age of eighteen.
 7. No home participating in the host family home shall be used for a rental income business or an adult foster care facility. A business of a nature that might be hazardous to the health, safety, morals or welfare of youth shall not be operated on the premises of the home.
 8. Mobile homes participating in the host family home shall have at least two exits, be skirted and properly installed and stabilized.
 9. The child placement agency or county shall establish procedures for safety issues related to swimming pools; trampoline or other outdoor equipment shall be assessed by the child placement agency or county, addressed by the county or state health department as necessary, and documented in the case file.
- D. A certificate shall be denied, suspended, revoked or made probationary for failure to repair or otherwise comply with any of the preceding requirements when a defect or noncompliance with such requirement has been noted by the

licensing or certifying representative of the child placement agency or county and brought to the attention of the licensee or applicant in writing.

7.721.932 Host Family Home Maintenance [Eff. 9/15/12]

Child placement agency or county shall ensure that homes participating in the host family home:

- A. Are kept in good repair and maintained in a safe, clean, and sanitary condition.
- B. All areas available to youth's activities including equipment, materials and furnishings shall be of sturdy, safe construction, easy to clean, and free of hazards, such as sharp points or corners, splinters, protruding nails, broken play and recreational equipment, or paint that contains lead or other poisonous materials and might be dangerous to the life or health of youth.
- C. Are kept free from accumulation of significant amounts of non-essential materials such as furnishings, newspapers, or magazines that could pose a fire or health hazard.
- D. Have provisions for collection, storage, and disposal of trash to prevent infestation by rodents.

7.721.933 Fire Safety [Eff. 9/15/12]

Child placement agency or county shall establish procedure for homes participating in the host family home that ensure:

- A. All homes participating in the host family home shall contain at least one Underwriters Laboratories (U.L.) approved fire extinguisher, highly visible, easily accessible, and in working condition, weighing not less than five pounds, that has a rating of 2A, 10BC. This requirement may be waived if more extensive fire-control measures are required by a local fire department.
- B. A smoke detector and carbon monoxide detector, in working condition, must be installed on each level of the home and near sleeping areas.
- C. Fire hazards, such as defective electrical appliances and electric cords, dangerous or defective heating equipment or flammable material stored in such a manner as to create a risk of fire shall be corrected or eliminated.
- D. No gas space heaters, open-flame gas or oil stoves, hot plates, or un-vented heaters are used in the home for heating purposes. No electric space heaters shall be used in the home for permanent heating purposes.
- E. Flammables, aerosol paints, insecticides, chemicals, and other dangerous materials shall be locked or stored so they are inaccessible to youth and must be stored in areas separate from sleeping or living areas. Flammables shall be

- stored in an approved container and not be stored near a furnace, hot water heater, or other heating device.
- F. Heating devices such as radiators, registers, fireplaces, wood-burning stoves, and steam and hot water pipes that pose a fire or burn hazard to youth shall be screened or otherwise protected.
 - G. There shall be no candles or other burnable objects permitted in youth's sleeping areas. Youth shall not be permitted to smoke inside host family home or in any vehicle used to transport.
 - H. Exit doors shall be clearly identified to all youth. No lock or fastening to prevent free escape from the inside of any room used by the youth shall be permitted.
 - I. Exit routes shall be kept free of discarded furniture, furnishings, laundry, and stacks of newspapers or magazines that could interfere with the prompt evacuation of the homes participating in the host family home.

7.721.94 Transportation [Eff.9/15/12]

- A. The child placement agency or county of the host family home shall ensure that youth are provided with the transportation necessary for implementing the youth's service plan.
- B. Homes participating in the host family home shall have means of transporting youth in cases of emergency.
- C. Any vehicle used by the host family home in transporting youth in care shall be properly licensed and the vehicle shall be maintained in accordance with Colorado law.
- D. Any host home parent or other person acting on behalf of the host family home operating a vehicle for purpose of transporting youth shall be properly licensed to operate the class of vehicle in accordance with Colorado law.
- E. Youth must be properly fastened into a restraint system that conforms to all applicable Federal Motor Vehicle Safety Standards and pursuant to Colorado law.
- F. The host family home shall not allow the number of persons in any vehicle used to transport youth to exceed the number of available seats in the vehicle.
- G. The vehicle shall be enclosed and provided with door locks.
- H. The child placement agency or county of the host family home shall ascertain the nature of any need or problem of youth in the program that might cause difficulty during transportation, such as seizures or a tendency toward motion sickness.

The host family home shall communicate this information to the driver of any vehicle transporting youth.

7.721.95 Records, Reports and Confidentiality

7.721.951 Confidentiality [Eff. 9/15/12]

- A. The child placement agency or county shall have a policy as to the maintenance, storage and confidentiality of records.
- B. Records shall be the property of the child placement agency or county and shall be protected against loss, tampering, or unauthorized use.
- C. Facts learned about youth and their families shall be kept confidential, with the following exceptions:
 - 1. In medical emergencies, and then only when the assistance and/or expertise is required of that unauthorized person; or,
 - 2. To the youth, his/her parent(s) or guardian(s) and their respective legal counsel(s), a court having jurisdiction over the youth, or an authorized public official, or licensing representative in performance of his/her mandated duties; or,
 - 3. If the parent(s) or guardian(s) has given voluntary, written consent facts maybe shared using confidential fax or encrypted email.
 - 4. Mandatory reporting of child abuse as required by law; and,
 - 5. Reporting to the appropriate authority a youth's attempt to harm his/herself or others.
 - 6. A youth's records may only be released upon the written consent of the youth, and the legal guardian for youth under eighteen years old; facts may be shared using confidential fax or encrypted email.

7.721.952 Records [Eff. 9/15/12]

- A. The child placement agency or county shall maintain complete records as required for the host family home.
- B. Records for youth shall be retained for at least three years. Retention of records for a longer period may be desirable when they reflect an accident, injury or other unusual circumstance.
- C. A record of admission shall be completed for each youth in the host family home at the time of admission. The admission record shall be maintained at the child

placement agency or county. The host family home where the youth where the youth resides shall received an abbreviated admissions record that contains:

1. Youth's name, date and place of birth (verified by a birth certificate when possible), gender, race, religious preferences, and date of admission.
 2. Psychiatric and psychological reports, when available.
 3. Medical/medication summary and reports, when available.
 4. Summary recording of significant contacts with parent(s), guardian(s) and other involved agencies.
- D. When a youth is removed or discharged from the host family home, the child placement agency or county shall complete a summary of the youth's discharge from the home which includes at a minimum:
1. The date of the discharge of the youth from the home.
 2. The plan for the youth.
 3. Circumstances that led to an unplanned discharge.
- E. Copies of a youth's file, including discharge information, shall be provided to parent(s) or guardian(s) upon request or to others by written consent.
- F. A copy of all policies developed by the child placement agency or county for the host family home shall be maintained for at least three years.

7.721.953 Reports [Eff. 9/15/12]

- A. The Child Placement Agency or County shall immediately attempt to notify the youth's parent(s), guardian(s), and/or the responsible agency of any serious illness or serious injury resulting in medical treatment away from the home, hospitalization or death involving a youth in the host family home.
- B. The child placement agency or county must immediately report in writing to the Department any accidents, injuries, serious illnesses, or fatalities that occurred at the host family home and that resulted in emergency medical treatment away from the host family home, hospitalization or death. The report must be made within 48 hours after the accident, illness, or death occurred.
- C. A report about a death must include:
1. The youth's name, birth date, address, and telephone number.
 2. If known, the names of the youth's parent(s) or guardian(s) and their address and telephone number if different from that of the youth.

3. Date of the fatality.
 4. Brief description of the incident or illness leading to the death.
 5. Names and addresses of witnesses or persons who were with the youth at the time of death.
 6. Name and address of police department or authority to which the report was made.
- D. The child placement agency or county of the host family home must immediately report in writing to the Department any change in the status of the home that would affect care of youth.
- E. The child placement agency or county must immediately report in writing to the Department any legal action brought against a home participating in the host family home regarding any youth, staff, or conduct of the home at the time of initiation of such legal action.

These rules will address the operation of a substitute placement agency that places a substitute child care teacher or director into a licensed child care facility for the purpose of providing substitute child care.

7.730 RULES REGULATING SUBSTITUTE PLACEMENT AGENCIES [Repealed eff. 03/05/2026]

Editor's Notes

History

Rules 7.702; 7.707; 7.712 eff. 05/01/2007.

Rules 7.705, 7.711, 7.712, 7.719 eff. 06/01/2007.

Rules 7.701.2; 7.701.3; 7.708; 7.709; 7.710 eff. 07/30/2007.

Rules 7.701.32-33, 7.710.56 A-J.5 emer. rules eff. 08/03/2007.

Rules 7.710.33 L-M, 7.710.36 A eff. 09/01/2007.

Rules 7.701.32-33; 7.710.56 eff. 10/30/2007.

Rules 7.710.52, 7.710.56 emer. rules eff. 12/07/2007.

Rules 7.710.52; 7.710.56 eff. 01/30/2008.

Rule 7.710.32 eff. 04/01/2008.

Rules B&P, 7.701.4, 7.707.42, 7.712.33, eff. 09/01/2008.

Rules 7.709, 7.710.34, 7.710.36 eff. 11/01/2008.

Rule 7.707 eff. 01/01/2010.

Rules 7.702.52-7.702.54, 7.702.55 D-7.702.56 A.5, 7.710.1-7.710.25, 7.710.33 A, J, L.6, M, 7.710.34-7.710.42, 7.710.45-7.710.98, 7.716.4-7.716.6 B eff. 05/01/2010.

Rule 7.701.33 A emer. rule eff. 09/10/2010.

Rule 7.701.33 A eff. 12/01/2010.

Rules 7.716.1, 7.716.2.A.7, 7.716.3 eff. 01/01/2011.

Rule 7.710.12 eff. 03/02/2011.

Rules 7.700, 7.701.11, 7.701.2, 7.701.33 A, 7.701.4, 7.720 eff. 04/01/2011.

Rules 7.701.33 A-D.1, 7.701.33 H-I emer. rules eff. 08/10/2011.

Rules 7.701.33 A-D.1, 7.701.33 H-I eff. 11/01/2011.

Rules 7.705.9-7.705.96 repealed eff. 01/01/2012; Rules 7.701.2, 7.705.1, 7.705.33, 7.705.42, 7.705.43, 7.714, 7.714.4, 7.714.932 eff. 01/01/2012.

Rule 7.17 repealed eff. 02/01/2012.

Rule 7.708 eff. 04/01/2012.

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